Descriptive Report ADA-PARC 2023

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1 Preface

1.1 Report Purpose & Context

In producing the descriptive report which follows, the guiding purpose was to support the current version of the ADA-PARC website (see appendix for link) with additional explanation of key findings for stakeholders, disability advocates, and other readers invested in disability-related outcomes in the United States. Rather than introducing new analysis, the pages that follow summarize what the dashboard shows, emphasizing broad patterns across domains and the ways those patterns vary by geography. It must be emphasized that the scope of this report is intended to be descriptive rather than causal. Results are presented as benchmarks and comparisons, without analyzing too deeply why such differences occur or what policies should follow from them.

Readers will see a consistent rhythm throughout. Each topic typically pairs a visual or table with a short interpretation that emphasizes (1) overall levels for people with disabilities, (2) how outcomes vary across geography, and (3) when comparable measures exist, how outcomes differ between people with and without disabilities. Geographies with 'best' and 'worst' results for people with disabilities are used as a descriptive aid meant to make the range of outcomes concrete, not necessarily to imply best or worst governance.

1.2 Data & Approach

All data in this analysis derives from the 2023 U.S. Census American Community Survey (ACS) 5-year estimates, a nationally representative survey that provides information on social, economic, and housing characteristics. Because these are estimates rather than a complete count, results should be read as the best available standardized picture of conditions across the country rather than as exact tallies. The five-year format pools multiple years of survey responses to improve reliability, especially for smaller geographies, one reason this data set works for the state and city comparisons used throughout the report

These raw measures form the foundation of the ADA-PARC Scorecards, which translate complex indicators into standardized metrics that can be compared across states and regions. The scorecards apply a multi-step transformation: (1) calculation of z-scores relative to the median state, clipped at ±3; (2) normalization into a 0–100 scale using the transformation $(50/3)z + 50$; and (3) aggregation into indices that combine multiple variables. This ensures that higher scores consistently indicate more favorable outcomes for people with disabilities (PWD)

Throughout the report, measures are presented for both PWD and people without disabilities (PWOD). For ease of reference, "PWD" and "PWOD" are used as shorthand. At every level of analysis, outcomes are compared both within the PWD population and between PWD and PWOD to highlight disparities. Leaders and laggards are identified, and the practical magnitude of observed differences is noted.

1.3 Populations & Comparisons

Discussion of outcomes for people with disabilities and people without disabilities are compared throughout the report and two main lenses are applied.

1. Outcomes within the disability population

   One lens focuses **within the disability population**: how outcomes for people with disabilities vary across states, regions, and cities. This is where geographic clustering shows up most clearly, and where it becomes possible to see whether places tend to fall into recognizable "tiers" rather than scattered, random-looking variation.

2. Differences between people with disabilities and people without disabilities

   The second lens benchmarks outcomes **between disability and non-disability populations**. When a measure is available for both groups, the report highlights the disability-related **gap** alongside the absolute level. This matters because two places can look similar on an overall outcome while showing very different levels of inclusion for people with disabilities.

Magnitude is treated as part of the story. A difference that is statistically or numerically "real" may still be small in practical terms, while some patterns—particularly in work/economic security and institutional living—are large enough to be difficult to ignore even before getting into deeper analysis.

1.4 Key Terms & Shorthand Acronyms

While a fuller list of terms is presented to guide with interpretation in the appendix, a small number of such terms are vital to understand:

- PWD (People with Disabilities): individuals identified as having a disability per ACS definitions
- PWOD (People without Disabilities): individuals identified as not having a disability per ACS definitions
- Disparity/gap: the difference between PWD and PWOD on the same measure. In this report, gaps are always calculated as PWD minus PWOD, though whether positive or negative values confer disadvantage for people with disabilities depends on the measure and will be noted as such.
- ACS / ACS-5: The Census Bureau's American Community Survey (ACS) 5-year estimates combine responses collected over five years to produce detailed information about people and housing for the U.S., states, and local communities. A "5-year estimate" means the number reflects an average picture across that full five-year period, not a single point-in-time year.
- ADA-PARC: Americans With Disabilities Act Participation Action Research Consortium. The ADA-PARC has research teams focused on Community Living, Community Participation & Work/Economic Participation measures and uses Regional Steering Committees in each of the ten participating regions to gather, review, and choose appropriate data to achieve the project's purposes.

1.5 Descriptive Report Contents

With a structure organized to move from "who and where" toward "how outcomes look across key participation domains," this descriptive report ends by identifying the key patterns that repeat across categories and geographies. The main sections appear in the following order:

- Demographics establishes baseline context including overall disability prevalence, disability types, and distributions across age, sex, race/ethnicity.

- Community Living examines living arrangements, identifying a core boundary between living in the community versus institutional settings.
- Community Participation focuses on access conditions that shape broader participation in society including insurance, education, transportation, and technology.
- Work & Economic focuses on conditions which affect economic security for people with disabilities including employment, labor force participation, poverty, and income.

1.6 ADA-PARC Scorecards

In addition to individual indicators, the ADA-PARC scorecards translate sets of related measures into standardized indices that can be compared more easily across places. Scorecards presented in this report compare outcomes for people with disabilities across states compared to each other, they do not account for outcomes for people without disabilities. All ADA-PARC scorecards are reported on a 100-point scale, where higher scores indicate more favorable outcomes for people with disabilities. As scores progress beyond 50 towards 100, states are increasingly above average in performance on the index, while scores which progress below 50 towards 0 indicates states that are increasingly below average in performance. States are also given a rank 1-51, (scorecards include the District of Columbia) and a quartile value for multiple layers of comparison. Q4 states are those in the top 25% of performers, Q3 states are those in the top 50% of performers, Q2 states are in the bottom 50% of performers, and Q1 states are in the bottom 25% of performers. Below is the overall structure of ADA-PARC scorecards, with their specific details and construction discussed later in each relevant section.

ADA-PARC Scorecards

Scorecards are organized in the hierarchy below, with details for each to be discussed as they are presented in the report.

Community Living Category:

- Communal Living
- Institutional Living

Community Participation Category:

- Insurance Access
- Education Access
- Transportation Access
- Technology Access

Work & Economic Category:

- Employment
- Wealth & Poverty

1.7 Equity Lens

As a complement to scorecards, which focus solely on outcomes for people with disabilities, an equity lens is also applied in presenting outcomes for many measures in the report. Smaller gaps across disability status generally suggest more equitable outcomes, while larger gaps indicate more pronounced exclusion or disadvantage for people with disabilities. Importantly, the equity lens is not limited to one domain. Gaps appear in living arrangements, access conditions that enable participation, and economic security, often in ways that reinforce one another across the life course.

1.8 ADA-PARC Geographies of Analysis

To keep comparisons meaningful while still place-specific, results are shown at four levels:

- **National Level**: Examines overall differences between PWD and PWOD at the broadest scale.
- **Regional Level**: Groups states into U.S. Census regions (Northeast, Midwest, South, West) to highlight geographic patterns.
- **State Level**: Focuses on differences among states, identifying both within-state disparities and cross-state contrasts.
- **City Level**: Zooms into urban contexts, examining outcomes in the most populous U.S. cities as well as identifying leaders and laggards among smaller cities.

A step by step 'zoom-in' approach helps reveal concentrated disadvantage that can be obscured at the national level.

1.9 Key Themes

- **Disability-related gaps are universal, not exceptional.** Across states, people with disabilities are less likely to live at home and more likely to be in group or institutional settings than people without disabilities; employment gaps likewise remain large everywhere.
- **Age shapes outcomes in distinct ways.** Institutional living disparities widen sharply with age, and poverty disparities are often most severe among working-age adults (18–64), even when older-age supports reduce gaps later in life.
- **Access barriers compound into downstream disadvantage.** Participation gaps appear across education, transportation, technology, and insurance coverage, aligning with the broader interpretation that barriers can emerge early and accumulate over time

2 Demographics

In this first section we take a big picture look at disability in the United States by examining the distribution of disability types, overall counts, and prevalence rates across multiple forms of personal identity and levels of geography. Rather than focusing on outcomes such as employment or income, the analysis here is intentionally descriptive and structural in nature. Its purpose is to establish a clear baseline for understanding who experiences disability, in what forms, and where, before turning to questions of participation or inequality in subsequent sections.

Disability is a major part of the United States' population story, and it is not evenly distributed. In 2023, an estimated 42.7 million people in the U.S. reported a disability, representing about 13% of the civilian non-institutionalized population. Those national totals provide essential scale, but they can also hide how sharply disability prevalence changes from place to place, and how much it differs across age groups, disability types, sex, and race/ethnicity.

Our first major section sets a baseline before the report moves into outcomes within community living, participation, and work/economic security. A main focus here is to describe who is most affected, where disability is more concentrated, and what types of disability are most commonly reported. That foundation matters because later comparisons often reflect underlying differences in population age structure, local economic conditions, and long term health patterns as much as they reflect circumstances specific to a single measure of participation.

For the purposes of this report, demographics are separated into these four sub-sections:

- **Counts, prevalence, and disability types** across national, regional, state, and city levels.
- **Age patterns**, since prevalence rises sharply with age and geographic differences often widen in later life.
- **Sex patterns**, which tend to be more modest than between place differences.
- **Race/ethnicity patterns**, where national differences provide a baseline, but local variation can be wide, especially in cities where smaller population groups can produce more extreme estimates.

2.1 Counts, Percentages, And Types of Disability

2.1.1 National Disability

National Totals: People With and Without Disabilities

PWD Total	PWOD Total	PWD Share of Population	PWOD Share of Population
42,703,063	284,722,215	13.0%	87.0%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: These totals summarize the overall size and share of the U.S. population with and without disabilities.

Table 1: National Totals: Number and share of people with and without disabilities.

National totals in Table 1 indicate that approximately 42.7 million people in the United States report having a disability, representing about 13% of the civilian non-institutionalized population. The remaining 284.7 million individuals, roughly 87% of the population, are people without disabilities.

These figures provide a national baseline for understanding the size and distribution of the disabled population in the demographic groups and geographic levels which follow . While the majority of U.S. residents do not report a disability, the proportion of people with disabilities remains substantial in absolute terms, underscoring the importance of examining how social, economic, and geographic factors shape their lived experiences.

Types of Disability: Nationally

Disability Type	Number with Disability Type	Percent with Disability Type	Prevalence among PWD
Mobility	20,564,367	6.7%	48.2%
Cognitive	16,770,218	5.4%	39.3%
Independent Living	14,883,921	5.9%	34.9%
Hearing	11,693,513	3.6%	27.4%
Vision	7,913,012	2.4%	18.5%
Self-Care	7,898,365	2.6%	18.5%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Disability types can overlap, so counts and percentages by type do not sum to the total number of people with disabilities. "Share of Disabled" is each type's count divided by the total number of people with any disability (not the sum across types).

Table 2: Types of disability nationally: totals, percentages, and share of the disabled population (2023).

Our second table above presents a national-level snapshot of disability types among people with disabilities (PWD) in the United States, showing both the number of individuals reporting each disability type and two important percentages - the percent of the total population who report that disability type, and the prevalence among individuals with disabilities. Since individuals can have more than one disability, this latter column's total will exceed 100%, for it reflects overlapping experiences rather than distinct population groups.

Mobility-related disabilities are the most prevalent, affecting approximately 20.6 million individuals, which corresponds to 6.7% of the total population and is experienced by almost half of the disability community. Cognitive disabilities are the second most common, reported by roughly 16.8 million people, which is experienced by 39.3% of people with disabilities and almost 5.4% of the total population.

Independent living difficulty ranks next by total number. Its higher percentage, despite a lower count than cognitive difficulty, reflects differences in measurement, since independent living difficulty is assessed only for adults (18+), whereas cognitive difficulty applies to the full population. Percentages therefore reference different underlying populations

Hearing, vision, and self-care disabilities occur less frequently in comparison but still affect millions of individuals. Hearing disabilities are reported by approximately 11.7 million people (3.6% of the population and a 27.4% prevalence among PWD), while vision and self-care disabilities each affect about 7.9 million individuals, accounting for roughly 2.4–2.6% of the population and an 18.5% rate within the disabled population. Although these categories represent smaller shares, their prevalence remains substantial in absolute terms.

Such distributions highlight the diversity of disability experiences in the United States and sets further context within the broader disability landscape. We turn next to prominence and variation in all-type disability at lower levels of geography.

2.1.2 Regional Disability

Regional Distribution of the Total Population vs. Disabled Population

Region	Share of Total Population	Share of Disabled Population
South	38.3%	40.3%
West	23.7%	22.2%
Midwest	20.8%	20.9%
Northeast	17.3%	16.6%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values show each region's share of the U.S. total population versus its share of the U.S. population with disabilities.

Table 3: Regions by share of the total population and share of the disabled population.

Examining the distribution of people with disabilities across U.S. regions reveals a pattern largely aligned with overall population size, but with subtle deviations which suggest important geographic dynamics. In line with having the largest share of the total population (38.3%) the South accounts for the largest share of the national disabled population (40.3%), However, its share of the disabled population is two percent greater than expected if disability were evenly distributed across regions. The West and Midwest each comprise roughly one-fifth of both the disabled and total populations, indicating proportional representation. Meanwhile, the Northeast contributes the smallest share of the national population and a similarly small share of people with disabilities.

It's imperative to note that these are *not* measures of prevalence, they do not indicate how likely residents in each region are to have a disability, but instead reflect how the national population of people with disabilities is distributed geographically. The South's modest over representation, for instance, may stem from regional differences in age structure, health, or other socioeconomic conditions which will be explored in the sections that follow. Understanding how such factors relate to one another may provide important context for planners and policymakers.

2.1.3 State Disability

Disability Counts & Percentages By State

State	PWD Count	PWOD Count	PWD %	PWOD %
Alabama	807,069	4,162,797	16.2%	83.8%
Alaska	92,451	611,682	13.1%	86.9%
Arizona	970,404	6,184,557	13.6%	86.4%
Arkansas	528,995	2,448,711	17.8%	82.2%
California	4,364,431	34,397,307	11.3%	88.7%
Colorado	638,686	5,075,117	11.2%	88.8%
Connecticut	429,568	3,126,135	12.1%	87.9%
Delaware	137,738	854,560	13.9%	86.1%
District of Columbia	72,611	590,403	11.0%	89.0%
Florida	2,924,178	18,667,410	13.5%	86.5%
Georgia	1,375,224	9,252,295	12.9%	87.1%
Hawaii	173,766	1,212,342	12.5%	87.5%
Idaho	257,204	1,609,581	13.8%	86.2%
Illinois	1,482,931	11,039,567	11.8%	88.2%
Indiana	927,862	5,787,049	13.8%	86.2%
Iowa	390,431	2,759,185	12.4%	87.6%
Kansas	386,716	2,489,083	13.4%	86.6%
Kentucky	784,920	3,644,413	17.7%	82.3%
Louisiana	729,316	3,793,755	16.1%	83.9%
Maine	212,043	1,147,764	15.6%	84.4%
Maryland	690,463	5,388,955	11.4%	88.6%
Massachusetts	835,832	6,091,058	12.1%	87.9%
Michigan	1,414,434	8,529,864	14.2%	85.8%
Minnesota	642,876	5,010,012	11.4%	88.6%
Mississippi	500,688	2,383,757	17.4%	82.6%
Missouri	886,543	5,176,246	14.6%	85.4%
Montana	156,093	932,083	14.3%	85.7%
Nebraska	235,106	1,702,436	12.1%	87.9%
Nevada	416,368	2,686,674	13.4%	86.6%
New Hampshire	178,623	1,192,880	13.0%	87.0%
New Jersey	975,308	8,199,100	10.6%	89.4%
New Mexico	346,122	1,731,749	16.7%	83.3%
New York	2,394,964	17,279,282	12.2%	87.8%
North Carolina	1,386,506	8,980,198	13.4%	86.6%
North Dakota	89,750	671,819	11.8%	88.2%
Ohio	1,646,041	9,966,743	14.2%	85.8%
Oklahoma	656,264	3,250,521	16.8%	83.2%
Oregon	635,797	3,561,149	15.1%	84.9%
Pennsylvania	1,820,613	10,979,922	14.2%	85.8%
Rhode Island	146,643	932,053	13.6%	86.4%
South Carolina	738,495	4,374,663	14.4%	85.6%
South Dakota	106,698	773,025	12.1%	87.9%
Tennessee	1,034,275	5,853,850	15.0%	85.0%
Texas	3,491,298	25,666,647	12.0%	88.0%
Utah	339,979	2,964,489	10.3%	89.7%
Vermont	92,624	546,485	14.5%	85.5%
Virginia	1,032,353	7,396,788	12.2%	87.8%
Washington	1,010,563	6,612,734	13.3%	86.7%
West Virginia	334,044	1,416,686	19.1%	80.9%
Wisconsin	701,774	5,126,293	12.0%	88.0%
Wyoming	79,382	490,341	13.9%	86.1%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages show each state's share of residents with and without disabilities; totals reflect ACS 5-year estimates.

Table 4: Disability counts and percentages by state.

Table 4 above provides a state-by-state breakdown of disability counts and percentages using the 2023 ACS 5-year estimates. Several clear patterns emerge. First, states with larger total populations such as California, Texas, Florida, and New York, as expected, have the highest absolute numbers of both people with disabilities and people without disabilities. These figures reflect population size rather than disproportionately high or low prevalence.

When turning to percentages, regional differences become more apparent. Many states in the South and Appalachia show comparatively higher disability prevalence, often exceeding 15% of the population. These patterns broadly align with long-standing demographic and health trends in these regions, including older age structures, higher rates of chronic health conditions, and socioeconomic constraints.

Conversely, several states in the West and Northeast report lower disability percentages, typically around 10–12%. These states often have preferable demographic and labor force characteristics such as a younger population and higher median incomes.

The PWOD percentages mirror these trends in an inverse manner, reinforcing how disability prevalence varies across the country. While these figures are descriptive rather than causal, they provide a foundational view of geographic disparities and set the stage for a more magnified analysis.

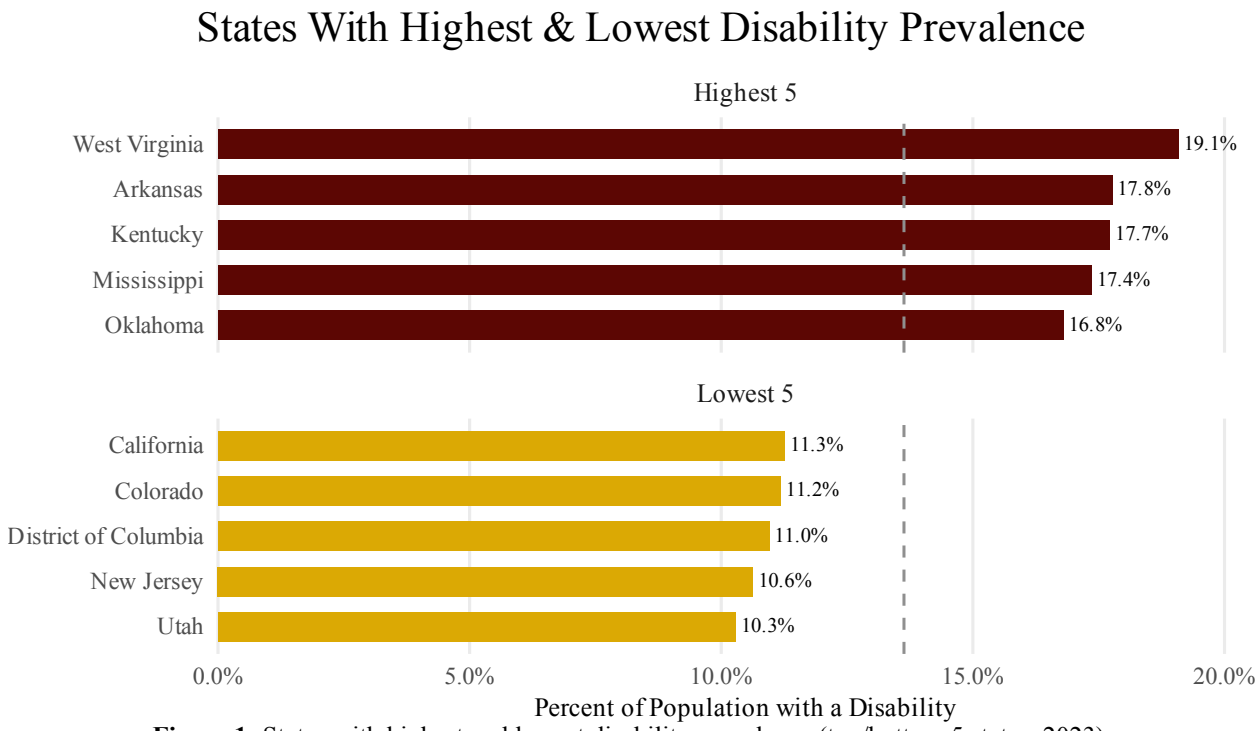


Figure 1: States with highest and lowest disability prevalence (top/bottom 5 states, 2023).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Bars show the five states with the highest and the five states with the lowest disability prevalence; the dashed line indicates the national average across states.

Zooming in on the leading and lagging states in terms of disability prevalence allows a deeper understanding of the range of outcomes across U.S. states. On the less favorable end West Virginia, Arkansas, Kentucky, Mississippi, and Oklahoma exhibit the highest prevalence rates, with values from 16.8% to 19.1%. These states have consistently ranked high in disability prevalence across Census releases, reflecting a both consistently worse conditions and a lack of success in their remediation.

At the opposite end of the distribution, Utah, New Jersey, the District of Columbia, Colorado, and California report the lowest disability prevalence, with approximately 10–11% of their populations identified as people with disabilities. Interestingly, these states differ substantially in terms of size and regional context, but they tend to converge on population characteristics predictive of such low disability prevalence including younger age profiles, higher average educational attainment, and labor market characteristics including a higher concentration of low-risk professions.

2.1.4 City Disability

Percent of Population With Disability: 10 Most Populated U.S. Cities

City	Total Population	Percent With Disability
New York, New York	8,460,246	11.7%
Los Angeles, California	3,829,863	11.1%
Chicago, Illinois	2,683,910	11.9%
Houston, Texas	2,281,731	11.0%
Phoenix, Arizona	1,611,990	11.5%
Philadelphia, Pennsylvania	1,570,102	17.4%
San Antonio, Texas	1,435,376	15.5%
San Diego, California	1,337,485	10.0%
Dallas, Texas	1,289,860	11.4%
San Jose, California	986,212	9.6%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Cities are ordered by population size.

Table 5: Percent of population with a disability in the 10 most populated U.S. cities.

Similar to the state level, we see variation in disability prevalence across cities, even when looking only at the largest-population urban centers in the country. Table 5 above summarizes disability prevalence among the ten most populous U.S. cities, highlighting both total population size and the percent of residents reporting a disability. Across this group, disability prevalence generally falls within a modest range, spanning from 9.6% in San Jose to 17.4% in Philadelphia. Most large cities—including New York, Los Angeles, Chicago, Houston, Phoenix, Dallas, and San Diego—cluster between 10% and 12%, indicating broadly similar prevalence levels despite substantial differences in population size and regional context.

Two cities stand out from this pattern. Philadelphia reports a notably higher disability prevalence at 17.4%, while San Antonio also exceeds the rest of the group at 15.5%. These higher rates distinguish them from other large cities and suggest that disability prevalence in urban areas can vary meaningfully even among cities of comparable scale.

While the nation's largest cities contain very large numbers of people with disabilities due to their population size, their disability prevalence rates are often comparable to or only modestly lower/higher than those observed elsewhere. It may be the case that such resource-abundant urban centers offer unique challenges and opportunities for people with disabilities which is to be explored further. Such framing helps contextualize urban disability patterns and should render one curious as to the complex ways in which access to services within urban centers may affect disability prevalence and quality-of-life (QOL) outcomes.

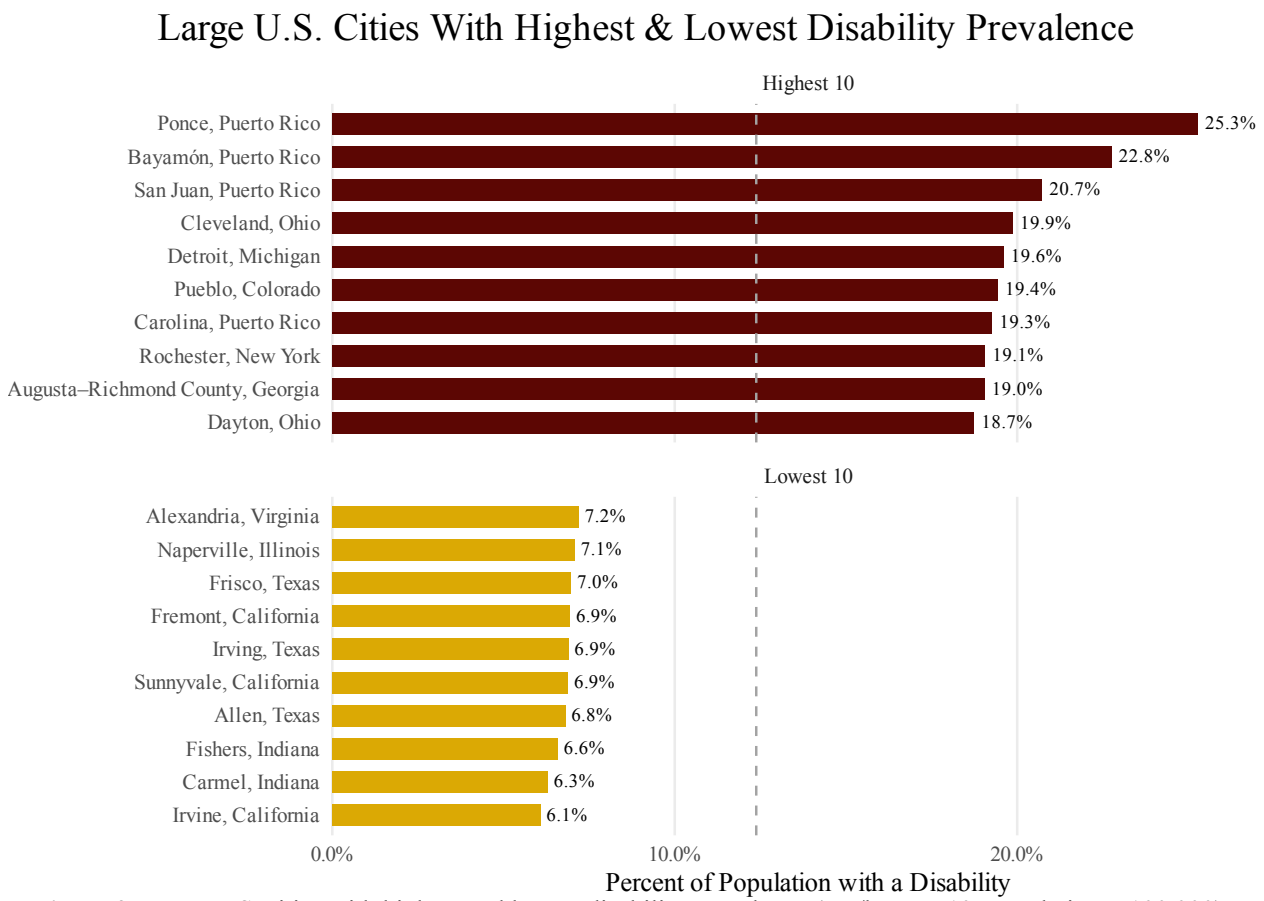


Figure 2: Large U.S. cities with highest and lowest disability prevalence (top/bottom 10; population ≥ 100,000).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Bars show the ten large cities (population ≥ 100,000) with the highest and the ten with the lowest disability prevalence; the dashed line indicates the mean across all eligible large cities.

Moving from the largest cities in the nation to just cities with a population exceeding 100,000, Figure 5 above illustrates substantial variation in disability prevalence among large U.S. cities. Cities in Puerto Rico appear prominently at the high end of the distribution. Ponce, Bayamón, and San Juan each report disability prevalence rates above 20%, placing them well above most large U.S. cities and at the upper extreme of the national urban distribution.

Several mainland and well-known cities also exhibit relatively high disability prevalence including Cleveland, Ohio, and Detroit, Michigan. Along with Pueblo, Colorado, they report rates approaching 19–20%, distinguishing them from other large cities and reflecting persistent differences in population health and demographic composition. While these cities vary in regional context, their elevated prevalence underscores that high disability rates are related to many factors of which geographic area is just one.

At the opposite end of the spectrum, a set of cities report notably low disability prevalence, generally clustered between 6% and 7%. Cities including Irvine, California, Carmel, Indiana, Naperville, Illinois, and Allen, Texas fall at the lower bound of the distribution. These cities tend to differ markedly from high-prevalence cities and national averages being younger, lower in poverty rates, higher in percentage of working age professionals with access to high-quality health care, among other factors.

Such contrast between the highest and lowest prevalence cities reveals a wide range of urban disability experiences, with rates in some cities more than three to four times those observed in others of similar size.

This degree of variation highlights the importance of local context when assessing disability prevalence and reinforces the need for place-sensitive understandings of the needs of people with disabilities.

Number & Type of Disability Summary

Disability in the United States is not evenly distributed across regional, state, and city geographies. At the national level, mobility, cognitive, and independent living disabilities account for the largest shares of reported disability, together comprising the majority of individuals with disabilities.

Further, the relative prevalence of disability varies systematically across geographic levels. States and cities which, relative to average, are older, higher poverty, lower income, and more blue-collar tend to exhibit especially high disability prevalence. This pattern is visible across much of the South and Appalachia, as well as in parts of the Midwest, where disability prevalence is consistently elevated. In contrast, younger or faster-growing metropolitan areas often show a relatively lower rate of disability prevalence generally.

Perhaps unsurprising, variance in disability prevalence becomes more pronounced at the city level. Large urban centers often exhibit more balanced distributions, reflecting demographic diversity and a wide range of health and occupational exposures. Cities with unfavorable workforce or demographic characteristics, as well as those generally economically distressed generally report higher rates of disability. Next, we look towards specific demographic groupings such as age, sex, and race before transitioning into our ADA-PARC categories of analysis.

2.2 Age

2.2.1 National Level Age

People With Disabilities By Age: Nationally

Age Group	Number With Disability	Percent With Disability
Under 18	3,378,203	4.7%
Ages 18–64	21,269,700	10.7%
Ages 65+	17,981,558	32.9%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect the share of each age group that reports any type of disability.

Table 6: People with disabilities by age — United States.

Age group variation of disability rates in the United States reveals clear life-course patterns that differ markedly across population groups. Among children under age 18, approximately 3.4 million individuals are part of the disability community, representing about 4.7% of the child population. Disability prevalence is comparatively low at younger ages, reflecting both developmental trajectories and the limited applicability of certain disability measures in childhood. Nevertheless, this population represents a substantial number of children with functional limitations, with important implications for educational access, early intervention services, and family support systems.

Among adults ages 18–64, approximately 21.3 million individuals report a disability, corresponding to a prevalence rate of 10.7% of the working-age population. Although prevalence in this group is well below that observed among older adults, the absolute number is the largest of any age group, underscoring the burden of

functional limitations is still quite significant during prime working, caregiving, and family-forming years. These patterns have direct implications for labor force participation, income stability, access to employment-based benefits, and the demand for workplace accommodations.

Disability prevalence increases sharply in later life. Among adults ages 65 and older, nearly 18.0 million individuals report a disability, thus roughly 1 in 3 people report a disability in this age range. Well-documented age-related changes in health status, mobility, sensory function, and independent living capacity are both to be expected and drive such a steep incline in prevalence. While the older population is smaller in absolute size than the working-age population, the much higher prevalence reflects a concentration of disability in later adulthood. a reality not to be considered as immune from improvement.

2.2.2 Regional Level Age

Average Disability Rates by Age: Regionally

Region	Percent of Age 18-64 with Disability	Percent of Age 65+ with Disability
Northeast	10.6%	30.2%
Midwest	10.8%	31.6%
South	12.6%	35.6%
West	11.0%	33.4%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values are region-level averages of state disability prevalence within each age group.

Table 7: Average disability rate by age group in U.S. regions .

Regional patterns in disability prevalence reveal meaningful variation across the United States, with differences that become more pronounced at older ages. Among children under age 18, disability rates are relatively low with minimal discrepancy across regions, ranging from approximately 4.4% in the Midwest and West to 5.2% in the South. Across regions approximately 1 in 20 children are reported to have a disability.

Among working-age adults ages 18–64, disability prevalence remains relatively consistent across regions, spanning from 10.6% in the Northeast to 12.6% in the South. A narrow spread indicates that disability during prime working years is a widespread national phenomenon rather than one concentrated in a particular region, although the South's slightly higher rate points to a greater overall burden within that population. Across regions approximately 1 in 10 working age adults have a disability.

Regional differences widen more among adults ages 65 and older. Disability prevalence in this age group is highest again in the South (35.6%), compared with roughly 30–33% in the Northeast, Midwest, and West. This larger regional gap in later life is consistent with well-documented differences in chronic disease burden, cumulative socioeconomic disadvantage, and access to resources and services supportive of functional health, all of which may exert stronger effects as individuals age.

These patterns indicate that regional variation in disability is relatively muted during childhood and working ages but becomes modestly pronounced in older adulthood. Such divergence highlights the importance of regional context in planning for age-related health needs, and services particularly for older adults.

2.2.3 State Level Age

Percent Of Population With Disability By Age Group & State

State	Under 18	18–64	65+
Alabama	4.7%	14.2%	38.3%
Alaska	4.5%	11.7%	35.4%
Arizona	4.9%	11.0%	32.2%
Arkansas	6.4%	15.8%	40.3%
California	3.8%	8.6%	33.3%
Colorado	4.0%	9.3%	29.5%
Connecticut	4.9%	9.6%	29.2%
Delaware	5.2%	11.5%	30.1%
District of Columbia	4.6%	8.8%	32.3%
Florida	5.0%	10.1%	31.4%
Georgia	4.8%	11.0%	34.4%
Hawaii	3.6%	8.9%	32.4%
Idaho	4.6%	11.9%	34.2%
Illinois	3.9%	9.5%	31.5%
Indiana	5.1%	12.1%	33.2%
Iowa	4.2%	10.4%	30.3%
Kansas	4.8%	11.7%	32.8%
Kentucky	6.2%	16.2%	39.0%
Louisiana	6.0%	14.1%	38.5%
Maine	5.9%	13.0%	31.0%
Maryland	4.3%	9.2%	29.6%
Massachusetts	4.9%	9.4%	30.2%
Michigan	4.9%	12.2%	32.2%
Minnesota	4.1%	9.4%	28.7%
Mississippi	5.9%	15.4%	41.0%
Missouri	5.0%	12.6%	34.4%
Montana	4.6%	11.8%	32.7%
Nebraska	3.9%	10.3%	31.7%
Nevada	4.6%	11.0%	34.1%
New Hampshire	4.8%	10.7%	28.6%
New Jersey	3.7%	8.0%	29.5%
New Mexico	5.1%	14.3%	37.6%
New York	4.3%	9.4%	31.6%
North Carolina	4.5%	11.1%	33.2%
North Dakota	3.9%	9.9%	31.3%
Ohio	5.3%	12.0%	32.9%
Oklahoma	5.7%	15.0%	40.6%
Oregon	5.2%	12.6%	34.4%
Pennsylvania	5.8%	11.7%	31.8%
Rhode Island	5.5%	11.1%	30.9%
South Carolina	4.9%	12.0%	33.6%
South Dakota	3.6%	10.1%	31.4%
Tennessee	5.1%	12.7%	36.7%
Texas	4.8%	10.0%	35.4%
Utah	4.0%	9.4%	30.4%
Vermont	5.4%	12.3%	29.2%
Virginia	4.6%	10.0%	31.3%
Washington	4.6%	11.0%	33.7%
West Virginia	5.6%	16.7%	39.5%
Wisconsin	4.3%	10.0%	28.6%
Wyoming	4.3%	11.4%	34.6%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect the share of each age group within each state that reports any type of disability.

Table 8: Percent of population with a disability, by age group and state.

Above table presents state-level disability prevalence across our three age groups, highlighting both life-course patterns and noteworthy geographic variation. Among children, disability prevalence is consistently lower than in adulthood, generally ranging from 3.6% in Hawaii and South Dakota to 6.4% in Arkansas. These differences likely reflect marginal advantages or disadvantages in child health, early-life socioeconomic conditions, access to diagnostic and support services, and reporting practices.

Among working-age adults (18–64), disability prevalence ranges from 8% in New Jersey to 16.7% in West

Virginia., Differences within this broad age range are likely due to a complex web of factors, including economic structure, occupational risk exposure, access to healthcare, and long-standing regional disparities that modify risk earlier in adulthood.

Disability prevalence and inter-state variance rises sharply in older adulthood across every state. Among individuals ages 65 and older, rates fall between 28.6% in New Jersey and 41% in Mississippi, underscoring the strong relationship between aging and functional limitation.

Collectively, these patterns demonstrate that disability is both age-structured and place-based. Prevalence increases steadily from childhood through working age and rises sharply in later life, while persistent cross-state differences particularly in the eldest age group point to the influence of broader social, economic, and health-related conditions.

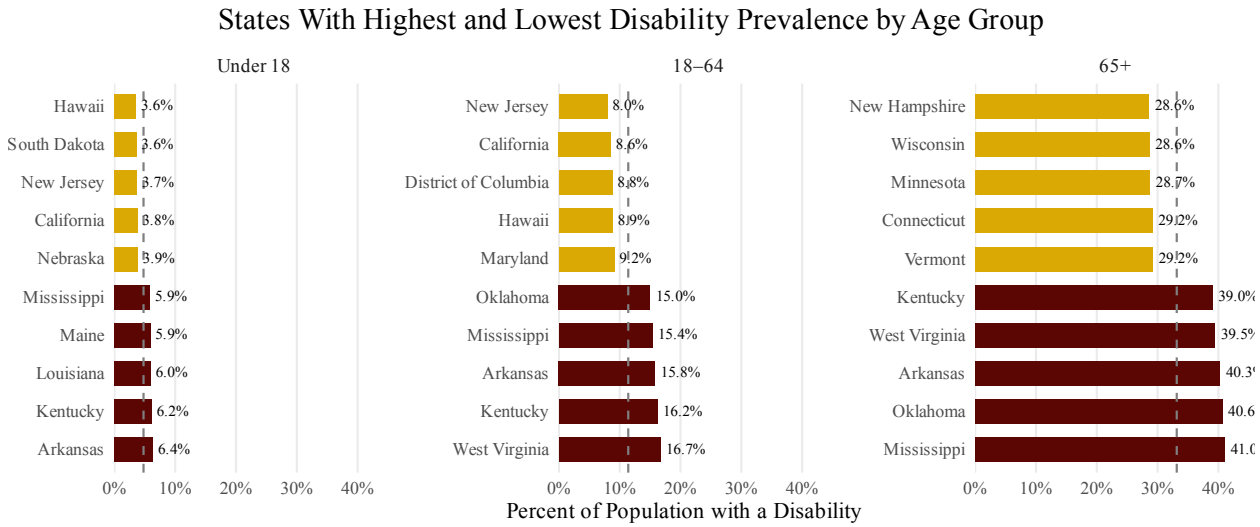


Figure 3: States with highest and lowest disability prevalence by age group (Under 18, 18–64, 65+), 2023.

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each panel shows the five states with the highest and the five with the lowest disability prevalence for the specified age group (Under 18, ages 18–64, and ages 65+). Disability prevalence for Under 18 is calculated as the number of children with a disability divided by the total under-18 population in each state; prevalence for ages 18–64 and 65+ is based on age-specific ACS percentages. Bars are colored by highest versus lowest group, and the dashed line indicates the mean disability prevalence across states within each age group.

Zooming in on state-level patterns, the figure highlights the five states with the highest and lowest disability prevalence across the same three age groups. Results in each panel are slightly different, but together they show that disability prevalence varies meaningfully by age and geography, with distinct regional patterns emerging at different stages of the life course.

Among working-age adults, disability prevalence in the extremes shows pronounced geographic concentration. States such as West Virginia, Kentucky, Arkansas, Mississippi, and Oklahoma report the highest rates, ranging from roughly 15% to 17%, modestly above the state average indicated by the dashed line. In contrast, Maryland, Hawaii, the District of Columbia, California, and New Jersey exhibit the lowest working-age disability prevalence, with rates between 8% to 9%.

Patterns among older adults reveal even sharper contrasts. Mississippi, Oklahoma, Arkansas, West Virginia, and Kentucky report the highest disability prevalence among older adults, with rates approaching or exceeding 40%. At the lower end of the distribution, Vermont, Connecticut, Minnesota, Wisconsin, and New Hampshire show substantially lower disability prevalence among adults ages 65+, around 28–30%.

Disability prevalence among children is markedly lower overall but still exhibits some state-level variation. States reporting the highest prevalence are Arkansas, Kentucky, Louisiana, Mississippi, and Maine, with rates of 6–6.5%, while Hawaii, South Dakota, New Jersey, California, and Nebraska fall at the lower end, closer to 3.5–4%.

It's noteworthy that in terms of low prevalence, California, New Jersey and Hawaii appear in both of the first two panels but not the third. Though perhaps. even more stark is the appearance of Arkansas, Mississippi, and Kentucky appearing in all three of the high-prevalence groupings, while Oklahoma and West Virginia appear in the latter two. As such, this visual demonstrates disability is not evenly distributed across states at any age range, and that regional disparities widen with age.

2.2.4 City Level Age

Disability By Age: 10 Most Populated U.S. Cities

City	Percent of Age 18+ with Disability	Percent of Age 65+ with Disability
New York, New York	8.5%	34.6%
Los Angeles, California	8.3%	36.3%
Chicago, Illinois	9.6%	35.4%
Houston, Texas	8.9%	35.1%
Phoenix, Arizona	10.2%	32.5%
Philadelphia, Pennsylvania	15.6%	40.8%
San Antonio, Texas	13.5%	41.8%
San Diego, California	7.3%	31.2%
Dallas, Texas	9.8%	34.9%
San Jose, California	6.3%	34.3%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect the share of each age group within each city that reports a disability.

Table 9: Percent of population with a disability by age group — top 10 most populated U.S. cities.

Across the ten most populated U.S. cities, disability prevalence displays a clear and consistent age gradient beginning in childhood and intensifying across adulthood. Among children under age 18, disability prevalence generally ranges between 3.5% and 5% in most cities. Philadelphia (7.7%) and San Antonio (6.6%) stand out with notably higher rates while San Diego (3.4%), and Los Angeles (3.5%) have the lowest rates .

Among working-age adults disability rates typically fall between 6% and 10% in most cities, with San Jose having a low rate of 6.3%, and Philadelphia the highest. (15.6%). Disability prevalence rises sharply among adults ages 65 and older, ranging from 31% to nearly 42% across the ten cities. San Diego (31.2%) and Phoenix (32.5%), report the lowest prevalence in this range while once again, San Antonio (41.8%) and Philadelphia (40.8%) appear at the upper end.

Still, the table demonstrates that age remains one of the strongest predictors of disability prevalence even within large metropolitan areas that differ substantially in geography, economic structure, and demographic composition. Consistency in terms of the localities with the highest and lowest prevalence suggest cause for deeper investigation.

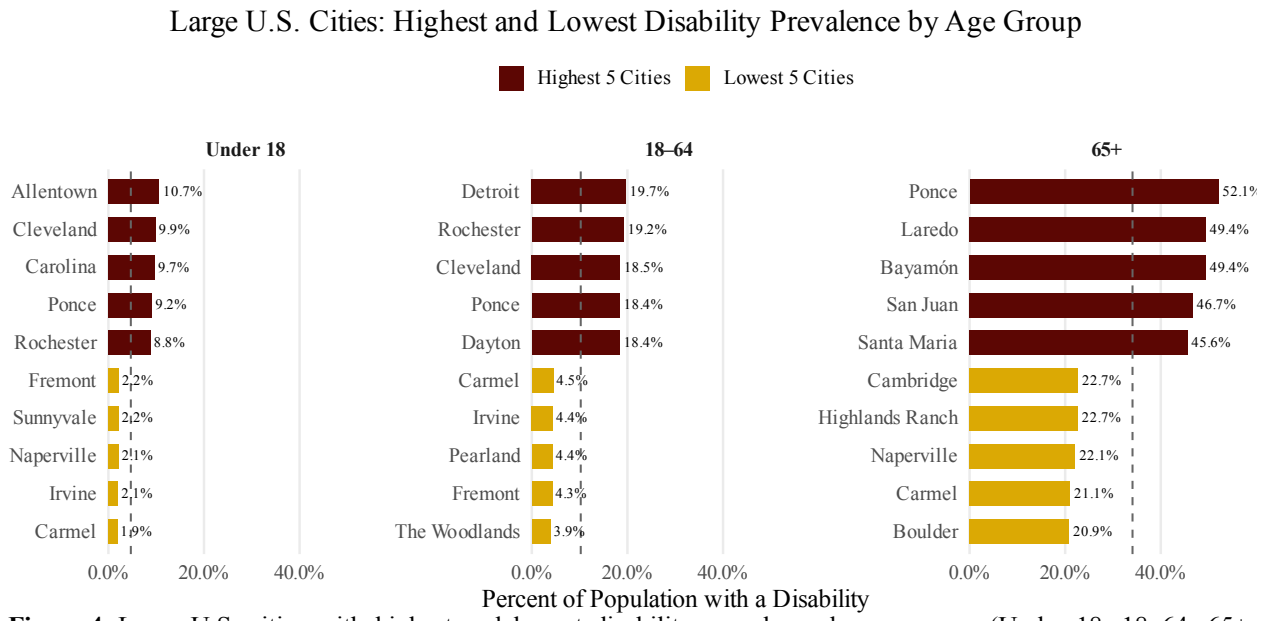


Figure 4: Large U.S. cities with highest and lowest disability prevalence by age group (Under 18, 18–64, 65+; top/bottom 5; population ≥ 100,000; 2023).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Bars show the five large cities (population ≥ 100,000) with the highest and the five with the lowest disability prevalence within each age group (Under 18, 18–64, 65+); the dashed line indicates the mean across all eligible large cities within each age group.

Moving from the largest U.S. cities to those with extreme outcomes, we get a better sense of the range of disability prevalence within large cities. Cities of Detroit, Rochester, Cleveland, Ponce, Puerto Rico and Dayton report the highest working-age disability rates, approaching 2x the large-city average indicated by the dashed line. In contrast, cities including Carmel, Irvine and Fremont, California, and in Texas Pearland, and Woodlands CDP, report substantially lower rates of 4% to 5%.

Disability prevalence among older adults is substantially higher across all large cities but also shows dispersion. Ponce, Bayamón, and San Juan, Puerto Rico, along with Laredo, Texas and Santa Maria, California, report the highest rates approaching or exceeding majorities. At the lower end of the distribution, cities such as Cambridge, Boulder, Carmel, Naperville, Illinois; and Highlands Ranch, Colorado report rates closer to a 1 in 5 individuals range.

Among children cities such as Allentown, Cleveland, Carolina and Ponce, Puerto Rico; and Rochester, exhibit the highest child disability prevalence, generally near or above 10%. By contrast, cities including Carmel, Irvine and Fremont, and Sunnyvale, California as well as Naperville, report markedly lower rates, closer to the 1 in 50 children range.

Across all three age groups, the consistent divergence from the large-city averages and the appearance of the same cities across multiple panels suggests that disability prevalence is also place-based. While aging remains a central driver of higher prevalence in later life, the persistence of wide geographic differences points to broader structural conditions which affect urban contexts differently.

Age Summary

Age is one of the strongest organizing dimensions of disability prevalence, and age-specific patterns remain remarkably consistent across geographic levels while still exhibiting meaningful local variation. Nationally, disability prevalence among adults ages 18–64 is substantially lower than among adults 65 and older, yet the

absolute number of working-age adults with disabilities is large, underscoring that disability is not confined to older adulthood.

At the regional level, working-age disability prevalence varies modestly, with the South consistently exhibiting higher rates than the Northeast, Midwest, or West. This regional pattern persists at the state level, where many Southern and Appalachian states show elevated disability prevalence among adults ages 18–64, while states in the Mountain West, Northeast Corridor, and parts of the Mid-Atlantic tend to report lower rates. These differences remain visible even after accounting for population size, indicating that prevalence—not just counts—varies meaningfully across states.

Among adults ages 65 and older, disability prevalence increases sharply everywhere, but geographic disparities widen substantially. States with the highest older-adult disability rates are heavily concentrated in the South and parts of Appalachia, while lower rates are more common in New England, the Upper Midwest, and select Western states. City-level patterns reinforce this divide: some cities exhibit disability prevalence among older adults exceeding 40%, while others remain closer to 30%, reflecting variation in population health, aging trajectories, and local demographic composition.

Importantly, age-based rankings of places are often stable across both age groups. States and cities with high working-age disability prevalence frequently also rank high among older adults, suggesting cumulative disadvantages that operate across the life course rather than age-specific anomalies.

As population aging continues in the United States, both the number and share of older adults with disabilities are likely to grow, increasing demand for accessible housing, long-term services and supports, and age-friendly community design. At the same time, the sizeable working-age population with disabilities highlights the importance of policies that promote economic inclusion, employment retention, and adequate health and social supports across adulthood, while early-life disability underscores the role of education and family-centered interventions.

2.3 Sex

2.3.1 National & Regional Level Sex

People with Disabilities by Sex: National and Regionally

Region	Sex	Number with a Disability	Percent with a Disability
United States	Female	21,985,723	13.2%
United States	Male	20,717,340	12.9%
Northeast	Female	3,745,307	13.0%
Northeast	Male	3,340,911	12.1%
Midwest	Female	4,550,277	13.3%
Midwest	Male	4,360,885	13.0%
South	Female	8,876,394	13.8%
South	Male	8,348,043	13.7%
West	Female	4,813,745	12.4%
West	Male	4,667,501	12.1%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect disability prevalence within each sex group (United States and each U.S. region).

Table 10: People with disabilities by sex — United States and regions (2023).

Nationally, disability prevalence is quite similar between females and males in the United States, with females exhibiting a slightly higher rate. Approximately 22.0 million females (13.2%) and 20.7 million males (12.9%) report having a disability. Although this sex-based difference is modest, it reflects a persistent pattern observed across ACS estimates rather than a one-year anomaly.

This small gap is likely driven primarily by differences in age composition. Women, on average, live longer than men and therefore comprise a larger share of the older adult population, where disability prevalence is substantially higher. These results underscore that disability is a common experience for both females and males and caution against overstating the role of sex in shaping overall disability prevalence.

Across regions, variation is slightly more pronounced than the differences between females and males within each region. In every Census region, the female–male gap remains small, generally within about one percentage point indicating that geography plays a relatively larger role than sex alone in shaping disability patterns.

The South reports the highest prevalence for both females (13.8%) and males (13.7%) and also contains the largest absolute number of people with disabilities. The Midwest and Northeast fall in the middle, with rates around 13% for females and slightly lower for males. The West has the lowest prevalence overall, at 12.4% for females and 12.1% for males.

2.3.2 State Level Sex

People With Disabilities By Sex — All States

State	Male: Number with a Disability	Male: Percent with a Disability	Female: Number with a Disability	Female: Percent with a Disability
Alabama	384,627	16.1%	422,442	16.4%
Alaska	50,294	14.0%	42,157	12.3%
Arizona	483,053	13.7%	487,351	13.5%
Arkansas	262,952	18.0%	266,043	17.5%
California	2,100,090	10.9%	2,264,341	11.6%
Colorado	319,640	11.1%	319,046	11.2%
Connecticut	206,520	11.9%	223,048	12.3%
Delaware	66,028	13.8%	71,710	13.9%
District of Columbia	31,178	9.9%	41,433	11.9%
Florida	1,429,375	13.6%	1,494,803	13.5%
Georgia	657,108	12.8%	718,116	13.1%
Hawaii	87,602	12.9%	86,164	12.2%
Idaho	133,165	14.3%	124,039	13.3%
Illinois	703,472	11.4%	779,459	12.2%
Indiana	448,732	13.5%	479,130	14.1%
Iowa	196,670	12.5%	193,761	12.3%
Kansas	193,742	13.6%	192,974	13.3%
Kentucky	385,391	17.7%	399,529	17.7%
Louisiana	348,677	16.0%	380,639	16.3%
Maine	107,499	16.1%	104,544	15.1%
Maryland	316,284	10.8%	374,179	11.9%
Massachusetts	394,005	11.7%	441,827	12.5%
Michigan	691,232	14.1%	723,202	14.4%
Minnesota	323,565	11.4%	319,311	11.3%
Mississippi	239,300	17.3%	261,388	17.4%
Missouri	435,549	14.7%	450,994	14.6%
Montana	83,154	15.1%	72,939	13.5%
Nebraska	117,591	12.1%	117,515	12.1%
Nevada	209,503	13.5%	206,865	13.3%
New Hampshire	89,265	13.1%	89,358	13.0%
New Jersey	452,858	10.1%	522,450	11.2%
New Mexico	172,711	16.9%	173,411	16.4%
New York	1,105,868	11.5%	1,289,096	12.8%
North Carolina	662,931	13.2%	723,575	13.5%
North Dakota	46,628	12.0%	43,122	11.5%
Ohio	795,900	14.0%	850,141	14.4%
Oklahoma	328,217	17.1%	328,047	16.5%
Oregon	314,390	15.1%	321,407	15.2%
Pennsylvania	868,024	13.8%	952,589	14.6%
Rhode Island	70,385	13.3%	76,258	13.8%
South Carolina	361,170	14.7%	377,325	14.2%
South Dakota	55,563	12.5%	51,135	11.7%
Tennessee	502,018	14.9%	532,257	15.1%
Texas	1,715,603	11.9%	1,775,695	12.0%
Utah	171,759	10.3%	168,220	10.3%
Vermont	46,487	14.7%	46,137	14.3%
Virginia	490,879	12.0%	541,474	12.5%
Washington	499,908	13.1%	510,655	13.4%
West Virginia	166,305	19.2%	167,739	19.0%
Wisconsin	352,241	12.1%	349,533	12.0%
Wyoming	42,232	14.6%	37,150	13.3%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect disability prevalence within each sex group in each state.

Table 11: People with disabilities by sex — all states (2023).

State level patterns in disability prevalence by sex reveal both consistent demographic trends and meaningful geographic variation. In most states, the number of females with a disability exceeds the number of males, reflecting the longer life expectancy of women and the higher prevalence of disability at older ages. This pattern is especially apparent in populous states and those with older age profiles such as Florida, Pennsylvania, New York, and West Virginia where absolute counts of females with disabilities are substantially higher.

However, when examining percentages rather than counts, differences between males and females are gen-

erally small. In many states, disability prevalence is very similar across sexes, while in some states males exhibit slightly higher rates, in most states females do. For example, males have marginally higher prevalence in states such as Arkansas, Idaho, Montana, and Wyoming, whereas females have higher prevalence in states such as New York, Pennsylvania, Indiana, and Massachusetts. In several states like Utah, Nebraska, and Kentucky rates are nearly identical for males and females.

Geographic patterns remain pronounced across both sexes. States in the South and Appalachia, including West Virginia, Kentucky, Mississippi, Arkansas, Alabama, and Oklahoma, consistently report some of the highest disability prevalence for both males and females. In contrast, states such as California, New Jersey, Colorado, Utah, and the District of Columbia exhibit lower disability rates across sexes.

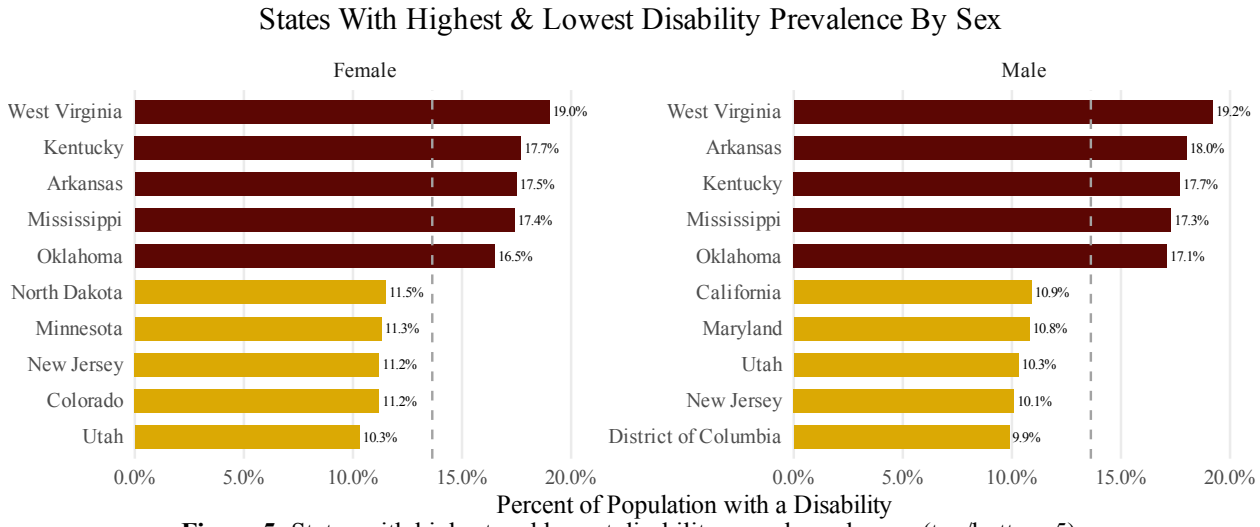


Figure 5: States with highest and lowest disability prevalence by sex (top/bottom 5).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Panels display the five states with the highest and the five with the lowest disability prevalence among females and males; dashed lines indicate the mean across states for each sex.

Focusing on the five states with the best and worst outcomes, the figure above displays states with the highest and lowest disability prevalence for females and males, with the dashed line indicating the national average for each sex. Regardless of sex, the highest disability prevalence states are the same, concentrated in the South, just in different order.

Within the female category, the ordering is as follows: West Virginia (19.0%), Kentucky (17.7%), Arkansas (17.5%), Mississippi (17.4%), and Oklahoma (16.3%). Ranging from 3% to 5% greater than the state average. In contrast, the lowest female disability rates are observed in Utah (10.3%), Colorado (11.2%), New Jersey (11.2%), Minnesota (11.3%), and North Dakota (11.5%), hovering from 3% to 5% below average meaning there is a roughly 10% range across states for females.

As mentioned, the male pattern is quite similar, as it goes West Virginia (19.2%), Arkansas (18.0%), Kentucky (17.7%), Mississippi (17.3%), and Oklahoma (17.1%) comprising the high-prevalence group, each exceeding the national male average by more than 3%. At the low end, California (10.9%), Maryland (10.8%), Utah (10.3%), New Jersey (10.1%), and the District of Columbia (9.9%) report the lowest male disability prevalence.

For both sexes, the same group of Southern and Appalachian states consistently appears at the top of the distribution, while Western, Northeastern, and Mid-Atlantic states tend to form the low-prevalence group.

2.3.3 City Level Sex

People With Disabilities By Sex In 10 Most Populated U.S. Cities

	Male		Female	
City	Number with a Disability	Percent with a Disability	Number with a Disability	Percent with a Disability
New York	426,163	10.5%	562,616	12.8%
Los Angeles	195,956	10.3%	230,837	12.0%
Chicago	143,086	11.0%	176,630	12.7%
Houston	116,484	10.4%	134,720	11.6%
Phoenix	90,119	11.2%	94,712	11.7%
Philadelphia	119,264	16.1%	154,421	18.6%
San Antonio	109,061	15.4%	113,632	15.6%
San Diego	65,088	9.8%	68,830	10.2%
Dallas	67,371	10.5%	79,225	12.2%
San Jose	44,753	8.9%	50,218	10.4%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Percentages reflect the share of males/females in each city who report a disability.

Table 12: People with disabilities by sex: Top 10 most populated U.S. cities.

Across the ten most populated U.S. cities, large numbers of people with disabilities are present among both males and females, reflecting the scale and diversity of major metropolitan areas. Although absolute counts vary with population size, disability prevalence by sex falls within a relatively narrow range across cities.

New York City contains the largest number of people with disabilities for both males and females, consistent with its overall size, yet its prevalence rates closely resemble those in other large cities. Los Angeles, Chicago, Houston, Phoenix, and Dallas show similar patterns, with disability prevalence for each sex generally clustering around the low-to-mid teens. This consistency suggests that, despite differences in geography and economic structure, large urban areas tend to exhibit comparable disability rates by sex.

A few cities stand out. Philadelphia and San Antonio report noticeably higher prevalence for both males and females, exceeding 15%, while cities such as San Jose and San Diego remain at the lower end of the distribution.

Overall, the table indicates that while large cities differ substantially in total population and absolute disability counts, sex-based differences in disability prevalence are modest and rates are broadly similar across major urban centers. These patterns point to the importance of underlying demographic and socioeconomic composition rather than city size alone in shaping disability prevalence.

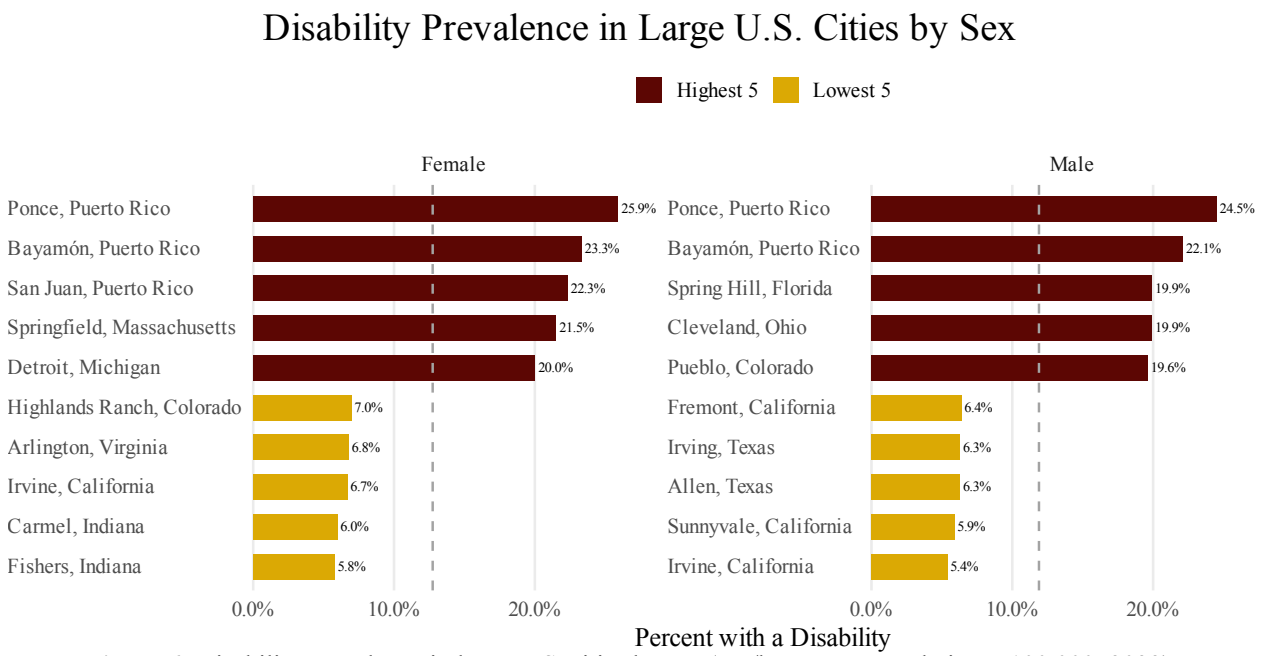


Figure 6: Disability prevalence in large U.S. cities by sex (top/bottom 5; population ≥ 100,000; 2023).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Bars show the five large cities (population ≥ 100,000) with the highest and the five with the lowest disability prevalence among females and males; separate panels are shown for each sex. The dashed line in each panel indicates the mean prevalence across all eligible large cities.

Across U.S. cities with populations greater than 100,000, disability prevalence both varies and follows a similar pattern for both females and males, with geographic context playing a noteworthy role. Puerto Rico's largest cities Ponce and Bayamón, along with San Juan for females rank among the highest-prevalence cities for both sexes, with rates far above the large-city average. Several legacy industrial cities including Detroit and Springfield (MA) for females, as well as Cleveland and Pueblo (CO), for males appear at the top of the distribution. In these locations, disability prevalence among both women and men approaches twice the average rate and 5x the rate in the lowest-prevalence cities.

At this end of the spectrum, the lowest disability rates derive from affluent or rapidly growing suburban and technology-oriented communities. We once again see Irvine, Fremont, and Sunnyvale California appear for males. Carmel and Fishersl Indiana, Highlands Ranch (CO), and Arlington (VA) are represented in the female category. These cities tend to share younger age structures and stronger socioeconomic profiles, which align with lower disability prevalence overall. Together, results indicate that disability prevalence in large urban areas is strongly place-based, reflecting differences in demographic composition, economic context, and access to resources.

Sex Summary

At the national level, disability prevalence among females and males is broadly similar, but geographic analysis reveals how sex differences intersect with place. Females account for a slightly larger share of the population with disabilities overall, driven largely by longer life expectancy and higher representation in older age groups where disability prevalence is greatest.

Across regions and states, disability prevalence by sex tends to move in parallel: areas with high disability prevalence among males almost always show high prevalence among females as well. This alignment is particularly evident in the South and Appalachia, where both sexes experience elevated disability rates

relative to national norms. Conversely, states with lower disability prevalence—such as Utah, New Jersey, Colorado, and California—consistently rank low for both males and females.

At the city level, sex differences remain modest relative to between-city variation. In most large cities, female disability prevalence slightly exceeds male prevalence, though the gap is usually small compared to the difference between high- and low-prevalence cities. Some cities with older populations show larger sex gaps, reflecting the concentration of disability among older women, while younger or rapidly growing cities exhibit tighter alignment between male and female rates.

Overall, these patterns indicate that geographic context dominates sex-based differences in shaping disability prevalence. Sex matters, but it operates within broader structural and demographic environments that influence disability risk across entire populations.

2.4 Race & Ethnicity

2.4.1 National Race & Ethnicity

People With Disabilities By Race/Ethnicity — United States

Racial/Ethnic Group	Number with a Disability	Percent of Group with a Disability
White	28,892,180	13.9%
White (Non-Hispanic)	27,231,154	14.3%
Black	5,767,051	14.5%
Asian	1,516,847	7.9%
Native Hawaiian or Other Pacific Islander	77,971	12.7%
American Indian or Alaska Native	449,827	15.7%
Some Other Race	2,175,598	10.0%
Two or More Races	3,823,589	10.9%
Hispanic or Latino (any race)	6,190,802	9.9%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect disability prevalence within each racial/ethnic group.

Table 13: People with disabilities by race/ethnicity — United States (2023).

Turning from the demographic category of sex to race, table above presents national disability estimates by race and ethnicity from the ACS-5 2023. For each group, it reports both the number of individuals with disabilities and the within-group prevalence, the percentage of people in that racial or ethnic group who report at least one ACS-defined disability.

Because racial and ethnic groups differ substantially in population size, the absolute counts primarily reflect national demographic composition. White and White (Non-Hispanic) individuals account for the largest number of people with disabilities nationally, consistent with their larger share of the U.S. population. In contrast, smaller population groups—such as Native Hawaiian or Pacific Islander and American Indian or Alaska Native individuals—have much lower absolute counts.

When examining percent disabled within each group, some clear disparities emerge. American Indian or Alaska Native individuals report the highest disability prevalence (15.7%), followed by Black (14.5%) and White (Non-Hispanic) individuals (14.3%). Conversely, Asian (7.9%) and Hispanic or Latino populations (9.9%) report comparatively lower disability prevalence. These differences, similar to those we observed with sex, are to some degree a result of younger age distributions rather than lower risk exposure to disability

per se. Importantly, the ACS classification of Hispanic or Latino as an ethnicity rather than a race means that this group overlaps with racial categories, and comparisons should be interpreted accordingly.

Another important note is that these differences do not reflect inherent or biological characteristics of racial or ethnic groups. Instead, they highlight how disability prevalence is shaped by social, economic, and structural conditions that vary systematically across the nation. This may become more clear as we move down the geographic levels.

2.4.2 Regional Race & Ethnicity

Percent Of Each Racial/Ethnic Group With A Disability By U.S. Region

Racial/Ethnic Group	Midwest	Northeast	South	West
American Indian or Alaska Native	16.9%	17.9%	17.3%	16.4%
Asian	6.8%	7.5%	7.3%	9.3%
Black or African American	13.0%	12.8%	15.4%	12.8%
Hispanic or Latino (any race)	9.3%	12.1%	9.1%	10.3%
Native Hawaiian or Other Pacific Islander	11.9%	18.3%	13.8%	14.0%
Some Other Race	10.1%	12.7%	9.2%	10.7%
Two or More Races	11.1%	11.9%	12.3%	11.6%
White	13.1%	13.4%	15.2%	14.1%
White (Non-Hispanic)	13.3%	13.5%	15.5%	14.7%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect disability prevalence within each racial/ethnic group and are averaged across states within each region.

Table 14: Percent of each racial/ethnic group with a disability by U.S. region (2023).

At the regional level, the table above compares group disability prevalence across racial and ethnic identities. Rather than reflecting population size, these percentages show the share of each racial or ethnic group within a given region allowing for clearer comparison across both demographic and geographic contexts.

Several consistent patterns emerge. American Indian or Alaska Native individuals exhibit the highest disability prevalence in every region, with rates fluctuating in a narrow band from 16–18%. This pattern aligns with well-documented structural inequities affecting Indigenous communities nationwide, including disparities in healthcare access, environmental exposure, chronic disease burden, and socioeconomic conditions that vary by region but remain persistent overall.

Black or African American populations also show elevated disability prevalence, particularly in the South, where rates exceed 15%. White and White (Non-Hispanic) populations follow a similar regional gradient, with higher prevalence in the South and Northeast than in the Midwest and West. By contrast, Asian individuals report the lowest disability prevalence in every region, with rates remaining below 10% across the country. These lower percentages are commonly associated with younger age distributions, migration patterns, and differences in health risk profiles rather than necesarilly lower exposure to disability risk. Hispanic or Latino populations display more regional variation, with notably higher disability prevalence in the Northeast compared to the South and Midwest.

Regional differences are evident across nearly all groups. In general, disability prevalence tends to be slighty higher on average in the South (12.8%) and Northeast (13.3%) and lower in the Midwest (11.7%) and West (12.7%), though the magnitude of these differences are modest. West states in particular, show more variance perhaps due the region's diverse population composition and varied socioeconomic contexts.

2.4.3 State Race & Ethnicity

Percent Of Each Racial/Ethnic Group With A Disability By State

State	White	White (Non-Hispanic)	Black	Asian	NHOPI	AIAN	Some Other Race	Two + Races	Hispanic
Alabama	16.7%	16.9%	16.7%	7.4%	18.5%	18.3%	6.8%	14.2%	8.6%
Alaska	13.3%	13.4%	12.2%	9.5%	12.7%	15.8%	12.6%	11.2%	11.3%
Arizona	14.9%	15.8%	12.4%	7.7%	11.0%	15.2%	10.5%	10.9%	10.3%
Arkansas	18.8%	19.1%	18.4%	7.9%	7.3%	20.9%	10.3%	13.1%	8.6%
California	12.9%	14.0%	15.7%	8.8%	12.7%	14.2%	9.2%	9.7%	9.2%
Colorado	11.4%	11.6%	12.5%	7.5%	15.0%	16.2%	11.2%	10.1%	10.0%
Connecticut	12.5%	12.5%	12.6%	6.3%	15.3%	14.9%	12.5%	11.2%	12.2%
Delaware	14.9%	15.1%	13.9%	7.0%	9.0%	18.5%	11.4%	10.9%	9.5%
District of Columbia	5.6%	5.6%	16.9%	4.7%	7.6%	20.0%	7.8%	10.0%	8.2%
Florida	15.1%	15.9%	12.2%	8.8%	11.7%	15.5%	11.2%	10.7%	10.5%
Georgia	13.9%	14.3%	13.6%	6.5%	10.3%	12.2%	7.2%	10.1%	7.7%
Hawaii	14.2%	14.4%	15.7%	13.2%	13.3%	19.0%	11.7%	9.7%	10.0%
Idaho	14.0%	14.3%	9.6%	10.2%	15.3%	18.2%	13.4%	12.6%	10.1%
Illinois	12.4%	12.7%	15.5%	7.4%	9.6%	11.0%	7.9%	8.8%	7.9%
Indiana	14.3%	14.4%	14.4%	5.9%	13.4%	15.7%	10.3%	12.5%	9.3%
Iowa	12.8%	12.9%	10.9%	6.0%	12.3%	18.0%	8.8%	10.8%	8.8%
Kansas	13.8%	14.1%	15.9%	6.9%	7.4%	16.8%	11.8%	11.5%	9.6%
Kentucky	18.4%	18.5%	15.8%	6.4%	13.1%	21.8%	10.2%	14.7%	9.7%
Louisiana	16.6%	16.7%	16.4%	8.7%	22.3%	16.8%	11.0%	14.5%	10.9%
Maine	15.7%	15.7%	11.3%	11.9%	22.3%	23.8%	12.8%	15.2%	12.6%
Maryland	12.3%	12.5%	12.3%	7.1%	9.6%	12.1%	5.8%	9.5%	6.8%
Massachusetts	12.5%	12.5%	13.6%	6.4%	16.6%	18.3%	12.4%	11.4%	12.9%
Michigan	14.2%	14.3%	17.3%	6.4%	16.8%	20.5%	12.0%	12.1%	10.4%
Minnesota	11.8%	11.9%	11.0%	7.4%	14.2%	19.2%	8.9%	9.9%	8.3%
Mississippi	17.9%	18.0%	17.3%	8.4%	16.9%	14.2%	9.9%	15.8%	10.3%
Missouri	14.9%	14.9%	16.0%	6.9%	10.0%	20.1%	10.3%	13.0%	9.9%
Montana	14.3%	14.4%	11.8%	9.8%	14.3%	16.3%	8.4%	14.5%	11.0%
Nebraska	12.7%	12.9%	12.4%	6.6%	13.2%	13.0%	8.2%	10.1%	8.5%
Nevada	15.4%	16.5%	14.0%	10.5%	13.8%	14.2%	9.8%	10.5%	9.4%
New Hampshire	13.4%	13.5%	10.8%	6.9%	37.5%	21.5%	9.7%	10.4%	10.0%
New Jersey	11.5%	11.7%	12.3%	5.9%	12.1%	9.7%	9.8%	9.3%	9.4%
New Mexico	17.4%	19.2%	16.3%	8.1%	21.8%	15.8%	15.9%	16.2%	15.3%
New York	12.5%	12.6%	13.7%	7.8%	15.5%	15.5%	12.0%	11.6%	12.2%
North Carolina	14.1%	14.4%	14.6%	6.0%	14.4%	17.5%	7.0%	10.1%	7.1%
North Dakota	11.8%	11.8%	6.7%	7.9%	6.5%	16.8%	10.3%	11.7%	9.4%
Ohio	14.3%	14.4%	15.5%	6.2%	13.9%	21.0%	13.2%	12.9%	11.7%
Oklahoma	17.8%	18.4%	17.0%	8.1%	11.3%	17.2%	11.9%	14.1%	9.7%
Oregon	16.2%	16.6%	14.9%	8.4%	11.3%	20.6%	9.7%	12.8%	9.8%
Pennsylvania	14.1%	14.1%	17.2%	8.0%	18.8%	17.4%	17.3%	12.7%	14.6%
Rhode Island	14.1%	14.2%	12.4%	7.4%	18.4%	13.5%	15.7%	11.3%	13.0%
South Carolina	14.8%	15.0%	15.3%	8.3%	9.9%	18.5%	8.8%	11.5%	8.6%
South Dakota	12.4%	12.5%	6.1%	7.3%	18.2%	12.9%	9.9%	10.6%	9.4%
Tennessee	15.9%	16.1%	14.2%	7.8%	9.4%	18.5%	6.9%	12.0%	7.7%
Texas	12.8%	13.9%	13.6%	6.5%	15.1%	14.0%	10.3%	10.7%	10.2%
Utah	10.7%	10.9%	8.9%	7.8%	9.9%	16.4%	7.3%	8.7%	8.0%
Vermont	14.7%	14.7%	11.1%	6.9%	8.4%	26.6%	11.9%	14.4%	11.6%
Virginia	13.0%	13.3%	14.0%	6.9%	18.0%	14.8%	7.7%	9.2%	7.7%
Washington	14.7%	15.0%	13.5%	7.6%	11.4%	17.7%	9.4%	10.9%	9.2%
West Virginia	19.3%	19.3%	19.3%	7.1%	29.4%	24.0%	11.6%	17.9%	12.2%
Wisconsin	12.3%	12.4%	14.4%	6.4%	6.7%	17.8%	9.1%	9.8%	8.7%
Wyoming	14.2%	14.4%	8.8%	11.5%	20.0%	13.1%	10.1%	13.4%	10.1%

Note: Native Hawaiian or Other Pacific Islander (NHOPI) and American Indian or Alaska Native (AIAN) are abbreviated in some narrative references.
Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values represent disability prevalence within each racial/ethnic group for each state.

Table 15: Percent of each racial/ethnic group with a disability — states (2023).

Moving down to the state level, the table above presents disability prevalence within and across these same racial and ethnic groups. Across most states, American Indian or Alaska Native (AI/AN) populations report some of the highest disability prevalence, often exceeding rates observed for other racial and ethnic groups within the same state. This result is especially notable given historical marginalization and long-standing structural inequities.

Black or African American populations also exhibit elevated disability prevalence in many states, we see here particularly in some Southern and Midwestern states such as Mississippi, Kentucky, Arkansas, Ohio, and Michigan. Rates of disability across the states for the White population categories often are comparable to or slightly lower than those among Black residents.

Native Hawaiian or Pacific Islander (NH/PI) populations show notable variability across states, with high rates in a small number of states like Vermont, West Virginia, and Maine, where estimates may be influenced by small population sizes but nonetheless signals the need for greater attention.

By contrast, Asian populations consistently report the lowest disability prevalence across most states, usually below 10%. Hispanic or Latino populations tend also to reflect lower disability prevalence than non-Hispanic White and other racial populations in many states, though this pattern varies considerably and may be owing largely by age profiles, though state-specific socioeconomic factors should also be considered.

In sum, the magnitude of racial and ethnic disparities differs meaningfully both within states across racial groups and also within racial groups across states. Some states especially in the South and West, exhibit larger gaps between highest and lowest prevalence groups, variation which further delineates the importance of place among other demographic factors.

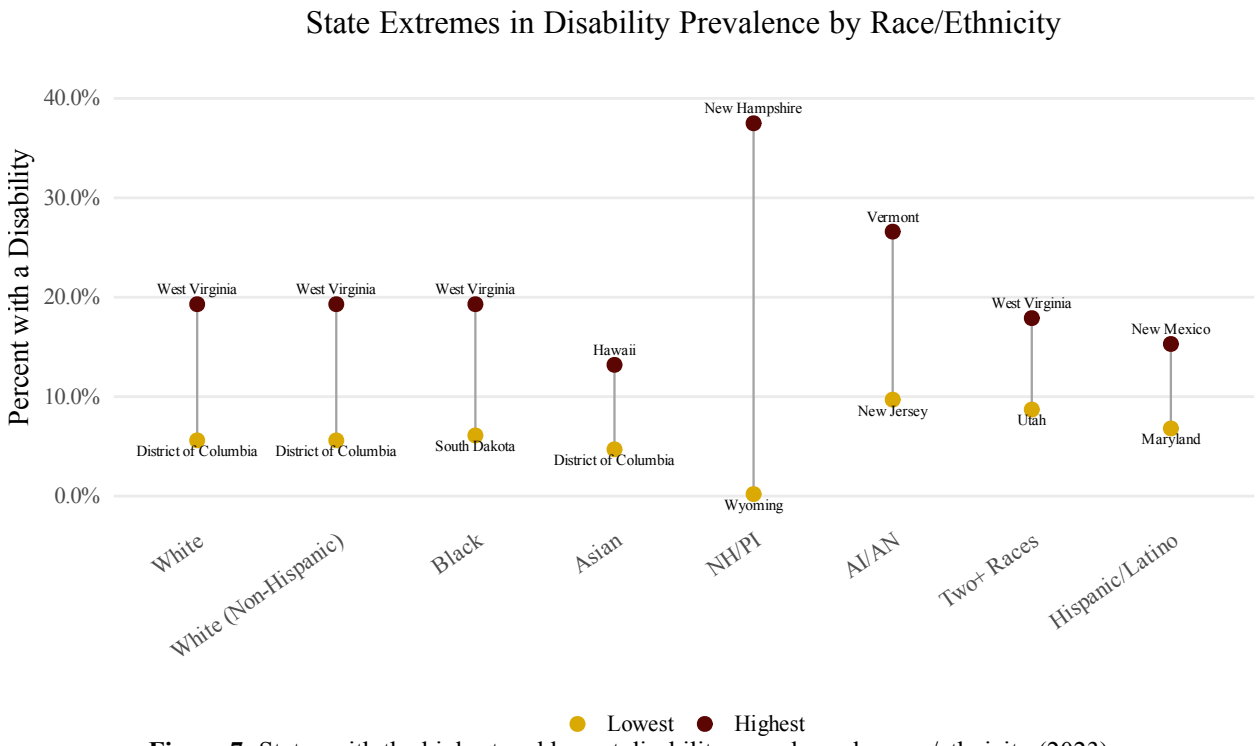


Figure 7: States with the highest and lowest disability prevalence by race/ethnicity (2023).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: For each racial/ethnic group, points mark the states with the lowest and highest disability prevalence; the vertical segment shows the range between them.

Figure above illustrates the states with the highest and lowest disability prevalence within each racial and ethnic group, highlighting how geographic context shapes disability outcomes differently across populations. Rather than showing national averages, the visualization emphasizes extremes the states at the low and high ends of prevalence for each group.

Across most racial and ethnic groups, the lowest disability prevalence repeatedly occurs in a small set of states, including the District of Columbia, New Jersey, Utah, Maryland, and North Dakota. The recurrence of these states across multiple groups suggests that certain state-level conditions such as younger age structures, stronger labor markets, or broader access to health and preventive services may be associated with comparatively lower disability prevalence across diverse populations.

At the high end of the distribution, West Virginia emerges most consistently, appearing as the highest-prevalence state for White, White (Non-Hispanic), and Black populations. Other states—including Vermont, New Hampshire, and New Mexico also appear as high-prevalence states for specific groups, indicating that elevated disability prevalence is not confined to a single region but reflects different combinations of demographic composition, population health, and socioeconomic conditions.

The degree of separation between the highest and lowest states varies substantially by racial and ethnic group. Groups such as Asian and Hispanic or Latino populations exhibit relatively narrower interstate ranges, while Native Hawaiian or Pacific Islander and American Indian or Alaska Native populations show markedly wider gaps between states. These larger spreads likely reflect a combination of geographic concentration or small population sizes in some states.

2.4.4 City Race & Ethnicity

Percent of Each Racial/Ethnic Group With A Disability: Top 10 Most Populated U.S. Cities

City	White	White (NH)	Black	Asian	NH/PI	AI/AN	Some Other Race	Two + Races	Hispanic
New York, New York	10.9%	10.5%	13.7%	8.2%	12.9%	15.4%	13.2%	12.4%	13.5%
Los Angeles, California	11.1%	11.8%	17.8%	10.6%	15.0%	14.4%	9.7%	10.1%	9.6%
Chicago, Illinois	9.7%	9.8%	18.4%	8.0%	5.4%	9.8%	9.4%	9.1%	9.1%
Houston, Texas	10.5%	11.6%	15.6%	8.0%	12.4%	13.1%	7.2%	10.5%	8.8%
Phoenix, Arizona	12.6%	13.5%	12.0%	8.2%	5.3%	14.6%	10.0%	9.6%	9.6%
Philadelphia, Pennsylvania	14.7%	14.7%	19.8%	11.0%	23.6%	19.6%	24.1%	16.8%	20.7%
San Antonio, Texas	15.5%	16.6%	16.3%	8.5%	26.3%	18.2%	18.5%	14.8%	15.4%
San Diego, California	10.2%	10.5%	13.9%	9.0%	11.5%	16.1%	10.1%	8.9%	9.5%
Dallas, Texas	10.2%	11.5%	18.1%	6.1%	7.4%	9.2%	8.2%	8.6%	7.8%
San Jose, California	12.0%	12.9%	13.3%	8.2%	16.5%	12.7%	8.0%	9.2%	8.8%

Source: U.S. Census Bureau, 2023 ACS-5.
Note: NH/PI = Native Hawaiian or Pacific Islander; AI/AN = American Indian or Alaska Native. White (NH) = White Non-Hispanic

Table 16: Percent of each racial/ethnic group with a disability — top 10 most populated U.S. cities (2023).

Across the ten most populated U.S. cities, disability prevalence varies meaningfully across racial and ethnic groups, reflecting both national disparity patterns and city-specific demographic contexts. While overall prevalence levels differ by city, several consistent trends emerge across urban areas.

Many of the major cities have American Indian or Alaska Native (AI/AN) and Native Hawaiian or Other Pacific Islander (NH/PI) populations exhibit some of the highest disability prevalence. By contrast, Asian

residents consistently report the lowest disability prevalence across nearly all of the largest cities, mirroring national ACS patterns and reflecting differences in age profiles and other systematic differences.

In the large coastal metros such as New York, Los Angeles, and San Diego, disability rates across White, Black, Asian, Hispanic/Latino, and multiracial populations tend to fall within a relatively narrower band. By contrast, cities such as Philadelphia and San Antonio exhibit wider spreads across groups, with notably higher prevalence among Black and Hispanic/Latino populations and elevated rates among race other and multiracial populations as well.

Generally, in these major cities, Black residents generally experience higher disability prevalence than White residents, while Asian populations consistently report the lowest rates. Hispanic or Latino populations often fall below Non-Hispanic White and Black populations, though this pattern varies by city and is less uniform than at the regional level.

Large U.S. Cities With Highest & Lowest Disability Rates By Race/Ethnicity (Pop ≥ 100,000)

Group	Highest City	%	Lowest City	%
White	Bayam'on, PR	26.2%	Washington, DC	5.6%
White (NH)	Ponce, PR	33.0%	Washington, DC	5.6%
Black	Escondido, California	29.3%	Carmel, Indiana	4.3%
Asian	Carolina, PR	45.1%	Topeka, Kansas	1.2%
NH/PI	Allentown, Pennsylvania	96.7%	Fullerton, California	1.2%
AI/AN	Westminster, Colorado	90.0%	Athens-Clarke, GA	1.1%
Other Race	Augusta-Richmond, GA	27.3%	Lafayette, Louisiana	1.3%
Two or More Races	Ponce, PR	29.9%	The Woodlands, Texas	3.3%
Hispanic	Ponce, PR	25.3%	Carmel, Indiana	2.6%

Source: U.S. Census Bureau, 2023 ACS-5.
Note: NH/PI = Native Hawaiian or Pacific Islander; AI/AN = American Indian or Alaska Native.

Table 17: Large U.S. cities with the highest and lowest disability rates by race/ethnicity (population ≥ 100,000, 2023).

Across large U.S. cities with populations of at least 100,000, interesting patterns emerge when examining the highest and lowest disability prevalence localities by racial and ethnic group. Municipalities in Puerto Rico appearing previously including Ponce, Bayamón, and Carolina are repeatedly among the highest-prevalence cities. Bayamón records the highest disability rate among White residents (26.2%), while Ponce ranks highest for White (Non-Hispanic) residents (33.0%), Hispanic residents (25.3%), and individuals identifying as Two or More Races (29.9%). Carolina reports the highest prevalence among Asian residents (45.1%). The recurrence of these cities across multiple groups suggests that local context plays an important role in shaping disability prevalence.

For Black residents, Escondido, California reports the highest disability rate (29.3%). Among those identifying as "Other Race," Augusta–Richmond County, Georgia has the highest prevalence (27.3%). At the other end of the distribution, several cities report comparatively low disability prevalence across groups. Washington, DC records the lowest rates for both White and White (Non-Hispanic) residents (5.6%), Carmel, Indiana reports the lowest rate among Black residents (4.3%) and Hispanic residents (2.6%), and Topeka, Kansas shows the lowest rate among Asian residents (1.2%).

Extremely high percentages appear for Native Hawaiian or Pacific Islander (NH/PI) and American Indian or Alaska Native (AI/AN) populations most notably Allentown, Pennsylvania (96.7% for NH/PI) and Westminster, Colorado (90.0% for AI/AN). Conversely, Fullerton, California (1.2% for NH/PI) and Athens–Clarke County, Georgia (1.1% for AI/AN) show the lowest reported rates. These very high or very low values

shouldn't be given too much weight, as ACS estimates for small subpopulations at the city level can be unstable.

Race & Ethnicity Summary

Disability prevalence varies substantially across racial and ethnic groups, and these differences become more complex—and more pronounced—when examined across geographic levels. Nationally, American Indian or Alaska Native and Black populations report some of the highest disability prevalence, while Asian and Hispanic or Latino populations generally report lower prevalence. These national patterns provide a baseline, but they obscure wide variation across states and cities.

At the regional level, racial and ethnic disparities in disability prevalence often widen in the South and Northeast, where several groups exhibit higher within-group disability rates compared to the West and Midwest. For example, Black disability prevalence is consistently higher in Southern states than in other regions, while American Indian or Alaska Native populations show elevated rates across nearly all regions, with particularly high prevalence in certain Western and Plains states.

State-level data reveal that the same racial or ethnic group can experience dramatically different disability prevalence depending on location. In some states, disability prevalence among a given group may be double that observed in another state. These differences reflect the interaction of demographic composition, local economic conditions, health environments, and historical context rather than uniform group-level characteristics.

City-level patterns further underscore this heterogeneity. Some cities—particularly in Puerto Rico—appear repeatedly among the highest-prevalence locations for multiple racial and ethnic groups, while affluent suburban or high-income metropolitan areas often anchor the lowest-prevalence end of the distribution. Notably, for many racial and ethnic groups, the gap between the highest- and lowest-prevalence cities is larger than the national difference between groups, highlighting the outsized role of place.

Because some racial and ethnic populations are small in certain cities, extreme values should be interpreted cautiously due to sampling variability. Nevertheless, the persistence of geographic clustering across multiple groups suggests that disability prevalence is shaped as much by where people live as by demographic identity alone.

2.5 Demographics Summary

- **Disability is widespread, but unevenly concentrated.** Nationally, about **13%** of the population reports a disability (≈ **42.7 million** people), but states and cities vary substantially around that baseline.
- **Mobility and cognitive disabilities are especially common.** Mobility disability affects about **20.6 million** people and is reported by roughly **48%** of people with disabilities; cognitive disability is also prevalent (≈ **16.8 million**, ~**39%** of people with disabilities).
- **Clear regional pattterns shows up at the state level.** Many states in the **South and Appalachia** exceed **15%** disability prevalence, while several states in the **West and Northeast** are closer to **10–12%**.
- **Highest prevalence states stand out sharply.** In the "highest" group, **West Virginia (19.1%)**, **Arkansas (17.8%)**, **Kentucky (17.7%)**, **Mississippi (17.4%)**, and **Oklahoma (16.8%)** appear at the upper end of the distribution.

- **At the city level, the spread is wider than among states.** Among large cities (population ≥ 100,000), several **Puerto Rico cities** appear at the high end (e.g., above **20%**), while low rate cities fall near the low end around **6–7%**.
- **Age is the strongest demographic predictor.** Disability prevalence rises from **4.7%** (under 18) to **10.7%** (ages 18–64) to **32.9%** (ages 65+).
- **Working-age adults account for the largest number of people with disabilities.** Even though prevalence is highest at ages 65+, the **18–64** group is the largest in absolute count (≈ **21.3 million**).
- **Sex differences are generally smaller than place-based differences.** Male and female rates tend to move together across geographies.
- **Race/ethnicity patterns vary nationally and locally, with a caution flag for small samples.** Some city level race/ethnicity estimates show extreme values, especially for smaller groups, so local rankings should be read carefully.

Apparent from the demographic results as a whole is that disability prevalence is patterned more strongly by places and their population composition than by any single demographic attribute on its own. Across multiple cuts of the data whether overall prevalence, sex, and race, many of the same places tend to reappear near the high and low ends, suggesting that local context (especially age structure) often shapes the baseline conditions in which disability is experienced.

Age stands out as the most powerful and universal demographic factor. Increase in prevalence among older adults is substantial across the map, but it is not identical everywhere. Some states and cities sit well above typical levels even after accounting for age thresholds, which is consistent with the idea that aging interacts with cumulative differences in health and socioeconomic conditions across regions. At the same time, the age 18-64 population remains central to the disability landscape in absolute terms, as the largest number of people with disabilities are working age.

City patterns reinforce the value of examining the range of local contexts. Even with the scope of large U.S. cities, this range is vast, with some cities reporting disability prevalence above 20%, while others are far lower at 6–7%. These results are an important a reminder that national averages can be poor guides to local realities, and that comparisons are most meaningful when paired with careful attention to the demographic and economic context of each place.

3 Community Living

First off the three ADA-PARC categories of analysis, Community Living is imperative for the broader scope of disability inclusion, as living settings can themselves constrain opportunities for participation. A dimension that is simple and concrete, whether people with disabilities are able to live in private households and non-institutional settings, rather than in institutional environments, shapes day-to-day autonomy, social life, access to services, and the expression of individuality.

This section uses 2023 ACS-5 estimates to describe community living along a continuum, not as single yes/no outcome. Two broad groupings, each with a dedicated ADA-PARC Scorecard, give it structure. Communal Living focuses on living at home and living in non-institutional group quarters, while Institutional Living focuses on nursing homes and correctional facilities. A consistent equity lens runs throughout, as outcomes for people with disabilities are shown alongside outcomes for people without disabilities to highlight disparity in residential settings.

Community Living sets the stage for the rest of the report. Living arrangements tend to move with other domains, as where people live can be very meaningful for employment and means of community participation. It should be expected that geographies with a higher percentage of people with disabilities living at home may perform more favorably in other areas of participation. Conversely, areas with a higher percentage of people with disabilities living in institutional settings may perform less favorably in other areas of participation.

Offering two layers of interpretation, results are presented both in terms of the two ADA-PARC Community Living scorecards, and the individual measures they capture. Scorecards will be detailed as they are presented, and summarize relative performance across states, while indicator level figures and tables show the underlying distributions. We begin first with a zoomed-out look at how all of the community living variables relate to each other.

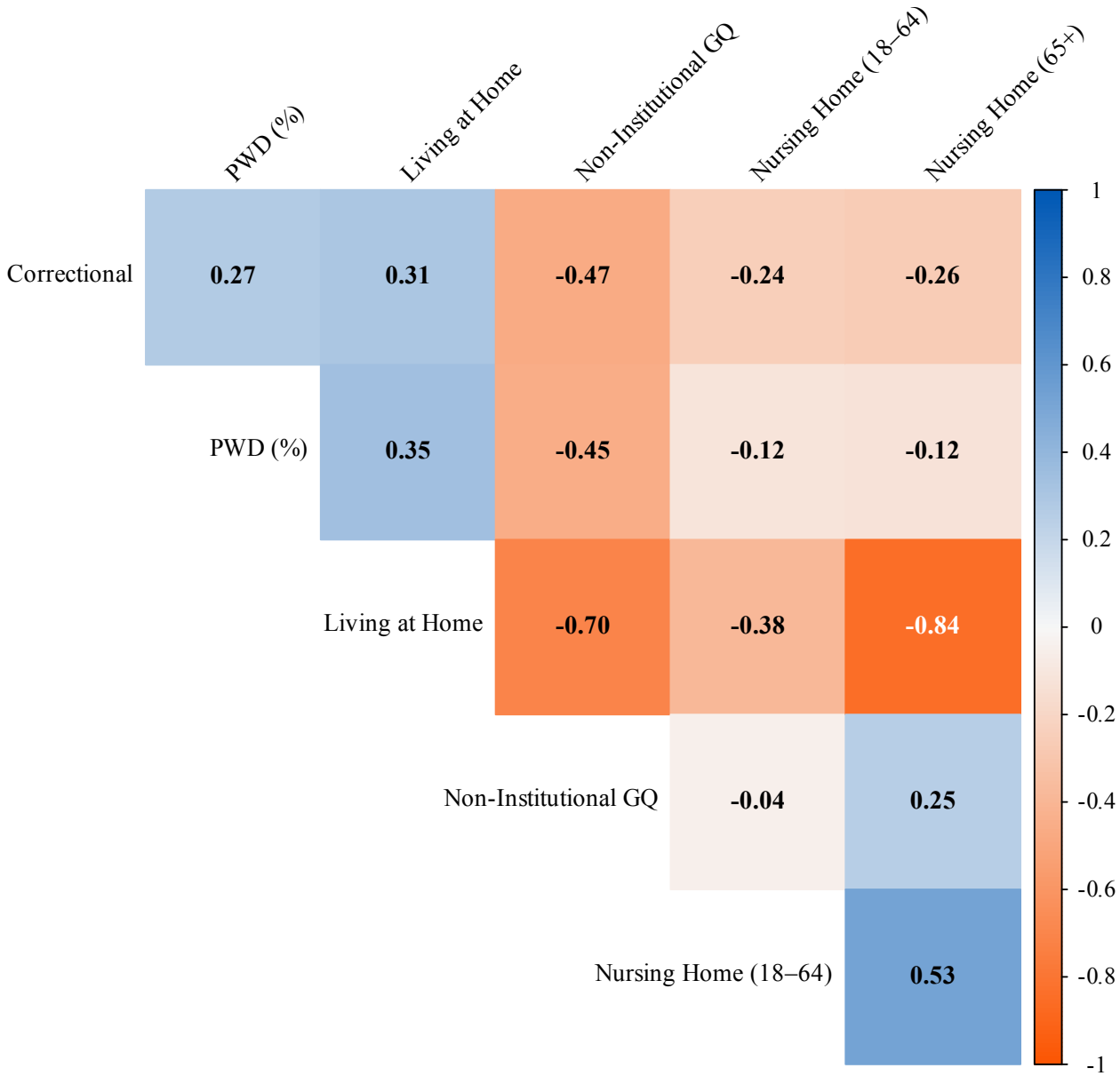


Figure 8: Correlations among community living variables (PWD, 2023).
Source: ADA-PARC 2023 Scorecards (constructed from U.S. Census Bureau, 2023 ACS-5).
Interpretation: Cells show pairwise correlations across states among the community living indicators for people with disabilities (PWD). Darker blue indicates a stronger positive correlation; darker orange indicates a stronger negative correlation.

A correlation matrix for community living variables lays above, which displays the strength of the positive or negative association between any two variables of the group across states. Results suggests that several community living indicators for people with disabilities move together in intuitive ways, while a few pairings show inverse relationships. The most prominent relationship is a strong negative correlation between living at home (PWD) and nursing home residence among older adults ($r = -0.84$), indicating that states with higher

shares of disabled older adults in nursing homes, as expected tend to have lower rates of living independently in the community. Unsurprisingly, a similar negative relationship exists between living at home and living in non-institutionalized group quarters (r = -.7).

Disability prevalence (PWD %) is positively related to people with disabilities living at home (r = 0.35) though also shows a small positive relationship with correctional facilities (PWD) (r = 0.27). In contrast, PWD (%) is moderately negatively correlated with non-institutional group quarters (r = −0.45). Its relationships with nursing home residence are comparatively small being−0.12 for ages 18 to 64 and −0.12 for ages 65+.

In sum, the matrix suggests that patterns of living for people with disabilities, and in particular the tradeoff between living in the community versus and institutional settings, drive the strongest associations. By contrast, disability prevalence is only modestly related to specific residential settings, suggesting that prevalence and living arrangement capture distinct dimensions of community living.

3.1 Communal Living

3.1.1 Scorecard ~ Communal Living Index

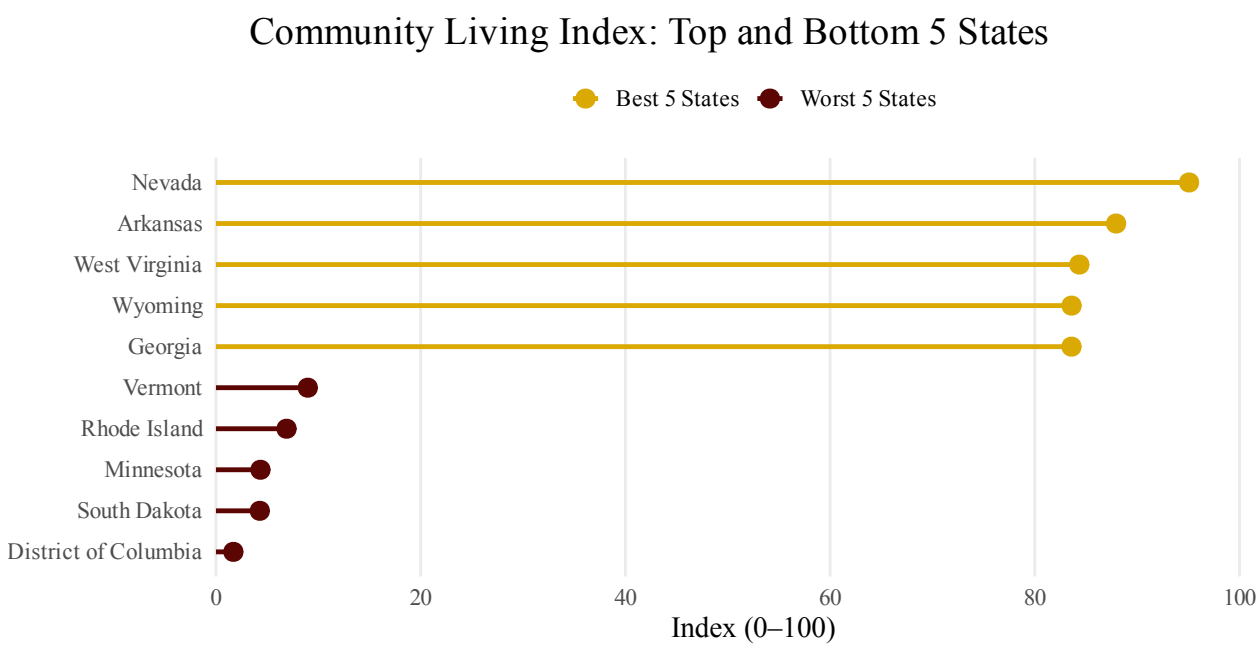


Figure 9: Community Living Index: Top and Bottom 5 States (2023).

Source: ADA-PARC scorecards (constructed from U.S. Census Bureau, 2023 ACS-5).

Interpretation: Dots show the five states with the highest and the five states with the lowest Community Living Index values (0–100). Higher values indicate relatively more favorable community living outcomes for people with disabilities.

First of our ADA-PARC scorecards, which are always on a 0 to 100 scale, is the Community Living Index, which highlights how states fare in terms of the percentage of people with disabilities who are living in in the community. Presented in the figure above are the highest and lowest performing states on this first index, with sates at the top including Nevada, Arkansas, West Virginia, Wyoming, and Georgia, each being significantly above average in this domain indicated by their proximity towards 100. While these states differ in demographic and geographic context, their scores suggest relatively lower reliance on institutional

settings and higher alignment with community-based living arrangements for people with disabilities in the measured time-frame.

In contrast, Vermont, Rhode Island, Minnesota, South Dakota, and the District of Columbia appear at the lower end of the distribution, with index values well below the national average. Lower scores reflect a different balance across the underlying indicators, including higher percentages of people with disabilities in institutional settings such as correctional facilities or institutionalized group quarters.

It's worth reminding that this index along with others that follow, is best interpreted as a comparative snapshot rather than a causal ranking, providing a framework for identifying broad patterns, we turn now to the full state rankings on Communal Living.

See next page for full Communal Living state rankings.

Communal Living Index — Full State Ranking

Rank	State	Communal Living Index	Quartile
1	Nevada	**95.08**	Q4
2	Arkansas	**87.95**	Q4
3	West Virginia	**84.36**	Q4
4	Wyoming	**83.59**	Q4
5	Georgia	**83.59**	Q4
6	Alabama	**79.82**	Q4
7	Louisiana	**79.30**	Q4
8	New Mexico	**79.29**	Q4
9	Texas	**79.20**	Q4
10	Kentucky	**79.12**	Q4
11	Tennessee	**78.91**	Q4
12	Mississippi	**77.87**	Q4
13	Delaware	**76.65**	Q3
14	Idaho	**76.32**	Q3
15	South Carolina	**76.26**	Q3
16	Oklahoma	**75.37**	Q3
17	Arizona	**73.35**	Q3
18	Utah	**72.53**	Q3
19	Florida	**72.30**	Q3
20	Virginia	**66.49**	Q3
21	North Carolina	**66.21**	Q3
22	Michigan	**65.61**	Q3
23	New Jersey	**64.05**	Q3
24	Indiana	**63.68**	Q3
25	Missouri	**63.05**	Q3
26	Ohio	**55.81**	Q2
27	Colorado	**55.36**	Q2
28	Nebraska	**53.49**	Q2
29	Illinois	**53.30**	Q2
30	Maryland	**49.82**	Q2
31	Washington	**46.87**	Q2
32	Montana	**46.68**	Q2
33	Alaska	**44.11**	Q2
34	Pennsylvania	**40.04**	Q2
35	Maine	**39.07**	Q2
36	Wisconsin	**35.70**	Q2
37	Hawaii	**33.15**	Q2
38	Kansas	**28.62**	Q2
39	Iowa	**27.79**	Q1
40	Oregon	**27.40**	Q1
41	New Hampshire	**24.35**	Q1
42	North Dakota	**23.35**	Q1
43	California	**22.09**	Q1
44	Massachusetts	**16.54**	Q1
45	Connecticut	**15.44**	Q1
46	New York	**13.77**	Q1
47	Vermont	**8.97**	Q1
48	Rhode Island	**6.90**	Q1
49	Minnesota	**4.35**	Q1
50	South Dakota	**4.29**	Q1
51	District of Columbia	**1.70**	Q1

Source: ADA-PARC analysis of 2023 American Community Survey (ACS) 5-year estimates.
Interpretation: higher index values indicate relatively more favorable communal living conditions for people with disabilities.
Quartiles computed with percentile, Q1≤28.20; Q2≤55.81; Q3≤76.48; Q4≤95.08.

Table 18: Communal Living Index — Full State Rankings (2023)

The full distribution of the Communal Living Index shows wide variation across states, with scores ranging from 1.7 in the District of Columbia to 95.1 in Nevada. States in the top quartile (Q4) cluster well above the national average, reflecting comparatively stronger performance on the combined community living indicators. This group includes several Southern, Mountain West, and Plains states, many of which exhibit lower

relative reliance on institutional settings and higher alignment with non-institutional living arrangements for people with disabilities.

States in the middle of the distribution (Q2 and Q3) span a broad range of index values, suggesting substantial heterogeneity in community living patterns even among states with similar overall scores. Within this middle range, differences may reflect variation in age composition, availability of long-term services and supports, housing stock, or state-specific approaches to residential care, rather than a single dominant factor.

The lowest-scoring states, concentrated in the bottom quartile (Q1), include several Northeastern states as well as the District of Columbia. These lower scores indicate a higher relative presence of institutional or group-quarters living arrangements among people with disabilities compared with national patterns. Importantly, these results should be interpreted in context: dense urban environments, legacy institutional infrastructure, and regional demographic characteristics can meaningfully shape community living profiles and influence composite index outcomes.

Overall, the spread of scores across quartiles highlights that community living arrangements for people with disabilities differ markedly across the United States. The Communal Living Index provides a comparative lens for understanding these differences at a high level, serving as a foundation for examining the underlying indicators that contribute to state-level variation rather than as a standalone assessment of policy effectiveness or quality.

3.1.2 Geographic Outcomes ~ Communal Living

3.1.2.1 National Level Communal Living

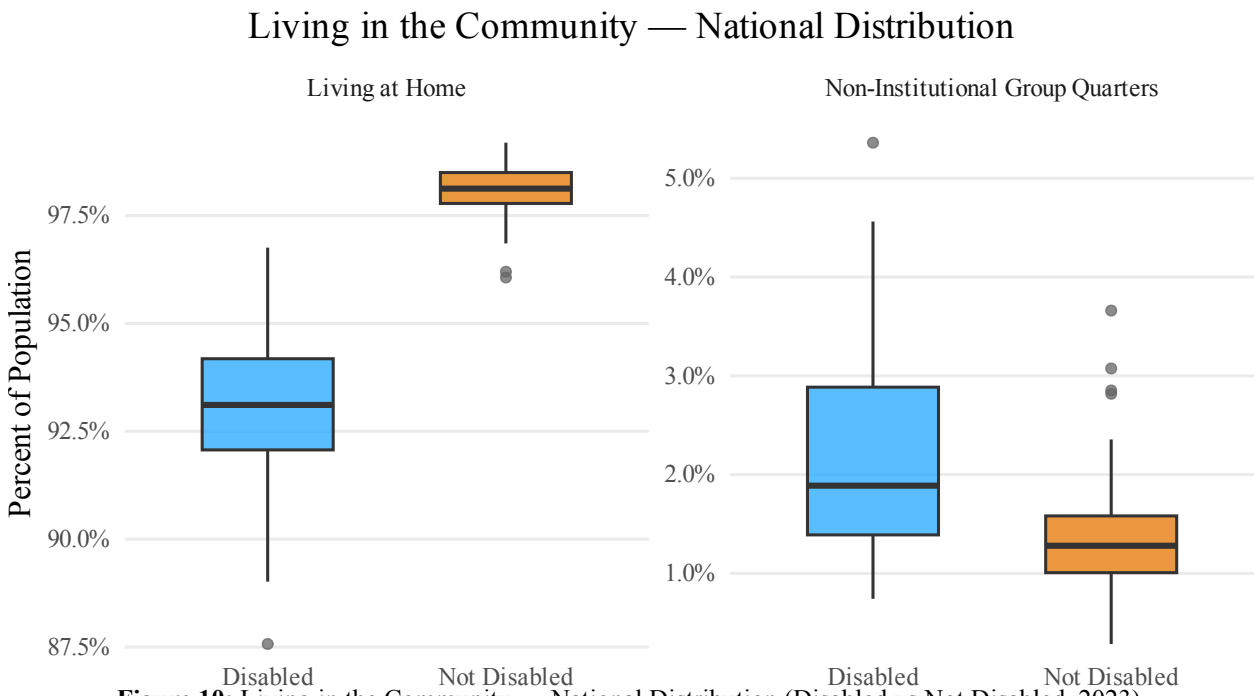


Figure 10: Living in the Community — National Distribution (Disabled vs Not Disabled, 2023).

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each box summarizes the distribution across states for the indicated living setting. The center line is the median;

boxes span the interquartile range (25th–75th percentiles); whiskers show the typical range beyond the IQR; points beyond whiskers are outliers.

Nationally, the vast majority of individuals living in the community are at home, regardless of disability status. Median state-level shares of people living at home are high for both groups, above 90%, though about five percent lower among people with disabilities. The distribution for individuals with disabilities also shows greater variation across states, both in the living at home category and the non-institutionalized group quarters category. Such is an indication that community residence patterns differ more widely depending on location for people with disabilities.

Differences are more pronounced when examining non-institutional group quarters. Although the overall percentages remain low for both groups, people with disabilities have a higher median share and a noticeably wider interquartile range across states. This suggests that, outside of institutional settings, individuals with disabilities are more likely to reside in alternative community-based arrangements and that reliance on these settings varies considerably by state.

3.1.2.2 Regional Level Living In The Community

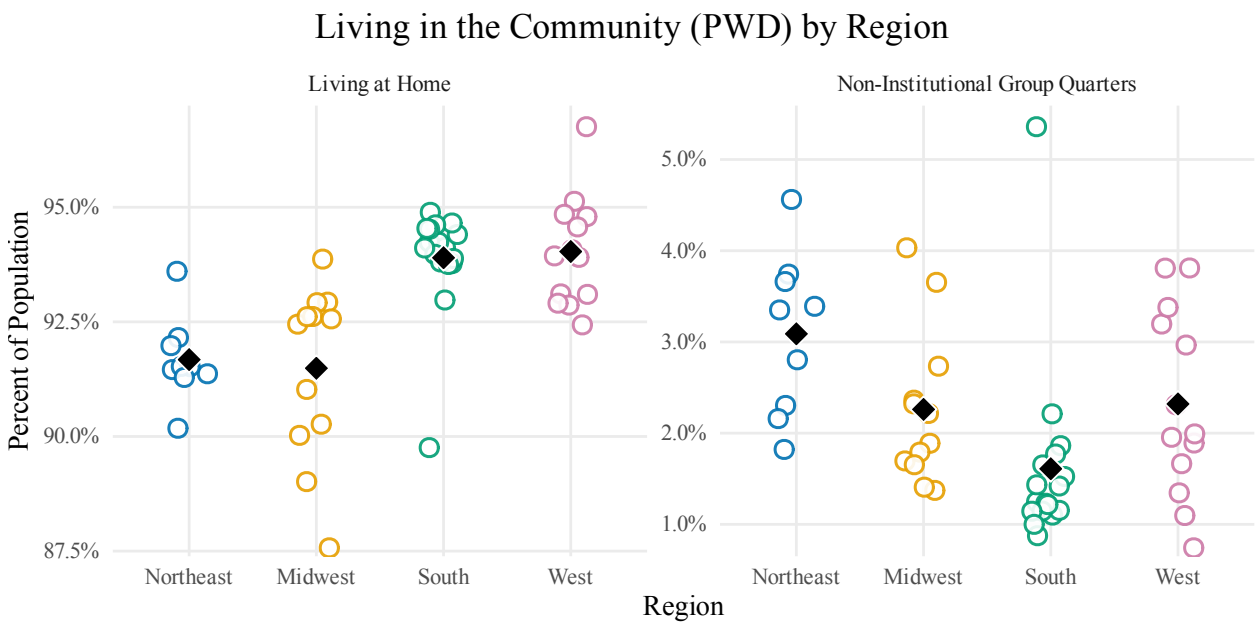


Figure 11: Living in the Community (PWD) by Region — State Distributions and Regional Means (2023).

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state within a region; the black diamond marks the regional mean across states.

Across all four regions, the overwhelming majority of people with disabilities live at home rather than in non-institutional group quarters. Regional means for living at home cluster tightly in the low- to mid-90 percent range, while non-institutional group quarters generally account for roughly 1–4 percent of the population. Some modest regional differences are visible. Southern and Western states show slightly higher average shares living at home, with many states concentrated near the top of the distribution. Northeastern and Midwestern states exhibit marginally lower means and somewhat greater dispersion, though differences across regions are only a few percentage points.

In the non-institutional group quarters panel, the Northeast and Midwest appear somewhat higher on average, with a few clear state-level outliers, while the South is more tightly clustered at the lower end though having

one significant outlier in the District of Columbia above 5%. Still, variation across regions on these masures is noticeable but not overwhelming. Regional context shapes small differences in distribution, yet the dominant pattern insofar as high rates of home-based living and comparatively low use of non-institutional group quarters remains preeminent.

See next page for full state results on measures of Communal Living.

3.1.2.3 State Level Communal Living

Community Living PWD: All States

State	Living at Home (Disabled)	Non-Institutional Group Quarters (Disabled)
Alabama	94.1%	1.1%
Alaska	93.9%	3.2%
Arizona	94.8%	1.9%
Arkansas	94.9%	0.9%
California	92.4%	3.8%
Colorado	93.1%	2.0%
Connecticut	91.4%	3.4%
Delaware	94.4%	1.5%
District of Columbia	89.8%	5.4%
Florida	93.8%	1.4%
Georgia	94.7%	1.2%
Hawaii	93.1%	3.4%
Idaho	94.1%	1.3%
Illinois	92.4%	1.7%
Indiana	92.9%	1.4%
Iowa	90.3%	2.2%
Kansas	91.0%	2.4%
Kentucky	94.2%	1.2%
Louisiana	94.2%	1.2%
Maine	92.2%	2.3%
Maryland	93.0%	2.2%
Massachusetts	91.5%	3.4%
Michigan	93.9%	1.9%
Minnesota	90.0%	4.0%
Mississippi	94.0%	1.1%
Missouri	92.9%	1.4%
Montana	92.9%	2.3%
Nebraska	92.6%	1.8%
Nevada	96.8%	0.7%
New Hampshire	91.5%	2.8%
New Jersey	93.6%	1.8%
New Mexico	95.1%	1.7%
New York	91.5%	3.7%
North Carolina	93.9%	1.9%
North Dakota	89.0%	2.3%
Ohio	92.6%	1.7%
Oklahoma	93.8%	1.2%
Oregon	92.9%	3.8%
Pennsylvania	92.0%	2.2%
Rhode Island	90.2%	3.7%
South Carolina	94.6%	1.7%
South Dakota	87.6%	3.7%
Tennessee	94.5%	1.4%
Texas	94.1%	1.1%
Utah	94.8%	2.0%
Vermont	91.3%	4.6%
Virginia	93.8%	1.8%
Washington	93.9%	3.0%
West Virginia	94.5%	1.0%
Wisconsin	92.6%	2.7%
Wyoming	94.6%	1.1%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values are the share of the disabled population in each living setting within each state.

Table 19: Community Living for PWD — All States (2023)

Across all states, the vast majority of people with disabilities reside in private households rather than group settings. In most states, between roughly 90% and 97% of people with disabilities live at home, confirming that household-based living is the predominant arrangement nationwide. Residence in non-institutional

group quarters is comparatively rare, typically ranging from about 1% to 3% across states. Even where shares are highest—such as in the District of Columbia (5.4%), Vermont (4.6%), Minnesota (4.0%), and several Northeastern states—non-institutional group quarters account for a relatively small bloc of the disabled population.

Variation is present but modest in magnitude. Nevada (96.8%), New Mexico (95.1%), and Wyoming (94.6%) report some of the highest shares living at home, while the District of Columbia and a handful of New England and Upper Midwest states show relatively greater use of non-institutional group quarters. However the distribution is tightly woven, indicating broadly similar community living patterns across states, with private homes serving as the primary setting for people regardless of disability status.

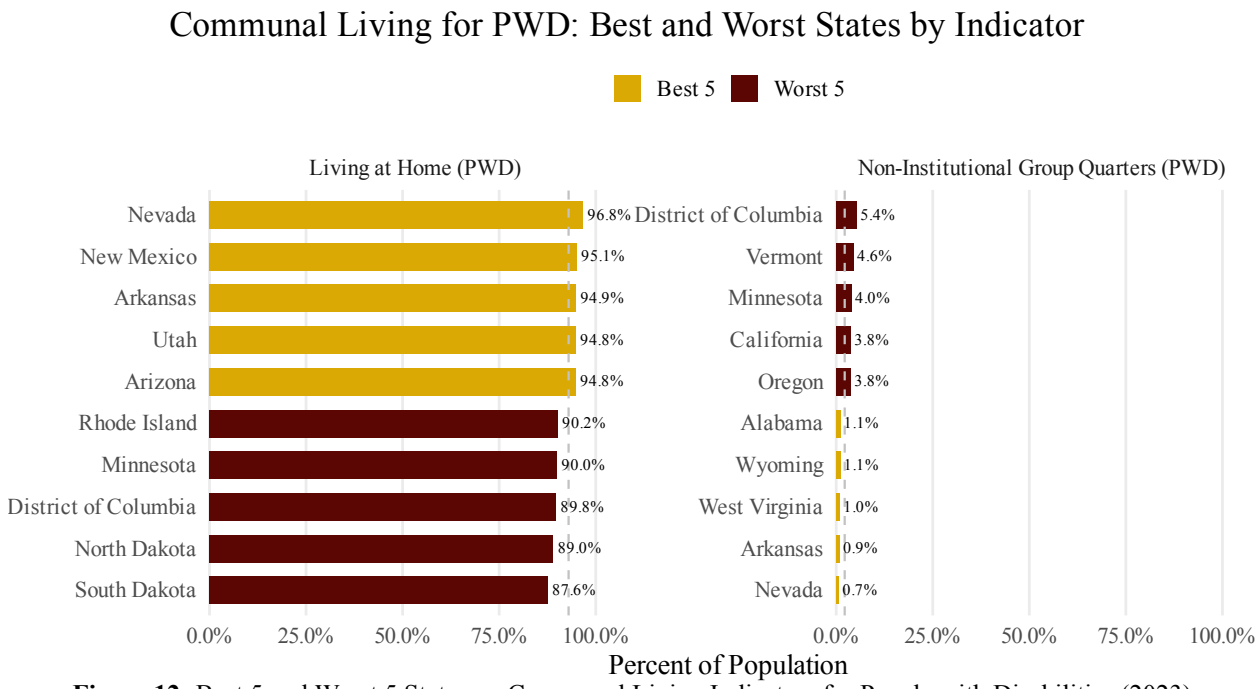


Figure 12: Best 5 and Worst 5 States on Communal Living Indicators for People with Disabilities (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: For *Living at Home*, higher values indicate better outcomes. For *Non-Institutional Group Quarters*, lower values indicate better outcomes. Gold bars mark the best states and maroon bars mark the worst states within each indicator. Dashed lines show national averages.

Focusing on top and bottom performing states on both metrics, we get a better idea of the range of outcomes within community living. Nevada, New Mexico, Arkansas, Utah, and Arizona reign in terms of percentages of people with disabilities living at home, roughly 95–97% . At the lower end, Rhode Island, Minnesota, the District of Columbia, North Dakota, and South Dakota creep toward and below the 90% range at 88–90%. While this gap is noticeable, even the lowest-ranked states maintain fairly high levels of home-based living.

Differences are somewhat sharper for non-institutional group quarters, while overall rates remain below 6%, the high rate states exceed the low rate states by multiple orders of magnitude. Alabama and Wyoming (both ~1.1%), West Virginia (~1.0%), Arkansas (~0.9%), and Nevada (~0.7%) form the low end of the distribution. States with the highest shares include the District of Columbia (~5.4%), Vermont (~4.6%), Minnesota (~4.0%), California and Oregon (both ~3.8%). Even in these highest states, non-institutional group quarters remain a far more uncommon living arrangment within the community/non-institutional bucket.

States With Greatest & Least Home Living Disparity

State	PWD Living at Home %	PWOD Living at Home %	Disparity
South Dakota	87.6%	97.8%	**-10.2%**
Minnesota	90.0%	98.6%	**-8.6%**
North Dakota	89.0%	97.5%	**-8.5%**
Iowa	90.3%	97.9%	**-7.7%**
Kansas	91.0%	98.0%	**-7.0%**
South Carolina	94.6%	97.8%	**-3.2%**
West Virginia	94.5%	97.7%	**-3.2%**
Arkansas	94.9%	97.8%	**-2.9%**
Nevada	96.8%	99.2%	**-2.4%**
Alaska	93.9%	96.2%	**-2.3%**

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values are percentage-point differences (PWD minus PWOD). Shading highlights the greatest and least absolute disparities.

Table 20: Living at Home Disparity (PWD − PWOD) — States with Greatest and Least Absolute Gaps (2023)

Applying more of an equity lens to the state level, we can see not just how PWD in one state compare to PWD in another state, but also how they fare relative to PWOD in their own state. Such is the purpose of our table above, which shows. that even in states with less disparity, PWOD have higher home-living rates. The disparities are systematic rather than isolated, ranging from roughly −2 to −10 percentage points (PWD minus PWOD).

In terms of the largest gaps states are as follows: South Dakota (−10.2), Minnesota (−8.6), North Dakota (−8.5), Iowa (−7.7), and Kansas (−7.0). Notably, both Dakota states and Minnesota were also among the lowest just looking at PWD rates, meaning these states perform poorly on this measure both in absolute and equity terms.

At the greater equality end, Alaska (−2.3), Nevada (−2.4), Arkansas (−2.9), West Virginia (−3.2), and South Carolina (−3.2) show closer alignment between PWD and PWOD. Nevada and Arkansas also appeared in the highest absolute rates for PWD meaning they perform well both in PWD-specific and equity terms. In total, there is nearly an eight-point difference between the largest and smallest within-state gaps, demonstrating meaningful variance in levels of living at home equality across disability status.

3.1.2.4 City Level Living In Community

ACS-5 year estimates unavailable at local level.

3.2 Institutional Living

3.2.1 Scorecard ~ Institutional Living Index

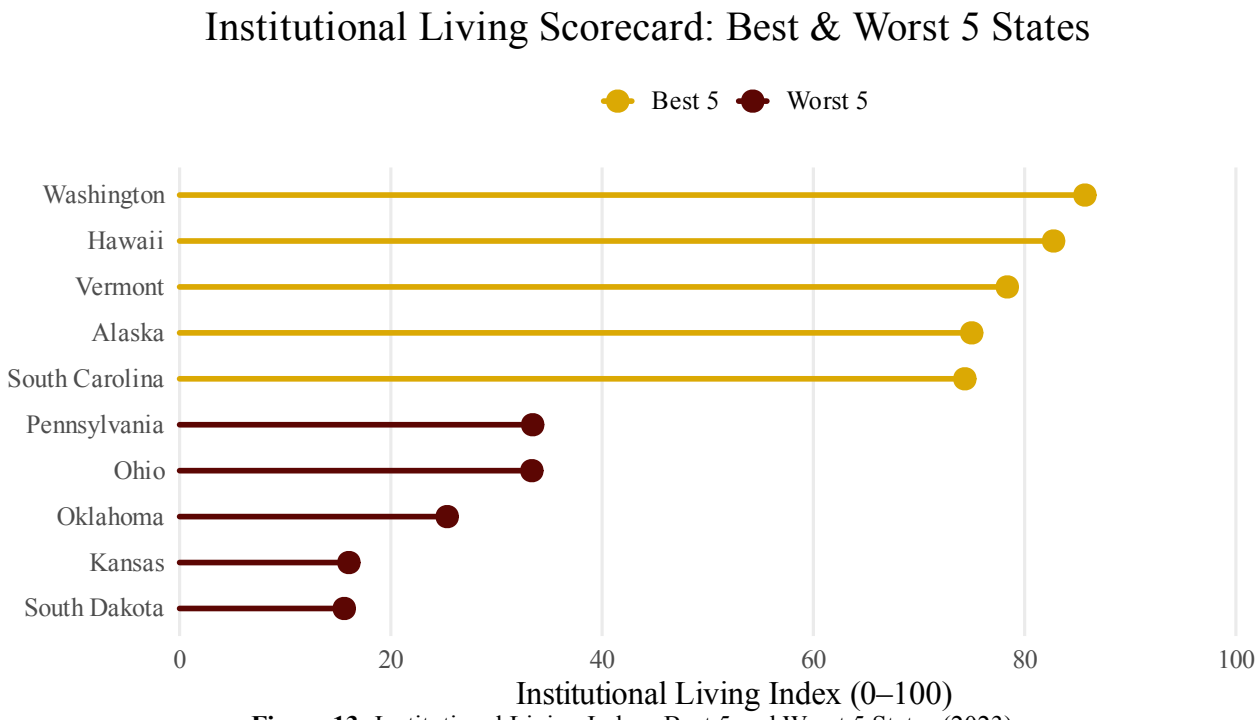


Figure 13: Institutional Living Index: Best 5 and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards (constructed from U.S. Census Bureau, 2023 ACS-5).

Interpretation: Higher index values indicate relatively more favorable institutional living outcomes for people with disabilities. Gold marks the best-performing states and maroon marks the worst-performing states on the 0–100 index.

Institutional Living Index, similar to the Community Living Index, rates states based on living arrangements for people with disabilities, but this time in the institutionalized context. To stay consisent across scorecards, the sacle remains 0-100 with higher scores indicating better performing states, meaning our measures for this index of living in correctional facilities or nursing homes, were inverted so that we could retain the higher = better logic. As such, higher scores on the index indicates lower shares of people with disabilities living in these settings.

Highest-scoring states include Washington, Hawaii, Vermont, Alaska, and South Carolina ranging from 74 to 85. These states stand out for having smaller proportions of their disabled population living in nursing homes or correctional facilities relative to other states. In contrast, South Dakota, Kansas, Oklahoma, Ohio, and Pennsylvania rank at the bottom of the index ranging from a score of 15 to 34. These states fall notably below the national average, indicating greater reliance on institutional settings for housing or confinement of disabled populations. Continue on for the full state rankings across this ADA-PARC scorecard index.

See next page for full state rankings on the Institutional Living Scorecard.

Institutional Living Scorecard — Full Ranking (All States)

Rank	State	Institutional Living Index	Quartile
1	Washington	**85.71**	Q4
2	Hawaii	**82.74**	Q4
3	Vermont	**78.36**	Q4
4	Alaska	**74.99**	Q4
5	South Carolina	**74.34**	Q4
6	Delaware	**68.33**	Q4
7	Oregon	**68.05**	Q4
8	New York	**67.30**	Q4
9	Wyoming	**66.96**	Q4
10	Massachusetts	**65.69**	Q4
11	New Jersey	**65.20**	Q4
12	Wisconsin	**65.10**	Q4
13	Nevada	**62.01**	Q3
14	Arkansas	**60.98**	Q3
15	Minnesota	**60.84**	Q3
16	New Hampshire	**59.96**	Q3
17	Maine	**59.79**	Q3
18	Utah	**59.58**	Q3
19	Connecticut	**59.24**	Q3
20	California	**59.04**	Q3
21	Arizona	**58.57**	Q3
22	North Carolina	**55.61**	Q3
23	Tennessee	**53.22**	Q3
24	Michigan	**51.51**	Q3
25	Montana	**51.36**	Q3
26	Idaho	**49.95**	Q2
27	New Mexico	**48.73**	Q2
28	Maryland	**47.35**	Q2
29	Nebraska	**47.31**	Q2
30	Virginia	**47.30**	Q2
31	District of Columbia	**46.94**	Q2
32	Illinois	**46.38**	Q2
33	Iowa	**45.56**	Q2
34	Rhode Island	**45.27**	Q2
35	West Virginia	**44.32**	Q2
36	Georgia	**41.38**	Q2
37	Florida	**40.78**	Q2
38	Indiana	**40.15**	Q2
39	Kentucky	**40.13**	Q1
40	Mississippi	**40.07**	Q1
41	Missouri	**39.98**	Q1
42	Texas	**36.39**	Q1
43	Colorado	**35.96**	Q1
44	Alabama	**35.34**	Q1
45	North Dakota	**34.71**	Q1
46	Louisiana	**34.36**	Q1
47	Pennsylvania	**33.43**	Q1
48	Ohio	**33.35**	Q1
49	Oklahoma	**25.34**	Q1
50	Kansas	**16.03**	Q1
51	South Dakota	**15.59**	Q1

Source: ADA-PARC 2023 Scorecards.
Quartiles computed with percentiles Q1≤40.14; Q2≤49.95; Q3≤61.50; Q4≤85.71.
Interpretation: Higher scores indicate stronger outcomes for people with disabilities on the Institutional Living Index. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

Table 21: Institutional Living Index — Full Ranking (All States, 2023)

Beyond the top and bottom five, the full ranking shows a fairly tiered distribution. States in the upper quartile (Q4) range between roughly 65 and 86, with much of the Northeast and West represented. Middle quartiles, perhaps as expected, have a tighter range of 45 to 62, suggesting that most states fall within a relatively small

window.

With a bit larger range is the lower quartile. Scores drop from just below 40 down to the mid-teens, indicating that the gap between mid-performing and lowest-performing states is considerable. In practical terms, this means that institutional residence among people with disabilities is not even across the country, but that a subset of states accounts for meaningfully higher rates relative to the national average.

3.2.2 Geographic Outcomes ~ Institutional Living

3.2.2.1 National Level Institutional Living

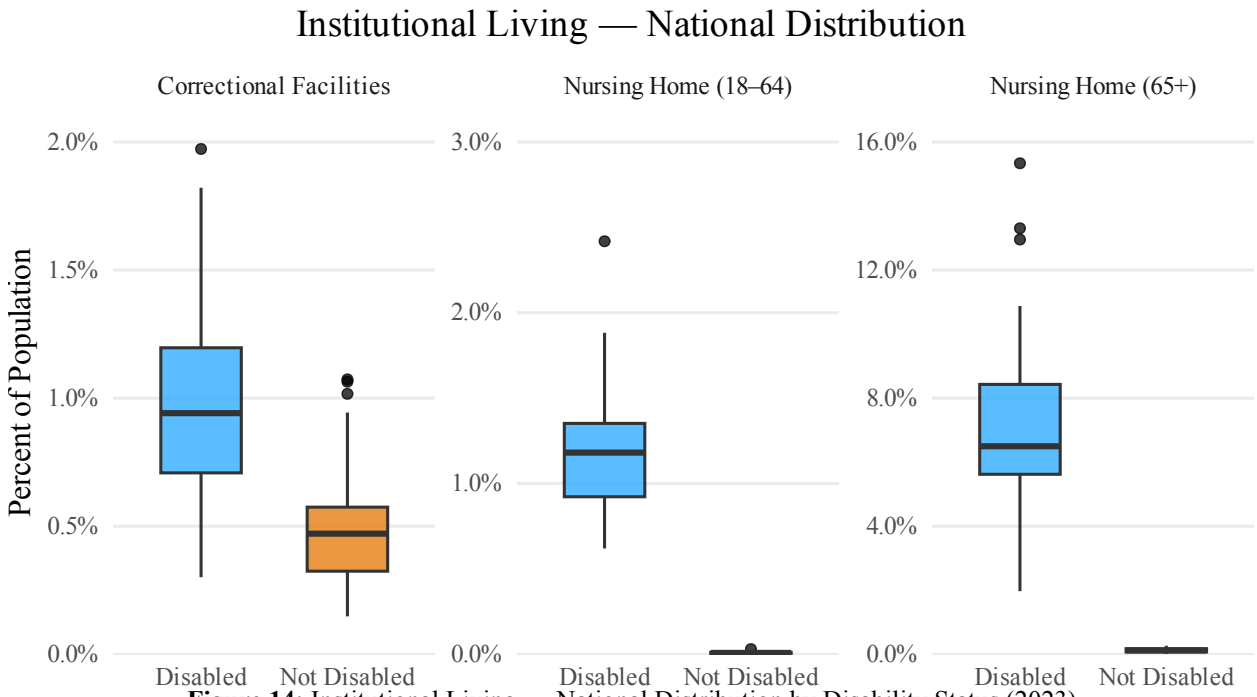


Figure 14: Institutional Living — National Distribution by Disability Status (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each box summarizes the distribution across states for the indicated setting. The center line is the median; boxes span the interquartile range (25th–75th percentiles); whiskers show the typical range beyond the IQR; points beyond whiskers are outliers.

Across all three institutional settings, people with disabilities (PWD) have higher rates of residence than people without disabilities (PWOD), though the size and shape of the gap differ by setting. Clearest separation appears in nursing homes for adults ages 65 and older. In every state, the share of PWD residing in 65+ nursing homes substantially exceeds that of PWOD, with medians for PWD clustered around the mid–single digits and several upper states approaching 2x the median. By contrast, PWOD rates in this setting are near zero across states, a pattern which reflects how strongly disability status aligns with institutional residence at older ages.

Among adults ages 18–64, the same directional relationship holds. However, the rates for PWD living in nursing homes is much lower for this age group which is to be expected. PWD rates are typically around one percent across states, while PWOD rates are close to zero. Correctional facilities show a smaller and more variable gap across disability status. PWD rates exceed PWOD rates in almost all states, with the medians

separated by half a percentage point. Interquartile ranges overlap more than in the nursing home panels with state variance also being more visible. Of course, this suggests that state-level factors may be more meaningful for determining institutionalization within correctional facilities compared to nursing homes.

3.2.2.2 Regional Level Institutional Living

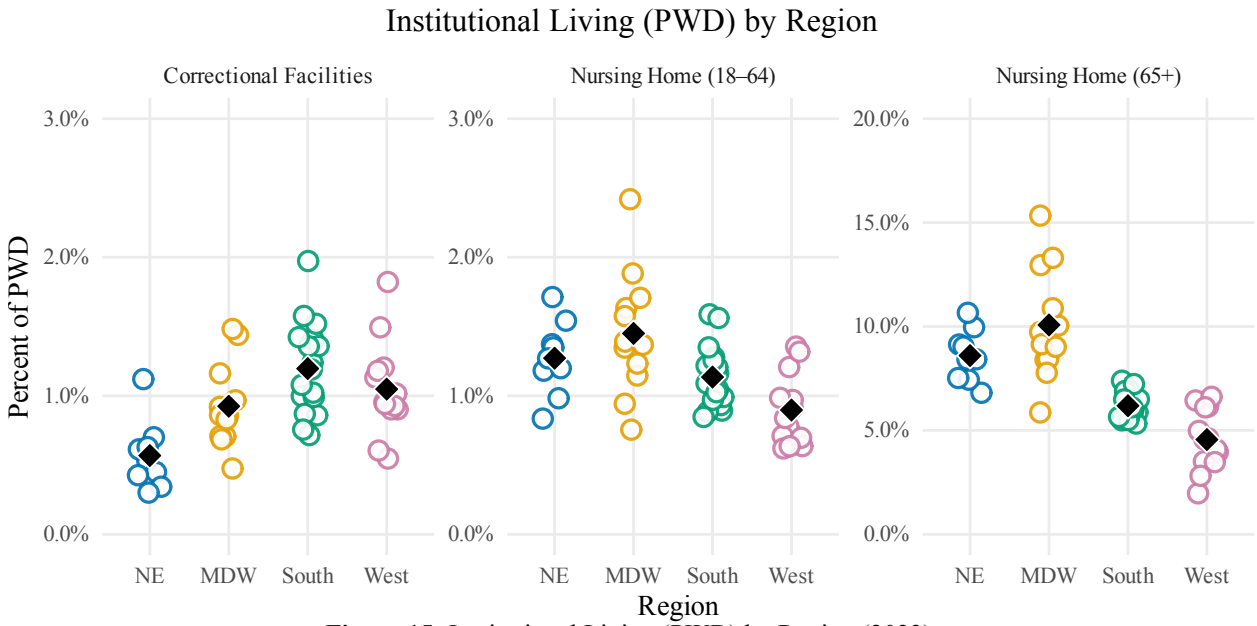


Figure 15: Institutional Living (PWD) by Region (2023).

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each dot represents one state within a region. Black diamonds indicate regional means across states. Wider vertical spread indicates greater cross-state variation within a region.

Across regions, institutional living among people with disabilities remains relatively uncommon, though clear differences emerge by setting and age group. Correctional facility residence among PWD is reasonably low in all regions, with medians ranging from .6% to 1.1%, being highest in the South and Lowest in the Northeast. Among working-age adults with disabilities, nursing home residence is also rare nationwide, typically near or below 2 percent. Midwestern states have the highest median and dispersion in this category with the West being lower on both.

In contrast, nursing home residence among older adults with disabilities (65+) shows much greater regional separation, though the regional medians are ordered the same Again, the Midwest exhibits the highest regional average and the widest spread across states, with several states reaching up to 15 percent. The Northeast follows with moderately elevated levels, while the South and West show lower regional means and tighter spreads. Patterns such as these suggest that there is meaningful regional differences in institutional living for PWD, which many relate to how states supportor rely on institutional care for adults with disabilities.

See next page for full table of Institutional Living measures by state.

3.2.2.3 State Level Institutional Living

Institutional Living (PWD) — All States

State	Correctional Facilities (PWD)	Nursing Home (18–64, PWD)	Nursing Home (65+, PWD)
Alabama	1.4%	1.2%	6.2%
Alaska	0.9%	0.6%	4.0%
Arizona	1.2%	0.8%	3.5%
Arkansas	0.7%	1.3%	6.9%
California	1.0%	1.2%	4.6%
Colorado	1.8%	0.9%	6.1%
Connecticut	0.5%	1.4%	8.4%
Delaware	0.9%	0.9%	5.9%
District of Columbia	1.0%	1.2%	7.4%
Florida	1.2%	1.2%	5.9%
Georgia	1.5%	0.9%	5.3%
Hawaii	0.5%	1.0%	4.6%
Idaho	1.1%	1.0%	6.4%
Illinois	0.7%	2.4%	8.4%
Indiana	0.9%	1.3%	9.7%
Iowa	0.7%	1.4%	13.0%
Kansas	1.4%	1.4%	10.0%
Kentucky	1.4%	1.0%	6.5%
Louisiana	1.4%	1.3%	6.1%
Maine	0.6%	1.2%	9.1%
Maryland	0.9%	1.6%	6.9%
Massachusetts	0.4%	1.2%	8.4%
Michigan	1.2%	0.9%	5.9%
Minnesota	0.5%	1.1%	10.9%
Mississippi	1.0%	1.6%	7.2%
Missouri	0.9%	1.9%	8.4%
Montana	0.9%	1.4%	6.6%
Nebraska	0.7%	1.6%	9.1%
Nevada	1.2%	0.7%	2.0%
New Hampshire	0.7%	1.0%	10.0%
New Jersey	0.3%	1.5%	6.8%
New Mexico	1.5%	0.8%	2.8%
New York	0.3%	1.3%	7.4%
North Carolina	1.0%	1.0%	6.0%
North Dakota	0.9%	1.6%	15.3%
Ohio	1.0%	1.7%	9.0%
Oklahoma	1.6%	1.3%	6.5%
Oregon	1.0%	0.7%	4.1%
Pennsylvania	1.1%	1.3%	9.0%
Rhode Island	0.6%	1.7%	10.7%
South Carolina	0.8%	0.9%	5.6%
South Dakota	1.5%	1.2%	13.3%
Tennessee	1.1%	1.1%	5.5%
Texas	2.0%	1.0%	5.5%
Utah	0.9%	1.3%	3.5%
Vermont	0.4%	0.8%	7.5%
Virginia	1.2%	1.0%	6.1%
Washington	0.6%	0.6%	5.0%
West Virginia	1.4%	0.8%	5.7%
Wisconsin	0.8%	0.8%	7.8%
Wyoming	0.9%	0.6%	6.1%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Values show the share of the PWD group-quarters universe in each institutional setting within each state; higher values indicate a larger concentration of PWD in that setting.

Table 22: Institutional Living (PWD) — All States (2023)

Across states, institutional living among people with disabilities is shaped primarily by nursing home residence among adults ages 65 and older. In most states, between roughly 5 and 10 percent of older adults with disabilities reside in nursing homes and several states such as North Dakota (15.3%), South Dakota

(13.3%), and Iowa (13.0%) exceeding ten percent. At the low end, states include Nevada (2.0%), New Mexico (2.8%), Arizona and Utah (3.5%), whith substantially smaller shares. Such a spread in this older-age category is significantly wider than both other institutional settings, making it the dominant driver of variation across states.

By comparison, correctional facility residence among people with disabilities is much lower and sees tighter variation in terms of absolute values but still some disparity particulary in terms of orders of magnitude. With Texas on the upper end at (2.0%), its rate of people with disabilities living in correctional facilities is multiple times higher than New York & New Jersey (0.3%) at the lower end. Nursing home residence among working-age adults (18–64) is similarly low in scale of the absolute values, ranging from about 0.6 to 1.6 percent, though Illinois (2.4%) stands out as a high-value outlier.

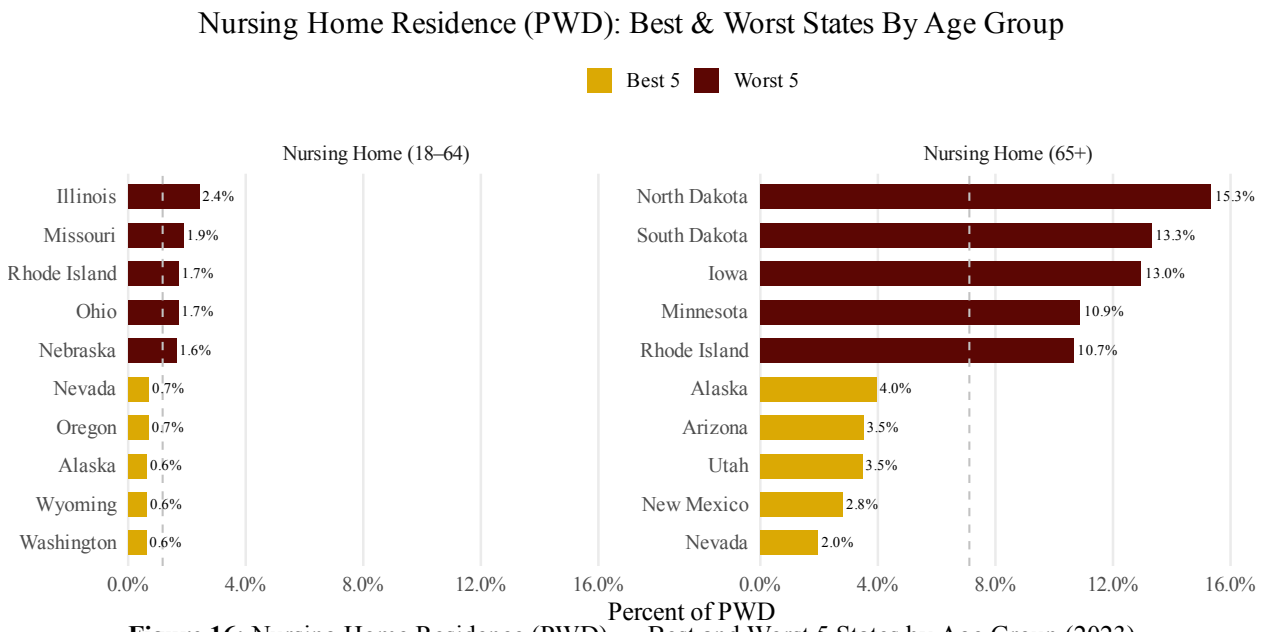


Figure 16: Nursing Home Residence (PWD) — Best and Worst 5 States by Age Group (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Gold bars indicate the five states with the lowest nursing home residence rates among people with disabilities (better outcomes), and maroon bars indicate the five states with the highest rates (worse outcomes). Dashed lines show national averages within each age group.

Looking closely at the top and bottom states for nursing home PWD populations, the figure above highlights the range of outcomes and shows how institutionalization risk rises sharply with age. Among adults age 18 to 64, states of Illinois, Missouri, Rhode Island, Ohio, and Nebraska have the highest prevalence ranging from 1.6% to 2.4%. In contrast, states of Washington, Wyoming, Alaska, Oregon, and Nevada fall farthest below the national mean ranging from .7% to .6%.

Interstate variation becomes far more pronounced and practically meaningful among older adults with disabilities. Here, nursing home residence is substantially more common overall, led by the states of North Dakota, South Dakota, Iowa, Minnesota, and Rhode Island, from 15.3% to 10.7%. Conversely, states of Nevada, New Mexico, Utah, Arizona, and Alaska show considerably lower rates, from 4% to 2%.

Such patterns suggest that age group differences correspond with significantly different outcomes in terms of the commonality with which PWD reside in nursing facilities across states. Interestingly, only Nebraska appears as a high rate state for both age ranges, while Alaska and Nevada appear as low rate states for both

age groups. Thus, there may be a complex relationship between the rates of nursing home living for PWD and state policy choices influencing institutional versus community-based living arrangements. of ong-term care facility may thus for in state long-term care systems.

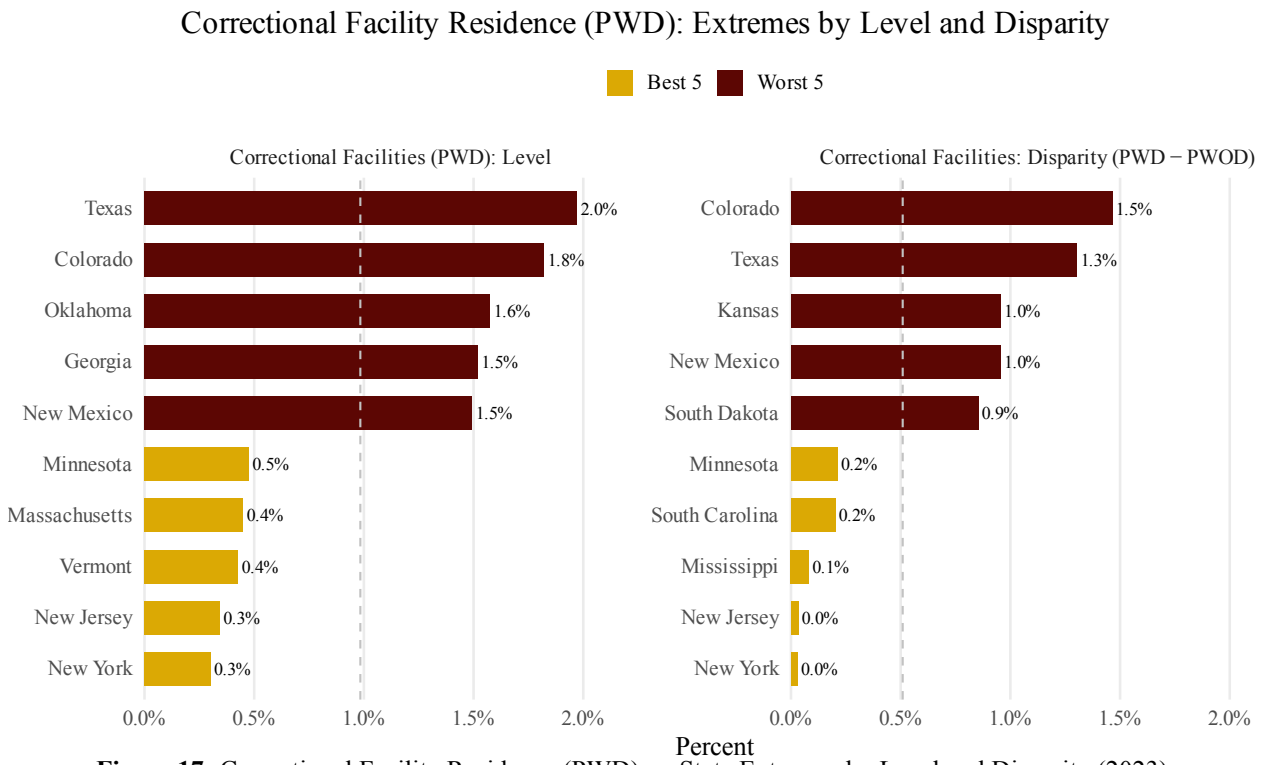


Figure 17: Correctional Facility Residence (PWD) — State Extremes by Level and Disparity (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Gold bars indicate the five states with the lowest correctional facility residence rates among people with disabilities (better outcomes), and maroon bars indicate the five states with the highest rates (worse outcomes). The dashed line shows the national average across states.

Looking at institutionalization in correctional facilities rather than nursing homes, figure above illustrates best and worst outcome states for people with disabilities. At its highest, correctional residence among PWD is two percent in Texas, a rate more than 6x the lowest state of New York. Other high-rate states include Colorado (1.8%), Oklahoma (1.6%), Georgia and New Mexico (1.5%). Although the percentages appear modest, these differences suggest meaningful variation in how states' criminal legal systems intersect with disability. On the low end of the distribution, states of New York (.3%), New Jersey (.3%), Vermont (.4%), Massachusetts (.4%), and Minnesota (.5%) exhibit lower rates of correctional residence among PWD. These lower rates may reflect differences in sentencing practices, re-entry programs, or a myriad of other factors.

Beyond absolute rates, the disparity panel adds an additional layer of insight. Worst two states in this domain are the same as the absolute levels but in opposite order, with Colorado (+1.5 percent gap) and Texas (+1.3). Kansas, New Mexico, and South Dakota round out the other three worst performers in terms of disparity. By contrast, states including New York (0.0), New Jersey (0.0), Mississippi (0.1), and South Carolina and Minnesota (0.2) display zero to small over representation of PWD. Put together, the two panels show that absolute rates and disparities often, but not always overlap. To what correctional facility residence among people with disabilities is a function state-level policy environments is beyond the scope of our report but of vital importance to ensuring the disability community is treated justly.

3.2.2.4 City Level Institutional Living

ACS-5 year estimates unavailable at local level.

3.3 Summary of Community Living Findings

Key Findings

- **Living at home is the norm for people with disabilities nationally.** In every state, living at home accounts for the overwhelming majority of residences for people with disabilities typically about 90–97%.
- **Even with high at home living rates for people with disabilities, the equity gap across disability status is universal.** People with disabilities are less likely than people without disabilities to live at home in every state, with negative gaps observed nationwide.
- **Non-institutional group quarters are small in absolute terms, but not trivial.** Their share is generally limited, yet state differences are enough to represent meaningful alternatives for living in the community.
- **Age worsens disability related institutionalization gaps.** Nursing home disparities across disability status are universal and widen sharply with age, often exceeding five percentage points for older adults and reaching double digits in several states.
- **Correctional facility residence rates are low, but are typically higher for people with disabilities.** Absolute rates are low, yet in most states people with disabilities are more likely than people without disabilities to be in correctional settings.
- **Certain states have consistently better or worse than average outcomes for people with disabilities across Community Living measures.** This pattern suggests outcomes are meaningfully shaped by state level systems and conditions rather than random variation.

Community living outcomes for people with disabilities show a clear through line across states, most people live at home, but the share living outside a private household in group quarters or institutional settings varies meaningfully Those differences are driven less by shifts in at-home living (which stays high almost everywhere) and more by how frequently people with disabilities are found in non-institutional group quarters, nursing homes, and correctional facilities. Each of these settings reflect different pathways and constraints for people with disabilities.

Comparisons to people without disabilities reveal outcomes which are consistently less favorable for people with disabilities. In every state, people with disabilities are less likely to live at home and more likely to live in group quarters or institutional settings. Disparity is therefore not about whether community living is still the most common living arrangement for people with disabilities, but to what extent their rates of institutionalization are elevated. It is in such settings where variation across states is especially consequential. Disability status gaps in nursing home residence widen sharply with age, often exceeding five percentage points and reaching double digits in several states, a pattern suggesting that state contexts affect how aging with a disability may coincide with institutional residence. Correctional facilities add a smaller but distinct dimension. Across disability status, (PWD ~ 1%, PWOD ~ .5% median), absolute levels are low, but a two times rate over representation nationally is concerning.

ADA-PARC Scorecards for community living sharpen the same conclusions, and organize states into performance tiers. On the Communal Living Index, Nevada, Arkansas, West Virginia, Wyoming, and Georgia have the most favorable outcomes for PWD, while Vermont, Rhode Island, Minnesota, South Dakota, and the District of Columbia perform poorly. On the Institutional Living Index, Washington, Hawaii, Vermont, Alaska, and South Carolina rank among the highest scoring states, reflecting lower shares of people with disabilities in nursing homes or correctional facilities, while South Dakota, Kansas, Oklahoma, Ohio, and Pennsylvania appear at the bottom.

As a whole, the community living results suggest that the favorability of living arrangement outcomes for PWD are sensitive to local contexts, while disparities compared to PWOD are systematic. Still, the largest and most relevant contrasts emerge in institutional settings rather than in the headline measure of living at home. How, or to what degree similar trends are found in the dimension of Community Participation is explored next.

4 Community Participation

Community participation reflects the degree to which people with disabilities are able to fully engage in the life of their communities as they see fit. Within the ADA-PARC framework, this category focuses on fundamental access to conditions that shape opportunities, to means of participating in and influencing society rather than participation outcomes themselves.

To capture these dynamics, the Community Participation category is organized into four sub-domains, each summarized through a scorecard constructed from multiple underlying indicators. Together, these scorecards provide a structured way to compare states across distinct but related participation factors.

Education Access

The Education Access scorecard captures differences in educational attainment among adults with disabilities, reflecting access to formal education pathways that shape long term participation in the labor market and civic life. Two measures are used in construction of the education access index:

- Percentage of adults with disabilities ages 25 and older whose highest level of education is a **bachelor's degree or higher.**
- Percentage of adults with disabilities ages 25 and older who have not obtained a **high school diploma or general education development (GED) equivalent.**

Transportation Access

The Transportation Access scorecard describes how people with disabilities navigate daily mobility, particularly in relation to work and community engagement. Measures emphasize commuting modes that reflect both independence and infrastructure availability, including:

- Percentage of workers with disabilities who **drive alone** to work as their primary commute mode.
- Percentage of workers with disabilities who **use public transit** as their primary commute mode.

Technology Access

The Technology Access scorecard captures digital connectivity, which has become increasingly important for participation and engagement in all other domains. Measures include:

- Percentage of individuals with disabiilties with **internet access.**
- Percentage of individuals with disabilities with **computer access.**
- Percentage of individuals with disabilities with **smartphone access.**

Health Insurance Access

The Health Insurance Access scorecard focuses on coverage among people with disabilities, recognizing healthcare access as a prerequisite for participation rather than an outcome. Measures in this sub-category examine:

- Percentage of individuals with disabilities with **public health insurance (Age 19-64).**
- individuals with disabilities with **public health insurance (Age 65+).**
- Percentage of individuals with disabilities with **private health insurance (19-64).**
- Percentage of individuals with disabilities with **private health insurance (65+).**
- Percentage of individuals with disabilities who are **uninsured (19-64).**
- Percentage of individuals with disabilities who are **uninsured (65+).**

Together, these four scorecards describe key participation enabling conditions for people with disabilities across states. In the sections that follow, descriptive results for each domain are presented, examining distributions, regional patterns, and disparities relative to people without disabilities. As with the Community Living section, the goal is to establish a baseline for understanding outcomes for PWD in this category, and how they vary across the United States.

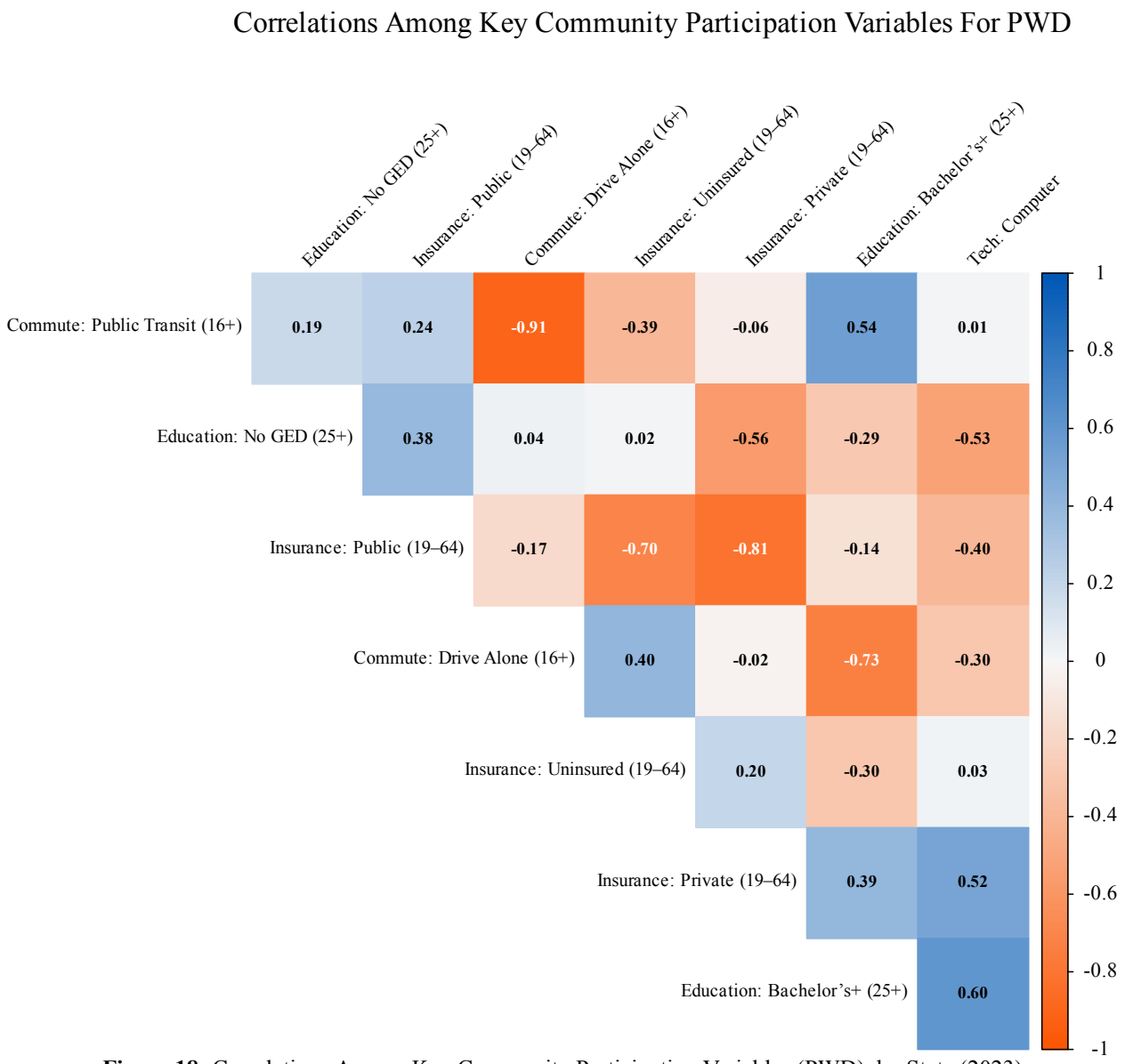


Figure 18: Correlations Among Key Community Participation Variables (PWD), by State (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Cell colors show Pearson correlations across states for the selected Community Participation measures (cooler tones indicate more positive correlation; warmer tones indicate more negative correlation; white indicates values near zero). Values are computed using pairwise-complete observations so missing data do not drop entire variables. Coefficients are printed in each cell with text color switching for readability at higher absolute correlations.

In advance of looking into our first scorecard for the category, we begin with figure above which summarizes the relationships among key community participation indicators for people with disabilities across states. Rather than operating independently, the community participation dimensions appear to group into broader patterns of participation related advantage and disadvantage.

Firstly, for educational attainment, states with higher shares of PWD holding a bachelor's degree or higher (25+) tend to have stronger digital access, especially to computers ($r = 0.6$) and are far less likely to have PWD with no GED ($r = -0.28$). Education also aligns with commute context, as bachelor's attainment is strongly negatively related to driving alone ($r = -0.7$) and positively related to public transit use ($r = 0.54$).

Technology indicators tend to move together in an expected manner. Computer and internet access are tightly linked (r = 0.9). Interestingly, tech access also seems to modestly track educational attainment in terms of computer and internet access but not smartphone access. Insurance measures show clearer patterns for public coverage than for private coverage. Among working-age adults, public coverage is strongly inversely related to private coverage (r = −0.8) and to being uninsured (r = −0.7). In contrast, private (19–64) is slightly positively related to uninsured (r = 0.201), suggesting that variation in uninsured rates is more tightly linked to public coverage levels than to private coverage alone.

Commute indicators are strongly opposed, and they connect to other measures in ways that likely reflect broader state context. Public transit and driving alone are nearly mirror images (r = −0.9). Driving alone is also moderately positively related to being uninsured (19–64) (r = 0.4) and smartphone access (r = 0.3), while transit use is moderately negatively related to uninsured (19–64) (r = −0.39). It's vital to re-emphasize, that these relationships and the matrix as a whole offers as a starting point for understanding outcomes, and suggest broad associations rather than causal connection.

4.1 Insurance Access

4.1.1 Scorecard ~ Insurance Access Index

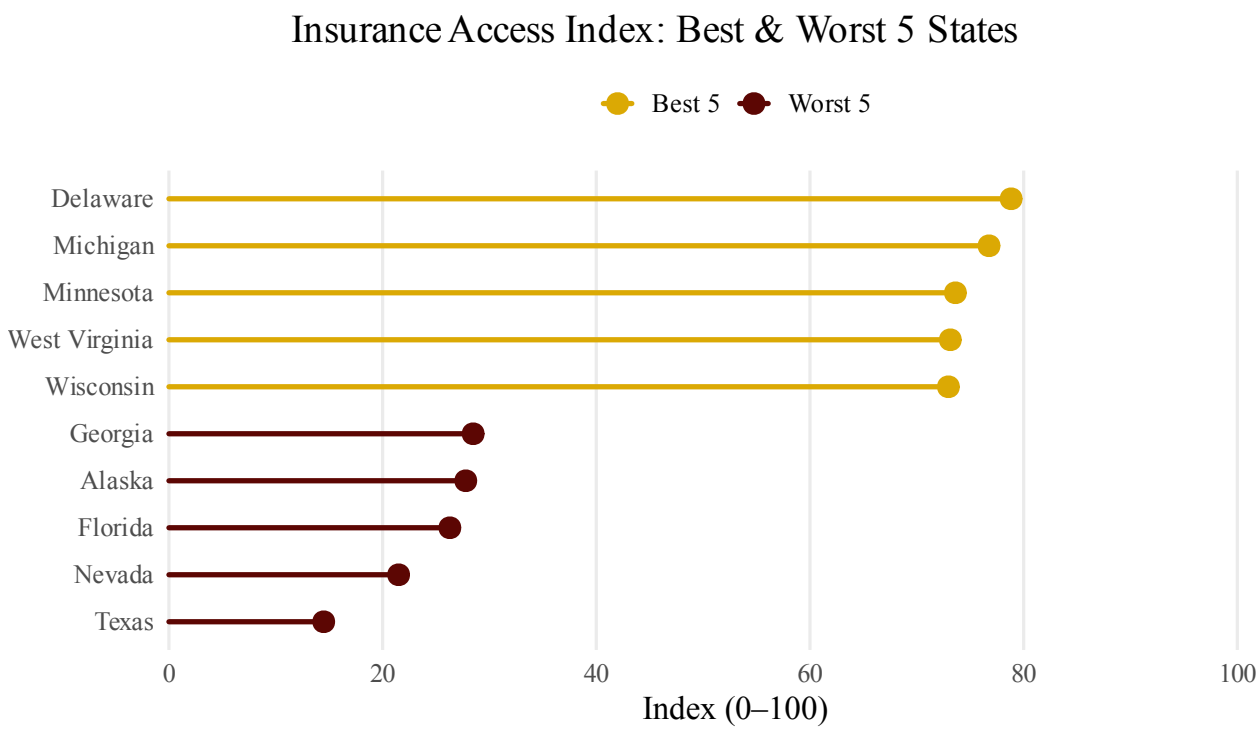


Figure 19: Insurance Access Index: Best and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards (constructed from U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates).

Interpretation: The Insurance Access Index is shown on a 0–100 scale where higher values indicate relatively better insurance access outcomes for people with disabilities. Gold marks the five highest-scoring states and maroon marks the five lowest-scoring states.

Now turning from broad relationships to our first scorecard within community participation - Insurance Access. Figure aboe highlights the five states with the highest and lowest insurance access 100-point scores

for PWD. Our insurance access index synthesizes multiple dimensions of coverage, capturing relative advantages and disadvantages rather than absolute coverage levels alone. Highest ranked states of Delaware, Michigan, Minnesota, West Virginia, and Wisconsin score in the mid-to-upper 70s, placing them well above the national average. In these states, higher overall coverage rates and comparatively lower uninsured shares combined produce strong index values. At the lower end, Texas, Nevada, Florida, Alaska, and Georgia record index values from the mid-teens to upper 20s. These scores reflect comparatively weaker coverage profiles and higher uninsured rates for PWD across age groups. Spread between the top and bottom groups is substantial considering the many variables which comprise this index.

Insurance Access Index —Full Ranking (All States)

Rank	State	Insurance Access Index	Quartile
1	Delaware	**78.8**	Q4
2	Michigan	**76.8**	Q4
3	Minnesota	**73.6**	Q4
4	West Virginia	**73.1**	Q4
5	Wisconsin	**73.0**	Q4
6	Iowa	**72.7**	Q4
7	Kentucky	**69.3**	Q4
8	Pennsylvania	**67.5**	Q4
9	Maine	**67.2**	Q4
10	Montana	**65.9**	Q4
11	Hawaii	**63.7**	Q4
12	Vermont	**63.0**	Q4
13	Massachusetts	**61.2**	Q3
14	New Hampshire	**59.7**	Q3
15	North Dakota	**59.5**	Q3
16	Wyoming	**57.4**	Q3
17	Arkansas	**57.3**	Q3
18	Indiana	**56.9**	Q3
19	Oregon	**56.7**	Q3
20	South Dakota	**55.2**	Q3
21	District of Columbia	**54.8**	Q3
22	Idaho	**54.7**	Q3
23	Maryland	**54.5**	Q3
24	Ohio	**53.7**	Q3
25	Alabama	**52.3**	Q3
26	South Carolina	**51.9**	Q2
27	Rhode Island	**50.2**	Q2
28	New York	**50.1**	Q2
29	Utah	**49.9**	Q2
30	Washington	**49.7**	Q2
31	North Carolina	**49.6**	Q2
32	Louisiana	**48.9**	Q2
33	Nebraska	**48.7**	Q2
34	Virginia	**47.8**	Q2
35	Kansas	**46.4**	Q2
36	Connecticut	**44.7**	Q2
37	Missouri	**44.6**	Q2
38	Mississippi	**44.0**	Q2
39	Colorado	**43.1**	Q1
40	Tennessee	**43.0**	Q1
41	New Mexico	**41.0**	Q1
42	Illinois	**39.9**	Q1
43	New Jersey	**38.9**	Q1
44	Arizona	**37.1**	Q1
45	Oklahoma	**35.4**	Q1
46	California	**32.4**	Q1
47	Georgia	**28.5**	Q1
48	Alaska	**27.8**	Q1
49	Florida	**26.3**	Q1
50	Nevada	**21.5**	Q1
51	Texas	**14.5**	Q1

Source: ADA-PARC 2023 Scorecards.
Interpretation: Higher index scores indicate stronger insurance access outcomes for people with disabilities (relative ranking across states).
Quartiles computed with percentile, cut points —Q1≤43.6; Q2≤51.9; Q3≤60.4; Q4≤78.8.

Table 23: Insurance Access Index — Full Ranking (All States, 2023)

Examining more than the top and bottom five states gives us a fuller picture of insurance access for PWD. Scores range from 78.8 in Delaware to 14.5 in Texas, with quartile cut points (Q1 ≤ 43.6; Q2 ≤ 51.9; Q3 ≤ 60.4; Q4 ≤ 78.8). States in the top quartile (Q4) generally combine lower uninsured shares and more consistent insurance coverage across age groups, with Midwestern states showing strength and to a lesser

degree the Northeast. Within the middle tiers (Q2 and Q3), states from multiple regions overlap in a fairly narrow score band—generally mid-40s through upper-50s and into the low-60s, in practical terms meaning only modest variation in outcomes for PWD.

From the lower end of Q3 into Q4, a disproportionate share of Southern states appear. Western states are more mixed, with some in the upper-middle range and others concentrated towards the bottom. Distribution of scores relays the uneven geography of insurance access for PWD. The wide spread between the highest and lowest ranked states reinforces the breadth of outcomes and potential for improvement through targeted policy interventions and cross-state learning.

4.1.2 Geographic Outcomes ~ Insurance Access

4.1.2.1 National Level Insurance Access

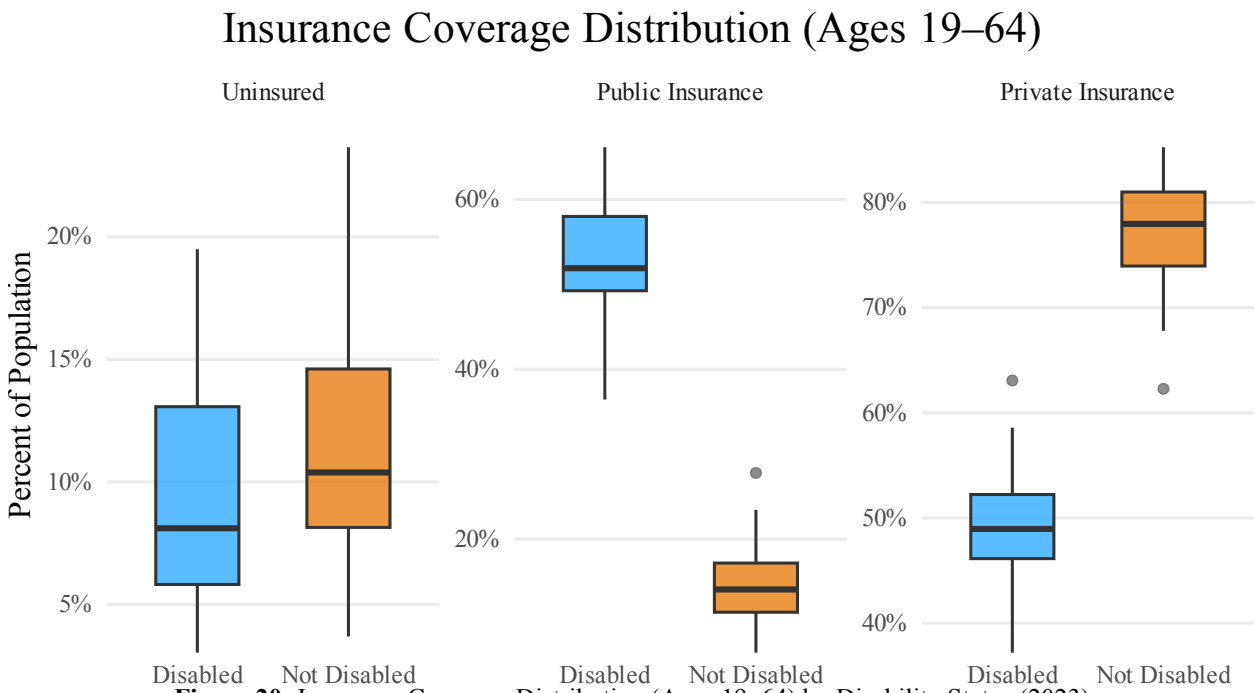


Figure 20: Insurance Coverage Distribution (Ages 19–64) by Disability Status (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each box summarizes the distribution across states (median line; boxes span the 25th–75th percentiles; whiskers show the typical range; points are outliers). Within each coverage type, compare Disabled vs Not Disabled to assess disability-related gaps among ages 19–64.

Moving from our Insurance Access Scorecard to the results of raw measures, figure above compares insurance coverage distributions for working-age adults (19–64) by disability status. Rather than emphasizing a single national estimate, the boxplots display medians and variation across states.

Among working-age adults, the median uninsured rate is lower for PWD, and most states fall between 7% to 13%, with several states extending above 15 percent. By contrast, the median uninsured rate for PWOD is higher, with most states falling between 8% to 15%, and some states extending beyond 20%. Both groups exhibit considerable state variation, though the upper tail is more pronounced among the non-disabled

Public insurance coverage shows major separation across disability status. For PWD, median public coverage is just above 50 percent, with many states falling between 45 and 60 percent. In contrast, public coverage for non-disabled adults is much lower, centered at 12 percent with minimal variation. A gap of approximately 35–40 percentage points in the median suggests the central role of Medicaid and other public programs in insuring working-age adults with disabilities.

Compared to public insurance, private insurance exhibits the inverse pattern. Non-disabled adults have private coverage rates grouped tightly between 75-80%, while PWD fall largely in the 48-52% range. closer to 45–50 percent. Separation between the medians for private insurance is substantial but slightly less than for public insurance, with an average gap of 25–30 percentage points.

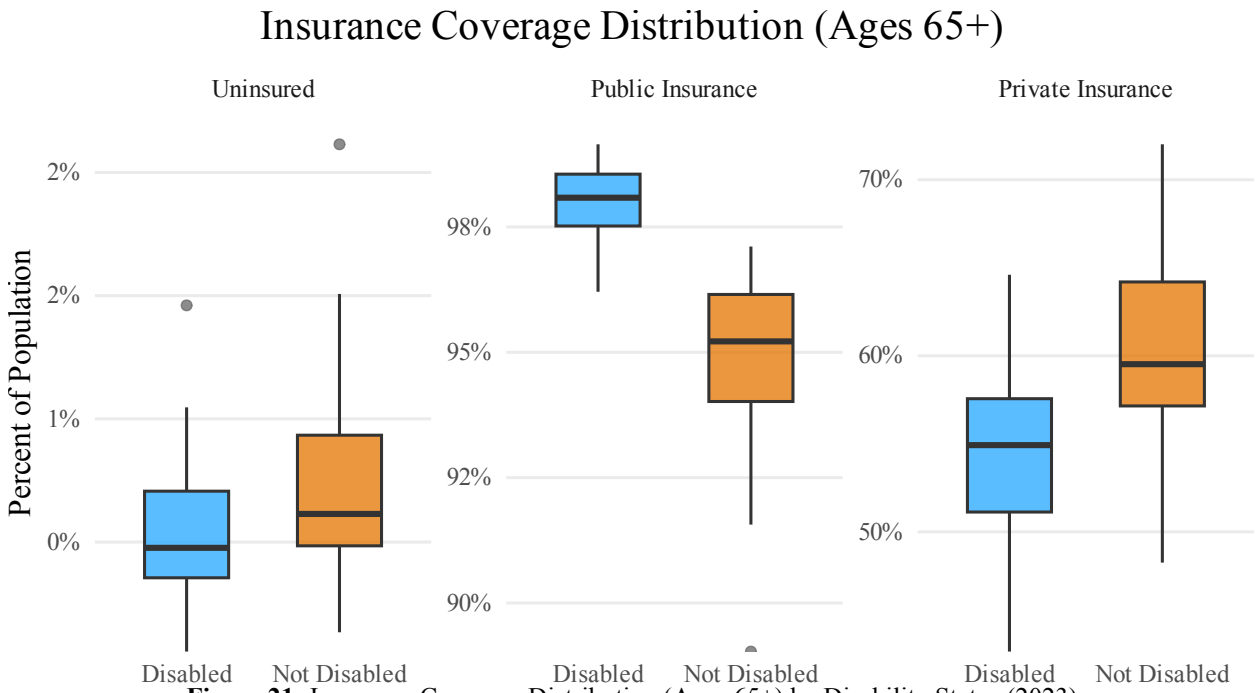


Figure 21: Insurance Coverage Distribution (Ages 65+) by Disability Status (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each box summarizes the distribution across states (median line; boxes span the 25th–75th percentiles; whiskers show the typical range; points are outliers). Within each coverage type, compare Disabled vs Not Disabled to assess disability-related gaps among ages 65+.

For adults ages 65 and older, insurance coverage patterns follow a similar pattern, though with some important distinctions to note. Public insurance coverage is nearly universal among older adults with disabilities. Median share of PWD ages 65+ covered by public insurance is above 98 percent across states with very little drop off. Even among older adults without disabilities, public coverage remains very high, above 95 percent on average, reflecting the central role of Medicare in this age group.

Private insurance among adults 65+ functions largely as supplemental coverage layered on top of public insurance rather than a substitute. As such, rates are still above 50% in most states for PWD and hover around 60% for PWOD. Variation in private coverage is modest, but relative to public insurance, comparatively greater. Remaining uninsured among adults 65+ is rare for both groups. In most states, uninsured rates are if not zero, quite close to it and almost always below 1%. Practical elimination of uninsured states at older ages stands in sharp contrast to the working-age disability population and reflects the stabilizing effect of public eligibility.

Regional Level Insurance Access

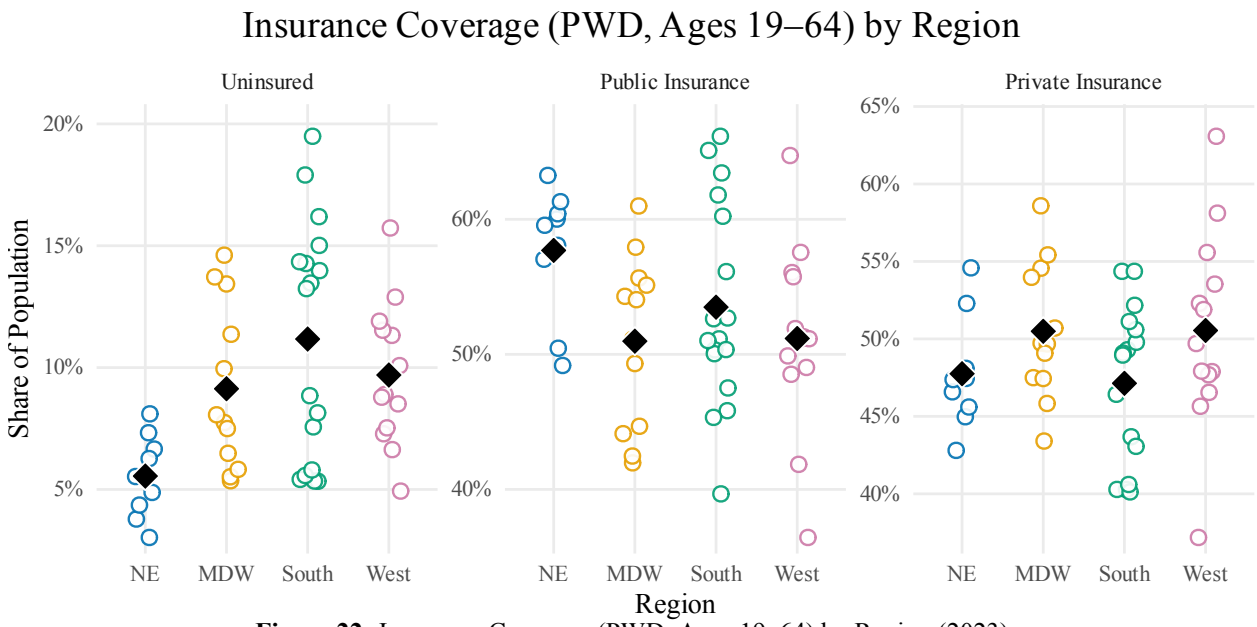


Figure 22: Insurance Coverage (PWD, Ages 19–64) by Region (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (ACS) 5-year estimates.

Interpretation: Each dot represents one state and the black diamond indicates the regional mean. Panels use separate y-scales so low-rate categories remain legible.

For our regional analysis in this domain, we look just at the working age PWD population, as there is minimal variation for PWD 65+ as explored previously. Thus, figure above compares state-level insurance coverage patterns for PWD ages 19–64, grouped by region. While regional averages differ modestly, the dispersion within regions is considerable, indicating that state-level variation remains meaningful. Private insurance coverage generally centers around the low-50 percent range across regions with the Midwest and West having higher rates than the Northeast and South. Public insurance coverage is similar with medians falling between 50 and 60 percent. The South and West show a noticeably higher range of outcomes.

In terms of the uninsured, here we get the most meaningful differences. On the low end, the Northeast has a median of 5% with rather minimal range. In contrast, the South's median is more than double at 11%, and has a much greater range with a few states around 5% and others approaching 20%. Between these two poles, the Midwest and West have both moderate medians around 9% also with modest dispersion. Overall, the figure suggests that while broad regional tendencies are visible particularly for uninsured rates, within region differences are often as large or larger than differences between regions.

4.1.2.2 State Level Insurance Access

Insurance Coverage (PWD) —All States

State	Uninsured (19–64)	Public (19–64)	Private (19–64)	Uninsured (65+)	Public (65+)	Private (65+)
Alabama	13.5%	51.1%	49.3%	0.4%	98.4%	54.9%
Alaska	12.9%	51.3%	47.9%	0.9%	97.0%	53.9%
Arizona	11.3%	51.1%	47.7%	0.7%	98.0%	50.1%
Arkansas	8.8%	61.8%	40.3%	0.3%	98.8%	48.3%
California	7.3%	56.1%	45.7%	0.8%	97.1%	43.2%
Colorado	8.5%	49.0%	53.5%	0.7%	97.5%	52.3%
Connecticut	5.5%	57.0%	46.6%	0.7%	97.5%	51.7%
Delaware	5.3%	56.1%	52.2%	0.4%	98.7%	63.4%
District of Columbia	5.3%	60.2%	43.7%	0.1%	96.2%	53.7%
Florida	14.0%	47.5%	49.8%	0.8%	97.8%	45.9%
Georgia	16.2%	45.8%	50.6%	0.7%	97.7%	50.4%
Hawaii	4.9%	51.2%	58.1%	0.5%	97.2%	64.6%
Idaho	11.5%	48.5%	52.3%	0.4%	98.5%	55.7%
Illinois	7.7%	51.1%	49.7%	0.7%	97.2%	53.2%
Indiana	8.1%	54.3%	47.5%	0.5%	98.4%	55.3%
Iowa	5.4%	55.7%	49.7%	0.4%	98.6%	58.9%
Kansas	14.6%	42.0%	54.6%	0.6%	98.1%	60.0%
Kentucky	5.4%	65.1%	40.3%	0.2%	98.8%	54.4%
Louisiana	7.6%	63.4%	40.1%	0.4%	97.9%	48.2%
Maine	7.3%	60.0%	45.0%	0.1%	98.3%	56.0%
Maryland	5.6%	52.6%	54.4%	0.7%	96.8%	63.2%
Massachusetts	3.0%	60.4%	47.4%	0.4%	97.3%	55.1%
Michigan	5.5%	61.0%	45.8%	0.3%	98.6%	64.2%
Minnesota	6.5%	54.0%	49.1%	0.3%	98.6%	63.9%
Mississippi	15.0%	52.7%	43.1%	0.3%	98.9%	46.9%
Missouri	13.4%	49.3%	47.4%	0.4%	98.5%	50.7%
Montana	8.9%	55.7%	47.9%	0.3%	98.5%	57.8%
Nebraska	11.4%	44.7%	55.4%	0.6%	98.1%	57.0%
Nevada	11.9%	49.9%	49.7%	1.0%	96.7%	44.8%
New Hampshire	6.7%	49.2%	54.6%	0.5%	97.7%	60.3%
New Jersey	8.1%	50.5%	52.3%	0.9%	97.1%	53.6%
New Mexico	8.8%	64.7%	37.2%	0.7%	98.0%	48.8%
New York	4.9%	61.3%	45.6%	0.6%	97.8%	49.2%
North Carolina	13.2%	50.3%	49.1%	0.4%	98.5%	53.8%
North Dakota	10.0%	42.5%	58.6%	0.5%	98.2%	64.2%
Ohio	7.5%	57.9%	43.4%	0.4%	98.1%	53.1%
Oklahoma	17.9%	45.3%	47.1%	0.6%	98.1%	55.8%
Oregon	6.6%	57.6%	46.5%	0.5%	98.1%	52.9%
Pennsylvania	6.3%	58.1%	48.1%	0.4%	98.2%	58.6%
Rhode Island	3.8%	59.6%	47.4%	0.7%	97.8%	49.4%
South Carolina	14.3%	50.0%	49.0%	0.4%	98.8%	54.7%
South Dakota	13.7%	44.1%	54.0%	0.3%	98.6%	55.5%
Tennessee	14.3%	51.0%	46.4%	0.5%	98.6%	51.6%
Texas	19.5%	39.7%	51.1%	1.5%	96.7%	46.2%
Utah	10.1%	36.4%	63.1%	0.5%	97.8%	55.3%
Vermont	4.4%	63.2%	42.8%	0.5%	98.2%	58.2%
Virginia	8.1%	50.4%	54.4%	0.8%	97.3%	58.5%
Washington	7.5%	51.9%	51.9%	0.7%	97.5%	57.1%
West Virginia	5.8%	66.1%	40.6%	0.2%	99.1%	57.4%
Wisconsin	5.8%	55.1%	50.7%	0.3%	98.7%	57.3%
Wyoming	15.7%	41.9%	55.6%	0.2%	99.0%	55.5%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are the share of people with disabilities who are uninsured, publicly insured, or privately insured within each age group.

Table 24: Insurance Coverage (PWD) — All States (2023)

Zooming in another level with the full state table, we get a clearer picture of the specifics amidst the regional trends. Most stark is the contrast between working-age and older adults in terms of coverage patterns, which are pronounced both in overall levels and in the degree of interstate variation. For adults age 19-64, uninsured rates range from 3.0 percent in Massachusetts to 19.5 percent in Texas. Several Southern and Mountain West

states, including Georgia (16.2%), Mississippi (15.0%), Wyoming (15.7%), and South Carolina (14.3%), also exhibit elevated uninsured shares. Public insurance is the most common coverage source in many states, exceeding 60 percent in West Virginia (66.1%), Kentucky (65.1%), New Mexico (64.7%), and Vermont (63.2%). Private coverage varies widely, from 37.2 percent in New Mexico to over 60 percent in Utah (63.1%). Importantly, these categories are not mutually exclusive in ACS reporting, so totals may exceed 100 percent, thus patterns reflect coverage prevalence, not exclusive enrollment shares.

Among adults ages 65 and older, coverage is much more uniform. Uninsured rates are near zero in most states, often between 0.2 and 0.7 percent, consistent with near-universal Medicare coverage. Public coverage exceeds 96 percent in every state and reaches 99.1 percent in West Virginia and 99.0 percent in Wyoming. Private coverage (often supplemental or retiree-based) shows greater variance, ranging from 43.2 percent in California to 64.6 percent in Hawaii and 64.2 percent in both Michigan and North Dakota. As a whole, interstate differences in insurance access for PWD are driven primarily by variation among working-age adults, while elder adults display consistently high public coverage and much fewer individuals uninsured.

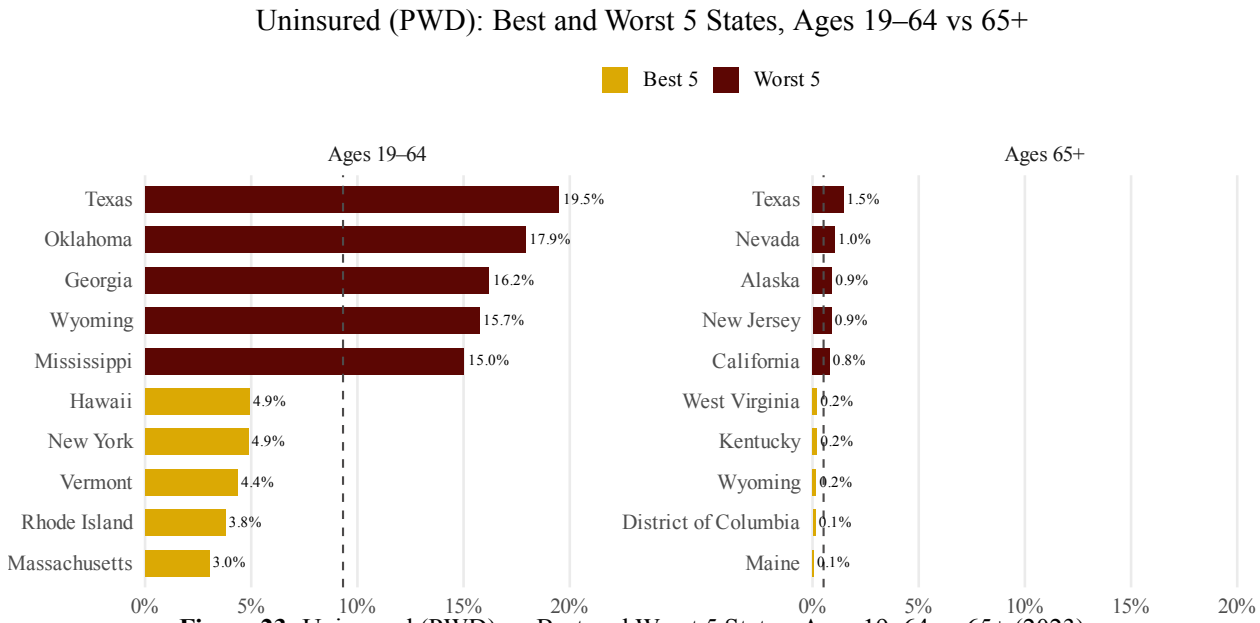


Figure 23: Uninsured (PWD) — Best and Worst 5 States, Ages 19–64 vs 65+ (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each panel shows the five best and five worst states in terms of percentage without insurance measure among people with disabilities ages 19–64 and 65+. Gold bars indicate better outcomes and maroon bars indicate worse outcomes. For *Uninsured*, lower values are better; for *Public Insurance* and *Private Insurance*, higher values are better. Dashed lines mark national averages within each panel.

Focusing on top and bottom performing states, figure above displays the five states with the highest and lowest uninsured rates among people with disabilities (PWD), shown separately for working-age adults (ages 19–64) and older adults (ages 65+). Among working-age adults, uninsured rates vary significantly and are many times larger than for elder adults. Texas (19.5%), Oklahoma (17.9%), Georgia (16.2%), Wyoming (15.7%), and Mississippi (15.0%) exhibit the highest uninsured rates. At the lower end, Massachusetts (3.0%), Rhode Island (3.8%), Vermont (4.4%), New York (4.9%), and Hawaii (4.9%) group around half of the 10% national average for the age group.

In contrast, uninsured rates among adults ages 65+ are quite low in all states. It is noteworthy that once again Texas has the highest rate of 1.5%, followed by Nevada (1.0%), Alaska (0.9%), New Jersey (0.9%),

and California (0.8%). Belonging to the lowest-uninsured states, are Maine (0.1%), the District of Columbia (0.1%), Wyoming, (0.2%), Kentucky (0.2%), and West Virginia (0.2%), who differ only marginally from the national mean. Wyoming is also an interesting case as it appears in the worst outcomes group for the younger age group while appearing in the best outcomes for the older. Spread between highest and lowest states in this age group is only 1.4 percentage points compared to over 16 percent for the younger population. Still, the panels highlight that meaningful interstate disparities in insurance coverage for PWD are almost largely within the population under 65.

Uninsured Disparity, Ages 19–64: Best and Worst States (PWD vs PWOD)

State	PWD Uninsured (19–64)	PWOD Uninsured (19–64)	Disparity
New Mexico	8.8%	14.8%	**-6.0%**
Arkansas	8.8%	13.7%	**-4.8%**
Louisiana	7.6%	12.4%	**-4.8%**
West Virginia	5.8%	10.3%	**-4.5%**
Delaware	5.3%	9.5%	**-4.2%**
South Dakota	13.7%	12.9%	**0.8%**
Nebraska	11.4%	10.4%	**1.0%**
District of Columbia	5.3%	4.0%	**1.3%**
North Dakota	10.0%	8.3%	**1.6%**
Kansas	14.6%	12.6%	**2.0%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Disparity is (PWD − PWOD) in percentage points. Gold = most negative (PWD lower uninsured). Maroon = most positive (PWD higher uninsured).

Table 25: Uninsured Disparity (PWD − PWOD), Ages 19–64: Best and Worst States (2023).

Switching to a more equity focused lens, this disparity table shifts from levels of uninsured to the gap between people with disabilities and people without disabilities in each state. Here, negative values indicate circumstances in which the uninsured rate is lower for PWD than PWOD, whereas more positive values indicate states where the uninsured rate is higher for PWD than PWOD. On the more favorable end of the distribution for PWD, several states show negative gaps, with New Mexico at −6.0%, followed by Arkansas and Louisiana (−4.8), West Virginia (−4.5) and Delaware (−4.2). In these states, uninsured rates for PWD are consistently below 10%.

In states with positive gaps, PWD in the 19-64 age group are uninsured at higher rates than PWOD. Kansas shows the largest positive disparity (+2.0), followed by North Dakota (+1.6), the District of Columbia (+1.3), Nebraska (+1.0), and South Dakota (+0.8). Commonly, these states creep above teh 10% uninsured rate for PWD, with the exception of DC, who has a reasonably low uninsured rate across disability statuts. In equity terms, these are the states where disability aligns with a higher likelihood of lacking insurance among working-age adults. As such, the table shows that disability-related inequities in insurance access are not uniform across the country. Some states exhibit large protective gaps for PWD relative to PWOD, while others show disadvantage for, PWD with uninsured rates remaining higher.

4.1.2.3 City Level Insurance Access

PWD Insurance Coverage in the 10 Most Populated U.S. Cities

	Ages 19–64 (PWD)			Ages 65+ (PWD)		
City	Unins.	Public	Private	Unins.65	Public65	Private65
New York	5.9%	63.6%	40.4%	1.1%	97.0%	35.8%
Los Angeles	9.7%	58.1%	38.5%	1.2%	96.8%	29.2%
Chicago	10.5%	52.4%	42.7%	1.4%	95.8%	37.9%
Houston	24.4%	40.0%	41.6%	1.9%	95.6%	36.7%
Phoenix	14.7%	49.0%	44.2%	1.8%	96.4%	41.8%
Philadelphia	7.4%	64.0%	39.3%	0.7%	97.2%	43.8%
San Antonio	19.7%	45.2%	46.3%	1.4%	97.1%	43.8%
San Diego	8.4%	50.6%	51.5%	1.2%	96.3%	46.0%
Dallas	22.6%	39.0%	44.8%	2.4%	95.6%	35.7%
San Jose	5.8%	52.9%	49.6%	0.9%	96.8%	39.0%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Entries are the share of people with disabilities in each age group who are uninsured, publicly insured, or privately insured.

Table 26: PWD Insurance Coverage in the 10 Most Populated U.S. Cities (2023)

Turning from the state to city level, the table above summarizes insurance coverage patterns among people with disabilities (PWD) in the ten most populated U.S. cities by age group (19–64 and 65+) and type of coverage. Focusing on large urban areas provides insight into how disability insurance access operates within dense labor markets and environments with complex demographics.

Among working-age adults with disabilities (ages 19–64), some variation is evident across cities, particularly in uninsured rates. Cities such as Houston, Dallas, and San Antonio exhibit notably high uninsured shares approaching or exceeding 20 percent while cities like New York, San Jose, and Philadelphia report uninsured rates below 8 percent. Across all cities, public insurance plays a central role for PWD ages 19–64 as we've seen previously, often covering half or more of the population. Private insurance coverage for working-age PWD remains consistently lower than public coverage and varies less dramatically across cities, generally ranging from the high 30s to low 50s.

For adults ages 65 and older with disabilities, coverage patterns are again more steady across cities. Public insurance coverage is nearly universal, reflecting the central role of Medicare, while uninsured rates hover between a low of .7% in Philadelphia to a high of 2.4% in Dallas. Private insurance among older PWD remains present but secondary, with coverage levels varying modestly. Macro-level patterns in the nation's most populous cities reinforce two key themes observed at the state level: firstly, that insurance access for PWD characterized by higher rates of public insurance rather than private market participation, and secondly, even within large and economically diverse cities, state context may exert meaningful shifts in coverage outcomes.

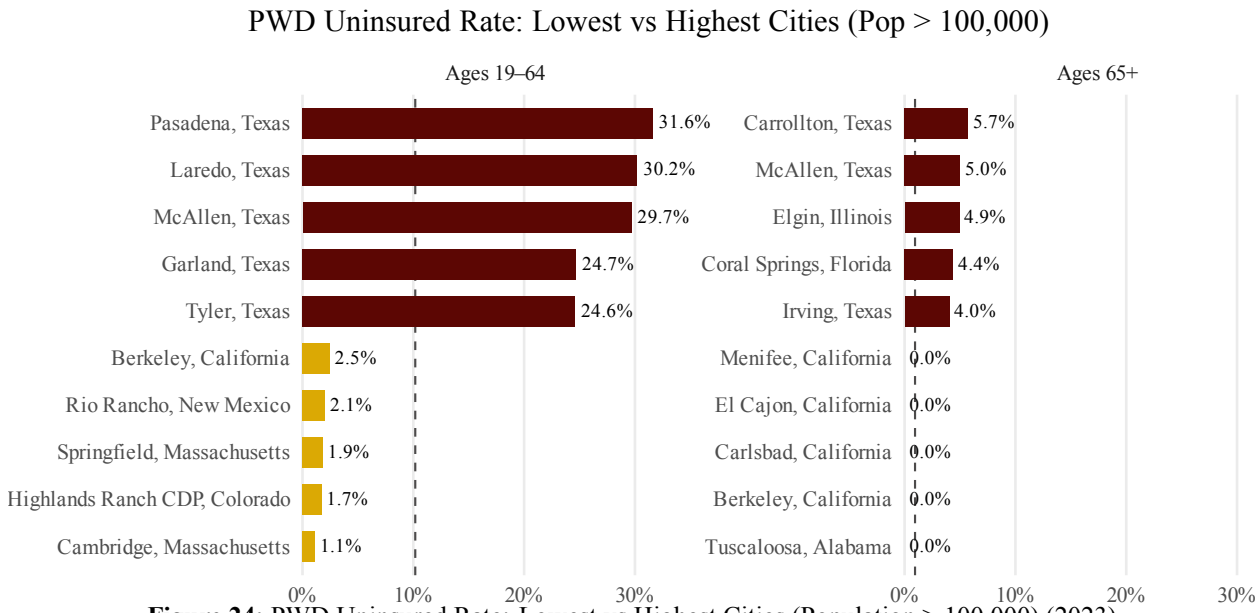


Figure 24: PWD Uninsured Rate: Lowest vs Highest Cities (Population > 100,000) (2023)

Source: U.S. Census Bureau, 2023 American Community Survey (5-year estimates).

Interpretation: Gold bars show the five cities with the lowest uninsured rates among people with disabilities (better outcomes) and maroon bars show the five cities with the highest uninsured rates (worse outcomes), among cities with civilian noninstitutionalized population greater than 100,000. Dashed lines mark the mean within each age group.

Proceeding to large cities with the best and worst outcomes for PWD, the figure above shows such localities with populations over 100,000, the dashed line in each panel representing the mean uninsured rate across all qualifying cities in that age group. In line with our state analysis, Texas cities dominate the highest uninsured rates across age groups, with California cities consistently appearing in the low-rate group.

For working-age adults, the range is striking. Cities of Texas make up the group exclusively, with Pasadena (31.6%), Laredo (30.2%), and McAllen (29.7%) reporting uninsured rates near or above 30 percent. Garland (24.7%) and Tyler (24.6%) follow closely. At the more favorable end for PWD, cities such as Cambridge, MA (1.1%), Highlands Ranch, CO (1.7%), Springfield, MA (1.9%), Rio Rancho, NM (2.1%), and Berkeley, CA (2.5%) report uninsured rates below 3 percent, an almost thirty-point spread between the extremes.

In the older population of ages 65+, the distribution is again much more compressed. Highest uninsured rates are those of Carrollton, (5.7%) and McAllen (5.0%), TX, followed by Elgin, Illinois (4.9%), Coral Springs, Flordia (4.4%), and Irving, TX (4.0%). remain well below the levels observed among working-age adults. On the other end, several cities report uninsured rate very close to zero (more decimals after one tenth of a percent not shown) four of which are Californian: Menifee, El Canjon, Carlsbad, and Berkeley, with Tuscaloosa rouding out the group. Gap between the highest and lowest cities here is less than six percentage points.

4.2 Education Access

4.2.1 Scorecard ~ Education Access Index Score

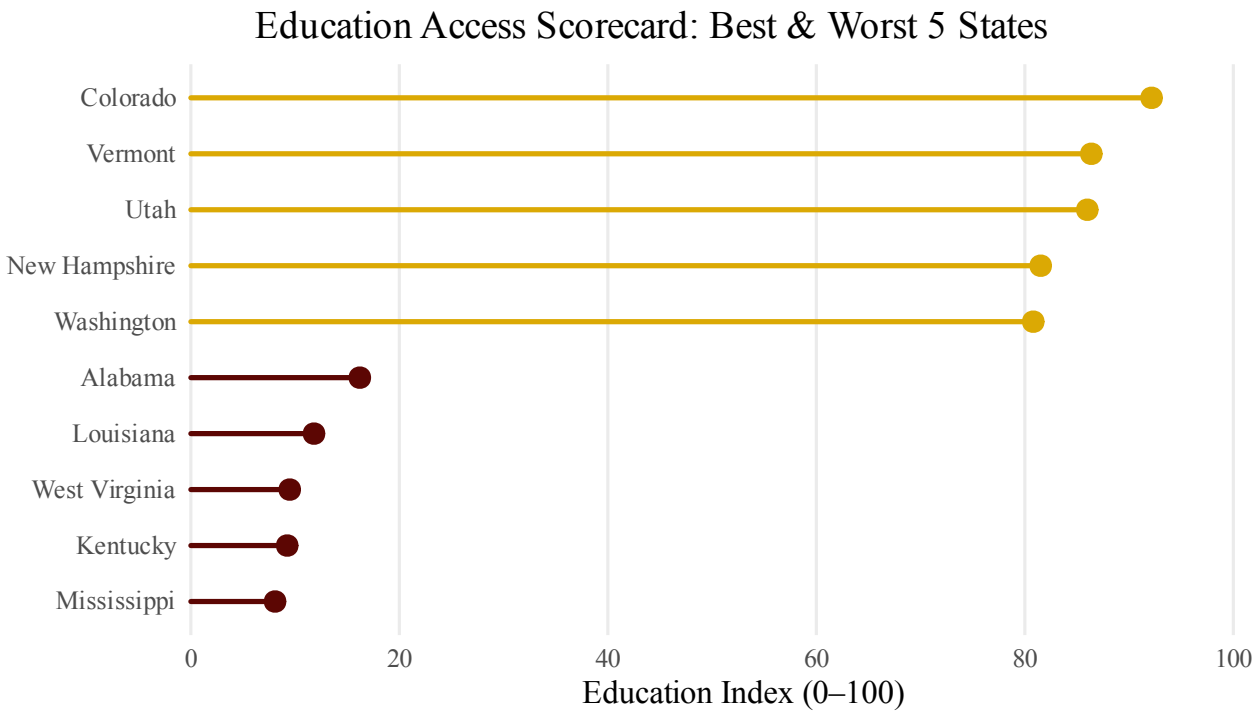


Figure 25: Education Index: Best and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards.

Interpretation: Scores range from 0 to 100, with higher values indicating stronger education outcomes for people with disabilities. Gold markers denote the five highest-scoring states (best outcomes) and maroon markers denote the five lowest-scoring states (worst outcomes).

Transitioning to our second scorecard within the Community Participation category, figure above presents five of the highest and lowest performing states on the Education Access Index for PWD. States at the top tier range from 80 to low 90s on the 100-point scale, with states from the Northeast and West represented. Colorado leads, followed closely by Vermont, Utah, New Hampshire, and Washington. These states tend to have lower shares of adults with disabilities lacking a high school credential and higher proportions attaining post-secondary education.

Bottom performers on this scorecard are concentrated in the South, with Mississippi, Kentucky, West Virginia, Louisiana, and Alabama falling below a score of 20. We can thus infer that these states are home to lower levels of educational attainment among PWD. Education Access Index scores set the stage for closer analysis of education outcomes for people with disabilities across the U.S. From this initial view, we are certain to find an uneven array of outcomes at the various levels of geography. Though before diving deeper, more detail can be derived from the full scorecard table.

Education Access Scorecard —Full Ranking (All States)

Rank	State	Education Score	Quartile
1	Colorado	92.2	Q4
2	Vermont	86.4	Q4
3	Utah	86.0	Q4
4	New Hampshire	81.5	Q4
5	Washington	80.8	Q4
6	Montana	80.3	Q4
7	Hawaii	77.5	Q4
8	Minnesota	74.4	Q4
9	Delaware	73.5	Q4
10	Maryland	73.0	Q4
11	Oregon	72.5	Q4
12	Maine	70.9	Q4
13	Wyoming	68.9	Q3
14	District of Columbia	68.3	Q3
15	Kansas	65.6	Q3
16	Virginia	64.7	Q3
17	Idaho	63.4	Q3
18	Nebraska	62.7	Q3
19	New Jersey	62.3	Q3
20	Arizona	61.5	Q3
21	Massachusetts	60.7	Q3
22	North Dakota	60.2	Q3
23	South Dakota	59.7	Q3
24	Wisconsin	58.4	Q3
25	Florida	58.1	Q3
26	Alaska	57.8	Q2
27	Connecticut	53.9	Q2
28	Iowa	53.5	Q2
29	Illinois	50.4	Q2
30	Nevada	46.4	Q2
31	Pennsylvania	40.5	Q2
32	Michigan	39.3	Q2
33	New York	39.2	Q2
34	California	38.5	Q2
35	North Carolina	38.0	Q2
36	Oklahoma	38.0	Q2
37	New Mexico	37.5	Q2
38	Rhode Island	36.8	Q2
39	Georgia	36.0	Q1
40	Missouri	35.1	Q1
41	Ohio	32.6	Q1
42	Texas	32.0	Q1
43	South Carolina	30.8	Q1
44	Indiana	26.5	Q1
45	Tennessee	21.5	Q1
46	Arkansas	18.9	Q1
47	Alabama	16.2	Q1
48	Louisiana	11.8	Q1
49	West Virginia	9.5	Q1
50	Kentucky	9.2	Q1
51	Mississippi	8.1	Q1

Source: ADA-PARC 2023 Scorecards.
Shading by quartiles: Q1 $\leq$ 36.4; Q2 $\leq$ 57.8; Q3 $\leq$ 68.6; Q4 > 68.6.
Interpretation: Higher scores indicate stronger outcomes for people with disabilities on the Education Index. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

Table 27: Education Index — Full State Ranking with Quartile Shading (2023).

Table above presents the complete state rankings for the Education Access Scorecard for PWD, highlighting the scale and nuances of interstate variation in educational outcomes. Scores range from 92.2 in Colorado to 8.1 in Mississippi. States in Q4, the top quartile (> 68.6) generally score above 70, and is dominated by states from the West and Northeast. States in the third quartile, (Q3: 57.9–68.6) sit just above the national median

and include Wyoming (68.9), the District of Columbia (68.3), Kansas (65.6), and Virginia (64.7), along with several Mountain West and Mid Atlantic states. As a group, they reflect solid, though not excellent, education outcomes for PWD. generally clustering in the low 60s.

Second quartile states, (Q2: 36.5–57.8) represent outcomes from slightly below average to considerably below average, and spans a wider geographic mix, including Connecticut (53.9), Illinois (50.4), Nevada (46.4), and Pennsylvania (40.5). In the bottom quartile, (Q1 ≤ 36.4) states display the most pronounced geographic concentration. Beyond the bottom five states, the South is represented by Missouri (36), South Carolina (30.8), and Tennessee (21.5). As a whole, the scorecard foreshadows the measure-specific trends in the geographic analysis to follow.

4.2.2 Geographic Outcomes ~ Education Access

4.2.2.1 National Level Education Access

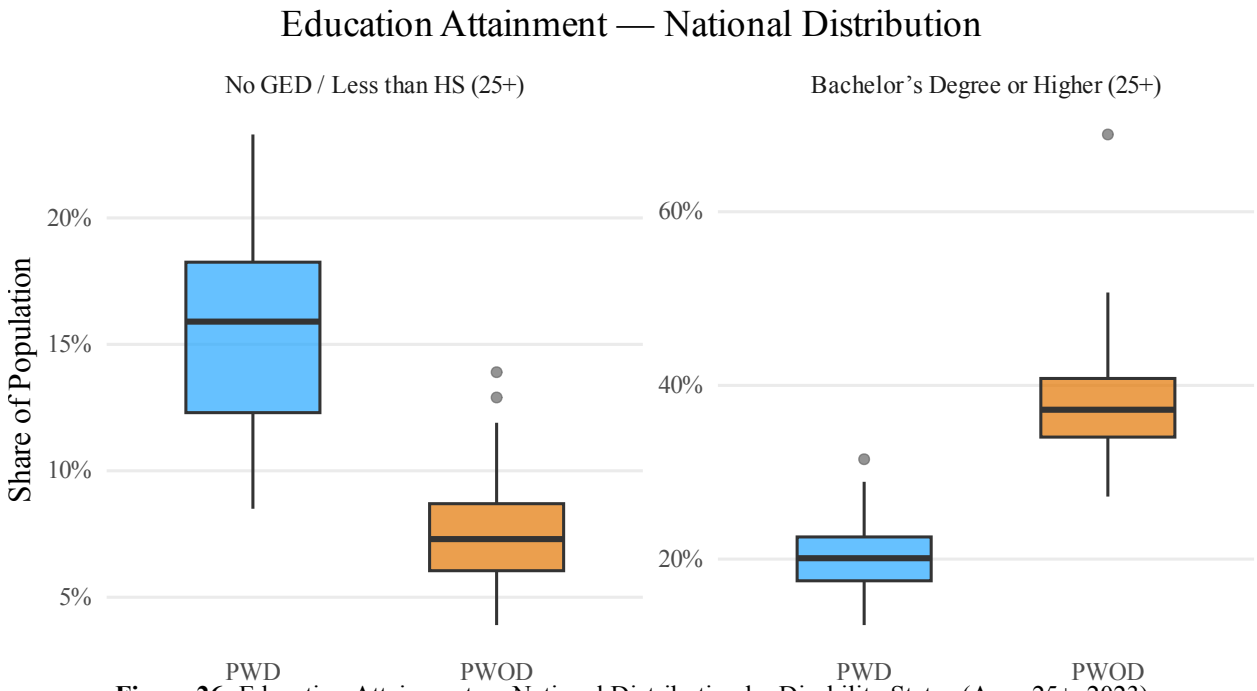


Figure 26: Education Attainment — National Distribution by Disability Status (Ages 25+, 2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each box summarizes the distribution across states; the center line is the median and points beyond the whiskers indicate outliers.

Figure above presents the national distribution of educational attainment among adults ages 25 and older, comparing people with disabilities and people without disabilities. Within the category of highest educational attainment at less than high school, the median rate for PWD is 16%, compared with 7% among PWOD, about a twofold difference. There is also noticeably more variation across states for people with disabilities relative to people without disabilities.

In terms of those whose educational attainment reaches a bachelor's degree , the pattern reverses and the size of the gap increases. Median share of PWOD with at least a bachelor's degree nears 40 percent, compared with around 20 percent among PWD. Only overlapping is the upper quartile for PWD and the lower quartile of PWOD. Viewed together, the boxplots show a dual disparity in the sense that adults with disabilities

are over represented among those with the least formal education and underrepresented among those with bachelor's degrees or higher.

4.2.2.2 Regional Level Education Access

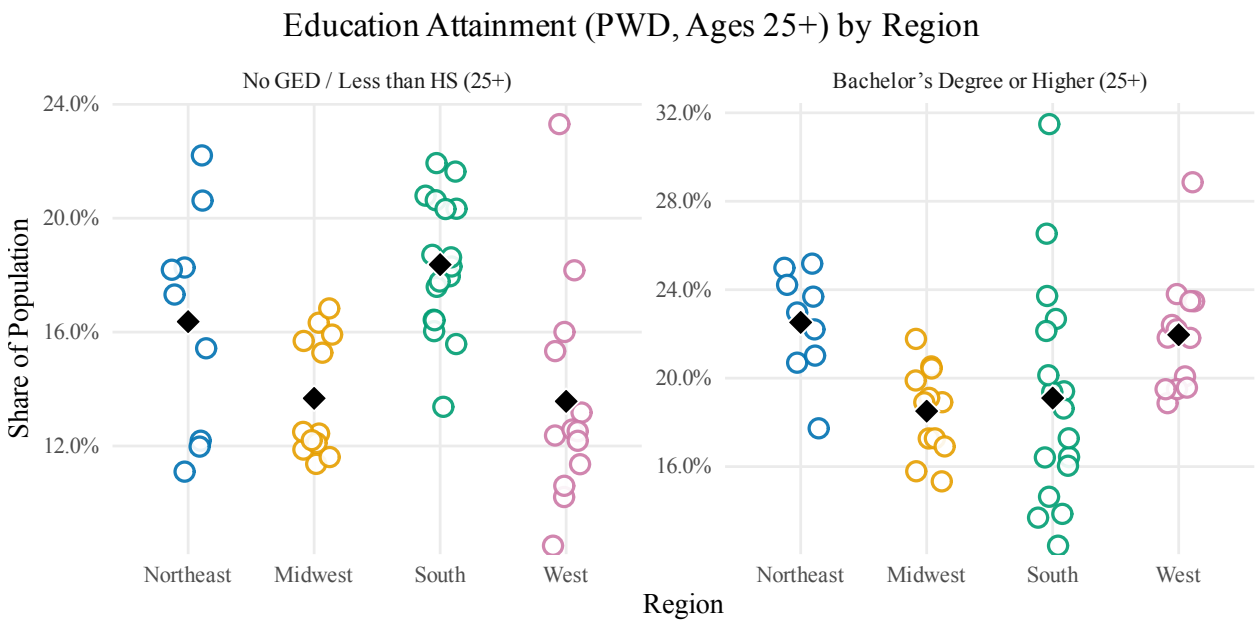


Figure 27: Education Attainment (PWD) by Region (Ages 25+, 2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state and the black diamond indicates the regional mean. Higher values indicate a larger share of people with disabilities ages 25+ in each education category; comparing regions shows how state-level distributions cluster and vary across the country.

Moving from the national to the regional level, figure above displays state-level educational attainment among adults with disabilities grouped by region. Black diamonds mark regional means. For lower educational attainment, the South stands out with the highest regional mean, and with numerous states exceeding twenty percent. The Northeast has the second highest regional mean at roughly 16% and a wide range of outcomes. Midwestern states have a regional mean of 13% with a small grouping of states around 12% and another at 16%. Western states have the lowest regional mean, also around 13% which is dragged up by one extreme outlier.

In the bachelor's degree panel, the Northeast posts the highest regional average in bachelor's attainment among PWD (22%), followed closely by the West. With a large variance, most Southern states fall below 20%, with a few high-scoring states pulling up the average. Midwestern states have the lowest average, at 18%. Two patterns are notable. Overall, regional context matters, but differences within regions are highly evident as well.

4.2.2.3 State Level Education Access

Education Attainment (PWD, Ages 25+) —All States

State	No GED / < HS (PWD, 25+)	Bachelor's+ (PWD, 25+)
Alabama	20.3%	16.4%
Alaska	12.4%	18.9%
Arizona	15.3%	21.8%
Arkansas	18.0%	13.9%
California	23.3%	22.4%
Colorado	11.4%	28.9%
Connecticut	18.3%	23.0%
Delaware	13.4%	22.7%
District of Columbia	17.6%	31.5%
Florida	16.4%	22.1%
Georgia	18.3%	19.4%
Hawaii	13.2%	23.5%
Idaho	12.6%	20.1%
Illinois	16.3%	20.5%
Indiana	16.8%	15.3%
Iowa	11.4%	17.3%
Kansas	12.4%	20.4%
Kentucky	20.8%	13.7%
Louisiana	21.6%	16.0%
Maine	11.1%	20.7%
Maryland	16.0%	26.5%
Massachusetts	18.2%	25.0%
Michigan	15.3%	17.3%
Minnesota	11.9%	21.8%
Mississippi	21.9%	14.6%
Missouri	15.9%	16.9%
Montana	10.2%	22.2%
Nebraska	12.5%	19.9%
Nevada	16.0%	19.5%
New Hampshire	12.2%	23.7%
New Jersey	17.3%	24.2%
New Mexico	18.2%	19.6%
New York	22.2%	22.2%
North Carolina	17.8%	19.4%
North Dakota	11.6%	18.9%
Ohio	15.7%	15.8%
Oklahoma	15.6%	17.3%
Oregon	12.5%	21.8%
Pennsylvania	15.4%	17.7%
Rhode Island	20.6%	21.0%
South Carolina	18.6%	18.6%
South Dakota	12.1%	19.1%
Tennessee	18.7%	16.4%
Texas	20.6%	20.1%
Utah	10.6%	23.8%
Vermont	12.0%	25.2%
Virginia	16.4%	23.7%
Washington	12.2%	23.5%
West Virginia	20.3%	12.4%
Wisconsin	12.2%	18.9%
Wyoming	8.5%	19.5%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are the share of the PWD population ages 25+ in each education category.

Table 28: Education Attainment (PWD, Ages 25+) — All States (2023).

Zooming in on the variation within regions, the table above presents state-level variation in educational attainment among adults with disabilities (PWD) ages 25 and older. Meaningful variation is evident across states. At the lower end of attainment, shares range from 8.5 percent in Wyoming to more than 20 percent in Mississippi (21.9%). A clear regional concentration emerges, with many of the lowest attainment states being located in the South and Appalachia. In contrast, several Mountain West and Upper Midwest states

group near 12 percent, suggesting comparatively lower levels of educational exclusion among PWD.

Bachelor's attainment displays a broad range. Highest shares appear in the District of Columbia (31.5%), and Colorado (28.9%), with the lowest, Kentucky (13.7%), and Mississippi (14.6%). Bottom performing states have rates nearly half those observed in the highest performing jurisdictions. Notably, the two measures of educational attainment do not move precisely in opposition. California (23.3% low attainment; 22.4% bachelor's) and New York (22.2%; 22.2%) are cases which exhibit relatively high shares at both ends of the distribution.

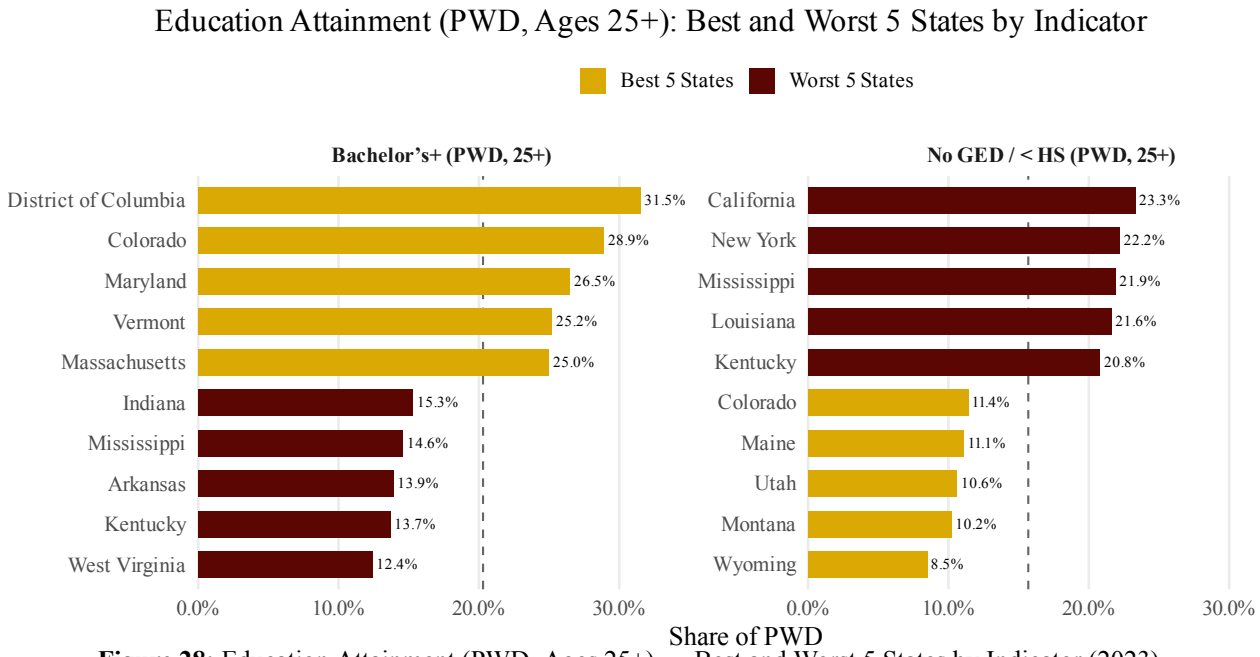


Figure 28: Education Attainment (PWD, Ages 25+) — Best and Worst 5 States by Indicator (2023).

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Gold bars show the five best states and maroon bars show the five worst states within each indicator. For Bachelor's+, higher values indicate better outcomes; for No GED / < HS, lower values indicate better outcomes. Dashed lines indicate national averages.

Examining best and worst performers, this figure presents the five highest and lowest performing states for adults with disabilities ages 25 and older across the two education indicators. A dashed reference lines denotes the national averages, approximately 20% for bachelor's attainment and 16% for no high school credential. Substantial variation appears in bachelor's attainment. District of Columbia leads at 31.5%, followed by Colorado (28.9%), Maryland (26.5%), Vermont (25.3%), and Massachusetts (25.1%) all well above the national benchmark. At the lower end, West Virginia (12.4%), Kentucky (13.7%), Arkansas (13.9%), Mississippi (14.6%), and Indiana (15.3%) fall roughly 5–8 percentage points below average rela and 10-15 percentage points below the highest performing states.

A similarly wide spread emerges for low educational attainment. California (23.3%) and New York (22.2%) report the largest shares of PWD without a high school credential, alongside Mississippi (21.9%), Louisiana (21.6%), and Kentucky (20.8%). In contrast, Wyoming (8.5%), Montana (10.2%), Utah (10.6%), Maine (11.0%), and Colorado (11.4%) have substantially lower rates, roughly half those observed in the worst states on this measure. Together, these ranges reveal the uneven educational landscape for people with disabilities, as such significant variation across states suggests the importance of geographic context for this domain.

No GED / < HS Disparity, Ages 25+: States with Greatest and Least Gaps

State	PWD No GED / < HS (25+)	PWOD No GED / < HS (25+)	Disparity
Kentucky	20.8%	8.3%	**12.5%**
District of Columbia	17.6%	5.4%	**12.2%**
Rhode Island	20.6%	8.4%	**12.2%**
West Virginia	20.3%	8.1%	**12.2%**
New York	22.2%	10.1%	**12.1%**
Oregon	12.5%	7.3%	**5.2%**
Colorado	11.4%	6.4%	**5.0%**
Utah	10.6%	6.0%	**4.6%**
Nevada	16.0%	11.9%	**4.1%**
Wyoming	8.5%	5.3%	**3.2%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are percentage-point differences (PWD − PWOD) shown with a % sign for readability. Maroon highlights the five states with the largest positive disparities (PWD higher than PWOD). Gold highlights the five states with the smallest disparities (lowest positive or negative).

Table 29: No GED / < HS Disparity (PWD − PWOD), Ages 25+: States with Greatest and Least Gaps (2023).

Viewing states now from an equity perspective with our state disparity table, it's clear that in all states PWD have higher rates of low educational attainment than PWOD. Largest disparities exceed 12 percentage points. In Kentucky, the gap is 12.5 percentage points, in DC, Rhode Island, and West Virginia, the gap is 12.2 points, and in New York 12.1 show similarly wide separations. In each case, the share of PWD whose highest educational attainment is less than high school is 2-3x the rate for PWOD.

Even in states with comparatively smaller disparities, meaningful gaps remain. Wyoming (3.2 percentage points), Nevada (4.1), Utah (4.6), Colorado (5.0), and Oregon (5.2) represent the narrowest divides among all states. Still, in these states PWD are more likely to lack a high school credential than their non-disabled peers. Overall, the spread in disparity from 3 to more than 12 percentage points is evidence of an educational inequality to be taken seriously, as it suggests that low educational attainment among PWD reflects structural barriers rather than isolated state-level conditions.

4.2.2.4 City Level Education Access

ACS-5 year estimates unavailable at local level.

4.3 Transportation Access

4.3.1 Scorecard ~ Transportation Access Index Score

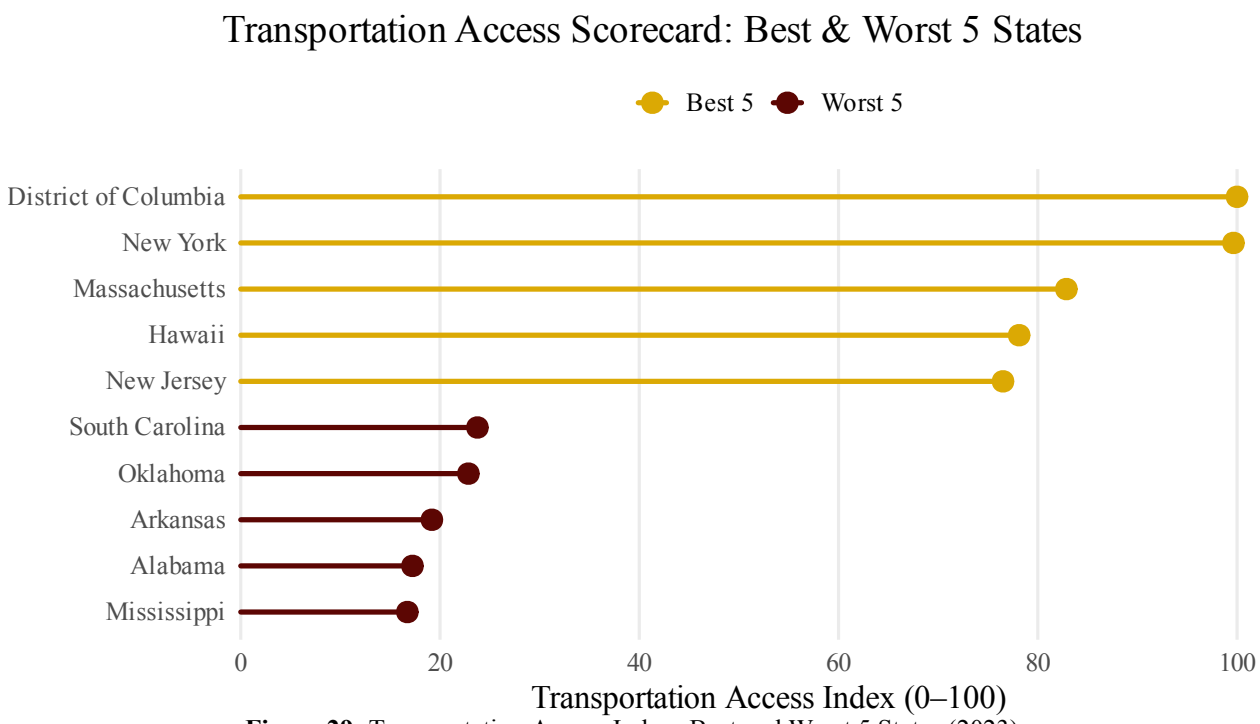


Figure 29: Transportation Access Index: Best and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards.

Interpretation: Higher index scores indicate stronger transportation access outcomes for people with disabilities. Gold marks the best-performing states; maroon marks the worst-performing states.

Switching gears from education to our next category of transportation, we begin with a peak into top and bottom performers according to the Transportation Access Scorecard for PWD. A scorecard constructed from two commute-based indicators, the share of PWD who primarily drive alone to work and the share who primarily use public transportation, higher scores reflect transportation environments that are more supportive of accessible and diversified commuting options for disabled workers. Immediately evident of the results from figure above, select geographies of DC and New York perform far above average in this domain and thus score on the maximum of the 100-pt scorecard scale. Also doing very well is Massachusetts with a score of 83, Hawaii (78), and New Jersey (76.5). These states combine relatively high public transit utilization among PWD with lower exclusive reliance on driving alone, suggesting more multimodal and transit-accessible commuting environments.

In contrast, the lowest-performing states all fall below 25 points, though are not outliers to the same degre as the highest performing states. South Carolina (24) and Oklahoma (23) score in the low-20s, while Arkansas (19), Alabama (17), and Mississippi (17) creep below twenty. In these states, commuting patterns are characterized by heavy reliance on driving alone and minimal public transit usage among PWD, reflecting transportation systems that offer fewer accessible alternatives. Because the index is derived directly from commuting behavior, these disparities likely capture differences, to a significant degree, in urban density and investment in inclusive transit infrastructure. Given the importance of transportation for employment

and healthcare access, this scorecard reflects another central domain for community participation among PWD.

Transportation Scorecard —Full Ranking (All States)

Rank	State	Transportation Score	Quartile
1	District of Columbia	**100.0**	Q4
2	New York	**99.6**	Q4
3	Massachusetts	**82.9**	Q4
4	Hawaii	**78.1**	Q4
5	New Jersey	**76.5**	Q4
6	Illinois	**74.8**	Q4
7	Maryland	**72.5**	Q4
8	Washington	**71.5**	Q4
9	Oregon	**69.2**	Q4
10	California	**67.8**	Q4
11	Pennsylvania	**67.3**	Q4
12	Minnesota	**65.1**	Q4
13	Connecticut	**58.2**	Q3
14	Alaska	**56.8**	Q3
15	Colorado	**56.1**	Q3
16	Utah	**55.5**	Q3
17	Arizona	**55.4**	Q3
18	Nevada	**54.8**	Q3
19	Vermont	**50.2**	Q3
20	Virginia	**47.9**	Q3
21	Delaware	**44.7**	Q3
22	Wisconsin	**43.3**	Q3
23	Florida	**41.8**	Q3
24	Maine	**40.9**	Q3
25	Rhode Island	**40.9**	Q3
26	Ohio	**39.6**	Q2
27	Michigan	**39.3**	Q2
28	Montana	**38.9**	Q2
29	New Hampshire	**38.0**	Q2
30	Georgia	**37.6**	Q2
31	Texas	**37.0**	Q2
32	Kansas	**36.0**	Q2
33	Missouri	**35.7**	Q2
34	South Dakota	**35.7**	Q2
35	Iowa	**35.2**	Q2
36	New Mexico	**34.8**	Q2
37	Wyoming	**34.5**	Q2
38	Idaho	**32.0**	Q2
39	North Carolina	**32.0**	Q1
40	Nebraska	**29.6**	Q1
41	West Virginia	**29.5**	Q1
42	Indiana	**28.5**	Q1
43	Kentucky	**28.3**	Q1
44	North Dakota	**26.7**	Q1
45	Louisiana	**25.8**	Q1
46	Tennessee	**25.4**	Q1
47	South Carolina	**23.8**	Q1
48	Oklahoma	**22.8**	Q1
49	Arkansas	**19.2**	Q1
50	Alabama	**17.2**	Q1
51	Mississippi	**16.7**	Q1

Source: ADA-PARC 2023 Scorecards.
Shading by quartiles: Q1 $\leq$ 32.0; Q2 $\leq$ 39.6; Q3 $\leq$ 57.5; Q4 > 57.5.
Interpretation: Higher scores indicate stronger transportation access outcomes for people with disabilities. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

Table 30: Transportation Index — Full Ranking (All States, 2023).

Expanding from the top and bottom five comparison, the full ranking reveals a pronounced geographic structure in transportation access outcomes. Scores span from a maximum of 100.0 in the District of Columbia to near max of 99.6 in New York, to 16.7 in Mississippi, a spread of more than 80 points on a the100 scale. Upper quartile states (Q4 > 57.5) are dominated by Northeast and West Coast states, and those with large urban centers such as Illinois (74.8). States in this upper range tend to feature dense metropolitan areas and established public transit systems, contributing to greater commuting options for among people with disabilities. Middle distribution states include those in the third quartile (Q3, 39.7–57.5) who post outcomes modestly above the national average. Colorado (56.1), Utah (55.5), and Arizona (55.4), sit near the upper bound of this range, while Florida (43.3) and Maine (40.9) fall closer to the national average.

By contrast, second quartile states (Q2, 32.1–39.6) group just below the national middle. Ohio (39.6), Michigan (39.3), and Montana (38.9), lead this group while Wyoming (34.5), and Idaho (32.0), just make the cut. While not among the lowest-performing states, these scores indicate transportation systems that remain predominantly auto-oriented, with limited public transit utilization for PWD. In the lowest quartile (Q1, ≤32.0) states from the South are considerably over-represented South, while also appearing are Nebraska (29.6), Indiana (28.5), and North Dakota (26.7). Regional concentration in the south points structural patterns of lower transit investment, and high reliance on driving alone. How the states separate out on these individual measures will become more clear in geographic analysis that follows.

4.3.2 Geographic Outcomes ~ Transportation Access

4.3.2.1 National Level Transportation

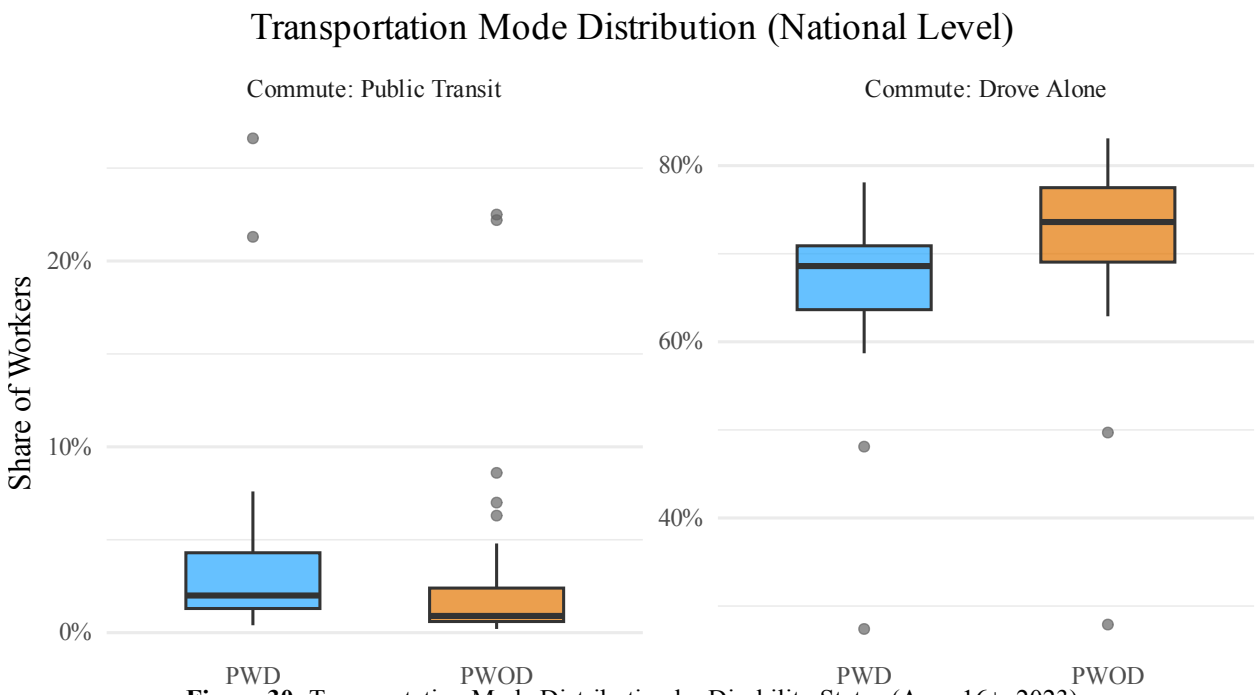


Figure 30: Transportation Mode Distribution by Disability Status (Ages 16+, 2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each box summarizes the distribution across states; the median is shown by the center line and points beyond the whiskers are outliers. Within each panel, comparing PWD and PWOD highlights disability-related differences in commuting mode shares.

At the national level, commuting patterns do differ in measurable ways between PWD and PWOD, though the big picture suggests state or local context is chiefly important. In the boxplot above it's clear that driving alone remains the dominant commute mode, the median state share for PWOD is above 70% and for PWD just below. In most states, this represents a gap of about 5 to 8 percentage points. Public transit use remains low nationally for both groups, but relative differences are evident, as transit shares for PWD have a lower floor of around 2–3 percent, compared with roughly .5 for PWOD. While these differences appear small in absolute terms, they represent proportionally higher reliance on transit among PWD, as the upper tail is particularly notable for PWD, suggesting that increased transit accessibility is likely more meaningful for PWD than PWOD.

4.3.2.2 Regional Level Transportation

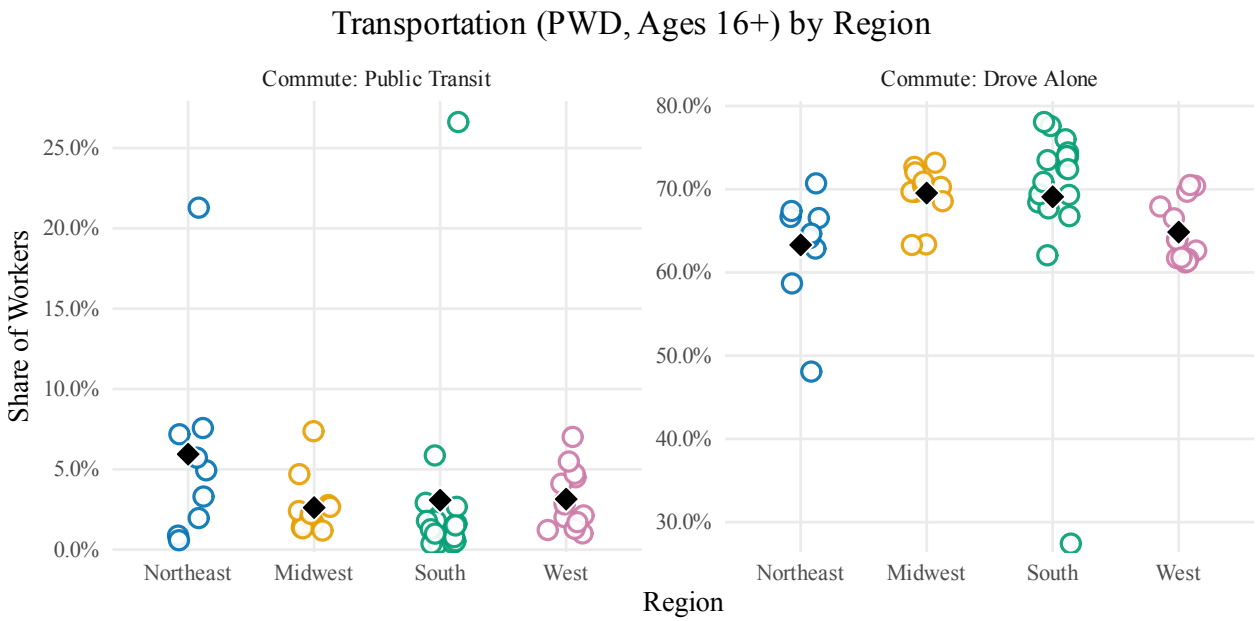


Figure 31: Transportation (PWD) by Region (Ages 16+, 2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state and the black diamond indicates the regional mean. Higher values indicate a larger share of workers with disabilities (ages 16+) using the specified commuting mode; comparing regions highlights how state-level distributions cluster and vary across the country.

Regional patterns sharpen the national picture by showing how commuting modes for people with disabilities (PWD) pan out geographically. Driving alone remains the dominant mode in every region, but average levels differ slightly on average. The Midwestern states group very tightly around 70%. The South is home to states with the highest driving reliance, though their regional mean is lower than the Midwest due to the outlier of DC. Western and Northeastern states have slightly lower regional means, with states ranging from 60-70% and New York below 50%. Relative to driving alone, public transit use shows a mirrorred pattern. In the Northeast, whose regional mean is the only to exceed 5%, a few states exceed this mark with New York above 20%. In the South, Midwest, and West, regional means are 2.5-3%, though they differ in how tightly they group. In the Midwest, most states tightly group around the regional mean with exception of Illinois at7 percent. Southern states group at 0-1% with an average dragged up by DC above 25%. Western states have a more even spread with a couple states right at the average, some around 1% and others just above 5%.

4.3.2.3 State Level Transportation

Transportation (PWD): Raw Levels by State

State	Public Transit (PWD)	Drove Alone (PWD)
Alabama	0.4%	77.6%
Alaska	2.0%	61.7%
Arizona	2.1%	62.6%
Arkansas	0.4%	76.0%
California	4.5%	61.6%
Colorado	3.0%	64.0%
Connecticut	4.9%	66.5%
Delaware	2.9%	68.4%
District of Columbia	26.6%	27.4%
Florida	1.8%	67.7%
Georgia	1.8%	69.4%
Hawaii	7.0%	61.2%
Idaho	1.0%	70.4%
Illinois	7.4%	63.4%
Indiana	1.4%	72.7%
Iowa	1.7%	70.2%
Kansas	1.6%	69.7%
Kentucky	1.3%	72.6%
Louisiana	1.5%	74.4%
Maine	0.9%	66.7%
Maryland	5.9%	62.1%
Massachusetts	7.2%	58.7%
Michigan	2.4%	69.7%
Minnesota	4.7%	63.3%
Mississippi	0.4%	78.1%
Missouri	2.0%	70.5%
Montana	1.2%	67.9%
Nebraska	1.3%	72.0%
Nevada	4.1%	66.5%
New Hampshire	0.6%	67.4%
New Jersey	7.6%	62.9%
New Mexico	1.3%	69.7%
New York	21.3%	48.1%
North Carolina	1.3%	70.9%
North Dakota	1.2%	73.2%
Ohio	2.8%	70.3%
Oklahoma	0.5%	73.9%
Oregon	4.7%	61.3%
Pennsylvania	5.7%	64.1%
Rhode Island	3.3%	70.7%
South Carolina	0.8%	74.0%
South Dakota	2.2%	70.9%
Tennessee	1.0%	73.5%
Texas	1.6%	69.3%
Utah	2.8%	63.9%
Vermont	2.0%	64.7%
Virginia	2.7%	66.8%
Washington	5.5%	61.8%
West Virginia	1.5%	72.4%
Wisconsin	2.7%	68.6%
Wyoming	1.7%	70.5%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values show the share of workers with disabilities commuting by public transit or driving alone. Higher values indicate greater use of that mode.

Table 31: Transportation (PWD) — Raw Levels by State (2023).

Looking inside each region the table above displays commute shares in all states, reinforcing how dominant automobile travel remains for workers with disabilities (PWD), while also revealing a small set of clear outliers. In most states, between 60 and 75 percent of PWD commute by driving alone. Highest shares appear in Mississippi (78.1%), and Alabama (77.6%). Several, mostly Midwest states, including North Dakota (73.2%), Indiana (72.7%), and Kentucky (72.6%) also exceed 72 percent. Low outliers include DC (27.4%) and New York (48.1%). These figures indicate that in much of the country, driving alone is not just common but very dominant. Public transit use, by contrast, remains below 3 percent in a majority of states. Alabama and Mississippi each report just 0.4 percent transit use among PWD. More average states are in the range of 1-3% such as Iowa (1.7%) and Michigan (2.4%) Even in many states with large populations and meaningful levels of urbanization, such as Texas (1.6%), and Florida (1.8%), transit plays only a marginal commuting role. Still, a small group of states diverge sharply with the District of Columbia (26.6%) and New York (21.3%) standing far above others. A secondary tier of transit states includes New Jersey (7.6%), Illinois (7.4%), Massachusetts (7.2%), Hawaii (7.0%), and Washington (5.5%).

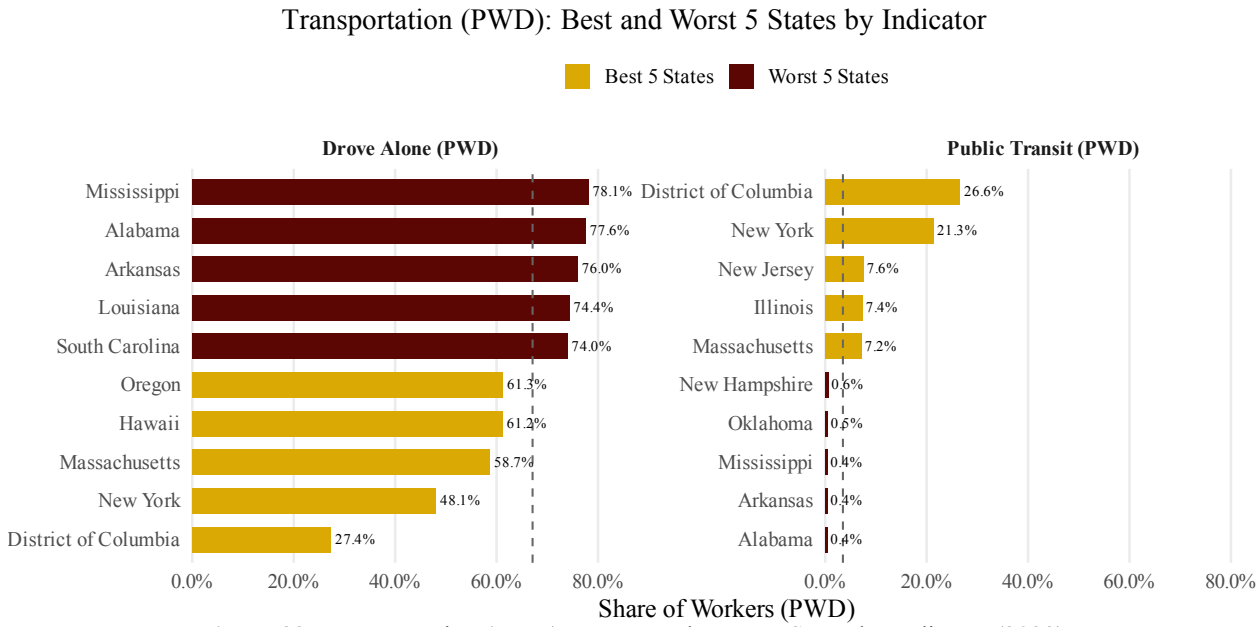


Figure 32: Transportation (PWD) — Best and Worst 5 States by Indicator (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Gold bars show the five best states and maroon bars show the five worst states within each indicator. For Public Transit, higher values are treated as better; for Drove Alone, lower values are treated as better. The dashed line shows the national average within each panel.

Focusing on just the states with more extreme outcomes, figure above emphasizes how sharply transportation mode use among workers with disabilities (PWD) diverges in specific geographic contexts. On the driving-alone dimension, the highest shares are concentrated in the South. Mississippi (78.1%), Alabama (77.6%), Arkansas (76.0%), Louisiana (74.4%), and South Carolina (74.0%) all report that roughly three in four PWD rely on driving alone for their commute. With the lowest rates of driving alone are The District of Columbia (27.4%) and New York (48.1%), followed by Massachusetts (58.7%), Hawaii (61.2%), and Oregon (61.3%).

Patterns for public transit use are even more polarized. The District of Columbia (26.6%) and New York (21.3%) again stand apart, with transit use among PWD far exceeding that of any other state. New Jersey (7.6%), Illinois (7.4%), and Massachusetts (7.2%) round out the top five a with rates still more than double

the average. On the lower extreme for transit reliance are Alabama, Arkansas, and Mississippi (each at 0.4%), all of which appear among the highest driving-alone states. Oklahoma (0.5%) and New Hampshire (0.6%) also appear in the bottom five for transit. On this dimension, the overlap across panels is meaningful, as states such as Mississippi, Alabama, and Arkansas have both very high driving-alone rates with near-zero transit use. Conversely, the District of Columbia and New York appear at the favorable extreme on both indicators, pairing low automobile dependence with high transit use. Together, these extremes give the impression that transportation for PWD is less about preference than the availability alternatives.

Public Transit Disparity, Ages 16+: States with Greatest and Least Gaps

State	PWD Public Transit (16+)	PWOD Public Transit (16+)	Disparity
District of Columbia	26.6%	22.2%	**4.4%**
Hawaii	7.0%	4.1%	**2.9%**
Minnesota	4.7%	1.9%	**2.8%**
Oregon	4.7%	2.5%	**2.2%**
Pennsylvania	5.7%	3.8%	**1.9%**
Alabama	0.4%	0.3%	**0.1%**
Arkansas	0.4%	0.3%	**0.1%**
New Hampshire	0.6%	0.6%	**0.0%**
New Jersey	7.6%	8.6%	**-1.0%**
New York	21.3%	22.5%	**-1.2%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are percentage-point differences (PWD − PWOD), displayed with a % sign. Gold indicates the five highest disparities (treated as better under this metric). Maroon indicates the five lowest disparities (treated as worse; negative values mean PWOD exceeds PWD).

Table 32: Public Transit Disparity (PWD − PWOD), Ages 16+: States with Greatest and Least Gaps (2023).

Public transit disparity reveals that differences between workers with disabilities (PWD) and those without disabilities (PWOD) are generally small, but vary across states and adds meaningful information to the larger image. Many states have rates of public transit commuting that are higher for PWD than PWOD. Chief among them is DC at a 4.4 percent gap, followed by Hawaii at 2.9 points, Minnesota (2.8), Oregon (2.2), and Pennsylvania (1.9). In these states, PWD rely on public transit at noticeably higher rates than PWOD, suggesting relatively strong accessibility and viable non-automobile commuting options.

At the lower end, differences show near parity or slightly lower rates of transit use among PWD compared to PWOD.New Hampshire (−0.8 points), New Jersey (−1.0), and New York (−1.1) show negative gaps, indicating marginally higher transit use among PWOD than PWD. Notably, New York combines high absolute transit use (21.3% for PWD) with a small negative disparity which may be related to persistent issues with reliably accessible transit systems. Overall, the spread from −1.2 to +4.4 percentage points is reasonably narrow.

4.3.2.4 City Level Transportation

ACS-5 year estimates unavailable at local level.

4.4 Tech Access

4.4.1 Scorecard ~ Tech Access Index Score

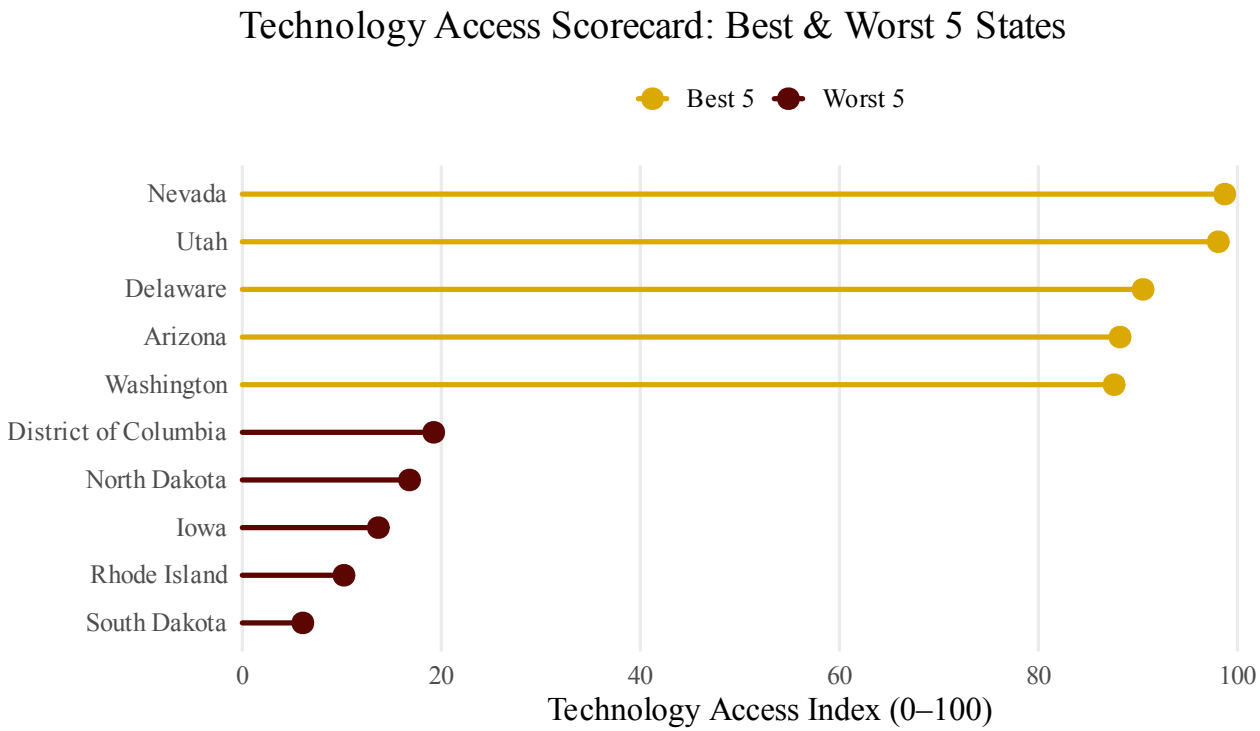


Figure 33: Technology Access Index: Best and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards.

Interpretation: Higher Technology Access Index scores indicate stronger access outcomes for people with disabilities. Gold marks the five highest-scoring states (best outcomes), while maroon marks the five lowest-scoring states (worst outcomes).

Last of our Community Participation scorecards is Technology Access. Figure above looks initially at top and bottom performers on the index (0–100 scale). In this sub-category, three indicators are synthesized to represent digital inclusion for PWD - access to the internet, access to a computer, and access to a smartphone. Top-performing states in this area are Nevada and Utah, with a score nearing the maximum of 100, followed closely by Delaware (90), Arizona (88), and Washington (87). Performing best on average across these indicators, PWD in these states have comparatively high levels of connectivity and device access. Notably, several of these states are located in the Mountain West and Southwest.

Outcomes for PWD are less favorable for the states represented by maroon dots. South Dakota is at the bottom with a score below 10, Rhode Island (10), Iowa (14), North Dakota (16.8), and the District of Columbia (19) round out the bottom five. Gaps between Nevada (near 100) and South Dakota (below 10) suggests that tech access is another domain in which there is significant cross-state variation for PWD, with some states performing far above or below, average. Given the increasingly important role of the digital world, uncovering these differences is imperative for understanding community participation overall for PWD.

Technology Access Scorecard —Full Ranking (All States)

Rank	State	Technology Access Score	Quartile
1	Nevada	**98.7**	Q4
2	Utah	**98.1**	Q4
3	Delaware	**90.5**	Q4
4	Arizona	**88.2**	Q4
5	Washington	**87.6**	Q4
6	Wyoming	**87.4**	Q4
7	Alaska	**86.8**	Q4
8	Florida	**78.8**	Q4
9	Idaho	**78.2**	Q4
10	New Jersey	**76.8**	Q4
11	Georgia	**72.2**	Q4
12	Colorado	**70.6**	Q4
13	Oregon	**70.0**	Q3
14	Maryland	**70.0**	Q3
15	Texas	**68.3**	Q3
16	California	**61.6**	Q3
17	North Carolina	**58.0**	Q3
18	New Mexico	**56.0**	Q3
19	Virginia	**55.2**	Q3
20	Michigan	**53.8**	Q3
21	Maine	**53.0**	Q3
22	New Hampshire	**52.5**	Q3
23	Vermont	**49.9**	Q3
24	South Carolina	**47.9**	Q3
25	Hawaii	**47.6**	Q3
26	Oklahoma	**47.4**	Q2
27	Wisconsin	**47.4**	Q2
28	Nebraska	**43.6**	Q2
29	Montana	**42.8**	Q2
30	Indiana	**41.1**	Q2
31	Illinois	**40.4**	Q2
32	Tennessee	**39.9**	Q2
33	Kentucky	**37.6**	Q2
34	Alabama	**35.5**	Q2
35	Kansas	**34.7**	Q2
36	Arkansas	**34.6**	Q2
37	Ohio	**32.1**	Q2
38	Massachusetts	**31.6**	Q2
39	Louisiana	**31.4**	Q1
40	Minnesota	**31.0**	Q1
41	West Virginia	**31.0**	Q1
42	Pennsylvania	**25.7**	Q1
43	Mississippi	**25.6**	Q1
44	Missouri	**24.4**	Q1
45	New York	**22.9**	Q1
46	Connecticut	**19.6**	Q1
47	District of Columbia	**19.2**	Q1
48	North Dakota	**16.8**	Q1
49	Iowa	**13.7**	Q1
50	Rhode Island	**10.2**	Q1
51	South Dakota	**6.1**	Q1

Source: ADA-PARC 2023 Scorecards.
Shading by quartiles: Q1 $\leq$ 31.5; Q2 $\leq$ 47.4; Q3 $\leq$ 70.0; Q4 > 70.0.
Interpretation: Higher scores indicate stronger outcomes for people with disabilities on the Technology Access Index. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

Table 33: Technology Access Score — Full State Ranking with Quartile Shading (2023).

Expanding from the extremes, full state rankings reveal a pronounced stratification in technology access outcomes for people with disabilities (PWD). High quartile (Q4, >70.0) states includs a mix of Mountain West, Mid-Atlantic, and Southern states. Florida (78.8), Idaho (78.2), and New Jersey (76.8) round out the upper tier. In the third quartile (Q3, 47.5–70.0) a broad middle group includes a diverse group of states including

California (61.6), North Carolina (58.0), Virginia (55.2), and Michigan (53.8), among others. These states post outcomes above the median, but fall short of the leading tier. ,

Lower quartile states are most commonly located in the Midwest and Northeast. Q2 (≤47.4) is led by Oklahoma (47.4), and at the bottom is Massachusetts (31.6). Some other states in the group are Illinois (40.4), Tennessee (39.9), Kentucky (37.6), and Alabama (35.5). In the lowest quartile (Q1, ≤31.5), Louisiana (31.4) just misses Q2, followed by Minnesota (31.0). Also in the bottom quartile are Pennsylvania (25.7), Mississippi (25.6), New York (22.9), Connecticut (19.6), the District of Columbia (19.2), and more. Interestingly, several large or urban areas including New York and the District of Columbia fall in the bottom tier, suggesting that some technology ecosystems do not necessarily translate into high access for PWD. Overall, the quartile distribution indicates that digital access for people with disabilities is complex, geographically unevenm and not reducible to simple urban–rural divide.

4.4.2 Geographic Outcomes ~ Tech Access

4.4.2.1 National Level Tech

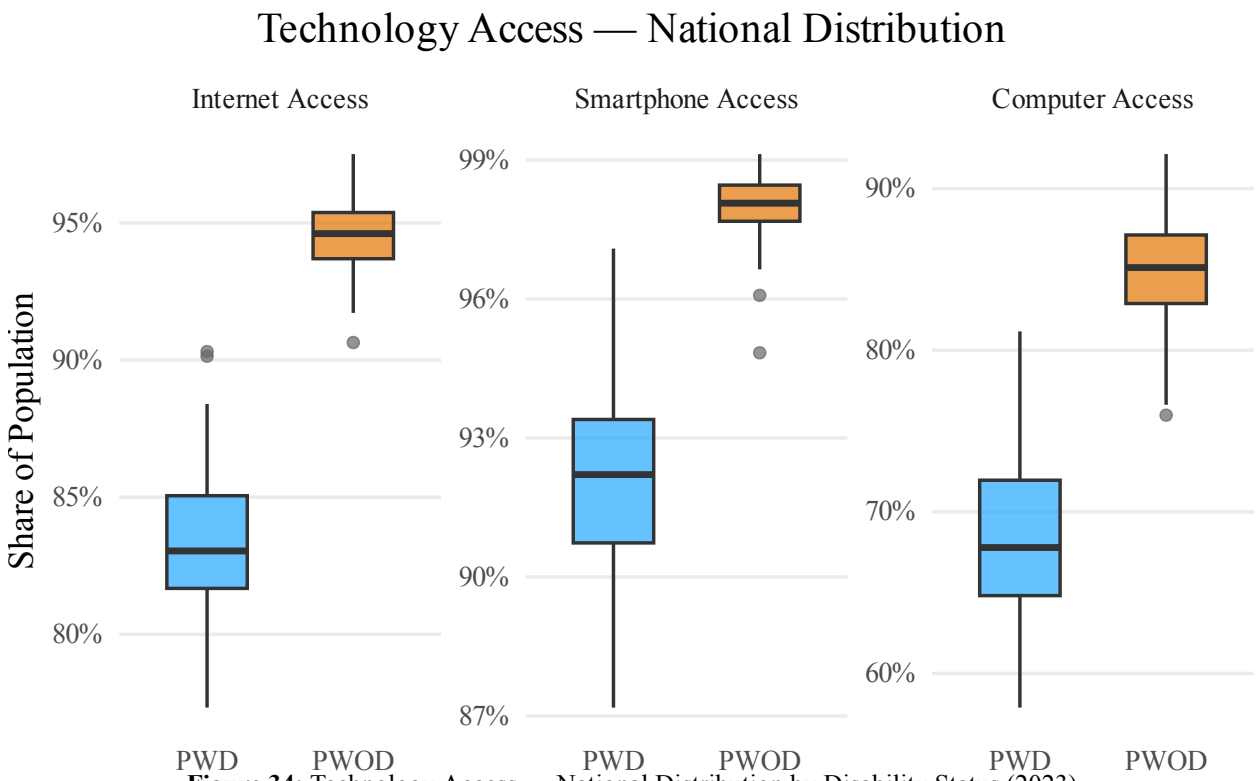


Figure 34: Technology Access — National Distribution by Disability Status (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Boxplots summarize the distribution across states; higher percentages indicate greater access.

Figure compares the national distributions of technology access for people with disabilities and people without disabilities across the three sub-domains of internet access, smartphone access, and computer access. Each panel displays the spread across states, allowing differences in both averages and variation to be evaluated. Across all three measures, the distribution for PWD is shifted downward relative to PWOD, indicating a consistent digital access gap. Most substantial is the separation in computer access, where the median share

for PWD is roughly in the upper 60s compared to the mid-80 percent range for PWOD. Variation across states for PWD is wider, spanning from the high 50s to the low 80s.

For internet access, overall levels are higher for both groups, but disparities remain consistent. Median level for PWD is just below 85%, while for PWOD it's 95%, although the average gap of roughly 10 percentage points is smaller than for computer access, it is still significant. Smartphone access percentages are the highest for both groups, with an average adoption of almost 99% for PWOD, and closer to 92% for PWD. Still, here there is consistently reduced access for people with disabilities relative to people without disabilities .As such, digital disparities affecting PWD are not confined to a single technology but pervasive across the measures within this scorecard.

4.4.2.2 Regional Level Tech

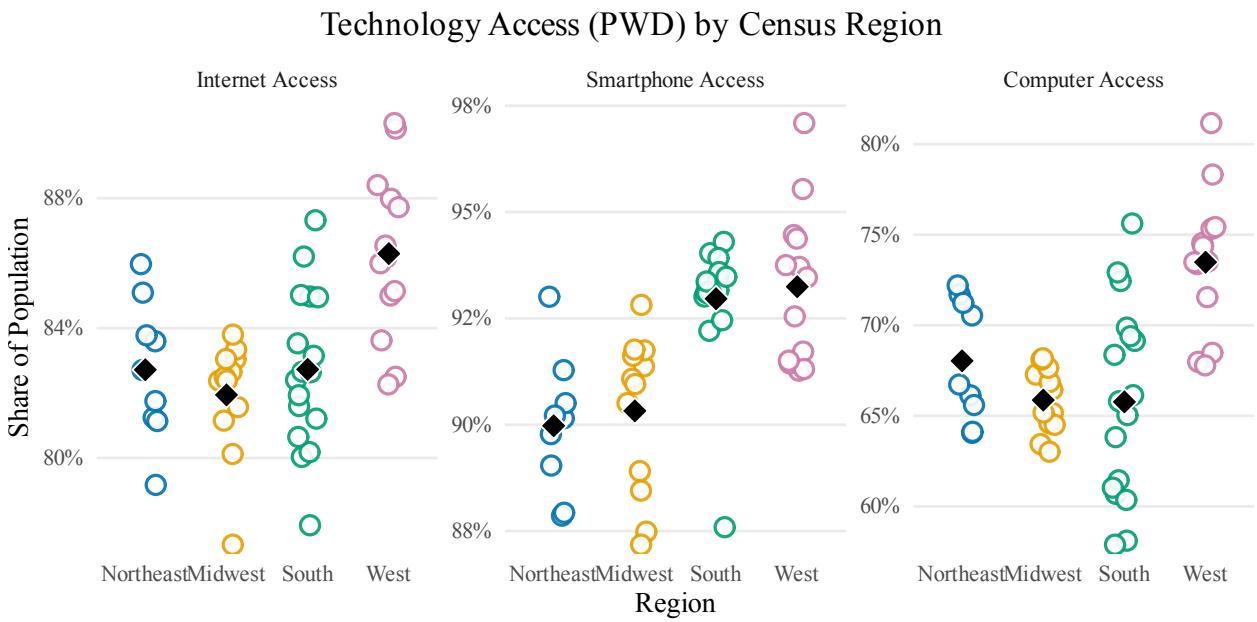


Figure 35: Technology Access (PWD) by Census Region — State Distributions (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state and the black diamond indicates the regional mean. Higher values indicate greater access; comparing regions highlights how state-level distributions cluster and vary across the country.

Regional distributions reinforce that technology access for people with disabilities (PWD) varies meaningfully across census regions, with the clearest differences emerging in computer ownership. In the West, values range from the high 60s to low 80s, with a regional mean around 74%. Southern states show the widest dispersion, spanning from high-50s to mid-70s, with a regional mean around 65%. In the Midwest, the regional mean is similar and values fall within a tight band at 65% plus or minus a few percentage points. Northeast rises slightly higher, with a regional mean around 68%, a small grouping of states at 65% and another exceeding 70%. For internet access, levels are higher overall with less range. Regional means range from 82% in the Midwest to 86% in the West, both the South and Northeast are around 83%. Within-region variation is fairly uniform, with values not exceeding or falling from the mean by more than five percent.

Smartphone access is again higher and even compressed within and across regions. Regional means generally fall between about 90 and 95 percent, with the West and South being around 93% and the Midwest and Northeast falling at 90-91%. . Again, spread is narrower than for computers, and most states, regardless of

region, are within the high 80s to mid 90s range. Overall, the regional gradient is consistent with the West tends to lead, the South trails and shows greater range, and the Midwest and Northeast occupy bottom or middle positions depending on the indicator.

4.4.2.3 State Level Tech

Tech Access —Raw Levels (PWD, All States)

State	Internet Access (PWD)	Smartphone Access (PWD)	Computer Access (PWD)
Alabama	81.6%	93.3%	60.7%
Alaska	88.0%	93.7%	73.5%
Arizona	86.5%	94.5%	74.5%
Arkansas	80.0%	94.0%	61.4%
California	85.0%	91.3%	71.5%
Colorado	85.1%	91.7%	75.3%
Connecticut	81.2%	87.9%	66.1%
Delaware	87.3%	94.3%	75.6%
District of Columbia	81.2%	87.6%	66.1%
Florida	86.2%	93.2%	72.4%
Georgia	85.0%	93.9%	69.9%
Hawaii	83.6%	91.4%	68.0%
Idaho	86.2%	92.5%	74.4%
Illinois	83.0%	91.4%	66.5%
Indiana	83.3%	91.8%	65.2%
Iowa	80.1%	88.5%	64.6%
Kansas	82.4%	90.5%	67.3%
Kentucky	82.4%	93.0%	61.0%
Louisiana	80.2%	93.4%	60.4%
Maine	83.6%	91.3%	70.5%
Maryland	85.0%	92.2%	72.9%
Massachusetts	82.7%	89.8%	66.7%
Michigan	83.8%	92.8%	66.9%
Minnesota	82.7%	88.9%	67.7%
Mississippi	77.9%	93.2%	58.1%
Missouri	81.2%	91.1%	63.4%
Montana	82.5%	91.3%	68.5%
Nebraska	82.5%	91.6%	68.1%
Nevada	90.1%	97.1%	78.3%
New Hampshire	85.1%	89.0%	71.7%
New Jersey	86.0%	93.0%	72.2%
New Mexico	82.3%	94.4%	67.8%
New York	81.8%	90.2%	64.1%
North Carolina	83.5%	93.1%	68.4%
North Dakota	81.6%	87.5%	64.5%
Ohio	82.4%	91.0%	65.2%
Oklahoma	82.7%	93.1%	65.8%
Oregon	86.0%	91.5%	73.4%
Pennsylvania	81.1%	90.5%	65.6%
Rhode Island	79.2%	87.9%	64.1%
South Carolina	82.6%	93.6%	65.0%
South Dakota	77.3%	87.2%	63.0%
Tennessee	81.9%	93.1%	63.8%
Texas	85.0%	93.5%	69.1%
Utah	90.3%	95.5%	81.2%
Vermont	83.8%	90.2%	71.2%
Virginia	83.2%	92.5%	69.4%
Washington	87.7%	93.5%	75.4%
West Virginia	80.7%	93.4%	57.9%
Wisconsin	83.0%	91.8%	68.2%
Wyoming	88.4%	93.8%	73.5%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are the share of people with disabilities with access to each technology. Higher values indicate greater access.

Table 34: Tech Access — Raw Levels (PWD) by State (2023)

Looking deeper, the table above reports access rates for all states across the three core technologies among people with disabilities. Internet access, smartphone access, and computer access are generally high nationwide, but some states perform consistently better than others. Smartphone access is the most universal. In nearly every state, more than 90 percent of PWD report access, with several states exceeding 95 percent in Nevada (97.1%), and Utah (95.5%). Even the lowest-performing states, such as South Dakota (87.2%) and North Dakota (87.5%), remain above the mid-80 percent range. . Internet access is just slightly lower and with a bit more variation. Most states group between 81 and 86 percent, but the range extends from 77.3 percent in South Dakota and to above 90 percent in Utah (90.3%). Thus the range across states lays at a 2–13 point difference. Perhaps most meaningful is the variation in computer access, which ranges from 57.9 percent in West Virginia and 58.1 percent in Mississippi to 81.2 percent in Utah, a spread of more than 23 percentage points. Overall, the raw data indicate that while smartphones are nearly ubiquitous among PWD, access to personal computing devices remains lower and uneven geographically.

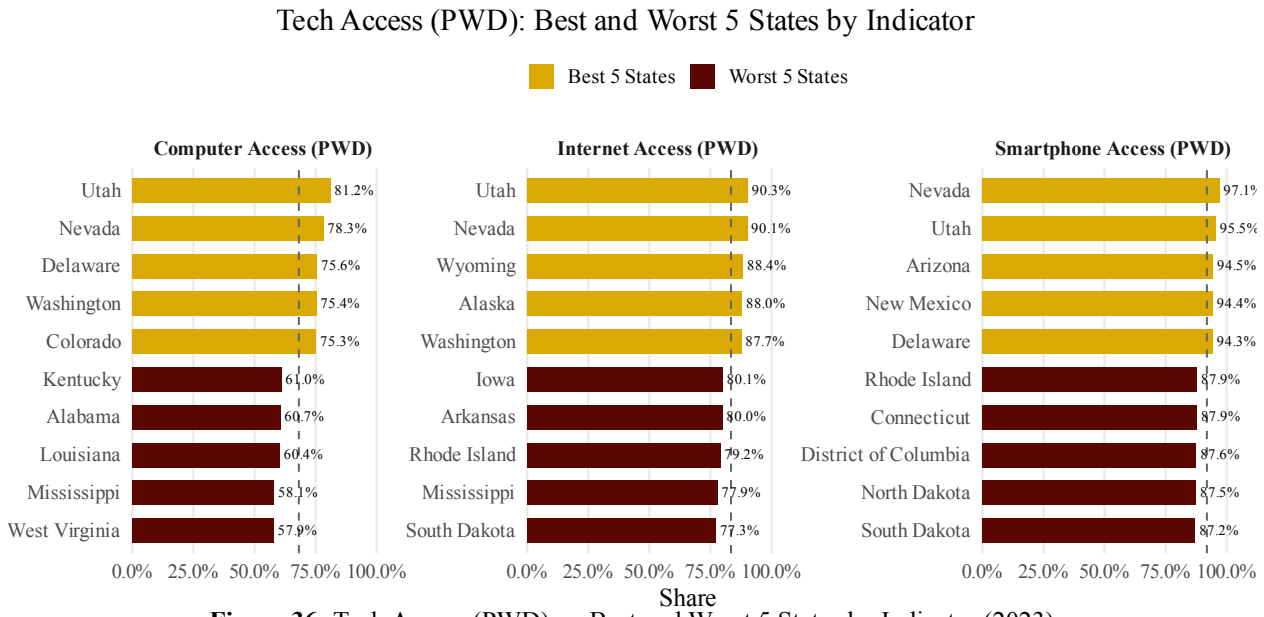


Figure 36: Tech Access (PWD) — Best and Worst 5 States by Indicator (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Gold bars indicate the five states with the highest access rates (better outcomes) and maroon bars indicate the five states with the lowest access rates (worse outcomes). The dashed line shows the national average within each panel.

Viewing top and bottom performing states more closely, figure above isolates the five states with the most extreme outcomes on each dimension of tech access among PWD. For computer access, the leading states are anchored at the top by Utah (81.2%) and Nevada (78.3%), followed by Washington (75.4%), Delaware (75.6%), and Arizona (74.5%). At the lower end are West Virginia (57.9%), Mississippi (58.1%), Louisiana (60.4%), Alabama (60.7%), and Kentucky (61.0%) each around or below 60%. For internet access, top-performing states include Utah (90.3%), Nevada (90.1%), Wyoming (88.4%), Alaska (88.0%), and Washington (87.7%), each approaching or exceeding 90 percent. In contrast, Mississippi (77.9%), South Dakota (77.3%), Arkansas (80.0%), and Louisiana (80.2%) fall roughly 8–13 points lower. Smartphone access is again both highest and more tightly distributed. Nevada (97.1%) and Utah (95.5%) again lead, with Arizona (94.5%) and Delaware (94.3%) close behind. Low states of South Dakota (87.2%), North Dakota (87.5%), Connecticut (87.9%), DC (87.6%), and Rhode Island (87.9%), remain in the high-80 percent range. Standing out from the figure is that , Utah and Nevada appear lead the nation in all three indicators, while Delaware- and Washington appear in two of the three top-performing groups. On the opposing end, Mississippi, South

Dakota, and Rhode Island appear in two of the three bottom-performing groups.

Internet Access Disparity: States with Greatest and Least Gaps

State	PWD Internet Access	PWOD Internet Access	Disparity
Alaska	88.0%	93.9%	**-5.9%**
Nevada	90.1%	96.4%	**-6.2%**
Wyoming	88.4%	95.3%	**-6.9%**
Utah	90.3%	97.5%	**-7.2%**
Arizona	86.5%	94.5%	**-8.0%**
Minnesota	82.7%	96.4%	**-13.8%**
Connecticut	81.2%	95.3%	**-14.1%**
Iowa	80.1%	94.5%	**-14.3%**
Rhode Island	79.2%	93.9%	**-14.7%**
South Dakota	77.3%	93.2%	**-15.9%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are percentage-point differences (PWD − PWOD), displayed with a % sign. Gold highlights the five states with the most positive gaps (better parity or PWD exceeding PWOD). Maroon highlights the five states with the most negative gaps (largest disadvantage for PWD).

Table 35: Internet Access Disparity (PWD − PWOD): States with Greatest and Least Gaps (2023).

Applying a an equity lens to internet access, table above quantifies the largest and smallest gaps in technology access between people with disabilities and people without disabilities, expressed as percentage point difference. Negative values indicate lower access among PWD, which in this case is ubiquitous, so even the states with the least disparity have some gap in access. In states with the smallest gaps, indicating relatively closer parity, are Alaska(−5.9 percentage points), Nevada (−6.5), Wyoming (−6.9), Utah (−7.0), and Arizona (−7.0). In these states, PWD internet access ranges from 88.0 to 90.3 percent, while PWOD access falls between roughly 94.7 and 96.4 percent. States characterized by deeper inequality are South Dakota (−15.9), Rhode Island (−14.7), Ohio (−14.3), Connecticut (−14.1), and Minnesota (−13.8). A full range from −5.9 to −15.9 percentage points indicates that while internet access is relatively high nationwide, parity has not been achieved in any state. Also noteworthy is the fact that Utah and Nevada perform well in both absolute and equity terms, while the opposite appears to be true for South Dakota and Rhode Island.

4.4.2.4 City Level Tech

ACS-5 year estimates unavailable at local level.

4.5 Summary of Community Participation Descriptive Findings

Key Findings for Community Participation

- **Participation gaps show up in every sub-category.** Across education, transportation, technology, and insurance coverage, people with disabilities consistently experience lower access or engagement than people without disabilities, though the size of the gap varies by domain and geography.
- **Education disparities are universal both in 'low-attainment' and 'high-attainment'.** In every state, PWD are less likely to have a bachelor's degree or higher and more likely to lack a high school credential, creating a persistent attainment gap rather than a single shortfall.

- **Transportation participation is constrained across disability status and often narrow in practice.** Most states show heavy reliance on driving alone among people with disabilities and people without disabilities. Public transit use remains limited outside a relatively small set of highly urbanized places.
- **Technology access reveals a modest digital divide.** Smartphone ownership among PWD is relatively high in many states, but more consequential gaps persist in internet and computer access.
- **Insurance coverage reflects more parity across disability status than many ADA-PARC domains.** PWD coverage rates are strong (often reflecting disability-linked eligibility pathways), though uneven coverage remains across states and in some states there is still a disability-related coverage gap.
- **No single indicator captures community participation.** State performance varies across sub-categories, meaning a state can look strong in one area (e.g., insurance) while still lagging in another (e.g., education or digital access).
- **Community participation shows domain-specific geographic paters rather than one 'overall' map:**. Education access tends to rank higher in parts of the **Northeast and West** and lower across much of the **South**. transportation access peaks in transit-rich places like **DC and New York** while several Southern states fall to the bottom, insurance access has its own leader/laggard set (e.g., **Delaware/Midwest leaders**) and technology access shows meaningful spread.

Community participation outcomes for people with disabilities show a consistent pattern across states, as access is uneven across multiple domains rather than concentrated in a single barrier. In the education measures, disadvantage is double-sided, as PWD are less likely to reach bachelor's level attainment and more likely to fall short of a high school credential. A noteworthy result, as a persistent gap at both ends of the distribution may influence job opportunities, earnings, and overall economic prospects.

Transportation measures highlight a different kind of constraint, one of limited choice. In most places, the dominant pattern is heavy reliance on driving alone and comparatively low public transit use among PWD and PWOD, pointing to participation that depends strongly on private vehicle access and the accessibility of local built environments. In highly urban contexts, public transit participation is strong, which may especially beneficial for PWD who are not able to drive alone Technology access shows a distint pattern. Smartphone ownership is relatively widespread among PWD in many states, but meaningful gaps persist for internet and computer access. Such forms of tech access are important for their connectivity to schooling, employment opportunities, telehealth, and accessing public services. Hence, device access is a less than satisfactory indicator of digital inclusion, as the more participation critical tools remain systematically less available to PWD. Insurance coverage is one of the few participation indicators where PWD seem to be doing well nationally on the headline measure. However, coverage gaps remain in some states, particularly within the working age population. For adults ages 65 and older with disabilities, health insurance coverage tends to be high and relatively stable across states.

Regional patterning in community participation is best read as subdomain-by-subdomain, with each component reflecting a different kind of place context. Education access shows the clearest divergence, with multiple high-scoring states appear in the Northeast and West, while the lowest education access outcomes concentrate in the South. Transportation access follows a distinct geography tied to infrastructure, as DC and New York sit at the favorable extreme, with several Southern states repeatedly falling at the lower end. Insurance access does not mirror either of those maps, as its leaders include Delaware and several Midwestern states, while a different set (including Texas, Nevada, Florida, Alaska, and Georgia) are at the bottom. Technology access adds a unique pattern by showing both regional tendencies and standout states. States in the West posts high rates, with Utah and Nevada consistently appearing at or near the top across internet, smartphone, and computer access for people with disabilities.

A main descriptive takeaway is that community participation for people with disabilities is complex and variable across states, rather than sorting neatly into consistent tiers across education, transportation, technology, and insurance. Though viewed from an equity lens, disability status gaps are striking, especially in the education and technology domains. PWD face lower access or engagement to education and technology in ways that are likely to compound disadvantage. As such, participation constraints are best understood in connection rather than isolated deficits that appear in only one domain.

5 Work & Economic

Work and economic outcomes are central to understanding the lived experience of disability in the United States. Employment access, labor force attachment, income adequacy, and exposure to poverty are more informative when analyzed together. Forming an interconnected system of material security, independence, and long term well-being, this final section is fundamental to assessing the extent to which our systems protects the dignity of people with disabilities.

To keep the section readable, results are organized into two sub-domains, each with a scorecard that mirrors the ADA-PARC dashboard structure - **Employment** and **Wealth & Poverty**. Each includes both a scorecard summary (PWD-only comparisons across states) and indicator level figures/tables that show the underlying levels and PWD–PWOD gaps

Employment

Designed to summarize labor market inclusion for working age people with disabilities using two core measures:

- **Employment rate** among people with disabilities age 18-64.
- Percentage of people with disabilities age 18-64 who are **not in the labor force.**

Wealth & Poverty

Designed to summarize economic security for people with disabilities using two core measures:

- Percentage of people with disabilities living below the federal **poverty** line.
- **Median individual income** for people with disabilities age 16+

Across states, these outcomes do tend to move together on average, though with many nuances and exceptions. Still, places with stronger labor force attachment among people with disabilities generally show stronger income benchmarks and lower poverty, while places with higher labor force exclusion frequently show a less favorable profile. The analysis which follows identifies recurring patterns and deviations across age groups, geographies, and disability status. Geography and equity are used as lenses rather than conclusions. National results establish baseline gaps between people with and without disabilities, while regional and state comparisons show both how those gaps widen or narrow across the country, and how disability populations compare to each other. City level results (where available) illustrate how wide the range of outcomes in economic security can be even in large U.S. cities. Throughout, the goal is to document how economic outcomes for people with disabilities vary in multiple ways, without assigning causes or prescribing solutions.

Correlations Among Work & Economic Variables (PWD)

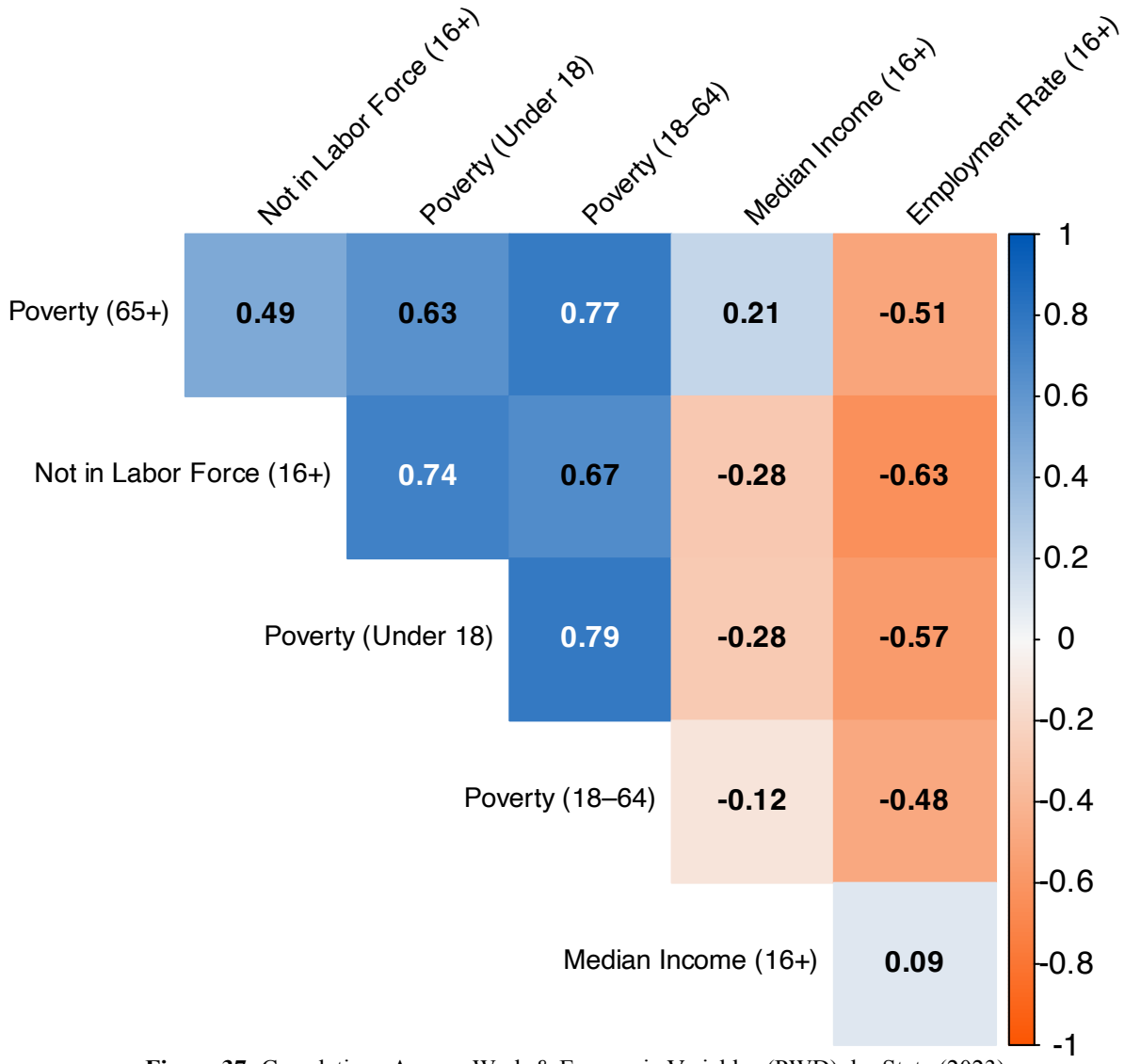


Figure 37: Correlations Among Work & Economic Variables (PWD), by State (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Colors show Pearson correlations across states (blue = positive, orange = negative, white ≈ 0). Cells are computed using pairwise-complete observations to avoid dropping variables with missing data.

As with our ADA-PARC categories of Community Living and Community Participation, we begin the Work Economic section with a macro-level picture of variable associations. Figure above is a correlation matrix summarizing how work and economic outcomes for people with disabilities relate across states, revealing an interconnected map of poverty, the labor force, and incomes. Poverty levels are strongly aligned across age groups. Child poverty and working-age poverty move robustly together ($r \approx 0.79$), and both are closely associated with poverty among older adults ($r \approx 0.63$–0.77). Such magnitudes indicate that states with elevated poverty among one segment of the disabled population tend to experience elevated poverty across others, pointing to foundational conditions rather than age-restricted effects.

Labor force participation also displays some strong and interesting relationships, lack of participation is correlated with child poverty ($r \approx 0.74$) and working-age poverty ($r \approx 0.67$) for PWD. Employment rates show the clearest inverse relationships, with higher employment among PWD being associated with lower poverty ($r \approx -0.48$ to -0.63) and lower labor force detachment ($r \approx -0.51$). States in which median income is higher PWD age 16+, tend to have less child poverty ($r \approx -0.28$) and working age poverty ($r \approx -0.12$), while showing a slight association with elderly poverty ($r \approx 0.21$), these relationships suggest that income differences are uneven within states among PWD, and that fixed benefits for the elderly disabled population may be insufficiently protective against poverty. As a whole, the matrix shows that work and economic indicators do not operate independently, they tend to move together in a manner which poverty, employment, and labor force participation are connected by state labor markets and policy environments.

5.1 Employment

5.1.1 Scorecard ~ Employment Index

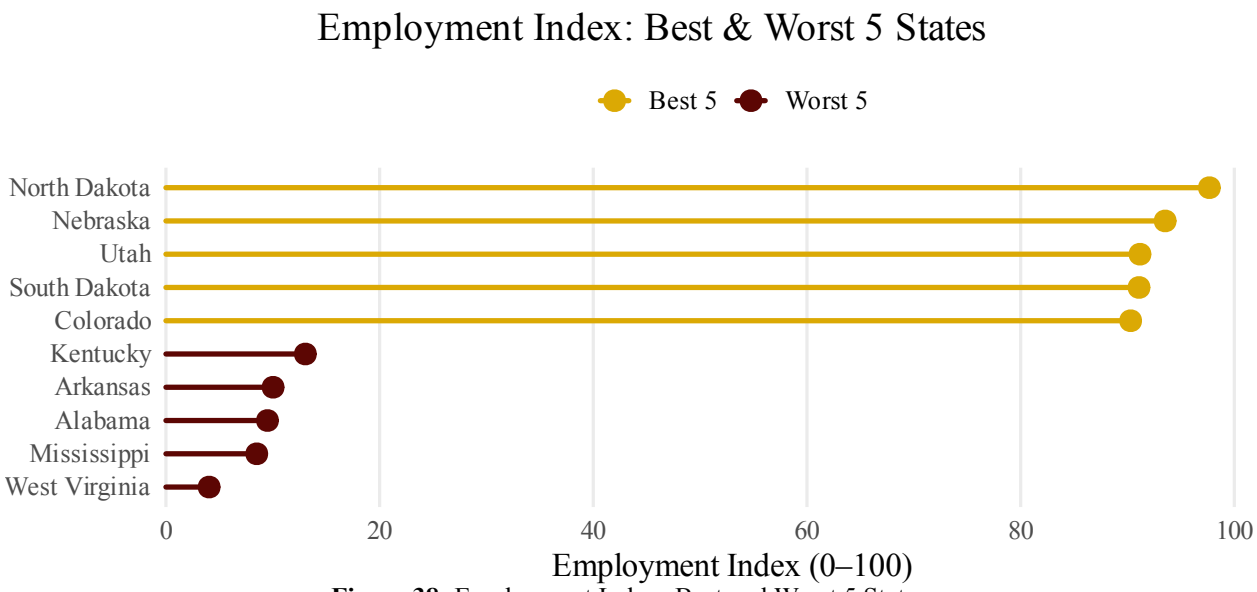


Figure 38: Employment Index: Best and Worst 5 States

Source: ADA-PARC 2023 Scorecards.

Interpretation: Gold bars indicate the five highest Employment Index scores (best performers) and maroon bars indicate the five lowest scores (worst performers). Scores are on a 0–100 scale, where higher values indicate stronger employment outcomes for people with disabilities.

First of two under the Work & Economic umbrella, the Employment Scorecard summarizes how states perform on labor market inclusion for working-age people with disabilities (PWD). The 0–100 point index combines two important measures - the employment rate among working-age PWD and the share of working-age PWD not participating in the labor force. Higher scores indicate better conditions for both metrics, with higher rates of employment and lower rates of labor force exclusion relative to other states. A pronounced divergence appears when looking at leaders and laggards on the Employment Index scale, as highest performers are concentrated in the Upper Plains and Mountain West, while the lowest in the South and Appalachia

North Dakota nears the upper limit of 100, followed closely by Nebraska and Utah in the mid-to-low 90s, with South Dakota and Colorado rounding out the top five. Such high scores indicate that these states are likely performing well on both sides of the index, with less disconnection from the labor market and higher rates of obtaining employment among PWD. In terms of lower performing states, West Virginia falls into the single digit range on the index, with Mississippi, Alabama, and Arkansas grouped around a socre of 10, and Kentucky at 15. In these states, lower employment rates coincide with elevated levels of labor force non-participation among working-age PWD. Proximity of top and bottom performers to the scale limits means that labor market inclusion for PWD varies significantly and warrants deeper investigation.

See next page for full Employment & Income Scorecard Rankings.

Employment Scorecard: Full State Ranking

Rank	State	Employment Score	Quartile
1	North Dakota	**97.7**	Q4
2	Nebraska	**93.5**	Q4
3	Utah	**91.2**	Q4
4	South Dakota	**91.1**	Q4
5	Colorado	**90.3**	Q4
6	Minnesota	**87.1**	Q4
7	Wyoming	**85.5**	Q4
8	Iowa	**82.9**	Q4
9	Kansas	**81.7**	Q4
10	Idaho	**79.2**	Q4
11	Montana	**77.0**	Q4
12	Wisconsin	**71.2**	Q4
13	Texas	**71.0**	Q3
14	New Hampshire	**68.1**	Q3
15	Alaska	**67.4**	Q3
16	Maryland	**66.4**	Q3
17	Washington	**62.6**	Q3
18	Nevada	**60.9**	Q3
19	District of Columbia	**59.7**	Q3
20	Arizona	**59.5**	Q3
21	Virginia	**58.7**	Q3
22	Connecticut	**57.7**	Q3
23	Oregon	**56.7**	Q3
24	New Jersey	**55.8**	Q3
25	Illinois	**54.9**	Q3
26	Delaware	**54.5**	Q2
27	Hawaii	**51.6**	Q2
28	Massachusetts	**48.1**	Q2
29	Oklahoma	**47.1**	Q2
30	Indiana	**46.3**	Q2
31	Rhode Island	**45.9**	Q2
32	California	**43.1**	Q2
33	Florida	**41.9**	Q2
34	Ohio	**39.1**	Q2
35	Pennsylvania	**38.9**	Q2
36	Vermont	**37.1**	Q2
37	Missouri	**35.1**	Q2
38	Georgia	**32.0**	Q2
39	Maine	**25.8**	Q1
40	North Carolina	**24.9**	Q1
41	New Mexico	**24.8**	Q1
42	Michigan	**23.3**	Q1
43	New York	**23.2**	Q1
44	South Carolina	**21.7**	Q1
45	Tennessee	**21.5**	Q1
46	Louisiana	**16.2**	Q1
47	Kentucky	**13.1**	Q1
48	Arkansas	**10.0**	Q1
49	Alabama	**9.5**	Q1
50	Mississippi	**8.5**	Q1
51	West Virginia	**4.0**	Q1

Source: ADA-PARC 2023 Scorecards.
Shading by quartiles Q1 $\leq$ 28.9; Q2 $\leq$ 54.5; Q3 $\leq$ 69.6; Q4 > 69.6.
Interpretation: Higher scores indicate stronger outcomes for people with disabilities on the Employment Index. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

Table 36: Employment Scorecard — Full Ranking with Quartile Shading (2023).

Full rankings reveal tiered employment landscape for PWD. Scores span from 97.7 in North Dakota to 4.0 in West Virginia, a spread of more than 90 points on the 0–100 scale. Employment in the top quartile (Q4, > 69.6) is dominated by states in the Upper Plains and Mountain West. In addition to the top five states discussed previously, Minnesota (87.1), Wyoming (85.5), and Kansas (81.7) and represented among others.

In the third quartile, (Q3, 54.6–69.6), states perform modestly above average. Bookended by Texas (71), and Illinois (54.9), several coastal and higher-income states appear here, indicating that broader economic strength does not automatically translate into the best disability employment outcomes.

The second quartile (Q2, 28.9–54.5) reflects state outcomes slightly below average with mixed or uneven performance. Delaware (54.5) and Hawaii (51.6), sit near the upper boundary, while Vermont (37.1), Missouri (35.1), and Georgia (32), group near the lower end. More large and economically diverse states fall here as well such as California, Florida, and Pennsylvania. In the bottom quartile, (Q1, $\leq$ 28.9) which is regionally concentrated in the South and Appalachia, Maine (25.8) marks the upper edge, followed closely by North Carolina, New Mexico, Michigan, and New York. Jointly, the quartiles are complex and diverse, though three broad dynamics are apparent - many of the Mountain West states are top performers, large complex state economies are mostly below average, and the lowest performing states are largely in the South.

5.1.2 Geographic Outcomes ~ Employment

We turn now to raw ACS-5 employment measures at different levels of geography.

5.1.2.1 National Level Employment

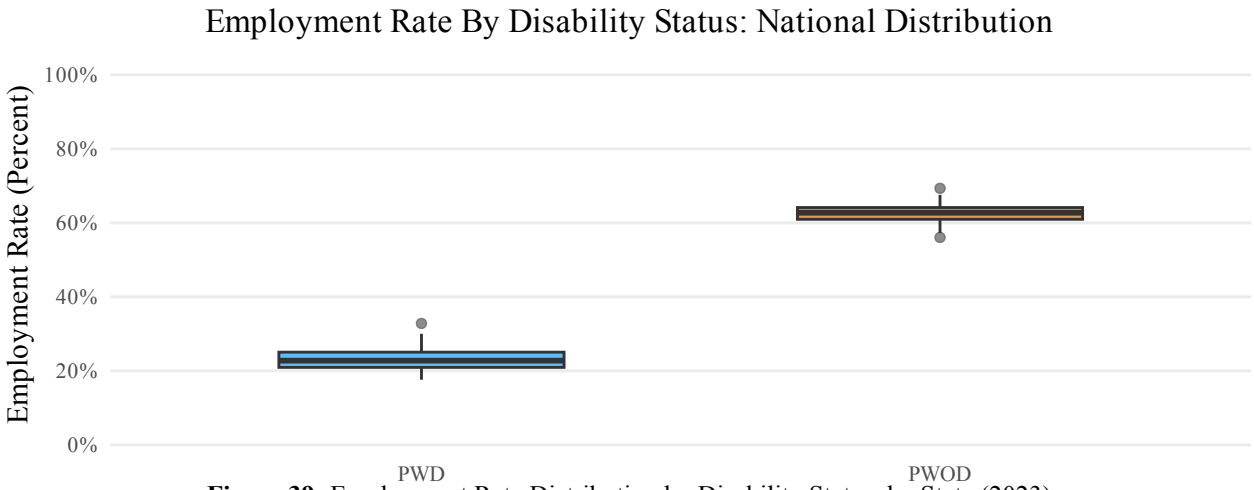


Figure 39: Employment Rate Distribution by Disability Status, by State (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each box summarizes the across-state distribution of the employment rate for the civilian noninstitutionalized population ages 16+. Rates are treated as percentages (0–100) and displayed on a 0–100 scale.

Beyond the Employment scorecard, an investigation of the raw measures of PWD employment are informative. Starting with figure above, the comparison boxplot shows a stark and consistent employment divide between working-age adults with disabilities and those without disabilities across states. Median employment rate for U.S. states among PWD is 22 percent, with a very tight spread, most falling between 20 and 25 percent. By contrast, PWOD employment centers at 62 percent, with most states ranging from 60 to 65 percent. Hence, the typical gap between the two groups is therefore on the order of 40 percentage points. A relatively narrow distribution of outcomes for PWD indicates that low employment among is not confined to a subset of states. Even higher-performing states rarely exceed the high-20 percent range, while the lowest fall closer to the upper teens. In contrast, PWOD employment rates are uniformly high, and even the lowest states for PWOD remain above 55 percent. In this domain, the dominant structural divide reflected by these

outcomes is disability status itself which suggest that improving PWD employment outcomes may require fundamental changes in labor market conditions which allow for the greater inclusion of individuals with disabilities.

5.1.2.2 Regional Level Employment

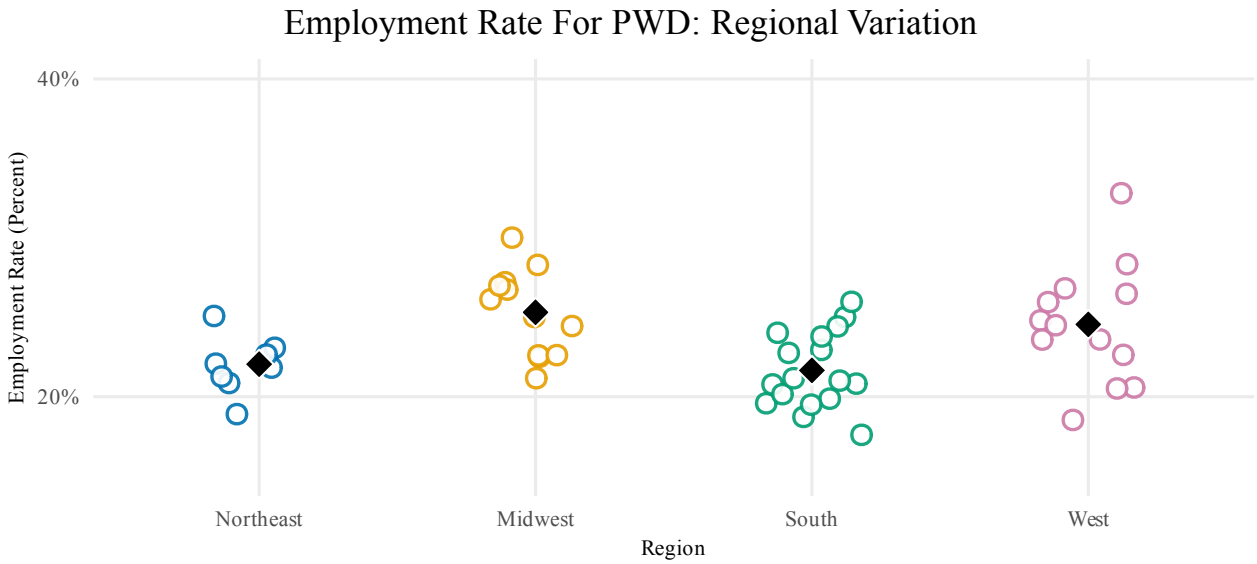


Figure 40: Employment Rate (PWD, Ages 16+) by Region, by State (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state and the black diamond indicates the regional mean. Rates are treated as percentages (0–100) and displayed on a percent scale.

As expected from the national distribution, employment rates for working-age people with disabilities group tightly across regions, but some differences remain visible. Regional means (black diamonds) range from 21-22 percent in the South and Northeast to 25-26 percent in the West and Midwest. Even at their highest, regional averages remain below 30 percent, while only a few high-performing states reach above that mark. Midwest states stand out for combining one of the highest regional means with several states reaching into the upper-20 percent range. With a slightly lower regional mean and higher variation, the West is home to the highest performing state. Southern states fall exclusively between the range of 17% to 25%. Also grouped very tightly together is the Northeast, whose states fall within a few percentage points of their 22% regional average. A key pattern across the figure is substantial overlap among regions with a lack of substantial variation within or across regions compared with many other measures examined previously.

5.1.2.3 State Level Employment

Employment Rates By Disability Status: All States

State	PWD Employment Rate	PWOD Employment Rate
Alabama	19.6%	59.6%
Alaska	26.8%	63.2%
Arizona	22.6%	59.7%
Arkansas	19.9%	60.8%
California	20.6%	61.6%
Colorado	28.3%	66.1%
Connecticut	22.7%	62.9%
Delaware	22.9%	60.4%
District of Columbia	25.0%	69.3%
Florida	18.7%	58.9%
Georgia	22.8%	63.1%
Hawaii	18.5%	60.0%
Idaho	26.5%	62.6%
Illinois	22.5%	63.1%
Indiana	24.4%	63.8%
Iowa	26.1%	65.2%
Kansas	27.2%	64.8%
Kentucky	20.8%	61.7%
Louisiana	20.8%	59.4%
Maine	20.9%	61.5%
Maryland	24.0%	64.5%
Massachusetts	22.1%	64.6%
Michigan	21.2%	60.6%
Minnesota	26.7%	66.6%
Mississippi	19.5%	58.9%
Missouri	22.6%	63.4%
Montana	24.8%	61.6%
Nebraska	28.3%	67.2%
Nevada	23.6%	60.9%
New Hampshire	25.1%	64.3%
New Jersey	21.3%	62.8%
New Mexico	20.5%	57.3%
New York	18.9%	61.0%
North Carolina	21.0%	61.9%
North Dakota	30.0%	67.6%
Ohio	22.6%	62.7%
Oklahoma	24.4%	62.1%
Oregon	23.6%	61.8%
Pennsylvania	21.8%	61.7%
Rhode Island	23.1%	63.0%
South Carolina	20.2%	60.0%
South Dakota	27.0%	65.7%
Tennessee	21.2%	62.5%
Texas	26.0%	63.9%
Utah	32.8%	67.2%
Vermont	22.6%	62.3%
Virginia	23.8%	63.7%
Washington	24.5%	63.5%
West Virginia	17.6%	56.0%
Wisconsin	25.0%	64.6%
Wyoming	26.0%	63.9%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are employment rates among the civilian noninstitutionalized population ages 16+.

Table 37: Employment Rate: PWD vs PWOD im all states.

Past broader regional analysis, the table above examines the breadth and specifics of state employment out-

comes for people with disabilities. State PWD employment rates range from 17.6 percent in West Virginia to 32.8 percent in Utah, a spread of more than 15 percentage points. Most states group in the 20–26 percent range, indicating that while variation exists, the national baseline for disability employment remains relatively low. Clear geographic patterns are visible. Higher PWD employment rates are mostly in the Upper Midwest and Mountain West, including the Dakotas, Colorado (28.3%), and Minnesota (26.7%). Conversely, several states in the Deep South have the lowest rates including West Virginia (17.6%), and Mississippi (19.5%), reflecting more constrained employment environments for working-age PWD. While outcomes are more tightly spread than some of our previous metrics, the differences remain meaningful, as an 18 versus 30 percent rate reflects a nearly two-thirds increase in employment likelihood for working-age PWD. Notably, the average gap between PWD and PWOD employment across states is 35 percentage points, reinforcing that PWD employment levels exist within a significant divide.

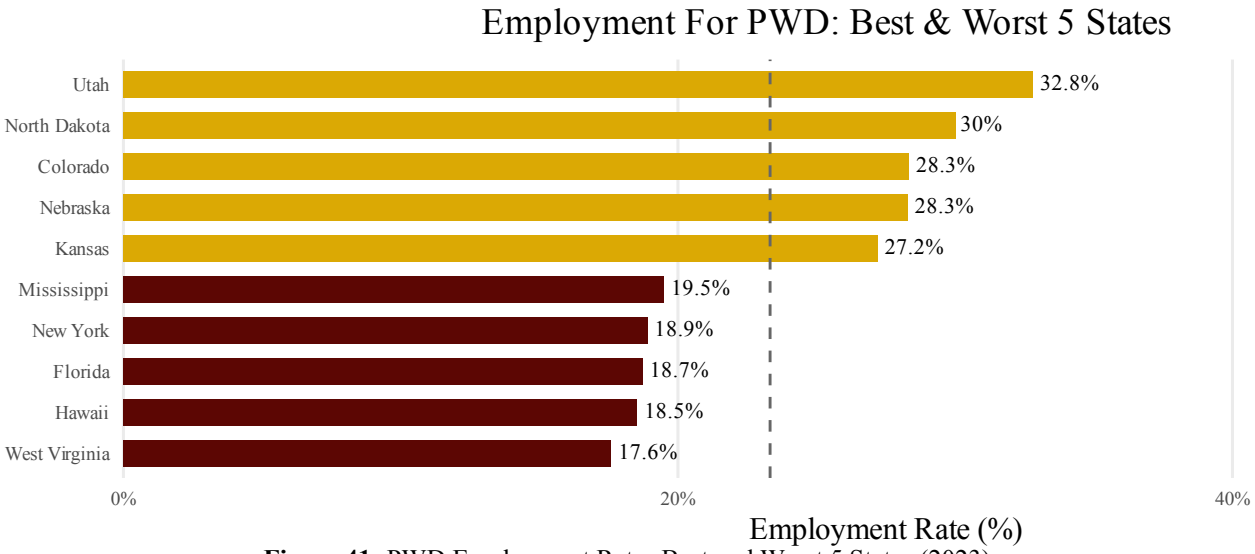


Figure 41: PWD Employment Rate: Best and Worst 5 States (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five states with the highest and the five states with the lowest employment rates among people with disabilities (ages 16+). The dashed line marks the national average across states.

Figure above isolates the five highest and lowest performing states for employment among working-age people with disabilities positioning them relative to the national midpoint (dashed line of 23 percent). The contrast between the two groups is consequential. In terms of the very best outcomes, Utah (32.8%), North Dakota (30.0%), Colorado (28.3%), Nebraska (28.3%), and Kansas (27.2%) lead, forming an upper tier. All exceed the national average by 4 to 9 percentage points, with Utah standing nearly 10 points above. And on the end with the most unfavorable outcomes for PWD, West Virginia (17.6%), Hawaii (18.5%), Florida (18.7%), New York (18.9%), and Mississippi (19.5%) all fall below 20 percent. Individually, they range from 4 to 5 percentage points below the national average, with West Virginia nearly 6 points beneath. States in this worst performer group are more geographically mixed, spanning Appalachia, the Deep South, the Northeast, and the Pacific, indicating that low disability employment is not confined to a single region. A total range of 15.2 percentage points, in relative terms, a working-age adult with a disability in Utah is nearly 85 percent more likely to be employed than in West Virginia. Overall, the figure reinforces that while disability employment rates remain low nationally, meaningful variation indicates that they are not fixed, with levels having important quality of life implications for PWD.

Employment Rate Disparity: States With Greatest & Least Gaps

State	PWD Employment Rate	PWOD Employment Rate	Disparity
District of Columbia	25.0%	69.3%	**-44.3%**
Massachusetts	22.1%	64.6%	**-42.5%**
New York	18.9%	61.0%	**-42.1%**
New Jersey	21.3%	62.8%	**-41.5%**
Hawaii	18.5%	60.0%	**-41.5%**
Montana	24.8%	61.6%	**-36.8%**
New Mexico	20.5%	57.3%	**-36.7%**
Alaska	26.8%	63.2%	**-36.4%**
Idaho	26.5%	62.6%	**-36.2%**
Utah	32.8%	67.2%	**-34.3%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are percentage differences (PWD − PWOD). Shading highlights the five states with the largest absolute gaps (maroon) and the five with the smallest absolute gaps (gold).

Table 38: States with Greatest and Least Employment Rate Disparity.

Understanding of employment outcomes for PWD improves when further compared against PWOD. Table above highlights where within state employment gaps are widest across disability stauts. Largest disparities occur in several large metro economies, DC with the widest (−44.3%), followed by Massachusetts (−42.5%), New York (−42.1%), New Jersey and Hawaii (both −41.5%). In these places, PWOD employment is relatively high while PWD employment remains low, producing exceptionally large separations. In the lowest-gap group, states still exhibit substantially lower rates for PWD in the mid-30 percentage point range. Utah has the smallest gap among the states (−34.3%), alongside Idaho (−36.2%), Alaska (−36.4%), New Mexico (−36.7%), and Montana (−36.8%). Notably, several of these states pair relatively higher PWD employment with moderately high PWOD employment, yielding gaps that are smaller than average. Main takeaways are the twofold. First, some jurisdictions with strong overall labor markets also exhibit the largest inequities, suggesting that broad employment strength does not guarantee disability inclusive access to work. Second, states that perform well on PWD employment still show extensive gaps relative to PWOD employment.

5.1.2.4 City Level Employment

Employment Rates By Disability Status: 10 Most Populated U.S. Cities

City	Total Population	PWD Employment Rate (16+)	PWOD Employment Rate (16+)
New York	8,460,246	17.4%	61.1%
Los Angeles	3,829,863	21.7%	63.3%
Chicago	2,683,910	23.7%	65.2%
Houston	2,281,731	25.9%	64.3%
Phoenix	1,611,990	29.2%	66.3%
Philadelphia	1,570,102	22.5%	63.3%
San Antonio	1,435,376	26.5%	65.3%
San Diego	1,337,485	22.8%	64.7%
Dallas	1,289,860	28.5%	67.0%
San Jose	986,212	19.3%	66.3%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Cities are ranked by total population. Values are employment rates for ages 16+.

Table 39: Employment Rate — PWD vs PWOD in the 10 Most Populated U.S. Cities.

Shifting from the state level to cities, table above begins with comparing employment rates for people with disabilities (PWD) and people without disabilities (PWOD) in the ten most populous U.S. cities, offering a look at disability inclusion within the nation's largest urban labor markets. Across all ten cities, employment rates for PWD range from 17.4 percent in New York to 29.2 percent in Phoenix, while PWOD employment ranges from New York's 61.1 percent to Dallas' 67.0 percent. Even in urban centers with such economic dynamism, employment rates for people with disabilities rarely exceeds 30 percent, reinforcing the broader national pattern. Employment gaps between PWD and PWOD are substantial in every city.

Of the select cities, the largest disparity appears in San Jose (−47.0 percentage points), where PWD employment stands at 19.3 percent compared with 66.3 percent for PWOD. New York also exhibits a wide gap (−43.7 points), followed closely by Los Angeles (−41.6 points). With less employment inequality across disability status, Houston (−38.4 points) and Phoenix (−37.1 points) show the smallest employment gaps among the ten cities. Notably, Phoenix combines the highest PWD employment rate (29.2 percent) with one of the smaller gaps, suggesting relatively stronger labor force inclusion for people with disabilities compared to peer cities. It appears that high-cost cities such as New York, Los Angeles, and San Jose have less favorable employment outcomes for PWD while the Sun Belt cities of Phoenix, Dallas, Houston, and San Antonio tend to exhibit relatively higher PWD employment rates, though again these relative differences are not massive but modest.

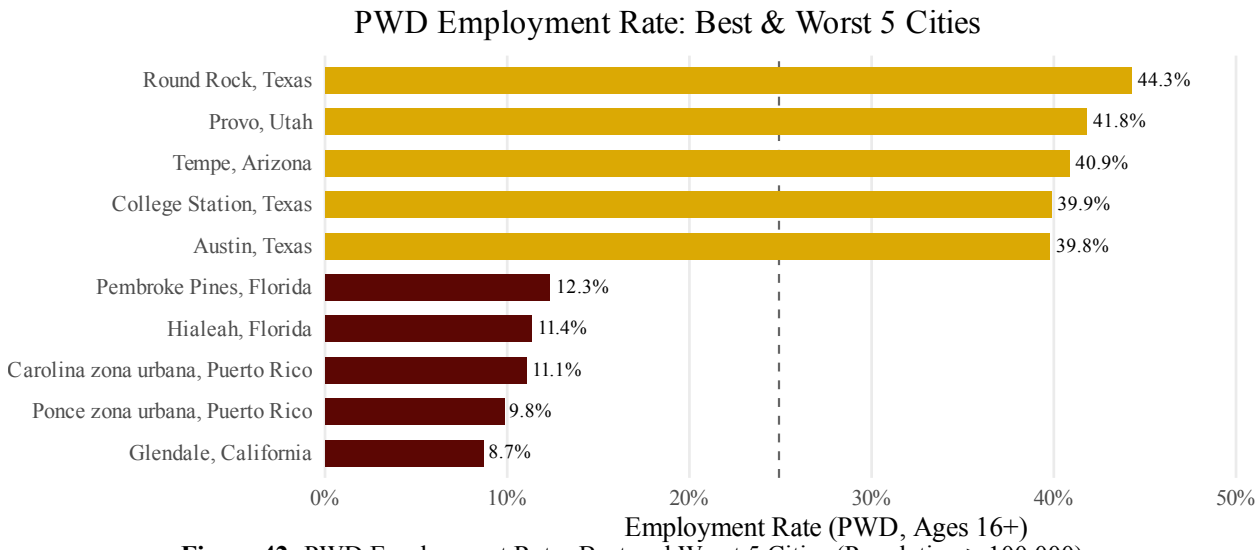


Figure 42: PWD Employment Rate: Best and Worst 5 Cities (Population > 100,000)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five cities with the highest and the five cities with the lowest employment rates among people with disabilities (ages 16+), restricted to cities with civilian noninstitutionalized population above 100,000. The dashed line marks the mean employment rate across eligible cities.

Viewing cities beyond the largest urban centers, we see that outcomes are more variable. Figure above contrasts the five highest and five lowest performing large cities (population 100,000+) on employment among working-age adults with disabilities, benchmarked against the large-city average (dashed line, roughly mid-20 percent). Leading this group is Round Rock, TX (44.3%), Provo, UT (41.8%), Tempe, AZ (40.9%), College Station, TX (39.9%), and Austin, TX (39.8%). All five post PWD employment rates near or above 40 percent, 15–20 percentage points above the large-city norm. In contrast, Glendale, CA (8.7%), Ponce, PR (9.8%), Carolina, PR (11.1%), Hialeah, FL (11.4%), and Pembroke Pines, FL (12.3%) anchor the lower end of the distribution. Each falls more than ten percentage points below average, and the lowest-performing city (Glendale) posts an employment rate more than five times lower than that of Round Rock. With two of Puerto Rico's urban zones and two South Florida municipalities appearing at the bottom, it's evident that local economic context matters significantly. Further suggesting that scale does not ensure inclusive outcomes, the highest-performing cities are predominantly mid-sized or suburban.

5.1.3 Geographic Outcomes ~ Labor Force

We turn now to the raw measure(s) of labor force participation for people with disabilities and people without disabilities at the national, regional, state, and city levels where data is available.

5.1.3.1 National Level Labor Force

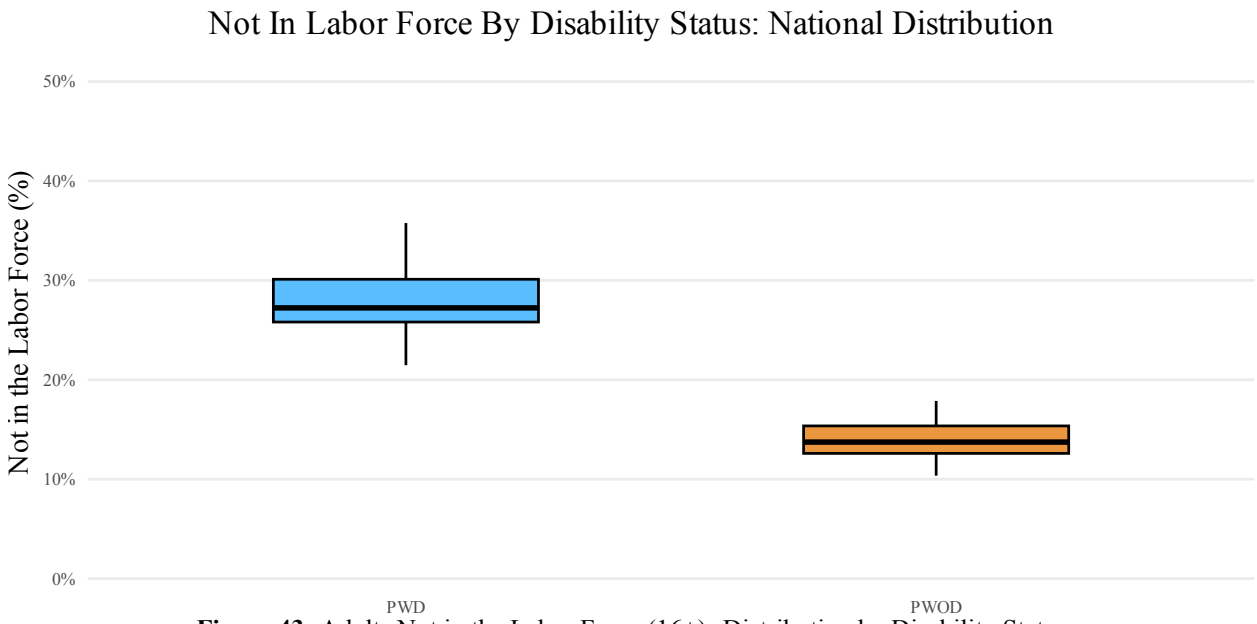


Figure 43: Adults Not in the Labor Force (16+): Distribution by Disability Status

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each box summarizes the across-state distribution of the share of adults ages 16+ who are not in the labor force for people with disabilities (PWD) and people without disabilities (PWOD). Values are percentages (0–100).

Beginning at the national level, figure above presents the distribution of the adults ages 16+ who are not in the labor force, comparing people with disabilitiesand people without disabilities. Fostering greater understanding of labor markets for PWD, it complements the employment rate results by focusing on levels of labor market detachment. Median not-in-labor-force rates for PWD and PWOD fall at 27%, and 13%, respectively. Thus, in practical terms, people with disabilities are roughly twice as likely to be outside the labor force as those without disabilities. Even at the lower end of the PWD distribution (roughly low-20 percent), rates remain well above the typical PWOD range. Further, the spread of outcomes is greater for PWD than PWOD, exhibited by the taller box and tails. Several states approach or exceed 30 percent of working-age PWD not participating in the labor force, while PWOD outcomes are tightly centered between 12 and 16 percent. That the entire PWD distribution sits above the PWOD distribution, a scenario similar to employment exists where greater labor force detachment for PWD is systematic nationwide.

5.1.3.2 Regional Level Labor Force

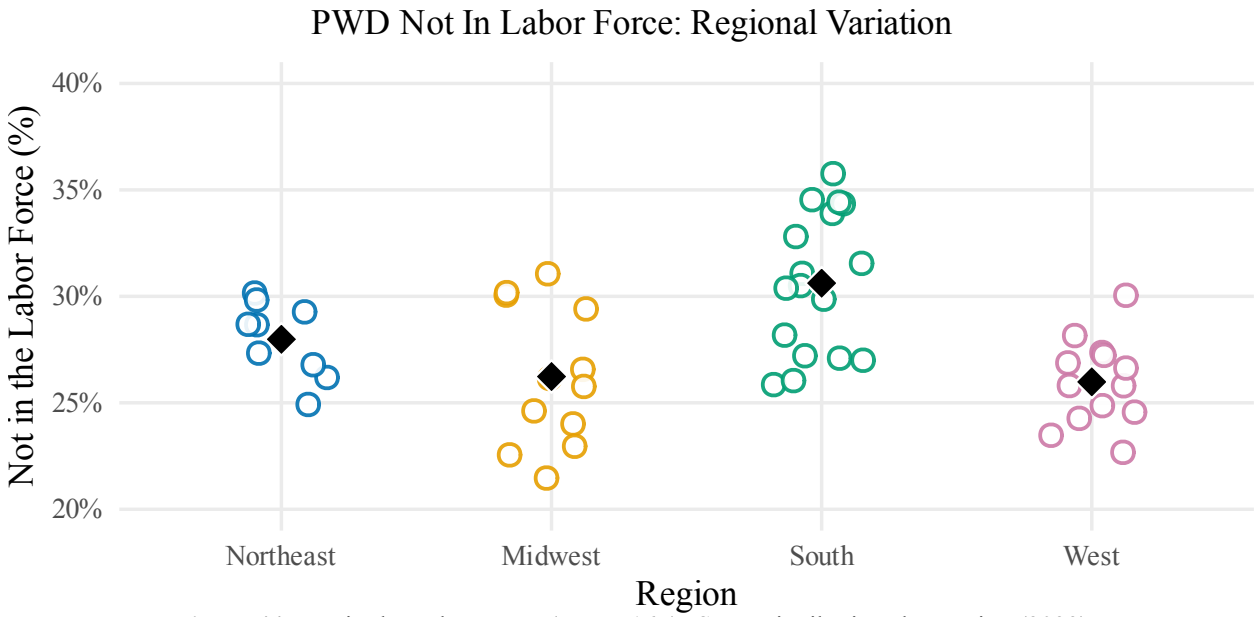


Figure 44: Not in the Labor Force (PWD, 16+): State Distributions by Region (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state's not-in-labor-force rate among adults with disabilities ages 16+, grouped by Census region. The black diamond marks the regional mean. Values are percentages (0–100).

Apart from the national level, figure above compares share of working-age adults with disabilities (PWD) who are not in the labor force across Census regions, with each point representing a state and black diamonds marking regional averages. Not much regional variation exists, with regional means falling between 26-31%. In the South, the regional mean is just above 30 percent, with several states reaching above 35 percent. The Northeast posts a regional average of 28 percent, with states falling between 25 and 30 percent. In the Midwest, there is more variation with states ranging from 21% to 31%, and a regional mean of 26%. Finally, in the West, states are rouped tighly from 23% to 30% with a regional mean just over 25%.

5.1.3.3 State Level Labor Force

Not In Labor Force By Disability Status: State Level

State	PWD Not in Labor Force (16+)	PWOD Not in Labor Force (16+)
Alabama	33.9%	17.3%
Alaska	28.2%	16.1%
Arizona	25.8%	15.3%
Arkansas	34.3%	16.0%
California	26.9%	16.3%
Colorado	24.3%	12.6%
Connecticut	26.2%	12.4%
Delaware	27.1%	12.9%
District of Columbia	28.2%	12.8%
Florida	25.9%	14.7%
Georgia	31.1%	15.5%
Hawaii	22.7%	13.7%
Idaho	25.8%	14.7%
Illinois	26.6%	13.5%
Indiana	30.1%	13.5%
Iowa	24.6%	11.2%
Kansas	25.8%	12.2%
Kentucky	35.8%	15.4%
Louisiana	32.8%	17.4%
Maine	30.2%	12.0%
Maryland	26.0%	12.6%
Massachusetts	27.3%	12.4%
Michigan	31.1%	14.5%
Minnesota	24.0%	10.4%
Mississippi	34.5%	17.9%
Missouri	30.2%	13.4%
Montana	24.6%	13.3%
Nebraska	23.0%	10.6%
Nevada	26.6%	15.2%
New Hampshire	26.8%	11.5%
New Jersey	24.9%	13.3%
New Mexico	30.1%	17.3%
New York	29.3%	15.4%
North Carolina	30.5%	15.0%
North Dakota	21.5%	11.2%
Ohio	29.4%	13.3%
Oklahoma	29.9%	15.9%
Oregon	27.4%	14.0%
Pennsylvania	28.7%	13.3%
Rhode Island	28.7%	13.1%
South Carolina	30.4%	15.4%
South Dakota	22.6%	10.8%
Tennessee	31.5%	15.0%
Texas	27.0%	16.0%
Utah	24.9%	13.7%
Vermont	29.8%	11.9%
Virginia	27.2%	13.8%
Washington	27.2%	14.3%
West Virginia	34.4%	17.8%
Wisconsin	26.1%	11.7%
Wyoming	23.5%	12.6%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are the shares of the civilian noninstitutionalized population ages 16+ who are not in the labor force, shown separately for people with disabilities (PWD) and people without disabilities (PWOD).

Table 40: Not in the Labor Force: PWD vs PWOD, state level.

Applying the state-level lens, table above presents a full look at shares of adults ages 16+ who are not in the labor force, by disability status. Across all states, people with disabilities are consistently more likely than people without disabilities to be outside the labor force. Rates for PWD range from 21.5 percent in North Dakota to 35.8 percent in Kentucky, with most states falling between 24 and 32 percent. In contrast, PWOD

rates are much lower and less variable, ranging from 10.4 percent (Minnesota) to 17.9 percent (Mississippi). Gaps between PWD and PWOD participation are therefore usually in the realm of 12 to 18 percentage points, exceeding that mark in several states.

States in the South are more frequently positioned toward the top of the distribution, while Upper Midwest states more often fall toward the lower portion. However, there is meaningful overlap across regions, underscoring that no region is uniformly high or low performing. Rates tend to vary more for PWD than for PWOD. Whereas PWOD non-participation rates vary within a comparatively narrow corridor (generally low-to-mid teens), PWD rates span more than 14 percentage points across states. This wider spread suggests that state context may exert a stronger influence on labor force attachment for people with disabilities than for those without.

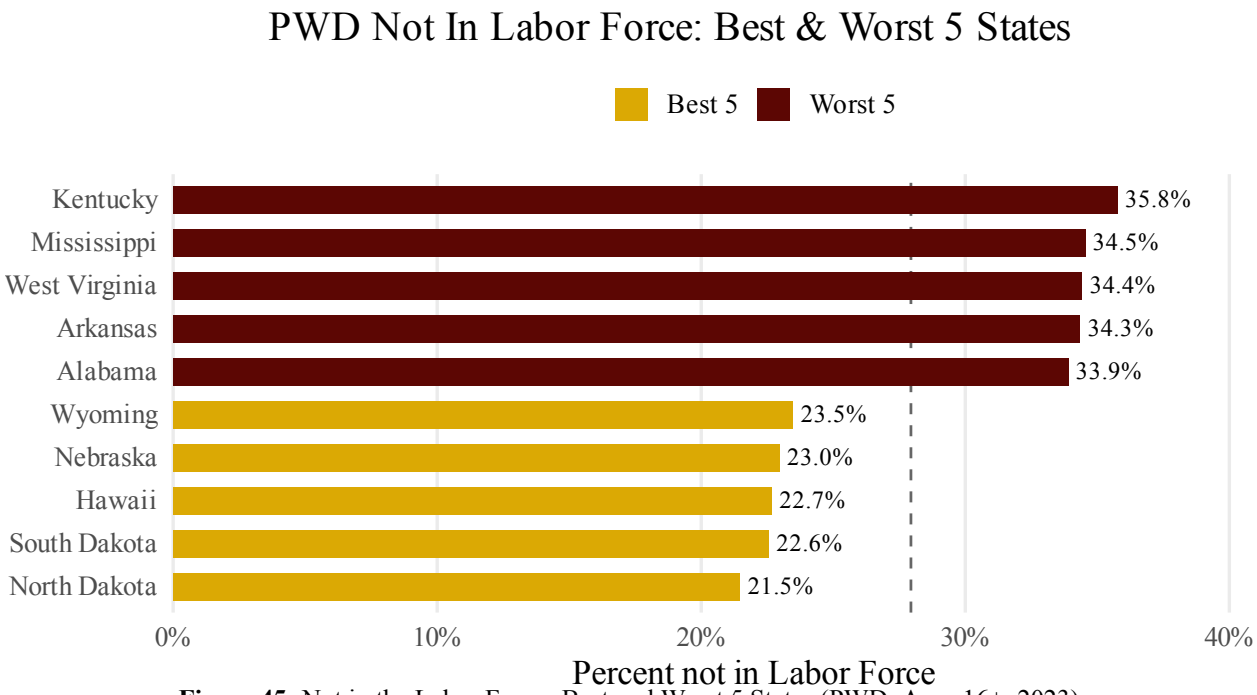


Figure 45: Not in the Labor Force: Best and Worst 5 States (PWD, Ages 16+, 2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five states with the lowest and the five states with the highest shares of people with disabilities of working age (16+) who are not in the labor force. Gold bars indicate the best outcomes (lowest shares) and maroon bars indicate the worst outcomes (highest shares). The dashed line marks the national average across states.

Examining top and bottom performing states more closely, figure above isolates the five highest and lowest rates of non-participation in the labor force for PWD. A dashed line is displayed indicating state average of 28% for comparison. Since higher values reflect greater labor market detachment, the contrast between the two groups captures where exclusion is most and least pronounced. With the highest rates, Kentucky (35.8%), Mississippi (34.5%), West Virginia (34.4%), Arkansas (34.3%), and Alabama (33.9%) crowd the 34 to 36 percent range. In each of these states, more than one in three working-age adults with disabilities are outside the labor force. All sit five to eight percent above the national midpoint, reinforcing an unfortunate concentration of elevated non-participation in parts of the Deep South.

With the lowest rates, North Dakota (21.5%), South Dakota (22.6%), Hawaii (22.7%), Nebraska (23.0%), and Wyoming (23.5%) coalesce in the low-20 percent range. Even in these. states with the most favorable outcomes for PWD, roughly one in five working-age adults with disabilities remains outside the labor force.

Divergence in the highest and lowest rate states surpasses 14 percentage points, a sizable difference in labor force attachment. Importantly, the states with the lowest nonparticipation rates overlap substantially with those that perform well on PWD employment, reinforcing the close relationship between entering the labor force and securing employment once attached. As such, while high labor force detachment for PWD persists nationally, state context is meaningful.

Not In Labor Force Disparity: Best & Worst States

State	PWD Not in Labor Force	PWOD Not in Labor Force	Disparity
Kentucky	35.8%	15.4%	**20.4%**
Arkansas	34.3%	16.0%	**18.3%**
Maine	30.2%	12.0%	**18.1%**
Vermont	29.8%	11.9%	**17.9%**
Missouri	30.2%	13.4%	**16.8%**
Wyoming	23.5%	12.6%	**10.9%**
California	26.9%	16.3%	**10.6%**
Arizona	25.8%	15.3%	**10.5%**
North Dakota	21.5%	11.2%	**10.3%**
Hawaii	22.7%	13.7%	**8.9%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Disparity is the difference between PWD and PWOD not-in-labor-force rates. Highlighting marks the largest and smallest absolute gaps (|Disparity|).

Table 41: Not in the Labor Force Disparity: Greatest and Least Gaps

Shifting the state analysis to an equity focus, table above shows where disability status is associated with the largest additional likelihood of being outside the labor force. Even among the smaller gap states shown here, none are close to parity. States with the largest labor force participation disparities have values in the high-teens to twenty percentage points. Led by Kentucky (+20.4%), close behind are Arkansas (+18.3%), Maine (+18.1%), Vermont (+17.9%), and Missouri (+16.8%). What stands out is that these large disparities can arise through different combinations of underlying rates. In Kentucky, for example, PWD nonparticipation is very high (35.8%) while PWOD is also elevated (15.4%). In contrast, Maine and Vermont pair moderate PWD levels (30.2% and 29.8%) with very low PWOD non-participation (12.0% and 11.9%). In states with smaller gaps, disparities still sit around 9–11 percentage points, with the lowest being Hawaii (+8.9%). North Dakota (+10.3%), Arizona (+10.5%), California (+10.6%), and Wyoming (+10.9%) also have relatively less disparity.

5.1.3.4 City Level Labor Force

ACS-5 year estimates unavailable at local level.

5.2 Wealth & Poverty

5.2.1 Scorecard ~ Wealth & Poverty Index

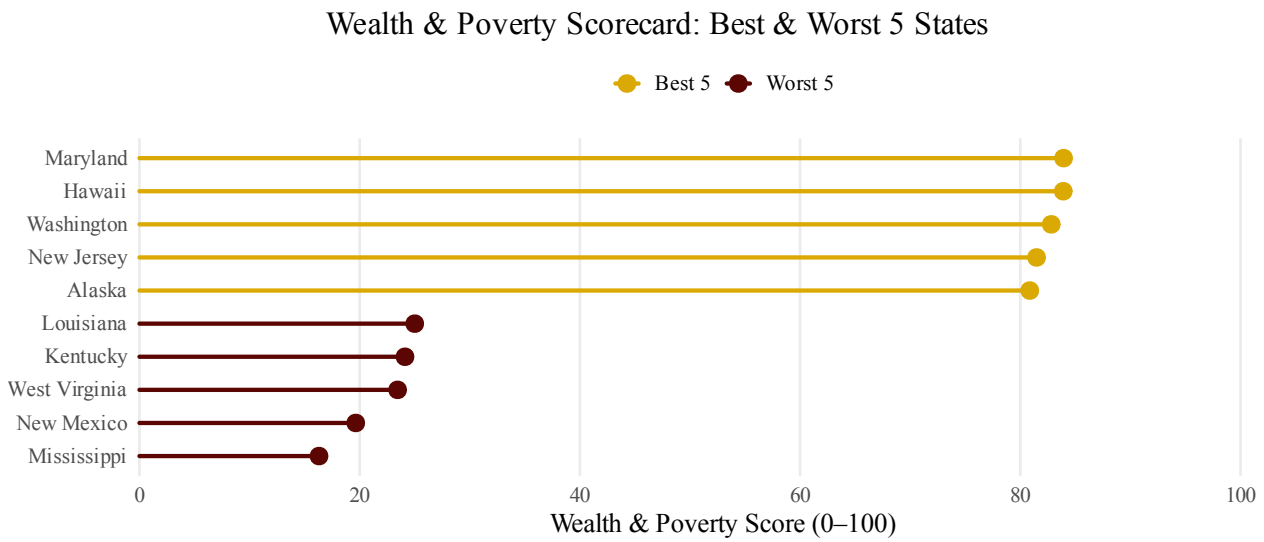


Figure 46: Wealth & Poverty Scorecard: Best and Worst 5 States (2023)

Source: ADA-PARC 2023 Scorecards.

Interpretation: Dots show the five states with the highest and the five states with the lowest Wealth & Poverty scores for 2023. Higher scores indicate better outcomes; gold denotes the best-performing states and maroon denotes the worst-performing states.

Figure above introduces the first of two Work & Economic scorecards, the Wealth & Poverty Index (0–100 scale) that combines two core indicators for people with disabilities. First, the share living below the federal poverty line and median individual income. Higher scores reflect states with lower PWD poverty rates and higher median incomes simultaneously, while lower scores indicate economic disadvantage which is compounded across both dimensions. Performing excellently on this scorecard are Maryland, Hawaii, Washington, New Jersey, and Alaska, each eclipsing a score of 80. These states sit near the upper bound of observed performance, indicating comparatively lower poverty prevalence among PWD and stronger income levels relative to other states.

With outcomes for PWD that are much less favorable, Mississippi, New Mexico, West Virginia, Kentucky, and Louisiana post scores ranging from the high teens to mid-20s. These values reflect a combined pattern of elevated poverty rates and lower median incomes for PWD. Distance between the bottom group and the leading states is 55–65 points, quite significant but not as high as some of the other ADA-PARC scorecards Several Southern states appear in the lower tier, while Mid-Atlantic, and Western Pacific states appear among the highest performers. Such regional representation in states with the most and least favorable outcomes for PWD aligns to some extent with patterns seen in employment and labor force participation. How these trends continue or to what degree they separate out will become more clear in the sub-sections to come.

See next page for full Poverty & Wealth Scorecard Rankings.

Wealth & Poverty Scorecard: Full State Rankings[1]

Rank	State	Wealth & Poverty	Quartile
1	Maryland	**83.9**	Q4
2	Hawaii	**83.9**	Q4
3	Washington	**82.8**	Q4
4	New Jersey	**81.5**	Q4
5	Alaska	**80.9**	Q4
6	Virginia	**79.2**	Q4
7	Colorado	**78.9**	Q4
8	Nevada	**74.6**	Q4
9	New Hampshire	**73.7**	Q4
10	California	**67.5**	Q4
11	North Dakota	**66.8**	Q4
12	Utah	**66.7**	Q4
13	Arizona	**62.3**	Q3
14	Delaware	**61.0**	Q3
15	Massachusetts	**60.1**	Q3
16	Rhode Island	**57.9**	Q3
17	Georgia	**57.4**	Q3
18	District of Columbia	**57.2**	Q3
19	Connecticut	**56.4**	Q3
20	Wyoming	**56.2**	Q3
21	Texas	**55.1**	Q3
22	Illinois	**54.9**	Q3
23	Florida	**54.3**	Q3
24	Nebraska	**53.6**	Q3
25	Wisconsin	**50.0**	Q3
26	Minnesota	**49.6**	Q2
27	Idaho	**48.9**	Q2
28	South Carolina	**47.7**	Q2
29	New York	**46.6**	Q2
30	Oregon	**45.9**	Q2
31	Kansas	**45.7**	Q2
32	Indiana	**45.5**	Q2
33	Oklahoma	**44.7**	Q2
34	North Carolina	**44.2**	Q2
35	Tennessee	**43.2**	Q2
36	Iowa	**42.1**	Q2
37	Pennsylvania	**41.7**	Q2
38	South Dakota	**41.2**	Q2
39	Maine	**41.1**	Q1
40	Montana	**40.9**	Q1
41	Missouri	**39.2**	Q1
42	Vermont	**36.1**	Q1
43	Alabama	**35.1**	Q1
44	Ohio	**31.9**	Q1
45	Michigan	**31.0**	Q1
46	Arkansas	**26.9**	Q1
47	Louisiana	**25.0**	Q1
48	Kentucky	**24.1**	Q1
49	West Virginia	**23.4**	Q1
50	New Mexico	**19.6**	Q1
51	Mississippi	**16.3**	Q1

[1]Quartiles calculated by percentile: Q1 $\leq$ 41.1; Q2 $\leq$ 49.6; Q3 $\leq$ 61.6; Q4 > 61.6.
Source: U.S. Census Bureau, 2023 ACS-5

Table 42: Full state ranking for the Wealth & Poverty Scorecard with ranks and quartiles highlighted (2023).

Full state rankings reveals the breadth of Wealth & Poverty scorecard results for people with disabilities across states. In the top tier, quartile four (Q4) states score above 61.6. Led by Maryland top states have scores above 80, followed by a second group in the 70s (e.g., Virginia 79.2, Colorado 78.9), and a third in the high 60s (e.g., California 67.5, Utah 66.7). This quartile spans multiple regions, from the Mid-Atlantic,

Pacific West, and Mountain West, ssuggesting that strong outcomes are achievable within multiple economic climates. Quartile 3 (Q3), scores between 49.7 and 61.6, and includes states such as Arizona (62.3), Delaware (61.0), and Massachusetts (60.1), at the upper end, with Florida (54.3), Nebraska (53.6), and Wisconsin (50.0) on the lower end. These states still sit above the national midpoint but don't perform as strongly as the top tier.

Slipping below the national median, quartile two (Q2) states score from 41.2 to 49.6, a tightly packed group. On top within this quartile are Minnesota (49.6), and Idaho (48.9), and anchoring the bottom are Pennsylvania (41.7), and South Dakota (41.2). At the bottom of the distribution, Quartile 1 (Q1) scores are below 41.2, with Maine (41.1) near the threshold and followed closely by Montana (40.9), and Missouri (39.2). These states exhibit dual disadvantage, elevated poverty rates alongside weaker income levels for PWD. Overall, the ranking makes clear that economic security for people with disabilities varies significantly at the state-level, warranting further investigation of such conditions vital to quality of life for PWD.

5.2.2 Geographic Outcomes ~ Poverty

We turn now to the raw measure(s) of poverty for people with disabilities and people without disabilities at the national, regional, state, and city levels where data is available.

5.2.2.1 National Level Poverty

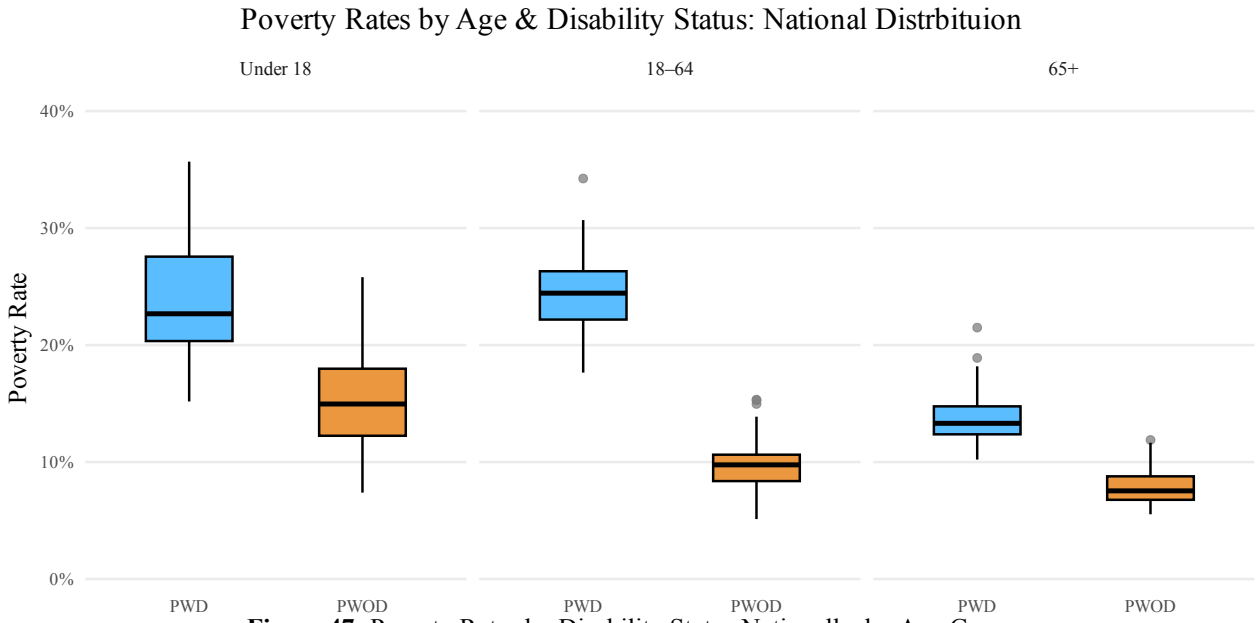


Figure 47: Poverty Rates by Disability Status Nationally, by Age Group

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each panel summarizes the distribution of state poverty rates for people with disabilities and people without disabilities in the indicated age group. Higher values indicate a larger share of the population living below the poverty line.

Looking first at poverty, figure above compares state-level poverty rate distributions for people with disabilities and people without disabilities across three age groups, highlighting how disability status and economic disadvantage are linked across the life course. For children under 18, the median poverty rate for PWD is 23%, compared to 15% for PWOD. With a range slightly larger for PWD, several states extend beyond

30%. Inequity across disability status becomes most pronounced for working-age adults. Here, the median poverty rate for PWD sits at 24%, higher than for children but with less variation. Meanwhile, for PWOD working age, the median is just below ten percent, and again with a tighter distribution. Among adults 65 and older, overall poverty levels decline for both groups. Median PWD poverty falls to 13%, with minimal variation barring a couple outliers in the 20% range. PWOD poverty centers at 7.5%. Thus, for the elderly disabled population, both the rate of poverty and gap relative to PWOD are lowest. While multiple factors likely contribute to this reality, it may suggest the possibility of outcome improvement through programs such as Social Security. PWD poverty rates exceed PWOD rates in almost all states in each age bracket, though the working-age population shows the largest disparities.

5.2.2.2 Regional Level Poverty

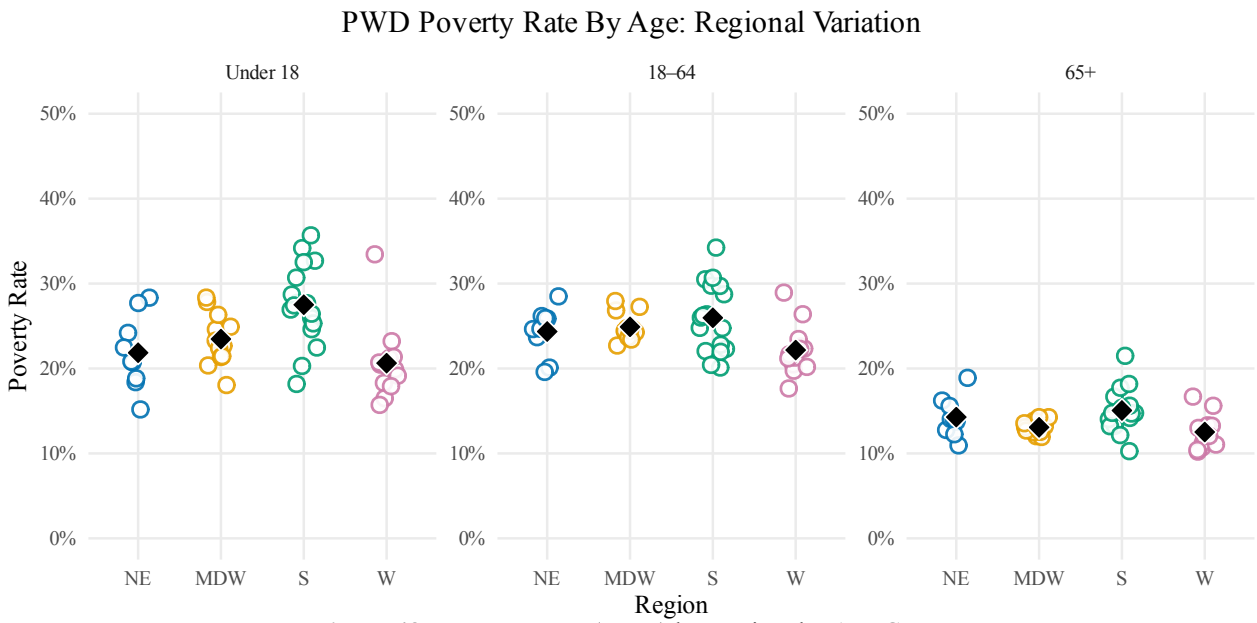


Figure 48: Poverty Rates (PWD) by Region, by Age Group

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state's poverty rate among people with disabilities in the indicated age group. Black diamonds mark regional means. Higher values indicate a larger share of the population living below the poverty line. NE = Northeast, MDW = Midwest, S = South, W = West

Moving down one geographic level, figure above separates out poverty among people with disabilities by region, revealing trends across age groups and neighbor states. For children under 18, the South has the highest regional average PWD poverty rate. With a full range of 19% to 36%, multiple states also fall right at the regional mean of 27.5. Posting the second and third highest averages, the Midwest and Northeast centera bit lower, with states group from the 15 to 30% range. In the under eighteen panel, the West has the lowest regional mean at 20%, with most states tightly grouped around the average. For working-age adults, there is a slightly more evenness across regions. Averages range from 22% in the West to 26% in the South, with states overall ranging from 18% to 34%. For adults 65 and older, poverty rates decline across all regions in line with the national distribution, and the regional gaps also compress. States range from 10% to 21% with the majority falling closer to regional means, each which hovers around 13-15%. Generally, the visualization shows that experiences of poverty for PWD are less variable across regions in older age.

5.2.2.3 State Level Poverty

Poverty Rates by Disability Status & Age: State Level

State	PWD Under 18	PWOD Under 18	PWD 18–64	PWOD 18–64	PWD 65+	PWOD 65+
Alabama	27.7%	21.2%	26.4%	12.5%	14.9%	9.2%
Alaska	18.3%	12.2%	20.5%	8.6%	10.7%	6.5%
Arizona	23.2%	16.7%	22.6%	10.7%	13.2%	8.3%
Arkansas	32.7%	20.6%	28.7%	12.5%	14.7%	8.9%
California	19.9%	15.0%	22.3%	9.9%	15.6%	9.2%
Colorado	16.5%	10.7%	19.7%	8.1%	11.2%	6.7%
Connecticut	24.2%	12.6%	23.7%	7.8%	12.8%	6.4%
Delaware	26.0%	15.2%	20.1%	8.8%	10.3%	5.5%
District of Columbia	27.7%	20.1%	34.2%	10.4%	21.5%	11.3%
Florida	22.5%	16.6%	22.3%	10.4%	14.8%	9.8%
Georgia	24.6%	18.4%	22.8%	10.7%	14.2%	9.1%
Hawaii	20.3%	12.2%	21.3%	8.5%	12.0%	7.5%
Idaho	19.2%	12.2%	20.2%	9.0%	11.0%	6.9%
Illinois	24.6%	15.0%	24.4%	9.4%	13.8%	7.9%
Indiana	26.3%	15.1%	24.8%	9.8%	12.0%	6.8%
Iowa	22.7%	12.3%	24.6%	9.3%	11.9%	6.6%
Kansas	23.3%	13.2%	24.4%	9.6%	12.5%	6.6%
Kentucky	30.7%	20.2%	30.5%	12.3%	16.7%	9.4%
Louisiana	34.2%	25.2%	29.7%	15.3%	17.7%	11.9%
Maine	20.7%	12.6%	26.2%	8.0%	14.0%	7.0%
Maryland	18.2%	11.2%	22.1%	7.2%	13.2%	7.3%
Massachusetts	22.5%	11.3%	24.6%	7.4%	16.2%	7.6%
Michigan	27.8%	16.9%	26.8%	10.4%	13.1%	7.5%
Minnesota	18.1%	10.3%	24.3%	7.2%	13.2%	6.5%
Mississippi	35.7%	25.8%	29.7%	15.0%	18.2%	11.7%
Missouri	24.9%	15.5%	27.3%	9.8%	14.3%	7.8%
Montana	21.2%	13.5%	23.5%	10.6%	13.3%	7.9%
Nebraska	20.4%	11.2%	22.7%	8.5%	12.7%	7.2%
Nevada	20.5%	16.6%	21.7%	10.5%	13.0%	9.1%
New Hampshire	15.2%	7.4%	20.1%	5.1%	10.9%	6.0%
New Jersey	18.4%	13.1%	19.6%	7.5%	14.0%	7.6%
New Mexico	33.4%	24.1%	28.9%	15.3%	16.7%	11.0%
New York	28.3%	17.7%	28.5%	10.6%	18.9%	9.8%
North Carolina	26.9%	17.7%	24.8%	10.5%	14.0%	8.3%
North Dakota	22.4%	10.1%	23.7%	9.4%	13.0%	7.5%
Ohio	28.3%	17.4%	27.9%	10.3%	13.6%	7.5%
Oklahoma	28.7%	19.6%	26.0%	12.6%	13.2%	8.1%
Oregon	21.4%	12.9%	26.4%	10.0%	13.3%	7.4%
Pennsylvania	27.7%	15.3%	25.9%	8.8%	13.8%	7.4%
Rhode Island	20.8%	12.8%	24.7%	8.1%	15.6%	8.3%
South Carolina	25.3%	19.4%	24.8%	11.5%	14.7%	8.7%
South Dakota	21.4%	14.7%	24.5%	9.9%	14.3%	7.0%
Tennessee	27.4%	18.3%	26.2%	10.7%	14.8%	8.5%
Texas	26.5%	18.5%	22.0%	11.0%	15.6%	9.5%
Utah	15.7%	8.6%	17.6%	7.7%	10.2%	5.8%
Vermont	18.8%	10.3%	25.9%	8.2%	12.3%	6.6%
Virginia	20.3%	12.3%	20.4%	8.1%	12.2%	6.5%
Washington	17.9%	11.6%	22.3%	8.0%	12.1%	6.7%
West Virginia	32.5%	20.8%	30.7%	13.9%	15.4%	8.4%
Wisconsin	21.4%	12.6%	23.4%	8.7%	12.5%	7.0%
Wyoming	20.7%	12.3%	21.2%	9.4%	10.4%	6.6%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values represent poverty rates, shown separately for people with disabilities (PWD) and people without disabilities (PWOD) within each age group.

Table 43: State-level poverty rates by disability status (PWD vs PWOD) and age group (Under 18, 18–64, 65+).

Zooming in further, table above provides a full state accounting of poverty by disability status and age group,

making visible both the consistency of disability associated disadvantage and the manner in which it varies across geographies. Two structural patterns emerge immediately. First, PWD poverty rates exceed PWOD poverty rates in every state and in every age group, there are zero reversals. Second, the magnitude of the gap varies systematically by age, with working-age adults showing the largest and most persistent disparities. Within the under eighteen group, PWD poverty rates frequently fall in the twenties, but exceed 30 percent in several states, including Mississippi (35.7%), Louisiana (34.2%), and New Mexico (33.4%). Even in comparatively lower-poverty states such as Utah (15.7%) and Colorado (16.5%), rates for children with disabilities remain markedly above those of non-disabled children.

Some large disparities emerge in among working-age adults as well. In many states, poverty among PWD approaches 25 percent, and in several cases surpasses 30 percent, such as the District of Columbia (34.2%), and West Virginia (30.7%). Conversely, PWOD poverty in this age group mostly stays within the 7–12 percent range. Ratios of PWD to PWOD poverty is often two to three times higher within states in these age groups. Among adults 65 and older, overall poverty levels drop significantly. Most PWD rates fall between 10 and 16 percent, with higher values in states such as Mississippi (18.2%), and New York (18.9%). Geographically, elevated poverty for PWD is most exacerbated but by no means limited to parts of the South. But even states with comparatively strong economic environments such as Colorado, Maryland, or Washington, still exhibit sizable disability-related gaps. This reinforces that overall state prosperity does not equate to parity in experiences of poverty across disability status.

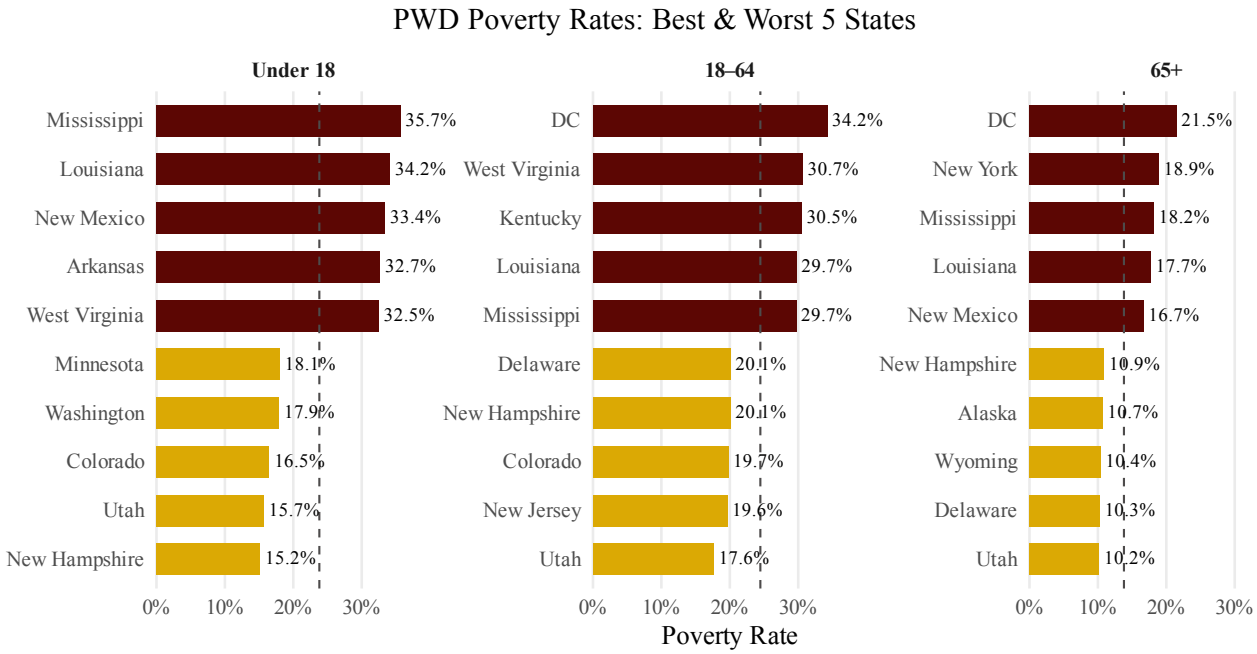


Figure 49: PWD Poverty Rates: Best and Worst 5 States by Age Group

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five states with the lowest poverty rates (best) and the five states with the highest poverty rates (worst) among people with disabilities, shown separately for Under 18, 18–64, and 65+. The dashed line marks the national average within each age group.

Building on the full state table and regional patterns, figure isolates the five highest and lowest poverty states for PWD within each age group, related to the national average (dashed line). For children, the highest poverty states of Mississippi (35.7%), Louisiana (34.2%), New Mexico (33.4%), Arkansas (32.7%), and West Virginia (32.5%), group tightly in the lower to middle 30 percent range. With the best outcomes for children with disabilities, New Hampshire (15.2%), Utah (15.7%), Colorado (16.5%), Washington (17.9%), and

Minnesota (18.1%) have rates around twenty percentage points, or two times lower than the worst performing states. Among working age adults the group with the highest poverty levels are DC (34.2%), West Virginia (30.7%), Kentucky (30.5%), Louisiana (29.7%), and Mississippi (29.7%), with rates nearing or above 30 percent. In contrast, Utah (17.6%), New Jersey (19.6%), Colorado (19.7%), New Hampshire (20.1%), and Delaware (20.1%) represent the lowest poverty group, though even in these states approximately one in five adults with disabilities live in poverty. Still, the gap between the highest and lowest performing state is about a 2x difference.

For adults 65 and older, DC (21.5%) once again posts the highest rate, followed closely by New York (18.9%), Mississippi (18.2%), Louisiana (17.7%), and New Mexico (16.7%). With a lower percent of elder PWD living in poverty are Utah (10.2%), Delaware (10.3%), Wyoming (10.4%), Alaska (10.7%), and New Hampshire (10.9%) . While the spread between worst and best outcomes narrows for this age group to eleven percentage points, still the rate is more than two times higher. Across age groups several patterns emerge. Firstly, the states of Mississippi and Louisiana recur among the highest poverty group in multiple age categories, while DC appears at the top for both adult categories. Conversely, New Hampshire and Utah appear as low poverty states in all three groups, while Colorado and Delaware appear twice.

Poverty Disparity (18–64): Best & Worst States

State	PWD (18–64)	PWOD (18–64)	Disparity
District of Columbia	34.2%	10.4%	**23.9%**
Kentucky	30.5%	12.3%	**18.2%**
Maine	26.2%	8.0%	**18.1%**
New York	28.5%	10.6%	**17.9%**
Vermont	25.9%	8.2%	**17.7%**
Delaware	20.1%	8.8%	**11.4%**
Idaho	20.2%	9.0%	**11.2%**
Nevada	21.7%	10.5%	**11.2%**
Texas	22.0%	11.0%	**11.0%**
Utah	17.6%	7.7%	**10.0%**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Disparity is shown in percentage-point terms as PWD minus PWOD poverty rates for ages 18–64. Higher values indicate worse outcomes (larger poverty gaps for PWD). Highlighting marks the five worst (largest gaps; maroon) and five best (smallest gaps; gold) states.

Table 44: Poverty Disparity (PWD − PWOD), Ages 18–64: Best and Worst States (2023).

Viewing state outcomes with a greater focus on equity, table identifies states with the greatest and least poverty gaps for working age adults. Disparity is defined as the percentage-point difference (PWD − PWOD), so larger values indicate wider inequality in poverty experience during prime working years. Largest gaps appear in the District of Columbia, Kentucky, Maine, New York, and Vermont. In these states, poverty rates for working-age adults with disabilities exceed those of their non-disabled peers by 18 to 24 percentage points. Notably, several of these places do not necessarily have the highest overall poverty rates for PWD, but they combine elevated disability poverty with low poverty among PWOD.

With results closer to parity, states such as Utah, Texas, Nevada, Idaho, and Delaware show the smallest working-age gaps. Utah stands out in particular for pairing a relatively low PWD poverty rate (17.6 percent) with the smallest disparity in the table (10 points), indicating lower poverty across the board. However, even in these states with the most equal outcomes, disparities remain substantial such that double digit gaps persist everywhere. Also of note, some states performed poorly on both absolute poverty outcomes and equality (e.g., Kentucky, DC), while others may appear moderate in absolute terms yet still exhibit pronounced inequality (e.g., Maine, Vermont). Examining these results in absolute and equity terms brings

forth the understanding that improving economic conditions for people with disabilities requires attention on both fronts.

5.2.2.4 City Level Poverty

PWD Poverty Rates by Age: 10 Most Populous U.S. Cities

City	Total Population	PWD Under 18	PWD 18–64	PWD 65+
New York	8,460,246	33.0%	32.8%	26.6%
Los Angeles	3,829,863	26.3%	28.9%	22.9%
Chicago	2,683,910	34.1%	30.4%	23.4%
Houston	2,281,731	36.5%	29.3%	21.3%
Phoenix	1,611,990	27.2%	24.6%	15.8%
Philadelphia	1,570,102	45.0%	37.6%	28.2%
San Antonio	1,435,376	32.8%	25.2%	18.9%
San Diego	1,337,485	19.2%	22.5%	14.8%
Dallas	1,289,860	29.3%	28.1%	20.6%
San Jose	986,212	11.8%	19.1%	15.7%

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Cities are ordered by total population. Values show the share of people with disabilities below the poverty line within each age group.

Table 45: PWD poverty rates by age group in the 10 most populous U.S. cities.

Across the ten most populous U.S. cities, table above offers a window into poverty among people with disabilities at every stage of life in high cost urban centers. Beginning with children, poverty rates are extremely high in multiple cities. Philadelphia reports a staggering 45%, followed by Houston (36.5%), Chicago (34.1%), and New York (33.0%). In these cities, an unconscionable one-third to nearly one-half of children with disabilities live below the poverty line. With much more favorable results, San Jose comes in at 11.8%, and San Diego at 19.2%, being the only two with sub-twenty percent results for children. Thus a spread between the highest and lowest-poverty cities exceeds 30 percentage points, an immense range of childhood disability poverty risk across major metropolitan areas.

Among adults age 18-64, there is both a higher floor and lower ceiling for PWD poverty. Philadelphia again records the highest rate at 37.6%, while New York (32.8%), Chicago (30.4%), and Houston (29.3%) still post rates near or above thiry percent. Best outcomes for this age group are found again in San Jose (19.1%) and San Diego (22.5%), though still near one in five adults with disabilities live in poverty. In the elder population of adults, poverty rates are higher in the cities of Philadelphia (28.2%), New York (26.6%), and Chicago (23.4%) and lower in San Diego (14.8%), San Jose (15.7%), and Phoenix (15.8%). Relevant trends include the consistently poor outcomes in Philadelphia, New York and Chicago, as well as the positive performance in the Californian cities, particularly San Jose and San Diego.

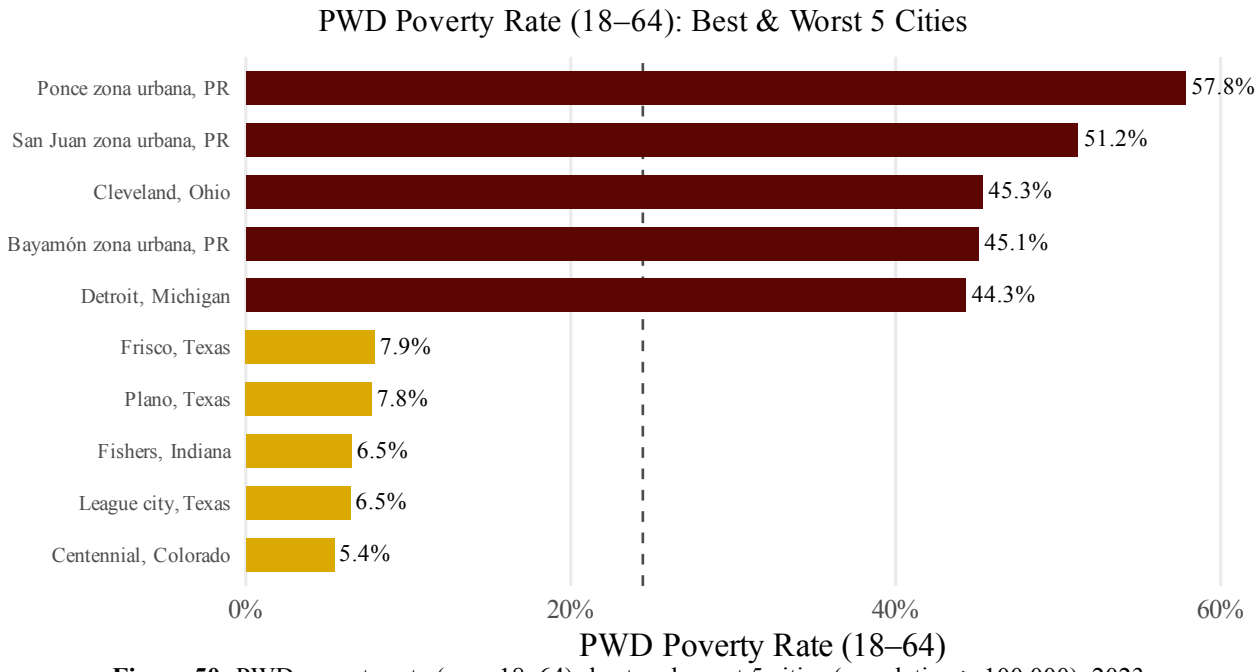


Figure 50: PWD poverty rate (ages 18–64): best and worst 5 cities (population > 100,000), 2023.

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five cities with the lowest and the five cities with the highest poverty rates among people with disabilities (PWD) age 18-64, among cities with a population greater than 100,000. The dashed line marks the mean poverty rate across eligible cities.

Reviewing best and worst performing large cities, rather than just the largest U.S. metros, figure captures the five localities, (population > 100,000) with the highest and lowest poverty rates among working age PWD. Dashed line marks the mean across all eligible cities, providing a benchmark for comparison. On the high end poverty rates are quite concerning, with Ponce, PR (57.8%) and San Juan, PR (51.2%) reporting rates exceeding one-half of working-age adults with disabilities living below the poverty line. Another Puerto Rican city, Bayamón (45.1%) also appears among the highest-poverty cities, alongside Cleveland (45.3%) and Detroit (44.3%). In each of these cities, rates of poverty for PWD in the age range are more than double the average across large cities. For multiple Puerto Rican municipalities to appear at the top signals structural economic strain, while the inclusion of legacy industrial cities such as Cleveland and Detroit underscores that severe disability poverty can materialize from distinct economic circumstances.

With a stark contrast in outcomes, Centennial, Colorado (5.4%), League City, Texas (6.5%), Fishers, Indiana (6.5%), Plano, Texas (7.8%), and Frisco, Texas (7.9%) all report poverty rates below 8 percent. Common between these cities is their largely suburban nature and location within high-growth metropolitan contexts. Each maintains a PWD poverty rate well below the large city mean and are 35–50 percentage points lower than the highest-poverty cities. With a ten times difference in PWD poverty rates for working age individuals, it's clear that local conditions can differ dramatically, and are deeply shaped by place based economic structures, constraints, or opportunities for people with disabilities.

5.2.3 Geographic Outcomes ~ Income

We turn now to the raw measure(s) of income participation for people with disabilities and people without disabilities at the national, regional, state, and city levels where data is available.

5.2.3.1 National Level Income

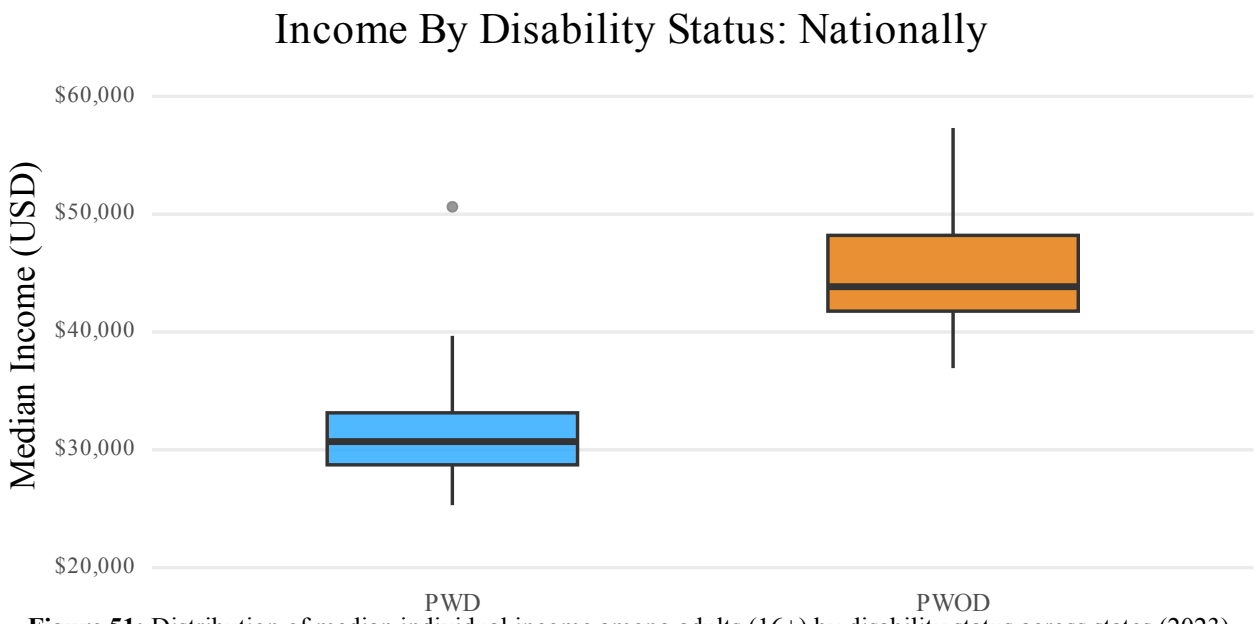


Figure 51: Distribution of median individual income among adults (16+) by disability status across states (2023).

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Boxplots summarize the distribution of median individual income (ages 16+) across states for the civilian noninstitutionalized population. Comparing people with disabilities (PWD) and people without disabilities (PWOD) highlights systematic income differences and the degree of cross-state variation within each group.

Starting with the big picture nationally, figure above presents the distribution of median individual income (ages 16+) by disability status, highlighting a pronounced earnings gap between adults with disabilities and adults without disabilities. Across states, median income for PWOD overall is just below $44,000, with the middle 50% of states ranging from $41,000 to $48,000. Median income distribution is also noticeably positively skewed, meaning that higher income states above the median result in a longer tail above the box. Median income for PWD is concentrated closer to the low $30,000s, with a pretty even distribution across states and less range relative to PWOD. There is one significant outlier state for PWD above the $50,000 range, which will be revealed in the state analysis which follows. Still, the broad takeaway is that in nearly every state, the typical adult with a disability earns substantially less than their non-disabled counterpart. Earnings gaps are not geographically isolated, but rather a persistent national pattern.

5.2.3.2 Regional Level Income

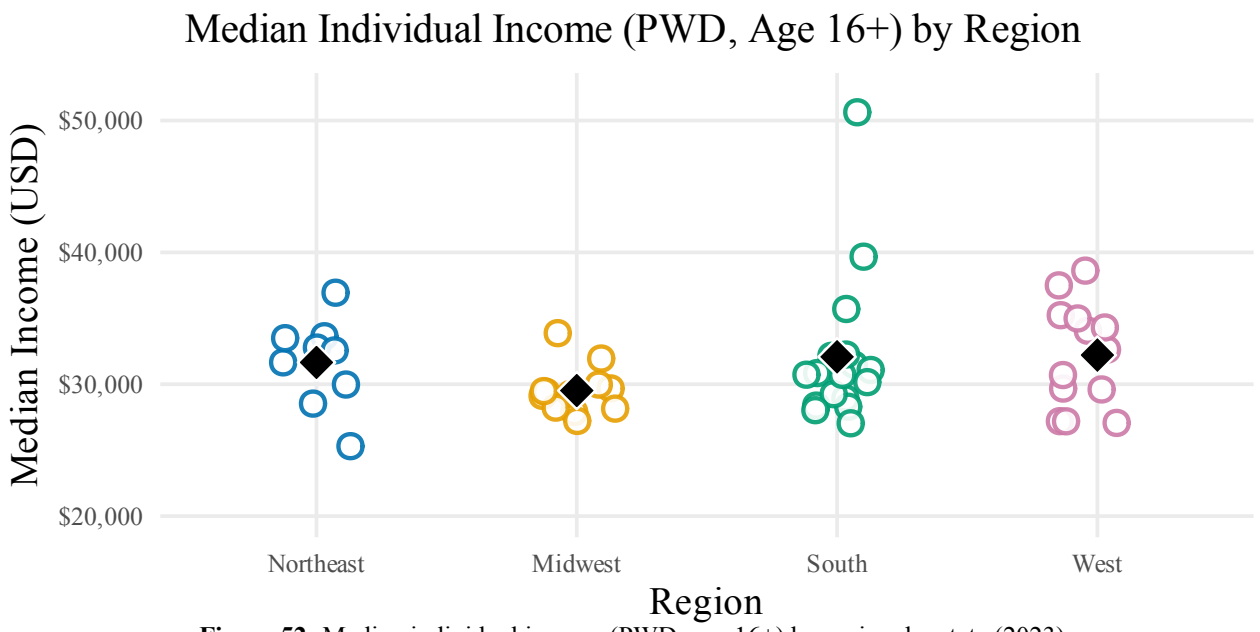


Figure 52: Median individual income (PWD, age 16+) by region, by state (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Each dot represents one state's median individual income for adults with disabilities (ages 16+), and the black diamond marks the regional mean. This view shows within-region spread and differences in typical income levels across regions.

Separating out the regional from the national, figure above displays median individual income (ages 16+) for PWD grouped by region, with black diamonds marking regional means. Modest regional variation and patterning emerges. Overall, regional means have a range just under $3,000, from $29,523 in the Midwest to $32,214 in the West. In the West, only one state lands right at the mean, while an even number of states fall above and below average, ranging from $28,000 to $39,000. With the next highest regional mean, just eclipsing $32,000, is the South. However, only three of the region's states post median values above the average, as one extreme outlier and two other high performers pull up the average. As a result, this. region also has the greatest range, from $26,000 to above $50,000. In The Northeast and Midwest, states are grouped more closely, all right around $30,000 in the Midwest, with a broader range of $25,000 to $38,000 in The Northeast. Chiefly, the regional distributions suggest that there are regional nuances to income for people with disabilities, but lower median incomes for PWD is a persistent national feature, with regional context influencing magnitude rather than direction.

5.2.3.3 State Level Income

Median Individual Income (Age 16+): PWD vs PWOD (All States)

State	PWD	PWOD
Alabama	$28,922	$40,008
Alaska	$35,237	$49,439
Arizona	$32,631	$44,694
Arkansas	$28,296	$38,832
California	$34,060	$48,279
Colorado	$34,305	$51,412
Connecticut	$31,660	$52,990
Delaware	$31,499	$46,180
District of Columbia	$50,628	$81,675
Florida	$31,084	$40,960
Georgia	$32,159	$42,835
Hawaii	$38,619	$47,495
Idaho	$27,202	$39,468
Illinois	$31,952	$47,979
Indiana	$29,684	$42,604
Iowa	$27,894	$44,177
Kansas	$29,122	$43,058
Kentucky	$28,382	$40,518
Louisiana	$29,248	$40,559
Maine	$28,532	$43,905
Maryland	$39,674	$56,564
Massachusetts	$33,642	$57,317
Michigan	$27,221	$42,627
Minnesota	$29,276	$50,689
Mississippi	$27,039	$36,935
Missouri	$29,415	$42,562
Montana	$27,190	$40,194
Nebraska	$29,975	$44,235
Nevada	$34,991	$42,310
New Hampshire	$32,785	$52,032
New Jersey	$36,941	$55,015
New Mexico	$27,070	$37,563
New York	$33,497	$50,704
North Carolina	$30,161	$42,212
North Dakota	$33,876	$46,071
Ohio	$28,227	$43,790
Oklahoma	$30,718	$40,201
Oregon	$29,673	$45,751
Pennsylvania	$29,979	$46,630
Rhode Island	$32,565	$48,813
South Carolina	$30,849	$40,966
South Dakota	$28,152	$42,507
Tennessee	$30,723	$41,625
Texas	$32,255	$43,736
Utah	$30,698	$42,664
Vermont	$25,305	$45,653
Virginia	$35,709	$50,379
Washington	$37,500	$53,841
West Virginia	$28,033	$39,738
Wisconsin	$29,482	$46,205
Wyoming	$29,605	$42,814

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Values are median individual income in U.S. dollars for the civilian noninstitutionalized population ages 16+.

Table 46: Median individual income among adults (age 16+) by disability status (PWD vs PWOD), across all U.S. states (2023).

Across every state, median individual income for people with disabilities (PWD) is lower than that of people without disabilities (PWOD), reinforcing the pervasiveness of disability-associated earnings inequality. In most states, the income gap falls in the range of roughly $12,000 to over $20,000 annually, indicating a substantial and persistent difference in economic position rather than a marginal variation. Magnitudes of

disparity vary meaningfully across states, and it is not always aligned with absolute income levels. High-income jurisdictions such as the District of Columbia, and Massachusetts, for example, report comparatively elevated median incomes for PWD in absolute terms. However, these same states also display some of the largest income gaps.

By contrast, some lower income states, particularly in parts of the South such as Mississippi, West Virginia, and Arkansas, tend to exhibit smaller absolute dollar gaps driven largely by lower earnings for both groups. Together, the pattern suggests that unlike many of our ADA-PARC measures, for PWD median income state performance in absolute and equity terms are often not aligned. To what extent this is the case will become more clear in unpacking the best and worst state performers, though what is certain is that income inequality across disability status persists ubiquitously.

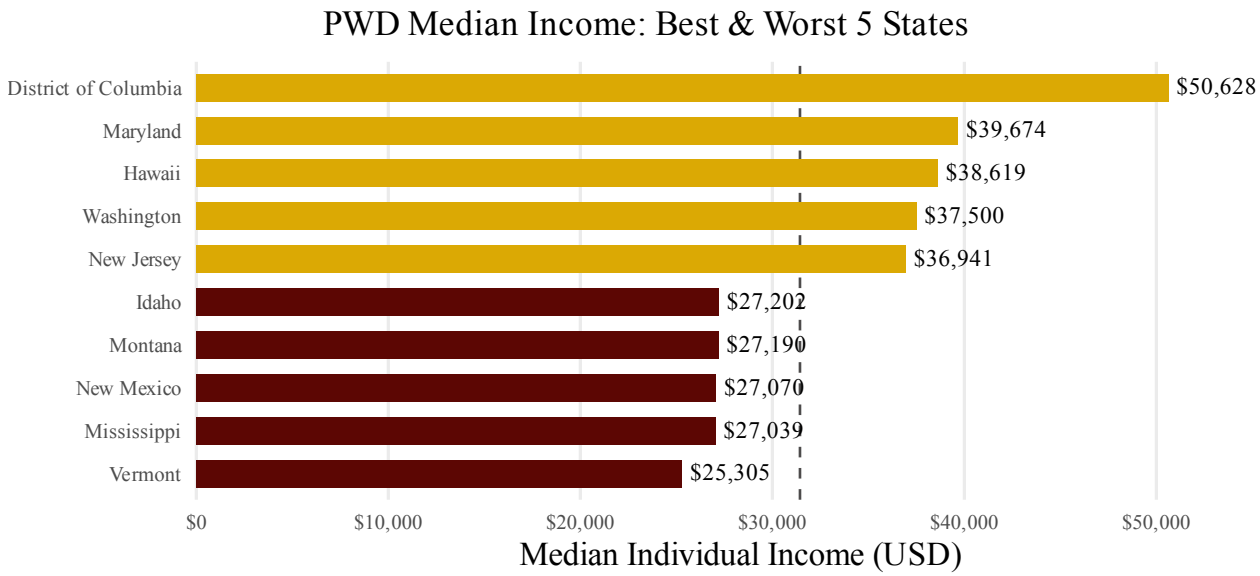


Figure 53: PWD Median Income: Best and Worst 5 States (2023)

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five states with the highest and the five states with the lowest median individual income among adults with disabilities (ages 16+). The dashed line marks the national average across states.

Figure above narrows the scope from all states, to the five highest and lowest income states for adults with disabilities (age 16+), bringing into focus how far state outcomes diverge in absolute terms. A dashed line marks the national median, providing a reference point for how these states compare in the broader landscape. Immediately evident is the greater divergence of highest income states from the average compared to the lowest income states. Standing apart is the District of Columbia with a median income of $50,628, more than $10,000 above the next highest state of Maryland ($39,674). Hawaii ($38,619), Washington ($37,500), and New Jersey ($36,941) complete the high income group. In the bottom tier, Vermont ($25,305), Mississippi ($27,039), New Mexico ($27,070), Montana ($27,190), and Idaho ($27,202) group tightly between $25,000 and $27,000, with values $3k-$5k below the average. Many of these states have smaller labor markets or lower overall wage structures, which appear to translate directly into lower median earnings for adults with disabilities. A $25,000-plus difference between the highest and lowest income states is striking, but as we've seen from the full state table, higher earnings do not automatically mean more parity.

Greatest & Least Median Income Disparity: State Level

State	PWD Median	PWOD Median	Disparity
District of Columbia	$50,628	$81,675	**-$31,047**
Massachusetts	$33,642	$57,317	**-$23,675**
Minnesota	$29,276	$50,689	**-$21,413**
Connecticut	$31,660	$52,990	**-$21,330**
Vermont	$25,305	$45,653	**-$20,348**
Mississippi	$27,039	$36,935	**-$9,896**
Florida	$31,084	$40,960	**-$9,876**
Oklahoma	$30,718	$40,201	**-$9,483**
Hawaii	$38,619	$47,495	**-$8,876**
Nevada	$34,991	$42,310	**-$7,319**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: States are selected by the absolute gap |PWD − PWOD|, while the disparity values retain direction (PWD − PWOD).

Table 47: States with the greatest and least median individual income disparity between adults with disabilities (PWD) and adults without disabilities (PWOD), age 16+ (2023).

Looking beyond income levels to the size of within-state gaps reveals how adults with disabilities are positioned relative to their non-disabled peers. Table highlights the five states with the largest and smallest median income disparities, calculated as PWD median income minus PWOD median income. All disparities are negative, even in states with the least inequality, adults with disabilities earn less than adults without disabilities. What varies significantly is the extent of that shortfall. Largest gaps appear to be in high income Northeastern and upper Midwestern states. The District of Columbia leads by a wide margin, with a difference of −$31,047 despite posting the highest median income for PWD ($50,628). Massachusetts (−$23,675), Minnesota (−$21,413), and Connecticut (−$21,330) follow closely, each exceeding a $21,000 gap. In these areas, strong wage environments elevate earnings disproportionaly for non-disabled workers. Vermont, though, is an exception in that its PWD income gap is large despite only moderate PWOD income. Smaller disparities are concentrated in a more geographically mixed group of states. Mississippi (−$9,896), Florida (−$9,876), Oklahoma (−$9,483), Hawaii (−$8,876), and Nevada (−$7,319) show gaps under $10,000. Here, smaller earnings gaps are driven more so by modest PWOD incomes than high PWD incomes. Looking closer at these cases reinforces earlier findings that higher economic prosperity does not necessarily correspond with narrower disability income gaps.

5.2.3.4 City Level Income

Median Income By Disability Status: 10 Most Populated U.S. Cities

City	Total Population	PWD Median Income (16+)	PWOD Median Income (16+)	Disparity (PWD — PWOD)
New York	8,460,246	$36,485	$51,399	-$14,914
Los Angeles	3,829,863	$31,045	$42,452	-$11,407
Chicago	2,683,910	$33,425	$50,550	-$17,125
Houston	2,281,731	$30,269	$40,502	-$10,233
Phoenix	1,611,990	$34,479	$44,544	-$10,065
Philadelphia	1,570,102	$30,983	$44,285	-$13,302
San Antonio	1,435,376	$27,820	$37,318	-$9,498
San Diego	1,337,485	$37,290	$55,197	-$17,907
Dallas	1,289,860	$32,312	$44,164	-$11,852
San Jose	986,212	$40,694	$62,985	-$22,291

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: Cities are ordered by population. Disparity is computed as PWD — PWOD.

Table 48: Median individual income disparity between adults with disabilities (PWD) and adults without disabilities (PWOD), age 16+, in the ten most populous U.S. cities.

Moving from states to the nation's largest cities shifts the focus to dense urban labor markets, where both opportunity and cost pressures are typically higher. Table above displays median income for adults with disabilities across the ten most populous U.S. cities. San Jose reports the highest PWD median income at $40,694, followed by San Diego ($37,290) and New York ($36,485). Chicago ($33,425) and Phoenix ($34,479) also sit above the $33,000 mark. On the low end, San Antonio records the lowest median income for PWD at $27,820, with Houston ($30,269), Philadelphia ($30,983), and Los Angeles ($31,045) grouping near the low $30,000 level. In this group of the largest U.S. cities, the spread in PWD median income is above $12,000.

Regional patterns are visible but not absolute. California metros occupy both the top tier (San Jose, San Diego) and the middle (Los Angeles). Northeastern cities such as New York and Philadelphia fall into the upper middle of the distribution, while Texas cities fall on the lower end. Gaps relative to non-disabled residents remain present in every city. San Jose and San Diego combine comparatively high PWD incomes with substantially higher PWOD earnings, producing some of the widest dollar gaps. By contrast, San Antonio's smaller disparity coincides with lower median incomes for both groups. In totality, city level outcomes follow similar trends to the state level insofar as large, economically dynamic geographies tend to raise absolute incomes for adults with disabilities, and yet significant economic inequality persists.

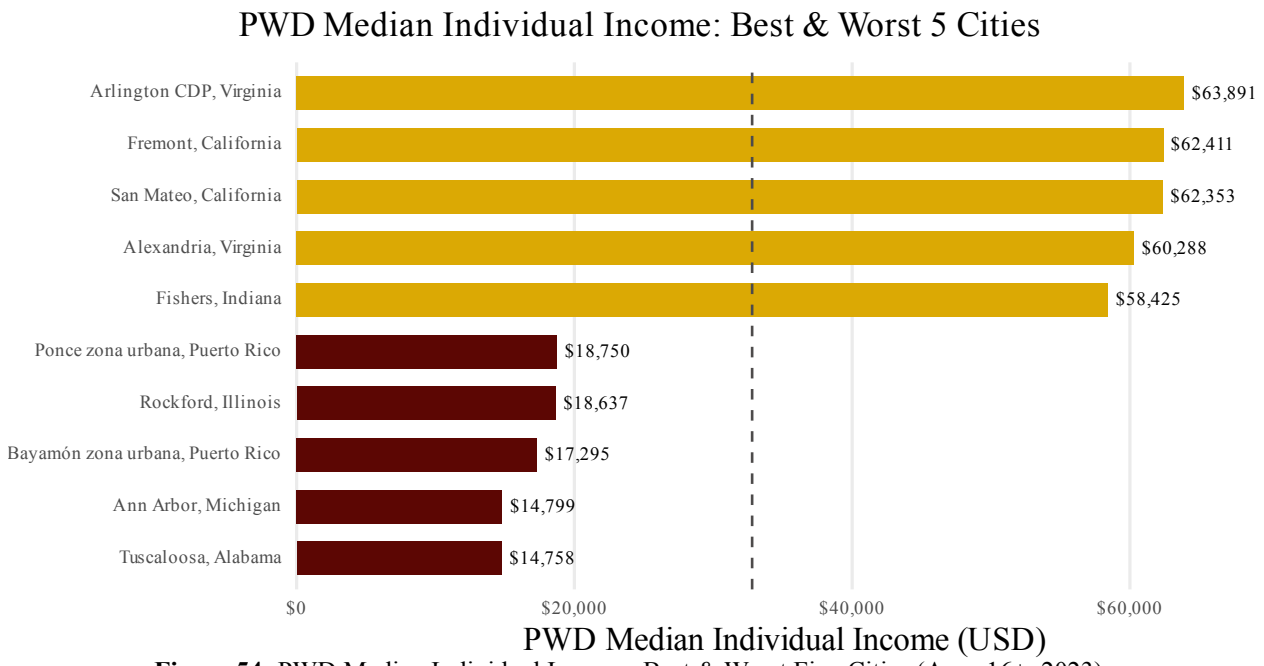


Figure 54: PWD Median Individual Income: Best & Worst Five Cities (Ages 16+, 2023).

Source: U.S. Census Bureau, 2023 ACS-5.

Interpretation: Bars show the five cities with the highest and the five cities with the lowest median individual income among people with disabilities (ages 16+) among cities with a population greater than 100,000. The dashed line marks the mean across all cities meeting the population threshold.

Median income for adults with disabilities differs dramatically across large U.S. cities. At the limit of the distribution, Arlington, Virginia reports a PWD median income of $63,891, with Fremont ($62,411) and San Mateo ($62,353) close behind. Alexandria, Virginia ($60,288) and Fishers, Indiana ($58,425) also exceed $58,000. In each of these cities, median income for people with disabilities is roughly double the levels observed in average income localities. A clear geographic pattern emerges among the highest performers, with cities in Northern Virginia and California's Bay Area dominating the upper tier, regions characterized by dense professional labor markets and high prevailing wages. Anchoring the bottom of the distribution, where median incomes fall sharply, are Ponce,($18,750) and Bayamón, Puerto Rico ($17,295). Rockford, Illinois ($18,637), Ann Arbor, Michigan ($14,799), and Tuscaloosa, Alabama ($14,758) also record values under $20,000. In these cities, PWD median income is less than one third of what is observed in Arlington. Such figures point to markedly different local wage environments and economic structures, with the the appearance of college towns for Ann Arbor and Tuscaloosa being perhaps the result of large student populations. Nonetheless, comparing the top and bottom cities brings into view the scale of dispersion, as the gap between Arlington and Tuscaloosa exceeds $49,000.

5.2.4 Work Economic Conclusions

Key Findings

- **Employment gaps are universal, and large.** No state comes even close to parity. Employment rates for people with disabilities remain well below those of people without disabilities, with **30–40 percentage point gaps** common in states across the nation.

- **Labor force exclusion is a dominant driver of economic disadvantage for PWD.** Across states, high shares of PWD are **not in the labor force**, often **above 30%**, meaning the employment gap is shaped more by labor force detachment than unemployment alone.
- **Income gaps are also widespread and significant across states.** Median individual income (16+) for PWD is lower than for PWOD in every state, many gaps exceeding **$15,000–$20,000** annually.
- **High income labor markets can coincide with large disability income gaps.** Even in high income states and cities (e.g., District of Columbia, Massachusetts, California metros, affluent suburban cities), PWD median incomes remain far below PWOD.
- **Poverty disparities appear in every state and every age group, with no reversals.** PWD poverty exceeds PWOD poverty in all states for children, working-age adults, and older adults. In many states, working-age PWD poverty approaches **25%** and sometimes exceeds **30%**. **Child poverty among PWD is widespread,** in the 20%s, exceeding **30%** in several states. **Older-adult poverty levels decline, but gaps persist.** Among adults 65+, PWD poverty commonly falls between **10–16%**.
- **Employment Index shows geographic patterns.** Top Employment Index states group in the Upper Plains and Mountain West, while the lowest scores concentrate in the South.
- **The Wealth & Poverty Index also shows clear regional patterning.** High scores are found in Mid-Atlantic and Western Pacific states while low scores include multiple Southern states.

Across the Work & Economic indicators, outcomes for people with disabilities show both a consistent disability status gap everywhere, and meaningful variation in the level of economic inclusion from place to place. On employment, the disability gap is not a marginal, but massive difference. Most states have 30–40 percentage point gaps between PWD and PWOD, and no state approaches parity. In most places, the central distinction is not a lack of employment alone, but broader exclusion in the form of large share of adult individuals with disabilities who are not in the labor force. Across states, this "not in the labor force" share often exceeds 30% among adults with disabilities. On the Employment Index, the top of the distribution is concentrated in the Upper Plains and Mountain West, with North Dakota near the top of the 0–100 scale and Nebraska, Utah, South Dakota, and Colorado close behind, while the lowest scores group in the Southern states of West Virginia, Mississippi, Alabama, Arkansas, and Kentucky.

On the Wealth & Poverty Index which combines PWD poverty and median individual income highest scores are in Maryland, Hawaii, Washington, New Jersey, and Alaska, while much lower scores appear in Mississippi, New Mexico, West Virginia, Kentucky, and Louisiana, reflecting a combined profile of higher poverty and lower income among PWD. Income outcomes track those patterns while adding their own kind of variation. Across every state and every city examined, median individual income is lower for adults with disabilities than for adults without disabilities, and many gaps exceed $15,000–$20,000 per year. Large city results make the spread visible, as Arlington, Virginia reports a PWD median income of $63,891, while several cities including Ponce and Bayamón (Puerto Rico) fall below $20,000. High-income metro areas also remain part of the story as places like the District of Columbia, Massachusetts, and California metros have PWD median incomes which remain far below those of PWOD. Poverty results reinforce how economic security for PWD varies across the life course, visible from the separating out of children, working-age PWD, and the elderly.

PWD poverty exceeds PWOD poverty in every state and in every age group, with zero reversals, though gaps are different across the ages. Working-age adults show some of the largest disparities. In many states, PWD poverty approaches 25% and in some exceeds 30% (e.g., DC 34.2%, West Virginia 30.7%), frequently putting PWD poverty at two to three times the level of PWOD within the same state. Child poverty among people

with disabilities is also widespread and reaches especially high levels in several states (e.g., Mississippi 35.7%, Louisiana 34.2%, New Mexico 33.4%), while still remaining elevated even in states with relatively low child poverty overall (e.g., Utah 15.7%, Colorado 16.5%). Among adults 65 and older, overall poverty levels decline, but disability related differences remain visible, with many states grouping in the 10–16% range for PWD and higher values in some places (e.g., Mississippi 18.2%, New York 18.9%).

Taken as a whole, the Work & Economic section documents a consistent national reality of large gaps by disability status while also showing that the level of economic inclusion experienced by people with disabilities varies substantially across states and cities. Even without assigning causes, the consistency and size of these gaps remain a stark, and difficult, finding across virtually every place and measure; whatever the underlying mix of factors, it is a saddening reality of widespread economic disadvantage for people with disabilities.

6 Cross-Category Findings & Conclusion

According to 2023 ACS-5 year outcomes for people with and without disabilities, cross-category patterns point to a consistent conclusion - disability status is associated with systematic disadvantage across where people live, the access conditions conducive to full societal participation, and core work–economic security. Beyond widespread inequality relative to people without disabilities, participatory and quality of life outcomes for people with disabilities varies meaningfully across state and local contexts.

6.1 Cross-Category Key Findings

- **Differences by disability status are consistent across domains, not isolated.** And differences are consistently in the direction of less favorable outcomes for people with disabilities.
- **Large gaps in outcomes for PWD across geographic contexts are evident on many measures.** Variation in state and large city performance is significant in many domains, suggesting structured differences rather than random variation.
- **Age steepens or reduces inequality for PWD depending on the domain.** I**nstitutional living** differences by disability status widen sharply with age, especially for nursing home residence among adults 65+. For poverty, the gap is typically most severe for children and working-age adults, while for health insurance coverage gaps are mitigated for adults 65+.
- **Community living rates are consistently lower for PWD across states, with their over representation in institutional settings.** Most people with disabilities live at home in every state, but PWD are still less likely than PWOD to live at home and more likely to reside in group quarters or institutional settings nationwide.
- **Community participation is complex, and rankings shift significantly by sub-category.** Disability status gaps appear in education, transportation, technology, and insurance, but states that look strong in one area do not necessarily look strong in the others, especially because transportation and technology outcomes are shaped by different local contexts than education
- **Economic security for PWD is constrained relative to PWOD across measures.** Nationwide, employment rates for adults with disabilities trail those of non-disabled adults by roughly **30–40 percentage points**, median individual income is lower by over **$15,000 annually** in most states, and poverty rates for working age adults with disabilities often approach **~25% or higher**, a rate 2-3x that of adults without disabilities depending on the state.

6.2 Discussion

Many interesting patterns and nuances are revealed in the body of the report, though a two-sided trend repeats throughout - outcomes for people with disabilities are consistently less favorable (relative to PWOD), and yet modestly to highly variable depending on place. This pattern holds true in the majority of cases examined across Community Living, Community Participation, and Work & Economics. Such consistency in the disadvantage associated with disability suggests structural inadequacy in the construction of systems meant to confer justice and well-being for all citizens. Especially when stacked together, inequities in living arrangements, access to conditions for participation, and economic security results in the constraint of autonomy.

Community living results make the report's core pattern easy to see. Across the country, most people with disabilities live in private households, yet the direction of difference across disability status never flips. Which is to say, in every state, people with disabilities are less likely to live at home and more likely to live in group or institutional settings than people without disabilities. Most consequential is the difference in institutional living, where people with disabilities appear over represented in correctional settings in most states, indicating that disability status is repeatedly associated with confinement and system contact beyond long-term care. Nursing home residence remains the larger driver of institutional living differences across disability status in numerical terms, with some states reaching double-digit levels. Together, these two pathways to institutional exposure particularly through correctional settings signals forms of exclusion that carry especially high human cost.

Community participation findings build on initial results, as disability disparity shows up in the inputs people rely on to move through daily life and to reach economic security. Across education, transportation, technology, and insurance, measures consistently place people with disabilities on the less favorable side of access or engagement compared with people without disabilities. Education differences are especially structural in the sense that they appear at both ends of the distribution in terms of greater 'low-attainment' and less 'high-attainment'. Transportation indicators suggest a different constraint, with limited practical options outside of highly urbanized contexts, as public transit use remains concentrated in places like DC and New York. Participation depends heavily on private vehicle access and usability, which can be especially constraining for some disability types. In the realm of technology access, large and consequential gaps persist in internet and computer access. Insurance coverage is the one area where headline rates are comparatively strong for people with disabilities, but unevenness remains, particularly for working-age adults. As a whole, community participation for PWD reads less like a single barrier and more like an access profile, where outcomes are better or worse depending on geographic context but with narrower participation pathways overall.

Work Economic outcomes are perhaps both the most uniform in direction and consequential in what they mean for people with disabilities. Disability-status separation appears across employment, labor force attachment, poverty, and income. State employment distributions show the scale immediately, with a typical employment gap of 40 percentage points. Further, adults with disabilities are not just less likely to be employed, they are also about twice as likely to be detached from the labor force altogether. Poverty and income results contextualize this workforce separation for measures of basic economic security. With no reversals, poverty for people with disabilities exceeds poverty for people without disabilities in every state and across all age groups. Elevated poverty among PWD is most pronounced among children and working age adults, with more than 1 in 3 children with disabilities living in poverty in some states. Unsurprisingly, then, income comparisons mirror the persistence of disadvantage for PWD while showing that overall prosperity doesn't accumulate equally. Across states, adults without disabilities posted a median income of $44,000, while adults with disabilities center in the low $30,000s, leaving a consistent earnings penalty nationwide.

Taken together, the Work & Economic findings describe a landscape where disability status repeatedly corresponds to reduced labor market attachment and diminished economic security, a foundational reality with major bearing on prosperity for people with disabilities.

Another way to describe the through line of findings in this report is the combination of universal-inequality with localized-expressions. The disadvantage by disability status is widespread across domains, but the degree of disadvantage is not uniform, as states and cities repeatedly group into higher and lower performance groups. A blend of persistent disability-gaps plus meaningful geographic variation underscores two realities at once, firstly that not enough is being done to promote parity, and that better outcomes for people with disabilities are within reach. It is difficult to read the current relities for people with disabilities across living arrangements, participation access, and economic security as an acceptable status quo. Certainly we can do much, much better.

7 Appendix

7.1 Data Source

United States Census Bureau ACS-5 2023

7.2 ADA-PARC Website

ADA-PARC

7.3 Interpreting Tables & Figures

7.3.1 Styles

Below is a list of the types of visual representations of results found in the report, each with an example and guidance on interpretation.

A) Basic Tables

Interpretation of tables should be in line with standard understandings. Tables presented in the report vary based on the metric(s) and geographic context(s) they represent, but each is labelled as such.

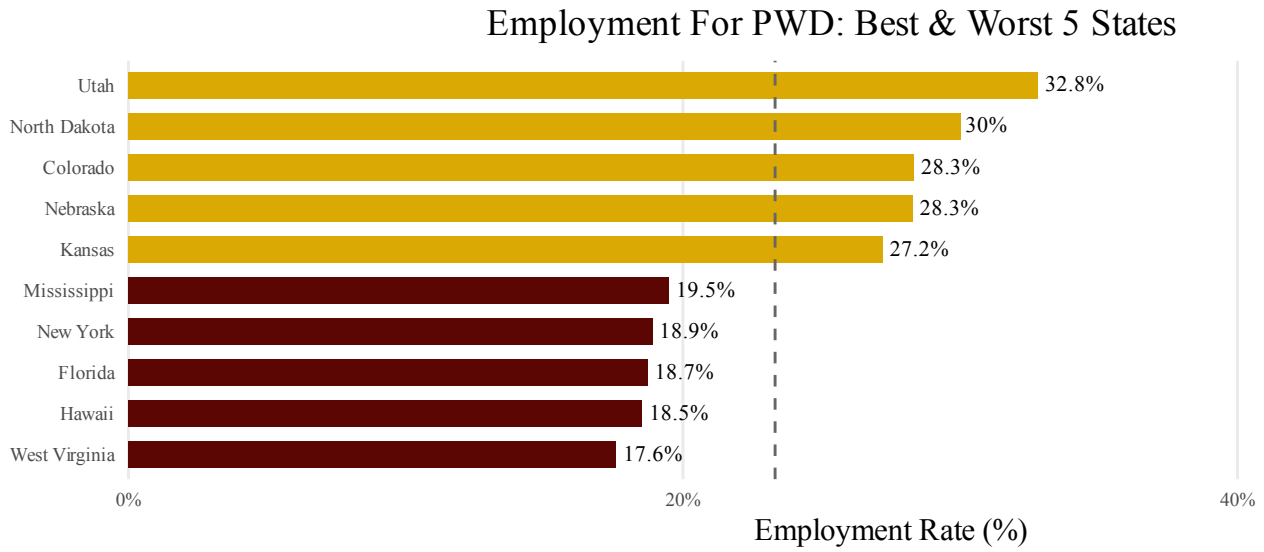
People With Disabilities By Age: Nationally

Age Group	Number With Disability	Percent With Disability
Under 18	3,378,203	4.7%
Ages 18–64	21,269,700	10.7%
Ages 65+	17,981,558	32.9%

Source: U.S. Census Bureau, 2023 ACS 5-year estimates.
Interpretation: Percentages reflect the share of each age group that reports any type of disability.

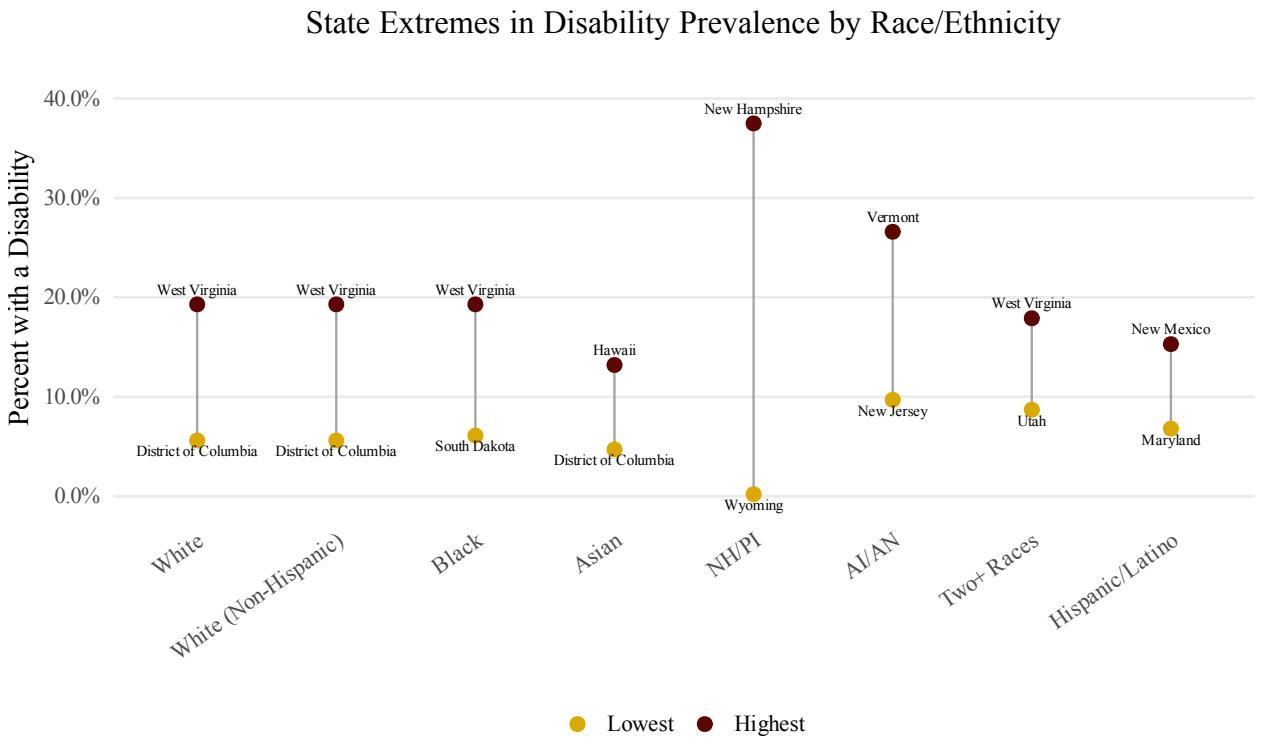
B) Horizontal bars

A horizontal bar chart is used in comparing the best/worst or highest/lowest state or city values on a given metric. Geographies with the least favorable outcomes for PWD are displayed in maroon, while those with the most favorable outcomes for PWD are displayed in gold. A dashed line of the average is displayed for comparison purposes.



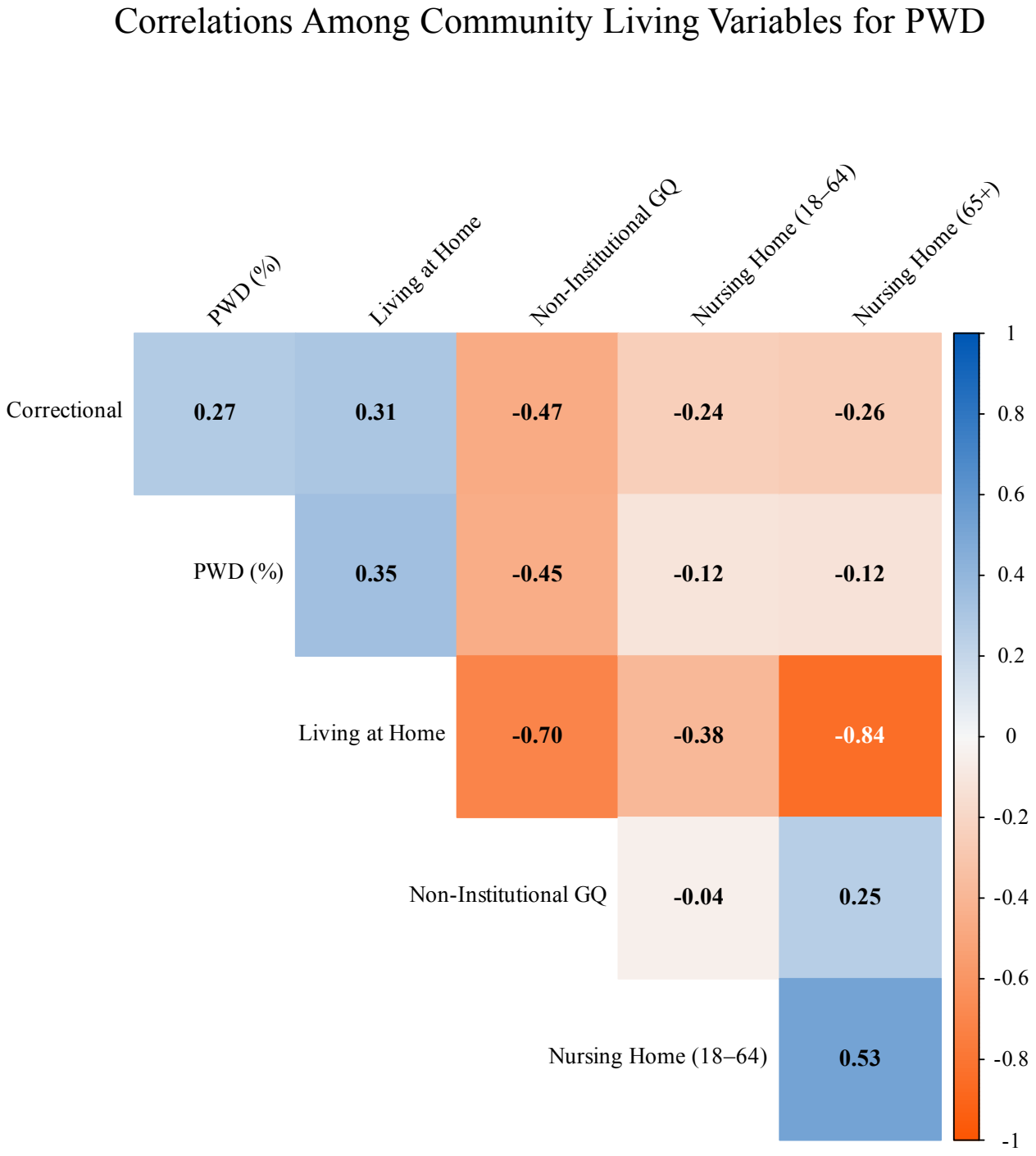
C) Disparity Lollipop Chart

A disparity lollipop chart is used in the race / ethnicity section to display, for each racial / ethnic group category, which state has the highest percentage with a disability (maroon), and which state has the lowest percentage with a disability (gold). The line connecting the two displays the percentage point gap between the two states.



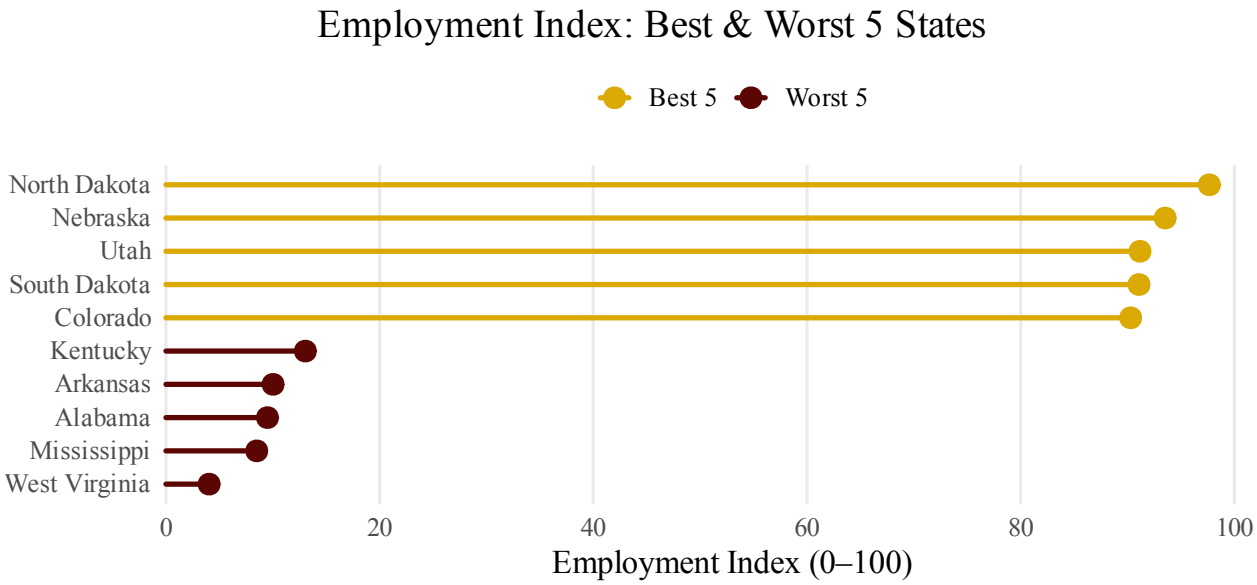
D) Correlation Plot/Matrix

A correlation matrix is used for each of the three ADA-PARC categories - Community Living, Community Participation, and Work / Economic, to display the associations between variables within the category. Each square represents the unique relationship between the two corresponding variables across states. More positive values, indicated with an increasingly dark blue color, represent that the two variables tend to move in the same direction - as one increases, so does the other, and vice versa. More negative values, indicated with an increasingly dark orange color, represent that the two variables tend to move in the opposite direction - as one increases, the other decreases, and vice versa.



E) Scorecard Lollipop Chart

A scorecard dot plot is used to display state performance on a composite 0–100 index scale. Each dot represents a state's score relative to the national distribution. States with more favorable outcomes for PWD appear on top, with bars extending farther toward the right (higher values), while those with less favorable outcomes appear on the bottom (lower values). This format emphasizes relative positioning and distribution rather than simply identifying extremes, helping readers understand how tightly grouped or widely dispersed state outcomes are within a category.



F) Scorecard Table

A scorecard table presents composite index scores alongside quartile classifications for all states. The index score reflects a normalized 0–100 scale, where higher values represent more favorable outcomes for PWD within that domain. States are grouped into quartiles to facilitate interpretation of relative standing. Quartiles divide states into four equal groups based on score distribution, from lowest performing (Q1) to highest performing (Q4). Shading or color cues reinforce quartile distinctions, allowing readers to quickly identify geographic patterns and relative rankings without relying solely on numeric values.

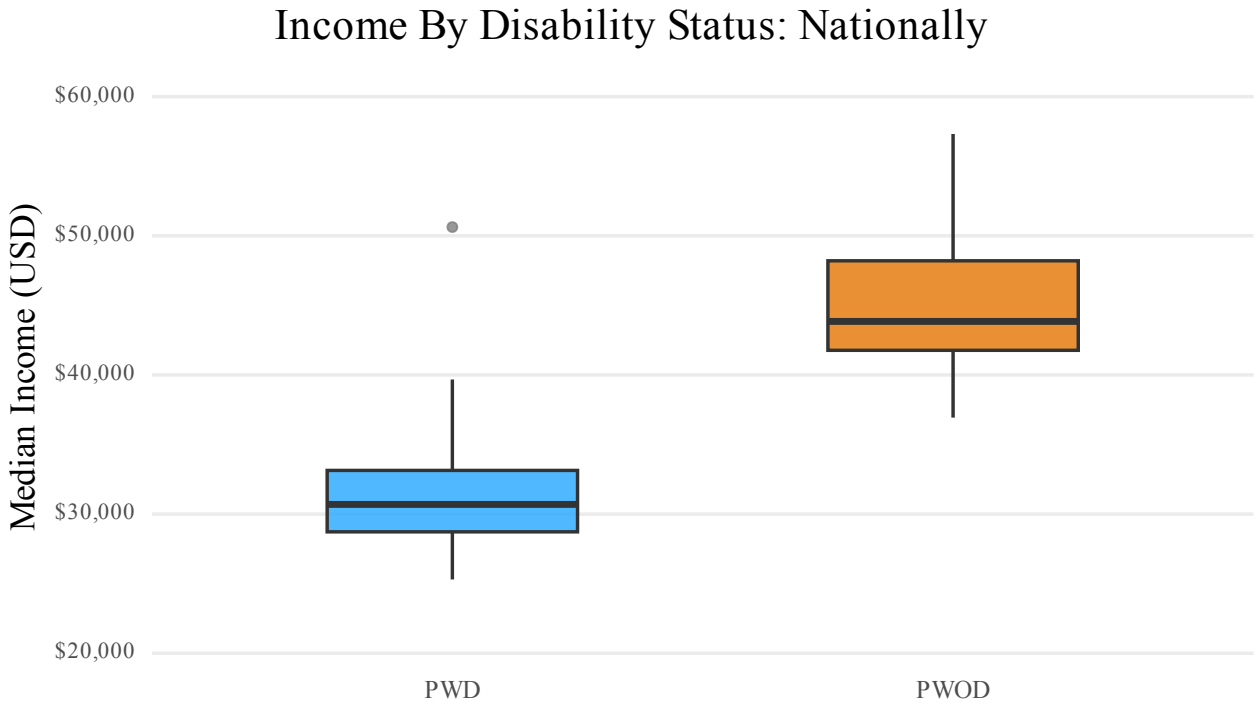
Employment Scorecard: Full State Ranking

Rank	State	Employment Score	Quartile
1	North Dakota	**97.7**	Q4
2	Nebraska	**93.5**	Q4
3	Utah	**91.2**	Q4
4	South Dakota	**91.1**	Q4
5	Colorado	**90.3**	Q4
6	Minnesota	**87.1**	Q4
7	Wyoming	**85.5**	Q4
8	Iowa	**82.9**	Q4
9	Kansas	**81.7**	Q4
10	Idaho	**79.2**	Q4
11	Montana	**77.0**	Q4
12	Wisconsin	**71.2**	Q4
13	Texas	**71.0**	Q3
14	New Hampshire	**68.1**	Q3
15	Alaska	**67.4**	Q3
16	Maryland	**66.4**	Q3
17	Washington	**62.6**	Q3
18	Nevada	**60.9**	Q3
19	District of Columbia	**59.7**	Q3
20	Arizona	**59.5**	Q3
21	Virginia	**58.7**	Q3
22	Connecticut	**57.7**	Q3
23	Oregon	**56.7**	Q3
24	New Jersey	**55.8**	Q3
25	Illinois	**54.9**	Q3
26	Delaware	**54.5**	Q2
27	Hawaii	**51.6**	Q2
28	Massachusetts	**48.1**	Q2
29	Oklahoma	**47.1**	Q2
30	Indiana	**46.3**	Q2
31	Rhode Island	**45.9**	Q2
32	California	**43.1**	Q2
33	Florida	**41.9**	Q2
34	Ohio	**39.1**	Q2
35	Pennsylvania	**38.9**	Q2
36	Vermont	**37.1**	Q2
37	Missouri	**35.1**	Q2
38	Georgia	**32.0**	Q2
39	Maine	**25.8**	Q1
40	North Carolina	**24.9**	Q1
41	New Mexico	**24.8**	Q1
42	Michigan	**23.3**	Q1
43	New York	**23.2**	Q1
44	South Carolina	**21.7**	Q1
45	Tennessee	**21.5**	Q1
46	Louisiana	**16.2**	Q1
47	Kentucky	**13.1**	Q1
48	Arkansas	**10.0**	Q1
49	Alabama	**9.5**	Q1
50	Mississippi	**8.5**	Q1
51	West Virginia	**4.0**	Q1

Source: ADA-PARC 2023 Scorecards.
Shading by quartiles Q1 $\leq$ 28.9; Q2 $\leq$ 54.5; Q3 $\leq$ 69.6; Q4 > 69.6.
Interpretation: Higher scores indicate stronger outcomes for people with disabilities on the Employment Index. Quartile shading ranks states relative to one another (Q1 = lowest scores; Q4 = highest scores).

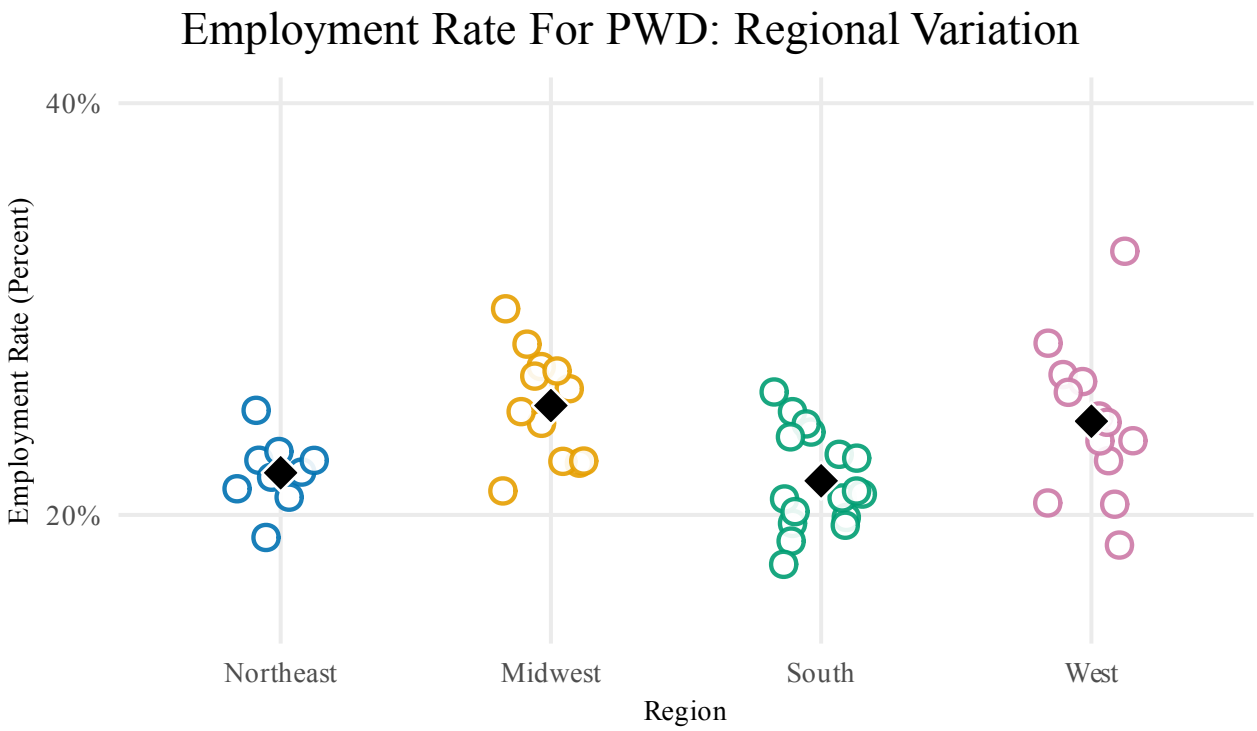
G) National Boxplot

A boxplot is used to summarize the distribution of a metric across states. The box represents the interquartile range (IQR), capturing the middle 50 percent of state values. The line inside the box indicates the median. Whiskers extend to show the range of values outside the middle 50 percent, excluding outliers. This visualization helps readers understand variability, clustering, and dispersion across states. A narrow box indicates relatively consistent values across states, while a wider box signals greater variation. Outliers may indicate states that differ substantially from national patterns and may warrant closer examination.



H) Regional Jitter Plot

A jitter plot displays individual state observations as separate points, slightly offset to reduce overlap. Each point represents a state's value for the selected metric. This format emphasizes the full distribution of state outcomes and allows readers to see clustering patterns, density, and dispersion. When combined with regional markers or national averages, jitter plots help illustrate how individual states compare within and across regions. Unlike aggregated charts, jitter plots preserve state-level detail while maintaining readability.



I) Disparity Table

A disparity table presents the difference between PWD and PWOD (People Without Disabilities) on a given metric. Differences are typically expressed in percentage points or dollar amounts, depending on the measure. Positive and negative values indicate the direction of the gap. A positive value may indicate that PWD have a higher rate of the measured outcome, while a negative value may indicate that PWD have a lower rate relative to PWOD. Interpretation depends on the metric's desirability. For example, a higher poverty rate among PWD represents a less favorable outcome, while a higher employment rate represents a more favorable outcome. Readers should interpret disparity values in conjunction with the underlying metric to determine whether the gap reflects advantage or disadvantage.

Greatest & Least Median Income Disparity: State Level

State	PWD Median	PWOD Median	Disparity
District of Columbia	$50,628	$81,675	**-$31,047**
Massachusetts	$33,642	$57,317	**-$23,675**
Minnesota	$29,276	$50,689	**-$21,413**
Connecticut	$31,660	$52,990	**-$21,330**
Vermont	$25,305	$45,653	**-$20,348**
Mississippi	$27,039	$36,935	**-$9,896**
Florida	$31,084	$40,960	**-$9,876**
Oklahoma	$30,718	$40,201	**-$9,483**
Hawaii	$38,619	$47,495	**-$8,876**
Nevada	$34,991	$42,310	**-$7,319**

Source: U.S. Census Bureau, 2023 ACS-5.
Interpretation: States are selected by the absolute gap |PWD − PWOD|, while the disparity values retain direction (PWD − PWOD).

Table 49: States with the greatest and least median individual income disparity between adults with disabilities (PWD) and adults without disabilities (PWOD), age 16+ (2023).

7.3.2 Interpretation Terms

Ranking

Ordering states or cities from highest to lowest (or vice versa) based on a specific measure or score.

Quartile (Q1–Q4)

A grouping that divides observations into four equal-sized groups:

- **Q1**: Lowest quarter of values
- **Q4**: Highest quarter of values

Quartile cut points

The numeric thresholds that separate quartiles for a specific distribution, boundaries between Q1-Q2, Q2-Q3, Q3-4 .

Benchmark

A high-performing group or value used as a reference point for comparison. Averages indicated with a dashed line are often used as benchmarks in this report.

Correlation Plot / Matrix

A table showing how indicators move together across places, used to illustrate relationships among outcomes. Positive values suggest that two variables move together - as one increases the other tends to as well, or as one decreases the other tends to as well. Negative values suggest the opposite - as one value increases the other decreases, and vice-versa. The more negative or positive the value, the stronger the relationship is.

7.4 General Term Dictionary

This glossary defines key terms, measures, and concepts used throughout the ADA-PARC descriptive report. Definitions are written for a broad audience and are intended to support interpretation rather than technical replication.

7.4.1 Program and Report Structure

ADA-PARC

A research and reporting initiative that compiles data on disability-related outcomes to describe participation, access, and equity across U.S. states and cities. See the link above to the ADA-PARC website for more information.

Descriptive analysis

An analytic approach that summarizes observed patterns, identifies associations, and key findings in data without making claims about cause and effect. This report describes differences across groups and when straightforward, suggests potentially influential variables, but does not suggest specific policy attributions or appropriate interventions.

Domain / Dimension / Category

Broad topic areas used to organize measures in the report (for example, Work–Economic, Community Participation, or Community Living).

Indicator / Measure

A single measurable statistic used to describe an outcome or condition (for example, poverty rate or median income).

Scorecard / Index

A summary measure that combines multiple indicators into a single value to support comparison across places.

7.4.2 Populations and Disability Groups

PWD (People with Disabilities)

Individuals identified as having a disability based on survey definitions used in the report - U.S. Census American Community Survey (ACS).

PWOD (People Without Disabilities)

Individuals identified as not having a disability, used at points as a comparison group.

Disparity / Gap

Difference between PWD and PWOD on the same measure (for example, income or poverty). In this report, disparities are calculated as the value for people with disabilities minus the value for people without disabilities (PWD - PWOD) and offer insight into equity across disability status.

Life course

An approach that examines outcomes across age groups, recognizing that economic and social conditions differ for children, working-age adults, and older adults.

7.4.3 Data Source and Survey Concepts

ACS (American Community Survey)

A large, nationally representative survey conducted by the U.S. Census Bureau that collects social, economic, housing, and demographic information.

ACS 5-year estimates

Survey estimates produced by pooling five years of data to improve reliability, especially for smaller geographic areas. This is the method used in the data unless otherwise specified.

Estimate

A statistical value derived from survey data rather than a complete count.

Median

The middle value in a distribution, where half of observations are higher and half are lower. Example - In the following set of values: 1, 5, 6, 9, 15, 18, 30 - value 9 is the median.

Mean / Average

The arithmetic average of a set of values. Example - In the following set of values: 1, 5, 6, 9, 15, 18, 30 - 12 is the average.

Variation / Dispersion

The spread or diversity of the quantitative values taken on by an individual measure or index across a categorical variable (geography in our case).

7.4.4 Geography

National level

Statistics summarized for the entire United States.

Region

Broad geographic groupings used for comparison: Northeast, Midwest, South, and West.

State-level

Statistics calculated separately for each U.S. state (and, where available and applicable, the District of Columbia & Puerto Rico).

City-level

Statistics calculated for cities meeting population and data reliability thresholds.

7.4.5 Demographic Concepts

Demographics

Statistical characteristics of a given section of the population, such as age, sex, race, ethnicity, and population size.

Age group / Age bracket

A defined range of ages used to summarize outcomes for populations at different life stages. Common groupings in this report include Under 18, 18–64, and 65+.

Working-age population

Individuals typically considered eligible for labor force participation, ages 18 to 64.

Older adults (65+)

Individuals aged 65 and older, analyzed separately due to distinct economic, health, and labor market dynamics.

Children / Youth (Under 18)

Individuals under the age of 18, analyzed separately because they are not expected to participate in the labor force.

Population share / Percentage

Proportion of a population that falls into a specific category, expressed as a percentage.

Civilian-Non-Institutionalized (CNI) Population Universe

Individuals living in community-based settings, excluding those residing in institutions such as nursing facilities, prisons, or long-term care hospitals. This is the population universe being used in the categories of Community Participation & Work Economic.

Group Quarters (GRP) Population Universe

Universe of individuals in the United States including those living in community-based settings and institutionalized settings such as nursing facilities, correctional facilities, or long-term care facilities. This is the population universe being used in the category of Community Living.

Group Quarters

Living arrangements where people reside in group settings rather than private households, including institutional and non-institutional group quarters.

Race

A demographic classification based on self-identification with one or more racial categories.

Ethnicity

A demographic classification indicating whether a person identifies as Hispanic or Latino, regardless of race.

Sex

A demographic classification typically recorded as male or female in survey data.

7.4.6 Work–Economic Concepts

Labor force

People who are either employed or unemployed and actively looking for work.

Employed

Individuals working for pay at the time of the survey.

Not In Labor Force

Individuals who are neither employed nor actively looking for work. This group includes people who are retired, unable to work, caregivers, discouraged from job seeking, or otherwise outside the labor market.

Median Individual Income (Age 16+)

The median income earned by individuals aged 16 and older.

Poverty

An income threshold that varies by family size and composition and is used to determine poverty status.

7.4.7 Community Participation Concepts

Community Participation

A domain capturing how people engage with society and the resources that enable participation, such as education, transportation, technology, and health insurance.

Technology Access

Availability of tools such as internet service, smartphones, or computers that enable communication and participation in modern economic and social life.

Transportation Access / Mode

How individuals travel to and from work, including driving, public transit, or other means.

Insurance Access / Coverage

Whether an individual has health insurance, separated out by type of insurance whether public or private, and also

7.4.8 Community Living Concepts

Community Living

A domain focused on living arrangements and the degree to which people with disabilities live in community-based versus institutional settings.

Institutional living

Living arrangements that involve institutional settings such as nursing facilities or correctional facilities.

Non-institutional living

Living arrangements in private households or community-based settings rather than institutions.